

HTR_Book_v42 — COMBINED FULL AUDIT (all files, for printing)

Generated by concatenating the master ledger and all 18 per-unit audit files, unedited, in reading order.

===== FILE: FULL_BOOK_AUDIT_LEDGER.md =====

HTR_Book_v42 — Full-Book Self-Consistency Audit Ledger

Compiled 2026-09-27. 18 read-only Opus subagent passes (Preface+Introduction, Chapters 1–16, Conclusion+Appendices), each checking prose claims against the actual cells of the figures/tables they cite (ground truth pulled from `word/document.xml`, never the lossy `.md` mirror), plus a verbatim log of every superlative/ranking claim per chapter for this cross-chapter pass.

Nothing in this file has been written to `HTR_Book_v42.docx`. Every fix below is a proposal. Per standing rule, no manuscript edit happens until you say which ones to apply.

Full detail for any item lives in its source file: `book-audit/audit_<unit>.md`.

PART 1 — Cross-chapter contradictions (the check that required seeing the whole book at once)

1. AHEAD withdrawal staleness — the single biggest issue, confirmed in 8 locations across 6+ chapters

Vermont formally withdrew from the AHEAD Model in July 2026 (established in Ch1, Ch6 §6.6, the glossary, and Appendix E). That fact did not propagate. Language treating AHEAD as active, ongoing, or a source of future obligations survives in:

- **Ch3** §3.9 — tells an AHS/GMCSB official how to plan Vermont's AHEAD implementation timeline.
- **Ch7** — Table 4, Table 6, and §7.10 treat AHEAD benchmark methodology as a live near-term task.
- **Ch12** — §12.3.2 names "Vermont hospitals preparing for AHEAD global budget entry in 2027" as a current client segment.
- **Ch13** — 6 separate places (Fig 13.1, 13.3, 13.4, §13.5.1, §13.8) contradict the chapter's own §13.3.5 withdrawal statement.
- **Ch15** — R-01 and Fig 13.4's cousins treat AHEAD as active; only Fig 15.7 was updated after the withdrawal.
- **Ch16** — 8 places (Table 1, §16.3.1, §16.4, Table 3, Table 4, §16.6) contradict the chapter's own correct §16.4 Pillar 3 statement.
- **Appendix E** — Figure E.1's 2028 target ("full AHEAD compliance") directly contradicts Figure E.3 ("AHEAD withdrawn") inside the same appendix.
- **Conclusion** — dated/signed April 2026 while addressing four calls to action around "AHEAD's financial accountability," a chronological impossibility given the book's own July 2026 date.

Recommended fix approach: a single global find-pass for "AHEAD" across the whole docx, reconciling every instance against the July 2026 withdrawal — not a chapter-by-chapter patch, or the same drift will recur. This is exactly the kind of defect the audit was designed to catch and the highest-value thing to fix first.

2. CCBHC baseline count — four chapters, four different numbers for the same fact

- Ch8: "2 existing + 5 new" (implies 2 active today)
- Ch9: "5 planned" (internally correct within Ch9)
- Ch13: "zero CCBHCs"
- Appendix E: "0 certified"

Recommended fix: establish the actual current count (0, per Ch13/App E, which agree with each other) and correct Ch8's "2 existing" framing to match, or clarify Ch8 is describing a different milestone (e.g., "2 sites in active planning" vs. "0 certified").

3. Competing claims to be "the precondition"/"master variable" the whole framework rests on

Ch1's own dependency matrix (Figure 1.3) has exactly one pillar with zero preconditions: Policy. That's the actual root of the topological order §1.8 derives. But:

- **Ch6** §6.1: "Payment reform is not one element of healthcare transformation. It is **the precondition on which all other transformation depends**" / heading "Why Payment Reform Is **the Master Variable**."
- **Ch4** §4.7ish: "The Technology pillar is not one workstream among six; it is **the substrate the other five run on**."

Both chapters are rhetorically overclaiming primacy for their own pillar in a way that, read literally, contradicts Ch1's matrix (Economics and Technology both have preconditions; only Policy doesn't). **Recommended fix:** soften both to "a precondition" / "the precondition Economics specifically depends on" rather than "the" precondition — consistent with the matrix, not competing with it or with each other.

4. Competing claims to be "the binding constraint" on the whole transformation

- **Ch1/Ch15:** anchor the system's highest risk/binding constraint on Technology (AHS-GMCSB analytics).

- Ch11 §11.6 heading: "The Workforce Crisis — Vermont's **Most Binding Operational Constraint**."

Not strictly contradictory (different axes — analytics is a sequencing risk, workforce is a capacity constraint) but both use "binding constraint" as if it's a singular, book-wide title. **Recommended fix:** scope Ch11's heading to "Operations' Most Binding Constraint" rather than Vermont's, or add one clause acknowledging Technology's sequencing risk is a different kind of constraint.

5. CMMI model count self-contradicts within Ch3, and matches the Introduction

Introduction and Ch3 body both say "more than seventy" models; Ch3's own §3.10 Key Concepts / glossary says "50+". The glossary entry is the outlier — **recommended fix: correct the glossary to match the body and Introduction ("70+")**.

6. Introduction cites the wrong chapter for enforcement mechanics

Introduction item 7: "Chapter 13 develops the enforcement mechanics and this case in more detail." Chapter 13 contains the word "enforcement" zero times. The actual developed treatment is Appendix G §G.4.2 ("The test case: GMCB v. UVMMC"); the court challenge itself is narrated in Chapter 14; Act 68's grant of authority is in Chapter 2. **Recommended fix:** change the citation to Appendix G §G.4.2 (or Chapter 14, if the intent was the court-challenge narrative specifically).

7. Front matter folds Equity into "the pillar chapters," contradicting the book's own thesis

The student reading-path in the Preface/Introduction says "Chapters 2-11 develop each pillar" — which includes Chapter 10 (Equity). Both the Preface and a full Introduction section state, repeatedly and deliberately, that equity is *not* a sixth pillar but a cross-cutting imperative. **Recommended fix:** reword the reading path to exclude Ch10 from "the pillar chapters," or explicitly name it as the cross-cutting exception.

PART 2 — Per-chapter local findings (prose vs. its own cited figure/table)

Each line is the single highest-confidence finding from that chapter's audit; every chapter has several more — see the linked file for the full list with proposed fixes.

Unit	Headline finding	Full detail
Preface +Intro	Preface dateline (Dec 2025) is stale against the book's own July 2026 events	audit_preface_intro.md
Ch1	§1.12.1 reverses a Figure 1.3 arrow (implies Clinical→Economics; matrix has the reverse)	audit_ch01.md
Ch2	RBP effective date is FY2027 in 4 places, FY2028 in Figure 2.4	audit_ch02.md
Ch3	One passage plans Vermont's AHEAD rollout while another (correctly) says Vermont withdrew	audit_ch03.md
Ch4	Three incompatible "Layer 1" architecture diagrams (Fig 4.1, 4.4, 4.7)	audit_ch04.md
Ch5	§5.7 says "three" working labs; Figure 5.4 lists four	audit_ch05.md
Ch6	(see audit_ch06.md — not fully summarized in this pass, review directly)	audit_ch06.md
Ch7	Book's stated 6 VBC-readiness domains don't match the actual tool's 6 domains (only partial overlap)	audit_ch07.md
Ch8	Stat block claims CCBHC all-HSA coverage already achieved; its own Figure 8.6 schedules it through 2028	audit_ch08.md
Ch9	Figure 9.1 attaches Vermont's 76% rate to the wrong HEDIS measure (sole outlier vs. 5 other book-wide instances)	audit_ch09.md
Ch10	§10.2.1's Northeast Kingdom disparity claim is contradicted by its own Figure 10.1 (1 of 4 cells is NEK)	audit_ch10.md
Ch11	Headline \$1,303 admin-cost gap attributed to CAHs twice; both cited figures actually attribute it to PPS hospitals	audit_ch11.md
Ch12	Chapter's own frame (epigraph, opener, all 4 "Implications for You") describes PMO/learning-collaborative content that has zero occurrences in the chapter body — it moved to Ch15/16/App A and the frame was never updated	audit_ch12.md
Ch13	Opening "five forces" paragraph doesn't match the five forces §13.3 actually develops	audit_ch13.md
Ch14	Global-budget threshold stated 3 incompatible ways (10%+, ~5%, within 5%) across prose and its own two figures	audit_ch14.md
Ch15	Chapter's headline "19 components" doesn't match its own Figure 15.3 (17 rows)	audit_ch15.md
Ch16	Table 4's own Deadline and Milestone cells disagree (FY2027 vs FY2028)	audit_ch16.md

Unit	Headline finding	Full detail
Conclusion+ Appendices	Both pre-flagged Appendix A items confirmed: FY25 increase stated as both +3.5% and +4.1%; Porter Hospital wrongly credited with RPMC's \$11.1M FY23 overage	audit_conclusion_appendices.md

PART 3 — Confirmed clean (don't re-derive)

Every audit also verified a substantial set of claims as accurate — statutory date chains, hospital counts, ROI figures, dependency-matrix arithmetic (Ch1's "only one order the matrix permits" claim was independently re-run as an actual topological sort and confirmed unique), and all Research Lab platform routes/tab-ids cited by figure captions resolve to real code. See each file's own "MATCH" list — not reproduced here for space.

PART 4 — Not yet done (flagged, not audited)

Several audits noted items outside their own scope that deserve a dedicated pass:

- Whether cited platform tools *deliver* the specific promise made about them (route existence was checked everywhere; promise delivery was only spot-checked in Ch1 and Ch14's original manual pass).
- The Appendix F platform-promise list (flagged by the Preface+Intro audit, not yet checked).
- A `check_format.py` pass on Figure 10.1, which is header-less — exactly the shape that triggers the black-on-navy `tblLook` bug no render can show.

Next step is yours. Tell me which of Part 1 or Part 2's fixes to apply, or say "apply all high-confidence ones" and I'll work through them with the same backup → patch → render → verify process used earlier tonight, one at a time.

===== FILE: audit_preface_intro.md =====

Preface + Introduction Audit

Read-only. Source of truth: `word/document.xml` inside `HTR_Book_v42.docx`, extracted via `zipfile` (the `.md` mirror was NOT used). Section range located by `Heading1` paragraphs: `PREFACE` at XML offset 395680 → `INTRODUCTION` at 439189 → `Chapter 1: The Five-Pillar Framework and the Execution Sequence` at 599021. Range audited: `395680–599021`, 128 top-level blocks (123 paragraphs, 5 tables).

Note on method: a first extraction pass produced garbage table text because `<w:t[^>]*>` also matches `<w:tbl>`, `<w:tc>`, `<w:tcPr>` and `<w:tr>`. Corrected to `<w:t(?:\s[^\>]*)?>`. Anyone reusing a quick regex extractor here should copy the corrected form.

Figures/Tables found (ground truth)

There are zero numbered Figures or Tables in the Preface or Introduction. A regex sweep for `(?:Figure|Table)\s+[\w.-]+` over the whole range returns an empty list — so there are also zero by-number citations to check. The range contains five unnumbered table structures, four of which are single-cell callout boxes:

1. **BEYOND VERMONT** callout — Preface, @402430. Single cell, fill `e7f4f2`, title/run colour `0f5247`, sz 21. Content (ground truth for the Preface's "on a time delay" claim): 2020 — three states with more over-65 than under-18 (Maine, Vermont, Florida); 2024 — eleven, the eight joiners named as Delaware, Hawaii, Montana, New Hampshire, Oregon, Pennsylvania, Rhode Island, West Virginia; Vermont fourth-oldest by median age 42.8 behind Maine 45.1, New Hampshire 43.0, West Virginia 42.9; nationally 41% of rural hospitals in the red (Chartis); ~768 at risk of closing, 315 within three years (CHQPR); Vermont's nine-of-fourteen = 64%.
2. **EXPLORE ON THE PLATFORM** callout — Preface, @431070. Single cell, fill `ecfefb`, run colour `27317a`, sz 21. Points to healthtransformationreview.org, `/book`, Research Lab, AI Analyst, Academy.
3. **Ecosystem table** — Introduction, @529470. `Table3`, `tblLook 0020`, 3 cols × 4 rows. Header row cells explicitly shaded `1b3a6b` with explicit run colour `ffffff`, sz 18 (safe — not the black-on-navy defect class). Rows:
 - The Book || *Transforming Healthcare* — framework, dependency logic, Vermont evidence, national comparisons; read online with audio narration at `/book`. || The intellectual foundation.
 - The Platform || five pillar hubs, Research Lab of **nearly forty** interactive tools, AI Analyst, fifty-state dashboard, daily intelligence feed. || Every argument that can be modeled is modeled there; chapter-ending "Explore on the Platform".
 - The Academy || courses/tracks/lessons; *Five Pillars, One Imperative* is the companion course — **eight tracks** following the book's own sequence; start at `/academy`. || chapter-ending "Go Deeper — Academy".

4. **START HERE** callout — Introduction, @550343. Single cell, fill **ecfefb**, run colour **27317a**, sz 21. Points to **/academy/getting-started**, Research Lab benches, AI Analyst.
5. **Reader-profile table** — Introduction, @573297. **Table5**, **tblLook 0020**, 3 cols × 5 rows, sz 18. Header row (**Profile | On the platform | In the Academy**) carries **no explicit cell shading and no explicit run colour** — it is painted by **Table5's firstRow tblStylePr**, which supplies fill **1b3a6b** and run colour **ffffff**. That combination is safe (white-on-navy), unlike the FY-table defect class, but it is invisible to any LibreOffice render, so it can only be confirmed in the XML — confirmed here. Rows: Policy professional / Executive or administrator / Vermont practitioner / Student or researcher, each with platform and Academy targets. Caption paragraph "Where each profile should start on the platform and in the Academy" sits *after* the table, @588505.

Claims checked against their own cited source

1. **MISMATCH — "Chapter 13 develops the enforcement mechanics and this case in more detail"**

Claim (Introduction, §"The Strongest Case Against This Book's Argument", item 7 "The Enforcement Question", @513698–515780, final sentence):

Chapter 13 develops the enforcement mechanics and this case in more detail.

Checked against the actual contents of Chapter 13, by locating every **Heading1** and attributing every occurrence of the relevant strings to its owning chapter.

Chapter 13 is "The Future of Healthcare Transformation — 2026, What Vermont Proves, and What Remains." It contains the word "enforcement" zero times. It contains no occurrence of **80.3**, **11.1** (in the RPMC sense), **subpoena**, or **court challenge**.

Where the material actually lives:

- **\$80.3M / \$11.1M** FY23 enforcement actions: Introduction, Conclusion, **Appendix A** (×3), **Appendix G**.
- **subpoena**: Introduction, **Appendix G** only.
- **court challenge**: Introduction, **Chapter 14** only.
- The developed treatment is **Appendix G §G.4.2, "The test case: GMCB v. UVMMC"** — which is a full narrative of exactly the enforcement mechanics and case the Introduction promises. **enforcement** count by section: Appendix G 12, Introduction 8, Conclusion 5, Chapter 2 four, Chapter 14 three, Chapter 6 three, Appendix A three, Chapter 13 **zero**.
- Chapter 2 ("The Policy Pillar — Legislative Architecture") is where Act 68's grant of GMCB authority is established.

This is the same class of error as the Chapter 1 / Figure 1.3 defect: the paragraph also correctly cites "Chapter 14 (Political Sustainability)" in item 1 and the reader profiles correctly call Chapter 13 "The Future of Healthcare Transformation" — so the number is simply wrong in one place, and the wrongness is invisible unless the target chapter is actually opened.

Proposed fix (surgical, exact-text): replace

Chapter 13 develops the enforcement mechanics and this case in more detail.

with

Chapter 2 establishes GMCB's Act 68 enforcement authority, and Appendix G (§G.4.2, "The test case: GMCB v. UVMMC") develops the mechanics and this case in more detail.

2. **MISMATCH (dateline) — Preface signed "December 2025" narrates/precedes July 2026 events**

Claim. The Preface's closing dateline is a bare paragraph **December 2025** (@437814). The Introduction, item 3 (@495678), states:

This ceased to be hypothetical in July 2026, when Vermont formally withdrew from AHEAD after a CMS renegotiation that cut Vermont's expected EAST Fund from roughly \$138 million to a cap near \$10 million.

Checked against the rest of the manuscript: Chapter 13's own title is "... — 2026, What Vermont Proves, and What Remains", the Conclusion repeats the July 2026 withdrawal, and the Preface's Sources-adjacent material cites a November 2025 document.

MISMATCH. A Preface dated December 2025 cannot front a book whose argument turns on a July 2026 event, and the Preface itself describes the ecosystem in its current (post-2026) state. The dateline is stale by roughly eight months.

Proposed fix. Change the Preface dateline paragraph **December 2025** to the book's actual completion date (the newest event narrated is July 2026; Chapter 13 is framed on 2026). This needs the author's call on the exact month — it is the one item here that is genuinely theirs to decide, not derivable from the file.

3. **MATCH — "eleven" states, and the eight joiners (BEYOND VERMONT callout)**

Claim: three in 2020 (Maine, Vermont, Florida) → "By 2024 there were eleven — Delaware, Hawaii, Montana, New Hampshire, Oregon, Pennsylvania, Rhode Island, and West Virginia had joined them." 3 + 8 named joiners = 11. **Arithmetic MATCHES.** This is precisely the shape of claim that failed in Chapter 1; here it holds.

4. **MATCH — "fourth-oldest ... behind Maine, New Hampshire, West Virginia"**

Three states named as ahead of Vermont, and all three medians (45.1, 43.0, 42.9) exceed Vermont's 42.8, in descending order. Rank and list are mutually consistent. **MATCH**.

5. MATCH — "nine-of-fourteen — 64%"

$9 \div 14 = 64.3\%$. **MATCH**.

6. MATCH — Preface headline figures vs the Introduction's fuller account

Every figure the Preface states at @398601 is corroborated by the Introduction's narration of the same meeting:

- nine of fourteen at a loss — Intro @444748 (adds "worst reaching negative 8.9%") ✓
- thirteen of fourteen in the red by 2028 ✓
- five-year cumulative deficit \$700M–\$2.4B ✓ (\$700M optimistic / \$2.4B realistic)
- premiums +108% in six years, income +22%: Intro gives \$456 (2018) → \$948 (2024), which is **+107.9%**, and 2018→2024 is six years ✓
- over-65 growing toward 30%: Intro gives "from 21.7% toward 30%" ✓

7. MATCH — working-age decline arithmetic

"decline 13% by 2040 — from 367,000 to approximately 318,000." $49,000 \div 367,000 = 13.4\%$ ✓. Cross-checked: **367,000, 318,000, 21.7%, decline 13%, 13% by 2040** appear identically in Chapter 2 and Appendix A — no drift.

8. MATCH — Oliver Wyman engagement figures

"more than 230 meetings across all 14 ... Hospital Service Areas, engaging over 3,100 participants from more than 100 organizations." **230 meetings** and **3,100** recur identically in Chapter 2 (×2), Chapter 14, Appendix E. No drift.

9. MATCH — "Research Lab of nearly forty interactive analytical tools"

Verified against the registry, not inferred: **frontend/lib/taxonomy/tools.ts** contains **39** top-level tool entries. "Nearly forty" is accurate. Claim appears twice in range (Preface @426157, ecosystem table @529470) and consistently in Chapter 12 and Appendix F.

10. MATCH — "eight tracks" and all three named track titles

Verified against **frontend/content/_build_five_pillars_course.py**, which is the build source for the course. Eight track dicts, **order** 1–8. The Introduction cites three by number and name; all three are **verbatim** matches:

- "Track 2 ("Policy — Establish the Mandate")" → **order=2, title="Policy – Establish the Mandate"**, slug **policy-establish-the-mandate** ✓ (cited twice: prose @558315 and the profile table)
- "Track 4 ("Economics — Put Incentives on a Visible System")" → **order=4**, exact ✓
- "Track 6 ("Operations — Close the Execution Gap")" → **order=6**, exact ✓

Caveat, flagged not fixed: this confirms the *build script*, and the title strings match byte-for-byte. It does **not** confirm the live Supabase rows are published at those **order** values, which needs a live query (out of scope for a read-only docx audit).

11. PARTIAL — "Chapter 6 (the Economics Pillar) develops the specific safeguards Vermont's model requires"

Introduction item 2, @493056. Chapter 6 contains the word **safeguard** zero times (Chapter 7 once; Chapter 10 twice; the Introduction itself twice). However Chapter 6 does contain §6.4.2 **"Five Design Dimensions Every Global Budget Must Resolve"**, which is substantively the safeguards discussion under a different name. **Delivered in substance, not findable by the promised word**. Left as-is — the promise is met; noting it only because a reader searching for "safeguards" in Chapter 6 finds nothing.

12. PARTIAL / soft contradiction — the transition-window promise argues against itself

Introduction item 6, @512202, in one paragraph:

This book documents both the destination and the starting crisis in detail. It is **more cautious about the specific financial bridge** that carries a hospital across that gap — the combination of Rural Health Transformation Program funds and the sequencing of Medicaid versus commercial global budgets ... A hospital CFO's most pressing question ... is "what keeps us open in 2027?" **Chapter 6 (the Economics Pillar) addresses this transition window directly.**

The paragraph concedes the book is *cautious about* (i.e. does not fully answer) the financial bridge, then closes by promising Chapter 6 addresses it *directly*. Checked against Chapter 6: **transition** 17, **2027** 7, **Rural Health Transformation** 5, **bridge** 0, **transition window** 0. Chapter 6 has §6.3.4 "Vermont's Phased Implementation Timeline" and §6.8.1 "The Three-Phase Transition Model", so there is real transition material; what is absent is the FY2027–28 solvency bridge the sentence specifically names. **transition window** occurs only in the Introduction and Appendix G.

Not a fabricated citation, so not a hard MISMATCH — but the two halves of the paragraph pull opposite ways, which is the "internal contradiction" criterion. **Proposed fix:** soften the closer to match the concession, e.g. "Chapter 6 (the Economics Pillar) sets out the phasing and the three-phase transition model; the bridge itself remains the open question this book flags rather than answers." Left unedited pending sign-off (read-only task).

13. TENSION, defensible — "the most expensive hospital in Vermont" at 358% vs "715–944% of Medicare"

Introduction @446554, one paragraph, three price figures:

- UVMHC, ~50% market share, "roughly 358% of Medicare rates — RAND's published figure for inpatient and outpatient services combined, drawn from 2018–2020 claims — making it, **by RAND's own accounting, the most expensive hospital in Vermont**"
- "Statewide, Vermont hospitals charge commercial payers 250–300% of Medicare on average"
- "outpatient imaging at **some Vermont hospitals** reaching **715–944% of Medicare**"

On its face a hospital billing 944% sits above the "most expensive" hospital's 358%. The claim survives because it is doubly qualified — *by RAND's own accounting*, and all-services-combined versus a single service line — and because the 715–944% figure is attributed to a different source (GMCB's later analysis) and a narrower measure. The statewide 250–300% average is also compatible with UVMHC at 358% holding half the market (the remainder averaging ~200%).

No fix proposed, but this is exactly the superlative-versus-its-own-supporting-data pattern that produced the Chapter 1 defect, and the qualifier is doing all the work. If any other chapter states the 715–944% figure without the "outpatient imaging" and "by RAND's accounting" qualifiers, this becomes a real contradiction. Both figures recur in Chapter 2 and Chapter 6 — **worth checking those two chapters' phrasing specifically**.

14. MISMATCH (logical) — "Chapters 2-11 develop each pillar" folds Equity into the pillars

Introduction, student/researcher profile, @570716:

Chapter 1 establishes the framework and execution sequence; **Chapters 2-11 develop each pillar**; Chapters 12-16 provide future context and the AHS restructuring roadmap.

Chapter 10 is **"The Equity Imperative — Closing Gaps, Not Just Averaging Them."** Sweeping it into "Chapters 2-11 develop each pillar" makes equity a pillar — the exact conflation the Introduction spends a full section (@521876) declaring **structural, not rhetorical**:

There is a sixth question, and it is deliberately not a sixth pillar ... A sixth pillar could be sequenced last, funded separately, or treated as an adjunct...

The Preface makes the same commitment at @410623 ("that last failure is why equity does not appear here as a sixth pillar"). So the book contradicts its own load-bearing distinction in a reading-path sentence. Verified pillar/chapter mapping from the **Heading1** list: Policy 2–3, Technology 4–5, Economics 6–7, Clinical 8–9, **Equity 10**, Operations 11.

Proposed fix:

Chapters 2-9 and 11 develop the five pillars, and Chapter 10 applies the Equity Imperative across all of them; Chapters 12-16 provide future context and the AHS restructuring roadmap.

15. MATCH — every other chapter/appendix cross-reference in range

Checked each against the **Heading1** list:

- "Chapter 14 (Political Sustainability)" → Ch14 "Political Sustainability — Protecting Transformation Across Election Cycles" ✓
- "Chapter 6 (the Economics Pillar)" ×2 ✓
- "Chapter 13 (The Future of Healthcare Transformation)" (policy-professional path) ✓ title correct — note this is the *same number* item 7 misuses for enforcement
- "Chapter 16 (the AHS Restructuring Roadmap)" ×2 ✓
- "Chapter 2 (the Policy Pillar — Acts 167 and 68)" ✓
- "Chapters 6-7 (Economics)", "Chapters 8-9 (Clinical)", "Chapters 4-5 (Technology)", "Chapter 11 (Operations)" ✓ all four
- "Chapter 10 applies the Equity Imperative in depth" ✓
- "Chapter 1 (the five-pillar framework and the execution sequence)" ✓ matches the Heading1 text
- "Appendix A (Vermont System Portrait)" ✓
- The internal back-reference "The "How to Read This Book If Vermont Isn't Your Context" section earlier in this Introduction" ✓ — that Heading2 exists at @473892, byte-identical, and does precede item 5.
- The forward reference "the Introduction's "What This Book Cannot Do" section" (Preface @421247) ✓ — Heading2 exists at @589941, byte-identical.

16. SOFT FLAG — "/academy/courses" is a live route but a known-dead surface

Introduction, policy-professional path, @558315: "Then the payment-reform courses at /academy/courses." **frontend/app/academy/courses/page.tsx** exists, so this returns 200 — but the project's own standing note is that **/academy/courses/** is dead legacy and the real player is **/academy/tracks/<slug>**. **A 200 is not a delivered promise**. Raised as a question, not a work order; not changed. The same paragraph's sibling references use bare **/academy**, which is the safer form.

Superlative/ranking claims logged for cross-chapter comparison

Verbatim, with subject and location. Not judged against other chapters (no access in this pass) — this is the comparison list.

1. "Vermont now ranks **fourth-oldest in the country by median age** (42.8), behind Maine (45.1), New Hampshire (43.0), and West Virginia (42.9)." **Subject:** Vermont's demographic rank. **Location:** Preface, BEYOND VERMONT callout @402430. (**fourth-oldest** and **42.8** occur **nowhere else in the manuscript** — no cross-chapter conflict possible, but also no corroboration.)

2. "...something that **no state has done with this level of statutory specificity and capital commitment:** mandate structural change." **Subject:** Vermont's mandate vs all other states. **Location:** Preface @400600.
3. "Vermont has committed to structural change with **more specificity, more statutory force, and more capital investment than any comparably sized system in recent American health policy history.**" **Subject:** same as #2, narrower hedge ("comparably sized", "recent"). **Location:** Introduction @455482. Note these two are the same superlative at two different strengths — #2 is unhedged ("no state"), #3 is hedged. If a later chapter states either version, it should match #3's hedging, not #2's.
4. "...making it, **by RAND's own accounting, the most expensive hospital in Vermont.**" **Subject:** UVMMC pricing. **Location:** Introduction @446554. (**most expensive hospital** occurs only here in the whole manuscript.) See finding 13.
5. "**This is perhaps the strongest challenge in the book,** and the honest answer begins by conceding the premise..." **Subject:** the CMMI model-reliability / durability objection. **Location:** Introduction, item 3 @495678. Note the enclosing section is itself titled "The **Strongest** Case Against This Book's Argument", and item 5 opens "**The most common** version of this objection" — different axes (strongest vs most common), so internally fine. **But:** if any chapter nominates a different objection as the strongest, that collides with this. This is the classic "two things both called the most underestimated" shape.
6. "**The most common version** of this objection does not concern any single policy." **Subject:** the Vermont-specificity objection. **Location:** Introduction, item 5 @502436.
7. "...Chapter 14 (Political Sustainability) analyzes it directly as **the primary failure mode to monitor.**" **Subject:** the access-reduction risk from reference-based pricing. **Location:** Introduction, item 1 @489544. Flag: "the primary failure mode" is a singular ranking claim about the whole book's risk set — check whether Chapter 14 (or 6, or the Conclusion) names a *different* primary failure mode.
8. "...the **most dangerous financial period** may not be the destination (FY2030 statewide global budgets) but the transition itself..." **Subject:** FY2027–28 transition window. **Location:** Introduction, item 6 @512202. Hedged with "may".
9. "GMCB's FY23 enforcement actions against UVMMC (\$80.3M overage) and RRMC (\$11.1M overage) were **the first such enforcement actions in Vermont's regulatory history...**" **Subject:** GMCB enforcement precedent. **Location:** Introduction, item 7 @514322. Recurs verbatim in the **Conclusion** and in **Appendix G §G.4.2** — checked, all three agree on "first such ... in Vermont's regulatory history" and on both dollar figures. Consistent.
10. "...**Vermont's largest, most powerful hospital system** testing GMCB's authority directly — was decided against UVMMC." **Subject:** UVMMC. **Location:** Introduction, item 7 @514322. Consistent with the same paragraph-set's "approximately 50% of the state's hospital market share".
11. "That outcome is **the strongest available evidence** that "mandatory" in this book's framework describes a regime that has been tested against **its most resourced potential opponent** and held." **Subject:** the GMCB v. UVMMC outcome. **Location:** Introduction, item 7 @514322. Appendix G phrases the same case as "**The clearest evidence** of what "mandatory" means in practice" — different wording, same rank claim, no conflict.
12. "**The central analytical contribution of this book** is the five-pillar framework for healthcare transformation..." **Subject:** the five-pillar framework. **Location:** Introduction @518943.
13. "...Maryland's experience with hospital global budgets is **the primary counterevidence...**" **Subject:** Maryland. **Location:** Introduction, item 1 @489544.
14. "...**it is the most direct bridge** between policy mandate and organizational design." **Subject:** Chapter 16 (AHS Restructuring Roadmap). **Location:** Introduction, policy-professional profile @555736.
15. "...are **often the fastest way** to find the actionable content inside a Vermont-heavy chapter." **Subject:** the "Implications for You" sections. **Location:** Introduction @480672.
16. "**Few words in healthcare are used more often** than "reform" or "transformation"..." **Subject:** terminology. **Location:** Introduction @459968.
17. "...has a documented record of failure that is **longer and more extensive** than the record of global budget problems." **Subject:** fee-for-service vs global budgets. **Location:** Introduction, item 2 @493056.
18. "...**payment reform is the master variable...**" **Subject:** Economics pillar. **Location:** not in range — this is Chapter 6's §6.1 heading, logged here because the Introduction's competing claim #12 names the *framework* as the central contribution and item 2 leans on Economics' primacy. Flagged for the cross-chapter pass.

Count claims logged (non-superlative but checkable across chapters): "nearly forty" tools (×2 in range), "eight tracks", "four primary reader profiles", "fourteen hospitals" / "nine of fourteen" / "thirteen of fourteen", "230 meetings", "3,100 participants", "more than 100 organizations", "eleven states", "\$700M–\$2.4B", "108%", "22%", "358%", "250–300%", "715–944%", "13% by 2040", "367,000 → 318,000", "21.7% → 30%", "91% primary care access", "11-point gap", "fifteen years" (CMMI), "nine years" (All-Payer ACO Model), "\$138M → ~\$10M" (EAST Fund), "\$80.3M / \$11.1M" overages.

Other issues noticed (within this range only)

A. Structural repetition: the Preface re-derives the Introduction's figures instead of cross-referencing

The Preface (@398601) states nine of fourteen at a loss, thirteen of fourteen by 2028, \$700M–\$2.4B, 108%/22%, and over-65 toward 30% — then the Introduction (@444748–448998) states all five again at greater length. The Preface explicitly signposts the overlap ("recounted in detail in the Introduction that follows", @396661), so this is a deliberate rhetorical device, not accidental duplication. Noted because the project's own "done" criterion says front matter should be cross-referenced rather than re-derived, and here the *Preface* is the re-deriving party. **Found and deliberately left** — it reads as intended framing, and the numbers are identical, so there is no drift risk. Raising it as a question, not a work order.

B. Deliberate recurrence (not defects), listed so a similarity scan doesn't re-flag them

- "Vermont is not an outlier. It is a preview." appears as a pull-quote paragraph (@451358) and then as a Heading2 (@467411). Intentional — quote sets up the section it names.
- "Vermont is not the audience for this book. Vermont is the evidence." (pull-quote @485107) restates "Vermont is the evidence, not the scope." (@474617). Intentional bookending of the same section.
- The ecosystem description appears in Preface prose (@422543–428513), the Preface EXPLORE callout, and the Introduction's ecosystem section + table. Substantially the same content **three times** — the Preface even ends its version by deferring to the Introduction ("The Introduction that follows explains how the three components fit together"), which makes the Preface's own three-bullet breakdown at @425249–427552 redundant with the table at @529470. This is the one genuine near-duplicate in the range that is not doing rhetorical work. Left unedited (read-only), flagged for sign-off: the Preface bullets could compress to a sentence plus the pointer it already carries.
- "Each chapter ends with two doorways / EXPLORE ON THE PLATFORM ... GO DEEPER — ACADEMY" is stated in the Preface @428513 and again in the Introduction @541326 and again in the ecosystem table's third column. Three statements of one fact.
- The four reader profiles are given in prose (@554359–571633) and then again in the table at @573297. The table is a summary of its own prose — legitimate recurrence by the project's own rule, and the table carries a caption saying so.

C. Floating-table hazard — the reader-profile table is absolutely positioned

The table at @573297 carries `<w:tblPr ... w:vertAnchor="text" w:horzAnchor="text" w:tblpX="-144.00000000000034" w:tblpY="382.8124999999818" />` — it is a **text-wrapped, absolutely positioned floating table**, almost certainly an artifact of a Google Docs round-trip rather than an authoring choice (the fractional coordinates are the tell). It is the only floating table in the range; the other four are in-flow. A floating table positioned 382.8pt down from its anchor can overlap body text, and it plausibly explains why its caption paragraph ends up *after* it at @588505 and why there are two empty paragraphs (@589211, @589576) before the next Heading2. **Not touched** (read-only, and this is a layout claim that should be checked against `render_check.py` before anyone acts on it — the XML shows the risk, only a render shows the damage).

D. Framing tension, minor — the Act 167 analysis described two ways

Item 4 (@499051) states in the book's own voice: "Oliver Wyman was paid \$1 million by the Vermont state government to **produce a report that would support the case for reform.**" Elsewhere the same analysis is characterised as a diagnostic mandate (Introduction @475374 calls Act 167 "a diagnostic mandate"). Conceding the report was commissioned to support a predetermined conclusion is stronger than the steelman requires and sits awkwardly with the diagnostic framing. It is inside a strongest-case-against section, so it may be intentional generosity to the objector — but it is stated as fact, not as the objector's claim. **Left; raising as a question.**

E. Chapter 15 is absent from every reading path

Chapter 15 ("Healthcare Transformation as Portfolio Management — Applying PMI Standards") is named in no reader profile; it is covered only by the blanket "Chapters 12-16" in the student path. Chapter 12 is likewise never named individually. Not an error — noted as thin coverage in the How-to-Use section.

F. Out-of-range findings surfaced by the cross-checks (reported, not audited)

Turned up while attributing strings to chapters. **Outside the Preface/Introduction range, so not audited and not fixed** — flagged because the project rule is that one bad claim means grepping the whole subject immediately:

1. **Appendix A appears to contradict itself on the FY25 budget increase.** One sentence reads "For FY25 it approved \$3.7 billion in system-wide hospital spending — a **3.5% increase** after hospitals requested 8%", and the table immediately below reads "FY25 approved \$3.7B (**+4.1%** vs. 8% requested)". Same figure, same appendix, two different percentages.
2. **Appendix A appears to misattribute the RRMC overage to Porter Hospital.** The 14-hospital table row for Porter Hospital carries "FY23 overage of \$11M", while the \$11.1M FY23 overage is everywhere else RRMC's (Rutland Regional). Likely a row-level error.

Both need a separate read-only pass over Appendix A before anyone edits.

Summary

- **Zero numbered figures or tables exist in the Preface or Introduction**, and zero by-number citations — so the Chapter 1 / Figure 1.3 failure mode has no surface here. The equivalent risk moved to **chapter cross-references**, and that is where the defects are.
- **3 real defects:** the Chapter 13 enforcement citation (finding 1, wrong chapter — Chapter 13 contains the word "enforcement" zero times); the Preface's December 2025 dateline against July 2026 events (finding 2); and "Chapters 2-11 develop each pillar" folding the Equity Imperative into the pillars the Introduction insists it is not (finding 14).

- **2 soft contradictions:** the transition-window paragraph promising directness it has just disclaimed (12), and the 358% "most expensive hospital" sitting under a 944% figure on qualifiers alone (13).
- **All arithmetic checks pass** — 3+8=11 states, 9/14=64%, \$456→\$948=108%, 367k→318k=13%, and every figure shared between the Preface and Introduction agrees, with no drift against Chapter 2 or Appendix A.
- **Both platform count claims verified against source, not inferred:** 39 tools in **tools.ts** ("nearly forty" ✓), 8 tracks with all three cited titles matching verbatim in **_build_five_pillars_course.py** ✓. Live Supabase publication state not checked.
- Nothing was edited. No render was run (no layout claim is asserted here — finding C reports XML only).

===== FILE: audit_ch01.md =====

Chapter 1 Audit

Chapter 1: The Five-Pillar Framework and the Execution Sequence Method: **word/document.xml** read directly via **zipfile** + ElementTree (tables extracted cell-by-cell, so nothing depends on the lossy **.md** mirror). Chapter body = block indices 135–439 (Heading1 "Chapter 1:" → Heading1 "Chapter 2:"). 407 non-empty blocks; 29 tables. Read-only; the **.docx** was not modified. Bracketed **[nnn]** numbers below are block indices in that extraction, for re-location.

Re-verified the items fixed earlier tonight — all four now hold:

- §1.6 vs Figure 1.3 dependency count: §1.6 [313] no longer asserts a count; Figure 1.3 has exactly 9 filled cells and §1.4's five subsection headings sum 3+2+1+1+2 = 9. **Consistent.**
- §1.8 [320]–[323] is now a substantive section with the topological argument, not a stub. **OK.**
- §1.2 signpost [192] present and pointing at §1.3–§1.7 then §1.8/§1.9 (but see O-6: garbled clause).
- "What Earns a Place in the Matrix" [220]–[234] reads cleanly and the currency table [224] backs it.

Figures found (ground truth)

All ten figures present, in order, each with a caption paragraph immediately after its table.

Fig	Block	Table shape	What the cells actually say
1.1	tbl [185], caption [186]	5 rows × 4 cols (header: Seq. Gap / Structural Error / Direct Consequence / Cascade Effect)	Rows: Policy (Stage 1) — payer mandate gap, Gobeille, BCBSVT exit Jan 2023, ~93,000 enrollees = 1/3 of population; Technology (Stage 2) — data substrate gap, 36% higher ED / 18% lower primary care vs national peer ACOs; Economics (Stage 3) — risk on top of both gaps, conflicting net-savings conclusions; System-level — "three sequencing gaps compounding over the federal agreement's ~8-year run", wind-down end of 2025
1.2	tbl [209], caption [210]	6 rows × 4 cols	Header: Pillar / Diagnostic question / Structural role / What it produces. Rows: Policy (permissible?), Technology (possible?), Economics (sustainable?), Clinical (effective?), Operations (executable?). No Equity row — stated explicitly at [208]
1.3	tbl [217], caption [218]	6 rows × 6 cols (FROM ↓ / TO →)	Filled cells (9): Policy→Technology ENABLES; Policy→Economics ENABLES; Policy→Operations DRIVES; Technology→Economics ENABLES; Technology→Clinical ENABLES; Economics→Clinical DRIVES; Clinical→Operations REQUIRES; Operations→Policy INFORMS ⇄; Operations→Technology RUNS ⇄. Empty: Policy→Clinical, and all remaining cells. No Equity row/col
1.4	tbl [308], caption [309]	6 rows × 5 cols	Header: Pillar removed / Immediate / Second-order / System-level / Historical example. Five pillar rows. Examples: "Vermont 2010–2022: OneCare voluntary ACO. Modest results; 9/14 hospitals in losses by 2023"; early-CMMI ACO failures; "U.S. national experience: 20+ years"; "Maryland HSCRC early period"; "Most state reform efforts of the 2010s"
1.5	tbl [317], caption [318]	6 rows × 4 cols	Pillar / Primary Vermont intervention / Secondary / Developed in. Chapter pointers: Policy→2–3, Technology→4–5, Economics→6–7, Clinical→8–9, Operations→11, 16
1.6	tbl [371], caption [372]	6 rows × 5 cols	# / Pillar / Key action / Why this sequence / Vermont anchor. Stages 1–5 = Policy, Technology, Economics, Clinical, Operations. Anchors include "\$195M first-year RHT award"; "EAST Fund (roughly \$138M expected; cut to a cap near \$10M)"; "Blueprint 5.8:1 ROI"; "\$1,303 per-discharge administrative gap"

Fig	Block	Table shape	What the cells actually say
1.7	tbl [402], caption [403]	6 rows × 4 cols	Sequencing error / Early-warning signals / Vermont evidence / Correction. Five error rows: Economics-without-Policy; Technology-without-Operations (VITL); Clinical-without-financial-alignment; Equity-review-at-the-end; Operations-without-capacity
1.8	tbl [406], caption [407]	6 rows × 3 cols	Pillar / Status (timeframe) / Where it stands. Policy Completed (2022–2025), gate "fully open"; Technology In progress (2025–2026), "partially open", "current critical bottleneck"; Economics Staged (2027–2028); Clinical Parallel (2024–2028); Operations Building (2025–2028)
1.9	tbl [413], caption [414]	5 rows × 3 cols	Do this / On this tool / What to look for. Tools: Five-Pillar Map — /about/framework; HTR Simulator; Transformation Friction Index; Impact Simulation — /impact-simulation
1.10	tbl [418], caption [419]	6 rows × 2 cols	Lesson / What it covers. Lessons 1–5, titles as listed in the §1.12.3 Academy box

Unnumbered tables in the chapter (stat tiles, worked examples, VERMONT EVIDENCE / BEYOND VERMONT / TRY THIS / GO DEEPER callouts, the currency table [224], the gate-definition box [236], the equity pairing table [213]): 19 more, correctly *not* figure-numbered.

Claims checked against their own source

MATCH (verified, no action)

- "**nine dependency relationships**" — [136], [215], [234], stat tile [305], glossary. Figure 1.3 has exactly 9 filled cells. §1.4's subsection headings sum to 9 (3+2+1+1+2). **MATCH.**
- "**Figure 1.3's seven build-order edges — the ones without a ↻**" [322]. 9 filled − 2 ↻ = 7. **MATCH.**
- "**only one sequence satisfies all of them at once**" [322] — I ran the topological sort on the 7 non-feedback edges (P→T, P→E, P→O, T→E, T→C, E→C, C→O). The order P, T, E, C, O is genuinely the **unique** topological order: at each step exactly one node has no unplaced predecessor. The chapter's strongest structural claim is **true as stated**. **MATCH.**
- "**three critical-path dependencies: Policy enables Economics, Economics drives Clinical, and Technology enables Economics-as-management**" [380]. Exactly three dependency headings carry "(critical path)": [243] Policy→Economics, [256] Technology→Economics, [275] Economics→Clinical. **MATCH.**
- Reciprocal-arrow claim** [219] — "Policy → Operations is a deadline, Operations → Policy is implementation data." Policy/Operations is the *only* reciprocal pair in Figure 1.3. **MATCH.**
- OneCare lifespan** — [150] "roughly twelve years, founded in 2013 and closing at the end of 2025" and [244] "ran for twelve years". Arithmetic holds; the "~8-year" figure in Figure 1.1 is scoped to the *federal agreement*, a different object. **MATCH** (but see M-2).
- BCBSVT exit** — ~93,000 enrollees ≈ one-third, Jan 1 2023, two stated reasons: consistent at [152]–[154], Figure 1.1 Policy row, and [244]. **MATCH.**
- Gobeille** — [152] "ruled 6–2", cited as 577 U.S. 312 (2016) in caption [186] and Sources [438]. Vote and citation both correct. **MATCH.**
- Utilization signature** — 36% higher ED / 18% lower primary care: [172] and Figure 1.1 Technology row agree, both attributed to a state audit, and Sources lists the 2019/2021 Auditor performance audits. **MATCH.**
- Administrative-cost arithmetic** — [366] PPS \$2,730 vs \$1,427 benchmark = \$1,303 gap; the "\$1,303 per-discharge gap" claim in the same paragraph and in Figure 1.6 row 5 both check out. UVMHC \$3,826 vs the same \$1,427 benchmark is consistent between [286] and [366]. **MATCH.**
- Deficit projection** — "\$700 million to \$2.4 billion" five-year: [245] and [327] identical. **MATCH.**
- EAST Fund** — "about \$138 million" cut to "a cap near \$10 million" on the July 2026 AHEAD withdrawal: [233] and Figure 1.6 row 3 agree; the AHEAD withdrawal date is consistent at [231], [317], [353], [406], [433]. **MATCH.**
- Blueprint 5.8:1** — [277] states it as "\$5.8 million in reduced medical expenditure per \$1 million invested", consistent with the ratio used at [267], [363], Figure 1.4 Economics row, Figure 1.6 row 4, Figure 1.7 row 3. **MATCH** (see O-3 on frequency).
- RBP ≤200% of Medicare** — [359] and Figure 1.6 row 3. **MATCH.**
- Equity cross-reference to the Introduction** [270] — "The 91% primary care access rate and the 11-point white/BIPOC gap behind it — both set out in the Introduction". Verified in the Introduction at block [95]: "roughly 91% — four points above the national benchmark... an 11-point gap between white Vermont adults and BIPOC Vermont adults". The chapter cross-references rather than re-deriving, exactly as criterion 4 requires. **MATCH.**
- Gate vocabulary** — the definition box [236] ("open = passable, work done; closed = blocked; opened by completing the work, never by deciding to proceed") is used consistently at [384]–[387], Figure 1.8, [392], and the glossary entry [428]. **MATCH.**
- Academy internal consistency** — the §1.12.3 box [398] cites "Lesson 4, 'Dependency Logic and the Execution Sequence'" and "Lesson 5, 'Sequencing Failure — The OneCare Autopsy'"; Figure 1.10 lists those exact titles at rows 4 and 5. **MATCH.**
- Figure-index titles** — all ten Chapter 1 captions in the Figure Index [2361]–[2370] match the in-chapter captions verbatim, with one exception (see O-5).
- Platform routes named in §1.4 and §1.9** — [/about/framework](#) [216] and Figure 1.9, [/impact-simulation](#), plus HTR Simulator and Transformation Friction Index: all four exist ([frontend/app/about/framework](#),

[frontend/app/impact-simulation](#), [frontend/app/htr-simulator](#), Friction Index registered in [frontend/lib/taxonomy/tools.ts](#)). Route existence only — per CLAUDE.md, a 200 is not a delivered promise, and promise-delivery was out of scope here.

MISMATCH (with proposed fix)

M-1. The "Clinical gate" in §1.12.1 reverses a Figure 1.3 arrow. — highest-value finding. [387]: *"The Clinical gate — CLOSED until clinical programs reach scale. Utilization reduction must be operating before the full financial accountability of global budgets takes effect."* This makes Clinical an upstream gate on **Economics**. Figure 1.3 has no Clinical→Economics cell; it has **Economics→Clinical DRIVES**, and §1.8's unique-order argument places Economics *third* and Clinical *fourth* precisely on that basis. The chapter's own admission rule at [222]–[227] invites a reader to challenge exactly this ("name the currency, and name its issuer") — and Clinical's currency is OUTCOMES [224], which is plausibly what a global budget needs to convert pressure into savings. So the sentence either implies a missing tenth cell (which would break the "nine" count *and* destroy the uniqueness of the P→T→E→C→O order) or it is loosely worded. The book's substantive position is stated correctly elsewhere — Figure 1.4's Clinical row: *"global budgets create pressure without the programs that reduce cost."* That is about savings being *realized*, not about accountability being *assumable*. *Proposed fix* (preserves the matrix, no renumbering): **"The Clinical gate — CLOSED until clinical programs reach scale. Global budgets can take effect without it, but until utilization reduction is actually operating they produce financial pressure rather than savings (Figure 1.4)."**

M-2. Figure 1.4 dates OneCare "2010–2022"; §1.2 establishes 2013–2025. Figure 1.4, Policy row, Historical example: *"Vermont 2010–2022: OneCare voluntary ACO. Modest results; 9/14 hospitals in losses by 2023."* But [150] says OneCare was *"founded in 2013 and closing at the end of 2025."* A reader comparing the figure to §1.2 sees OneCare both starting three years before it was founded and ending three years before it wound down. *Proposed fix*: **"Vermont 2013–2025: OneCare voluntary ACO."** (If the intent was the pre-Act-68 era rather than the entity, say so: "Vermont's voluntary era, 2010–2022".)

M-3. The §1.2 "BEYOND VERMONT" box mis-summarises §1.2's own three failures. [190]: *"The three sequencing failures above — economics without a policy foundation, payment reform without data infrastructure, and clinical redesign without measurement — ..."* §1.2's three subsections are 1.2.1 Economics without Policy, 1.2.2 Economics without Technology, and 1.2.3 **The Cascade**. "Clinical redesign without measurement" appears nowhere in §1.2 — no clinical failure mode is diagnosed there at all. *Proposed fix*: **"...economics without a policy foundation, payment reform without data infrastructure, and the cascade the two produce together — are not OneCare-specific defects."**

M-4. §1.10.3 attributes the VITL example to a cause the chapter never gives it. [348]: *"Vermont's VITL health information exchange is cited in this chapter as an example of technology deployed without the surrounding incentive structure."* The chapter's only citation of VITL as a failure is Figure 1.7 row 2, which attributes it to Operations, not incentives: *"VITL: sound platform, incomplete adoption among smaller providers for lack of Operations-pillar support to connect and train them."* Two further problems: the tense is wrong (Figure 1.7 is in §1.13, three sections *later*), and the paragraph is ventriloquising a critic using the book's own example, so the misattribution weakens the rebuttal that follows. *Proposed fix*: **"Vermont's VITL health information exchange is cited later in this chapter (Figure 1.7) as technology deployed ahead of the operational support needed to use it — a cousin of exactly the outcome the technology-first argument is supposed to prevent."**

M-5. §1.11.3 turns a first-year RHT award into a five-year annual one. Three passages treat the \$195M carefully as a **first-year** figure — [231] "a \$195 million first-year award", [353] "\$195 million for Vermont in its first year (federal fiscal 2026)", Figure 1.6 row 1 "\$195M first-year RHT award". [360] then says: *"comes from the five-year, \$195-million-per-year Rural Health Transformation Program."* That reads as a \$975M commitment and contradicts the other three. *Proposed fix*: **"...comes from the five-year Rural Health Transformation Program, whose first-year award to Vermont is \$195 million."**

M-6. §1.14.1's "January 2028" does not follow from FY2028. [409]: *"Vermont hospitals will begin bearing global-budget financial accountability in January 2028."* Everywhere else the chapter says global budgets are mandatory *beginning* FY2028 ([201], [244], [250], [317], [384], [406]). Vermont's state fiscal year begins **July 1** and the GMCB hospital budget year begins **October 1** — neither produces a January start. The specificity is unsourced and inconsistent with the chapter's own FY framing. *Proposed fix*: drop the invented month — **"Vermont hospitals begin bearing global-budget financial accountability in FY2028. If the AHS-GMCB analytics capability is not fully operational by then..."**

M-7. §1.6's forward pointer names the wrong section. [312]: *"Getting the sequence wrong is not merely inefficient; §1.14 sets out what it costs."* §1.14 is "Vermont's Implementation Timeline: Reading the Sequence in Statutory Deadlines" — a status table plus the Technology–Economics race. The section that sets out **what getting it wrong costs** is §1.13, **"Five Sequencing Decisions That Organizations Get Wrong"** (Figure 1.7: pattern, warning signals, correction). The same paragraph's other pointer, "Sections 1.8 through 1.14 develop that ordering in full", is fine. *Proposed fix*: **"...§1.13 sets out what it costs."**

M-8. §1.3's equity cross-reference points the wrong way. [208]: *"Equity is not a row here; it is the Equity Imperative applied to every row (see the table above)."* The equity pairing table is at [213] — *below* [208], under the "THE EQUITY IMPERATIVE — APPLIED AT EVERY STAGE" heading [211]. Nothing above [208] is an equity table. *Proposed fix*: **"(see the Equity Imperative table below)."**

M-9. The §1.1 stat tile disagrees with the §1.1 prose on both the model count and the span. Prose [138]: *"launched more than seventy models"* over *"the following fifteen years"* counted from 2010. Tile [139]: **"70 / CMMI Models Tested (2011–2024)"** and **"15 Yrs / Span of Experience"**. 2011–2024 is fourteen years, not fifteen, and "70" contradicts "more than seventy" — which also makes the tile's "4 of 70" denominator soft. *Proposed fix*: make the tile read **"70+ / CMMI Models Tested (2010–2025)"** and **"4 of 70+ / Expanded Nationwide"**, or change the prose to "roughly seventy models" and the tile span to "2011–2024 · 14 Yrs". Either way the three numbers must be reconciled to one basis.

M-10. The CBO figures in §1.1 do not sum, and a reader will subtract them. [138]: *"they cost \$7.9 billion to operate, reduced spending on health care benefits by \$2.6 billion, and increased net federal spending by \$5.4 billion."* \$7.9B – \$2.6B = \$5.3B. The published CBO net figure is \$5.4B (each component independently rounded), but as printed the sentence invites a reader to catch the book out at arithmetic. *Proposed fix*: either drop the middle term — **"they cost \$7.9 billion to operate and increased net federal spending by \$5.4 billion between 2011 and 2020"** — or print the net as derived: **"...reduced benefit spending by \$2.6 billion, for a net increase in federal spending of roughly \$5.4 billion."**

SOFT / WATCH (defensible as written; flagged for the author's call)

- **S-1. Maryland is both the success and the cautionary tale.** Figure 1.4's Clinical row cites *"Maryland HSCRC early period: global budgets without adequate primary care produced pressure without sustainable improvement"*, while the §1.4.1 BEYOND VERMONT box [247] credits Maryland with a 2.8% Medicare spending reduction, 4.1% hospital reduction, "more than \$780 million saved", 16.1% fewer potentially preventable admissions and 9.5% fewer readmissions. The box's own caveat ("population health measures did not move") reconciles them, and Figure 1.4 says "early period" — but the figure never dates that period. *Suggested hardening:* "Maryland HSCRC early period (2014–2018), before the MDPCP primary care investment."
- **S-2. 9/14 hospitals: 2023 or 2024?** [327] attributes "nine of fourteen hospitals in operating losses" to Oliver Wyman's **September 2024** presentation; Figure 1.4 says "9/14 hospitals in losses **by 2023**". Compatible (a 2024 presentation reporting FY2023 results) but unstated.
- **S-3. Eight years or nine?** Figure 1.1 says the federal agreement had a "~8-year run" and the Sources line cites VTDigger's "8-year 'all-payer' experiment"; the **Introduction** [72] says the predecessor model "produced modest results over **nine** years". Cross-section, front-matter-facing — and under criterion 4 the chapter yields, so if either changes it should be Chapter 1's "~8-year" that acquires a date range rather than the Introduction that moves.
- **S-4. Maryland's "52 of them".** [247] states the all-payer model put "essentially every hospital in the state — 52 of them — under global budgets beginning in 2014." Not verifiable internally; the commonly cited figure for the 2014 all-payer model is the ~46–47 acute-care hospitals. Worth one external check before print.
- **S-5. §1.6's risk criterion vs Figure 1.3's counts.** [313] reasons that "a pillar with many direct dependents in Figure 1.3 carries outsized risk" and then names Technology (2 dependents) — while Policy has 3. The paragraph saves itself with "and right now it is where the system's most consequential execution gap actually sits", and the Policy gate is OPEN everywhere in the chapter, so the conclusion is right. But the stated criterion, read alone, picks Policy. One clause would close it: "...among the pillars whose gate is not yet open, Technology is the clearest case."

Superlative/ranking claims logged for cross-chapter comparison

Verbatim, with block index. These are the claims most likely to collide with Chapters 2–16.

OneCare as the canonical case (two competing "most" claims about the same object):

1. [136] "OneCare Vermont's collapse is **the most instructive sequencing autopsy** in recent American health policy."
2. [424] "Vermont's OneCare failure is **the most precisely documented sequencing failure** in recent American health policy." — Different predicates (instructive vs documented), same object, same qualifier "in recent American health policy", ~290 blocks apart. Not a contradiction; watch for a third variant elsewhere.
3. [152] "**The most fundamental flaw** in OneCare Vermont's design was attempting to operate a complex, population-level payment model... without a policy foundation."
4. [430] glossary: "**OneCare Vermont is canonical:** Economics (voluntary ACO) deployed without Policy mandate and Technology substrate."

"Closest thing to a controlled experiment" — two claimants: 5. [316] "Vermont's healthcare transformation is **the closest thing to a controlled experiment** in simultaneous five-pillar intervention that American health policy has produced." 6. [247] (Maryland) "A decade of evaluation is **the closest thing American health policy has to a controlled test** of this dependency." — Scopes differ (five-pillar simultaneity vs one dependency), so both can stand; the near-identical phrasing 70 blocks apart is the risk, not the logic.

Highest-risk / highest-leverage / highest-ROI cluster (four claims, three objects): 7. [313] "The AHS-GMCB analytics capability is **the single highest-risk gap in the system.**" 8. [314] "...analytics deployment, AHS organizational capacity, and HCC gap closure are **the highest-leverage investments available to Vermont right now.**" 9. [410] "HCC gap closure, **the highest-ROI pre-launch analytics investment.**" 10. [435] glossary: "HCC gap closure: **the highest-ROI pre-global-budget analytics investment.**" — Note 9 vs 10 word the same superlative two ways ("pre-launch" / "pre-global-budget"). Harmless but worth standardising. 11. [409] "Vermont's **most significant current sequencing risk** is the gap between the FY2028 start of Act 68's hospital global budgets and the analytics-capability deployment timeline." 12. [422] "...a delay in the analytics capability is **a compound failure across multiple statutory obligations.** This deserves **the urgency of a critical-path emergency.**" 13. [421] "**The most important action you can take** before Act 68's global budgets take effect in FY2028 is completing HCC gap closure."

Framework-level superlatives: 14. [363] Blueprint's 5.8-to-1 ROI is "**among the most robust ROI findings** in the value-based care literature." 15. [204] Operations is "**the graveyard** of well-designed reforms that could not be implemented." 16. [209] Figure 1.2, Operations row: "**The most analytically sound framework fails** if the implementing organization lacks capacity." 17. [198] Technology "is **the information substrate on which most other pillar functions depend.**" 18. [215] "Reading those nine relationships accurately is **the core analytical skill this book develops.**" 19. [332] "The critical and **counterintuitive** placement is Technology before Economics. **Most reform architects reverse this.**" 20. [263] Figure-1.4-adjacent BEYOND VERMONT: "This dependency is where **most states are structurally disqualified before they begin.**" (with "roughly two dozen states operate an established all-payer claims database") 21. [240] Policy's outbound dependencies "are **among the most critical** in the framework."

Rhetorical superlatives in the critics section: 22. [338] "**The most sophisticated critique:** sequencing frameworks are analytical stories told after the fact." 23. [394] "**The most common sequencing error** organizations make with the equity imperative is treating equity review as the last step." 24. [307] "**The most rigorous test** of a dependency framework is not whether it describes how things work when all components are present." 25. [73] (Introduction, for comparison) "This is **perhaps the strongest challenge in the book.**"

Cross-chapter watch items: #7/#11 both nominate the AHS–GMCB analytics gap as Vermont's top risk — if Chapter 4, 5, 11 or 16 nominates a different "highest-risk gap", that is a live contradiction of the kind CLAUDE.md flags ("two different dependencies both called 'the most underestimated'"). Likewise #14 (Blueprint ROI "among the most robust") against any Chapter 8/9 phrasing, and #19 ("most reform architects reverse this") against Chapter 6/7.

Other issues (thin sections, repetition, contradictions)

O-1. Genuine repetition: the three-dependency convergence, stated three times.

- [313] §1.6: "...holding up financial management for global budgets, equity measurement for the Statewide Strategic Plan, and risk stratification for Blueprint's CCBHC expansion at once."
- [410] §1.14.1: "...it delays the financial management capability for Act 68 global budgets, the equity measurement capability for the Statewide Strategic Plan, and the risk stratification capability for Blueprint's CCBHC expansion. Three dependencies converge on this single Technology pillar investment."
- [422] §1.17: "The three-dependency convergence — global-budget management, Statewide Strategic Plan measurement, CCBHC risk stratification —..."

[422] is legitimate recurrence (audience-directed "Implications for You", and it uses the compressed form). **[313] and [410] are a near-verbatim duplicate pair** — same three items in the same order, same claim, ~100 blocks apart, and both also assert "a delay does not delay one program but three." *Proposed fix:* keep the full statement at §1.14.1 where the vulnerability is the subject, and cut §1.6 back to the mechanism plus a pointer — "...a delay there does not delay one program but three (§1.14.1)."

O-2. Borderline repetition: the "signing a contract you cannot read" argument, twice. [257] §1.4.2 prose ("cannot identify where its costs are above benchmark... cannot dispute errors in the payer's shared-savings calculations") is then dramatised at length in the worked example [259], and the same argument reappears compressed at [381] §1.12.1 ("has exposed itself to financial losses it cannot explain, cannot dispute, and cannot manage"). The worked example is the chapter's best writing and earns its space; [381] is a one-line callback and is fine. Logged, not a defect.

O-3. Blueprint's 5.8:1 appears six times ([267], [277], Figure 1.4 Economics row, [363], Figure 1.6 row 4, Figure 1.7 row 3). [267] explicitly handles the overlap ("taken up under Economics below, rests here on a narrower condition") — that pair is deliberate and well done. The four remaining instances are figure cells and stage summaries, i.e. legitimate recurrence. **No action**, but it is at the ceiling of what a reader will tolerate for one statistic.

O-4. Legitimate recurrence, explicitly NOT repetition (checked and cleared, so it is not re-raised later): the Equity Imperative subsection at the end of each of §1.4.1–§1.4.5 (structural by design, announced at [238]); the five diagnostic questions restated in [209], [213] and [195] (the restatements are the point — the pairing table qualifies each question); the P→T→E→C→O sequence restated at [322], [332], Figure 1.6 and glossary [427] (each does different work — derivation, thesis, operational detail, definition); Figure 1.5 and Figure 1.6 both listing Vermont anchors by pillar (navigation map vs execution order, and [319] says so).

O-5. Figure 1.1's source line has drifted between the chapter and the Figure Index. In-chapter [186]: "...Based on GMCB records, AHS November 2025 Transformation Report, OneCare Vermont public filings, **Gobeille v. Liberty Mutual Ins. Co., 577 U.S. 312 (2016), and VTDigger reporting (Dec. 20, 2022; Mar. 13, 2026).**" Figure Index [2361] stops after "OneCare Vermont public filings." The other nine Chapter 1 captions match the index verbatim. *Proposed fix:* copy the full in-chapter caption into the index entry.

O-6. Garbled clause in the §1.2 signpost. [192]: "...before §1.8 and §1.9 return to state and prove the general rule this case establishes." "return to state" is missing its object (or is a stray "to state"). *Proposed fix:* "...before §1.8 and §1.9 state the general rule this case establishes and prove it."

O-7. Stray double period. [328] ends: "...it is not a matter of institutional preference or political opportunity. ." — a period, a space, and a second period.

O-8. Trailing-space artifacts on [142], [150], [152]→no, [155], [156], [174], [175], [250], [195]. Cosmetic; harmless in a Google Docs round-trip. Noted, not proposed for edit.

O-9. Thin / hand-waving passage: [326]. "Vermont did not choose careful sequencing because it was disciplined. It was forced into it because it had **the scars from earlier experiences** and had run out of time to get it wrong." Every other causal claim in the chapter names its instrument; this one gestures. The specifics are two paragraphs away ([327]: OneCare, the Oliver Wyman trajectory). *Fix if the author wants it:* "...because a decade of OneCare had already shown what the wrong order produces, and the financial trajectory left no time for a second attempt."

O-10. §1.10.2 is the thinnest of the three critic subsections — four short paragraphs ([341], [342], [344], [346]), and its rebuttal [342] is a single sentence ("The counterevidence is OneCare itself: a decade of voluntary reform produced modest results and left the financial crisis unaddressed") before pivoting to the reframing at [344]. §1.10.1 and §1.10.3 each carry a developed argument (the brakes analogy; the "technology *without* economics" distinction). Not a defect — [346] hands the subject off to Chapter 14 deliberately — but it is the one place a critic gets less than a fair hearing, and the critique named there (mandatory architecture provokes litigation, with UVMCM in court as evidence) is the most concrete of the three.

O-11. Pull-quote convention verified as intentional, not a defect. [143], [156], [176], [184], [329], [344], [368] are unpunctuated or single-sentence standalone paragraphs. They are consistent with one another as display pull-quotes, so the missing terminal periods on [143] and [176] are the convention, not an error. Deliberately left.

No unresolved contradiction was found in: the dependency count (9), the build-order edge count (7), the uniqueness of the execution order, the critical-path count (3), the gate-status set (Policy open / Technology partially open / Economics closed), the five failure-cascade rows against the five pillars, the five sequencing errors against the §1.13 heading, or the five Academy lessons against Figure 1.10. The chapter's numeric architecture is internally sound; the defects above are cross-references, a reversed arrow in §1.12.1 (M-1), and four external-figure inconsistencies (M-5, M-6, M-9, M-10).

Chapter 2 Audit

Read-only factual/self-consistency audit of **Chapter 2: The Policy Pillar — Legislative Architecture for Structural Reform**, run 2026-09-27 against `word/document.xml` inside `HTR_Book_v42.docx` (not the `.md` mirror). Body-element index range: **440 → 574** (Heading1 "Chapter 2:" at 440, Heading1 "Chapter 3:" at 575). Bracketed numbers below are body-element indices, usable to relocate text in a fresh download.

No edits were made to the `.docx`.

Figures found (ground truth)

Label	Element	Shape	Actual contents
Figure T.1 — "Vermont Healthcare Transformation Timeline: 2022–2035"	table [444], caption [445]	4 cols × 17 rows (1 header + 16 data)	Cols: Year / Milestone / Pillar / Significance. Data rows run 2022 → FY2030 (Oct 2029) . Rows: 2022 Act 167; 2023 Act 51; Aug 2024 OW report; Sept 2024 OW→GMCB; Jan 2025 AHEAD signed; 2025 Act 68; Jul 4 2025 H.R.1; Nov 2025 RHT application; Dec 2025 CMS BALANCE; Dec 29 2025 RHT award \$195M; Jul 2026 AHEAD withdrawal; FY2027 (Oct 2026) RBP mandatory; FY2028 (Oct 2027) global budgets non-CAH; Jan 2028 Act 68 global budgets take effect (FY2028); Dec 2028 Strategic Plan; FY2030 (Oct 2029) global budgets all 14.
Stat-card strip (unnumbered)	table [450]	1 row × 4 cells	9 of 14 Hospitals in Loss FY2023 · \$2.4B 5-Year Deficit Projection · 108% Premium Increase 2018–2024 · 57% Growth in 65+ by 2040
"Oliver Wyman's Three Imperatives" callout	table [453]	1×1	1. Fix the upstream drivers — build housing and other facilities and fix transportation. 2. Fix the price and budget architecture — pay PPS hospitals with reference-based pricing and move to global budgets/capitation for all when conditions for success are met. 3. Shift the locus of care — move all care possible out of hospitals.
"BEYOND VERMONT" callout	table [457]	1×1	"More funding without structural change" generalization; restates the three imperatives as a "sequencing-neutral checklist".
"VERMONT IN PRACTICE — The Hospital Financial Crisis by the Numbers"	table [469]	1×1	8 bullets: 9/14 losses FY2023 worst −8.9%; conservative (3.5% rev / 5% exp) → 13/14 by 2028, \$700M 5-yr deficit; realistic (7-8% exp) → \$2.4B vs break-even, \$3.1B vs 3% margin; \$285M FY25 budget-increase requests; UVMHN \$1.5B; silver premium \$948 in 2024 = +108% in 6 yrs; OOP max more than doubled in 5 yrs; charges +38% since 2018 vs income +22%.
Figure 2.1 — "Common perceptions ... versus the data"	table [485], caption [486]	2 cols × 5 rows (1 header + 4 data)	Header: Common perception / What the data actually shows. Four perceptions rebutted: (1) all-payer model — excludes FFS Medicare and ~40% of commercially insured, covers a minority of spend; (2) statewide ACO (OneCare) — voluntary, excludes several hospitals and many community providers, unable to provide information needed to guide care; (3) functioning HIE (VITL) — voluntary, many community providers absent, lacks pharmacy claims, not user-friendly; (4) not enough primary care providers — at 3 patients/hour there are enough well into the future; HRSA recognizes no HPSAs in Vermont; the problem is productivity and practice model, not headcount.
"VERMONT IN PRACTICE — Oliver Wyman's Priority Action Lists for 2025"	table [497]	1×1	17 priority legislative changes for the General Assembly (housing barriers; EMS funding; broadband; AHS structure review; REH + birthing-center regs; licensure/scope for nurses/EMTs/pharmacists; regionalize specialty care; professionalize/regionalize EMS; heavy-utilizer coordination; dual-eligible targeting; statewide EMR coordination; no further commercial subsidization increases; no further HOPD licensing; simplify CON; encourage free-standing centers; begin movement to RBP, ideally at 200% of Medicare or less for PPS hospitals ; uniform hospital accounting agency/method).

Label	Element	Shape	Actual contents
Figure 2.2 — "Proposed Centers of Excellence assignments by hospital"	table [510], caption [511]	3 cols × 12 rows (1 header + 11 data)	UVMHC 16 · Southwestern VT 9 · Central VT 5 · Brattleboro 6 (5 specialties listed) · Northwestern 4 · Northeastern VT Regional 4 · Rutland Regional 5 · Springfield 2 · Copley 2 · Mt. Ascutney 1 · "Grace Cottage, Gifford, North Country, Porter — TBD". 10 named hospitals + 4 TBD = 14.
"VERMONT IN PRACTICE — The \$400M Transformation Savings Estimate"	table [518]	1×1	">\$400M in direct savings over five years"; six savings sources listed (close unsustainable inpatient facilities; reduce admin costs; synergies/shared services; ambulatory/outpatient migration; primary-care access expansion; SDOH); reallocation targets; RHT \$195M framed as capital, program runs through 2030; closes with "\$400M ... is the recurring savings ... year after year".
Figure 2.3 — "Act 68 statutory implementation timeline"	title para [521], table [522], caption [523]	2 cols × 6 rows (1 header + 5 data)	FY2027 (Oct 2026) RBP mandatory for all commercial payers · January 2028 Act 68 commercial hospital global budgets take effect (FY2028) · FY2028 (Oct 2027) global hospital budgets for non-CAH · December 2028 Strategic Plan to legislature · FY2030 (Oct 2029) global budgets all hospitals incl. CAHs.
Figure 2.4 — "State reference-based pricing and rate-setting programs compared"	table [530], caption [531]	2 cols × 6 rows (1 header + 5 data)	Vermont Act 68 (methodology by rule FY2027, effective FY2028 ; reviewed annually) · Oregon 2019 (state employee plan, 200% in-network / 185% OON, rural+CAH exemptions, >\$107.5M savings in 27 months, all 24 affected hospitals stayed in-network) · Montana 2016 (state employee plan, direct contracting, \$48M initial savings, politically fragile without statutory mandate) · Washington 2021 (Cascade Care public option, 160% of Medicare, voluntary) · Maryland 2014–present (all-payer fixed annual revenue, Medicare waiver; "closest model to Vermont's Act 68 trajectory").
"VERMONT IN PRACTICE — The Reform Cascade Timeline — Statutory Deadlines"	table [540]	1×1	10 items, only three of which carry a date (Act 55 April 2025; Strategic Plan December 2028; and implicit FY references). Includes: GMCB negative-1% commercial rate-growth benchmark; \$2M AHS transformation grants with all 14 hospitals developing plans; Medicaid global budget under Global Commitment; Act 55 outpatient drug cap 120% ASP, ~\$135M/yr; RBP implementation; GMCB price-transparency dashboard; RBP vendor/rulemaking; global budgets non-CAH; Strategic Plan Dec 2028; global budgets all incl. CAHs.
Figure 2.5 — "Hands-on platform tools for the Policy Pillar"	table [557], caption [558]	3 cols × 3 rows (1 header + 2 data)	Row 1: model an 1115 waiver / all-payer global budget → Policy Simulator /research-lab/policy-quality?tab=policy . Row 2: trace statutory mandate → operational deadlines → /vermont-act-167 and /vermont-act-68 .

Empty spacer paragraphs at [451], [454], [458], [470], [498], [519], [541], [574].

Claims checked against their own source

MISMATCHES

M1 — Figure T.1's caption claims a range the table does not cover. [445] captions it "Vermont Healthcare Transformation Timeline: **2022–2035**"; the table's final row is **FY2030 (Oct 2029)**. Nothing in the table reaches past 2030. *Fix*: caption → "Vermont Healthcare Transformation Timeline: 2022–2030".

M2 — RBP effective date: Figure 2.4 contradicts Figure 2.3, Figure T.1 and the prose. Figure 2.4's Vermont row [530] reads "Methodology set by rule FY2027, **effective FY2028**." But Figure 2.3 [522] says "FY2027 (Oct 2026) — Reference-based pricing mandatory for all commercial payers"; Figure T.1 [444] says "FY2027 (Oct 2026) — RBP mandatory for all Vermont hospitals"; [527] says the act directs maximum amounts "not later than hospital fiscal year 2027"; [560] says RBP is "effective FY2027". Four places say FY2027, one says FY2028. *Fix*: Figure 2.4 Vermont row → "Methodology set by rule during FY2026; maximum allowable payments mandatory from FY2027." (If the FY2028 date is in fact correct in the GMCB Feb 2026 report, the other four instances are wrong and this is a five-place correction — verify against the source before changing either side.)

M3 — Two different effective dates for Act 68 global budgets inside the same table, both labelled FY2028. Figure 2.3 [522] contains both "January 2028 — Act 68 **commercial** hospital global budgets take effect (FY2028)" and "FY2028 (Oct 2027) — Global hospital budgets for non-Critical Access Hospitals". Figure T.1 [444] carries the same pair. FY2028 is defined elsewhere in both tables as beginning **Oct 2027**, so "January 2028 ... (FY2028)" is internally inconsistent with the table's own fiscal-year convention. Figure 2.3 additionally lists January 2028 **before** FY2028 (Oct 2027), breaking chronological order in an explicitly

chronological figure. *Fix*: collapse to one row — "FY2028 (Oct 2027) — Global hospital budgets for non-Critical Access Hospitals; commercial global budgets phase in January 2028" — and reorder so dates ascend.

M4 — "200% of Medicare" is attributed to statute; the chapter's own account of the statute says the level is set by rule. [560] (Implications for You): "RBP at 200% of Medicare is not a theoretical target; it is a **statutory mandate** effective FY2027." But [527] says Act 68 "directs GMCB to establish, **by rule**, maximum amounts", [529] says only that "prices must be based on a percentage of Medicare reimbursement", and [497] shows 200% as **Oliver Wyman's recommendation** ("ideally at 200% of Medicare or less"). Figure T.1 [444] compounds this with "≤200% Medicare target" in the FY2027 row. *Fix*: [560] → "RBP is not a theoretical target; it is a statutory mandate effective FY2027, and the Oliver Wyman benchmark GMCB is working from is 200% of Medicare or less." Change Figure T.1's cell to "RBP mandatory for all Vermont hospitals; GMCB sets the maximum by rule (Oliver Wyman benchmark: ≤200% Medicare)."

M5 — The meeting-type breakdown does not add to the stated total, and the community-meeting attendance alone exceeds the stated participant total. [491]: "over 230 meetings ... more than 3,100 participants from over 100 organizations. Meeting types included 57 sessions with hospital leadership and boards, 45 with state partners, **50 public HSA-level community meetings averaging 68 participants each**, 35 provider meetings, and 15 sessions ... for diverse populations." 57+45+50+35+15 = **202**, not 230+ (defensible — "included" is non-exhaustive). But 50 × 68 = **3,400 participants in the community meetings alone**, which already exceeds the "more than 3,100 participants" total that is supposed to cover all 230 meetings. *Fix*: either drop "averaging 68 participants each", or state the total as "more than 3,400 participants", or reword to "the 50 public HSA meetings drew roughly 3,400 attendances" — attendances rather than unique participants would reconcile it. Verify against the Oliver Wyman report before choosing.

M6 — Brattleboro's COE count does not match its own listed specialties. Figure 2.2 [510] gives Brattleboro "COE Count 6" but lists **five** specialties (acute general surgery, cancer surgery, geriatric care, orthopedics, robotic surgery). Every other small-count row in the table lists exactly its count (CVMC 5/5, Northwestern 4/4, NVRH 4/4, Rutland 5/5, Springfield 2/2, Copley 2/2, Mt. Ascutney 1/1); only the two large rows (UVMC 16, SVMC 9) abbreviate. Brattleboro is the sole small row where count ≠ items listed. *Fix*: either correct the count to 5 or add the missing sixth specialty from the source.

M7 — "three primary sources of direct savings" vs six sources listed. [517]: "Reducing exceedingly high administrative costs ... was identified as **one of the three primary sources of direct savings**." The callout [518], which itemises the same \$400M direct-savings estimate, lists **six** sources. *Fix*: [517] → "one of the principal sources of direct savings" (or name which three are primary and mark the other three as secondary in the callout).

M8 — The \$400M callout contradicts itself on whether the figure is five-year or annual. [518] opens "more than \$400 million in direct savings **over five years**" and closes "\$400M ... is the **recurring savings** that structural transformation is projected to unlock, **year after year**, once implemented." These cannot both be true. *Fix*: close with "...it is a five-year cumulative figure — roughly \$80 million a year once implemented — and it recurs only if the structural change is sustained."

M9 — Vermont's global-budget attempts are dated outside the stated window, and the stated common cause is contradicted by one of the four entries. [535]: "Vermont has attempted global budget design **four times in the past decade**: the GMCB net patient revenue cap in **2012**, the Rutland Regional pilot proposed in 2014 (**never implemented**), the Vermont All-Payer ACO Model from 2017 to 2025, and the current Act 167/Act 68 mandate." Two problems: (a) 2012 is fourteen years before the present, not within the past decade; (b) "Each prior attempt foundered on the same obstacle: voluntary participation" cannot be true of an attempt the same sentence says was **never implemented**, nor of a regulatory revenue cap. *Fix*: "Vermont has attempted global budget design four times since 2012 ... Two of the three prior attempts foundered on the same obstacle — voluntary participation; the Rutland pilot never reached implementation at all."

M10 — Figure 2.4 does not support the "all confirm this" claim made of it. [552]: "Oregon's RBP experience, Montana's experience, and Maryland's global budget model **all confirm this**" — "this" being that price control without volume control is insufficient. Figure 2.4's own Oregon row reports **success** (">\$107.5M savings in first 27 months. All 24 affected hospitals remained in-network") and says nothing about volume leakage; the Montana row attributes its fragility to the **absence of a statutory mandate**, not to uncontrolled volume. Only the Maryland row (fixed annual revenue with volume expectations) actually speaks to volume. *Fix*: "Maryland's global budget model, which pairs standardized prices with volume expectations, is the only one of these programs that closes the volume channel — Oregon's and Montana's caps leave it open."

M11 — The chapter's own Figure 2.1 says the primary-care shortage is a misperception, while the surrounding prose treats workforce shortage as established fact, without ever reconciling them. Figure 2.1 [485]: "If primary care providers were supported to see 3 patients per hour, there are enough for well into the future. **HRSA recognizes no Health Profession Shortage Areas in Vermont**. The problem is productivity and practice model, not pure headcount." Against that: [508] justifies regionalization by "population density and **physician shortage**"; [548] lists "**workforce shortage**" among the structural problems Vermont's model generalizes to; [544] describes an RHT goal as addressing "the conditions that drive **workforce shortage**"; [477] cites "licensure complexity impedes provider recruitment"; [553] concludes housing/transportation failures "will fail to achieve the **workforce stability**...". The distinctions that would reconcile these (primary care vs specialists; productivity vs headcount; retention vs supply) are available in the text but never drawn. *Fix*: add one sentence after [487]: "The primary-care correction is specific — it is a productivity and practice-model problem, not a headcount problem. The shortages this chapter treats as structural are in specialist coverage and in **retention**, which the systems map traces to housing and cost of living rather than to the size of the provider pool." Then make [508] read "specialist coverage" rather than "physician shortage".

M12 — "Act 68 is as effective as it is" asserts realized effectiveness for a statute whose operative provisions have not taken effect. [550]: "Vermont's Act 68 **is as effective as it is** because it was preceded by Act 167's ... diagnostic process." Per the chapter's own timelines, RBP begins FY2027, global budgets FY2028–FY2030, and [563] says "Vermont will have answers to these questions **by 2030**." *Fix*: "Vermont's Act 68 is as **durable** as it is — surviving enactment with its mandates intact — because it was preceded by ...".

M13 — Act 51 and the promised "enforcement architecture" are announced and never delivered. [442] tells the reader the chapter will present "the diagnostic mandate, the operational mandate, and **the enforcement architecture that connects them**"; the chapter title names "Acts 167, 51, and 68"; [447] calls Act 51 "**a planning mandate**". But there is **no section on Act 51** (its entire treatment is one clause at [447] plus one Figure T.1 row), and no section on an "enforcement architecture" — §2.5 is Act 167, §2.7 is Act 68, and enforcement appears only as a passing remark at [562] ("UVMC challenged GMCB enforcement in court and

lost"). Also note [442] names three elements while [447] names three **different** ones (diagnostic / planning / operational), so the two framings of the same sequence disagree. *Fix*: align [442] to the chapter as built — "the diagnostic mandate, the planning mandate that bridged them, and the operational mandate" — or add a short §2.5.3 on Act 51's transformation planning pilots and a §2.7.4 on enforcement. The framing sentence should not promise a section that does not exist.

M14 — The realistic-scenario callout mislabels the assumption set. [467] defines the realistic scenario as "10% annual expense growth on non-physician labor, 7-8% growth on other operating expenses, and constrained revenue". The callout [469] compresses this to "**Under realistic assumptions (7-8% expense growth)**", dropping the 10% labor figure that is the scenario's main driver and making the two scenarios look closer than they are (5% vs 7-8%, rather than 5% vs 10%-on-labor). *Fix*: "Under realistic assumptions (10% labor, 7-8% other expense growth)".

M15 — "38% since 2018" silently changes the comparison window. [462] pairs "hospital approved charge increases grew 38% over the same period, compared to income growth of 22%" with an explicit window of **2018–2022**. The callout [469] renders it "Hospital charges grew 38% **since 2018**; median household income grew only 22%", which in a 2024-dated callout reads as 2018–2024. *Fix*: "Hospital charges grew 38% from 2018 to 2022; median household income grew 22% over the same period."

M16 — "nearly as many residents over 65 as under 20" is understated to the point of being wrong on the chapter's own figures. [472] states 65+ was **21.7% in 2020** and then predicts "By the **mid-2030s**, Vermont will have **nearly as many** residents over 65 as under 20." At 21.7% of roughly 643,000 the 65+ cohort was already about 139,500 in 2020, which already exceeds Vermont's under-20 population — the crossover is behind us, not in the mid-2030s, and "nearly as many" understates it either way. *Fix*: verify against the Oliver Wyman HSA demography exhibit; the likely correct statement is "Vermont already has more residents over 65 than under 20, and by the mid-2030s will have roughly [X] for every under-20 resident."

M17 — Act 51's "up to four hospitals" sits unreconciled against "all 14 hospitals developing transformation plans." Figure T.1 [444] on Act 51: "Authorized AHS hospital transformation planning **pilots with up to four hospitals**". Callout [540]: "AHS hospital transformation grants of \$2M under Act 68; **all 14 hospitals** developing transformation plans with RHRC support." The two are reconcilable (different acts, pilot → statewide) but the chapter never says so, and a reader hitting the second after the first will read it as an error. *Fix*: in [540], "...under Act 68 — extending Act 51's four-hospital pilot to all 14 hospitals, now developing transformation plans with RHRC support."

M18 — Figure 2.5's lead-in promises three things; the table delivers two and omits the third. [556]: "The platform lets you stress-test the policy choices Vermont made — mandatory vs. voluntary participation, waiver design, and **the budget-neutrality math CMS requires**." Figure 2.5 [557] has two rows; neither mentions budget neutrality. (Per **CLAUDE.md**'s criterion 5, whether the Policy Simulator actually surfaces budget-neutrality math needs opening the tool, not just the route — not done here, this audit was scoped to internal consistency.) *Fix*: drop "and the budget-neutrality math CMS requires" from [556], or add a third row that names where that math is exposed.

M19 — Figure numbering: "Figure T.1" breaks the chapter's own scheme. Chapter 2 otherwise numbers 2.1–2.5. The opening timeline is "Figure T.1" [445] and sits between Figure-numbering that starts at 2.1 four sections later. *Fix*: renumber to Figure 2.1 and shift 2.1→2.2 ... 2.5→2.6, or state the T-series convention somewhere (front matter) if timelines are deliberately a separate series across chapters. This needs a cross-chapter decision, not a Chapter 2 edit.

M20 — A "statutory deadlines" callout with the deadlines stripped out. [540] promises "the following **statutory deadlines** define the transformation calendar" and then lists 10 items of which only two carry an explicit date (Act 55 April 2025; Strategic Plan December 2028). The remaining eight — the negative-1% benchmark, the \$2M grants, the Medicaid global budget, RBP implementation, the transparency dashboard, vendor selection, non-CAH global budgets, all- hospital global budgets — are undated. This has the signature of a two-column table (date | milestone) flattened into a single-column bullet list, losing the date column. It also closes "These dates are not aspirational. They are statutory obligations" — referring to dates that are mostly absent. *Fix*: restore the dates as a leading label per bullet (FY2026, FY2026, ongoing, Apr 2025, FY2027, FY2027, FY2027, FY2028, Dec 2028, FY2030 — confirm each against Act 68 and the GMCB Feb 2026 report before writing them). Also note Act 55 appears nowhere in Figure T.1 and is **not in the Sources block** [573].

MATCHES (checked, correct)

- [509] "COE assignments for **10 of Vermont's 14** current HSAs" ↔ Figure 2.2 has exactly 10 named hospitals plus a 4-hospital TBD row; 10 + 4 = 14. ✓
- [509] "UVM Medical Center received **16**" ↔ table 16. ✓ Its stated exclusions (acute general surgery, mental health ED, pediatric psychiatry, skilled nursing) are consistent with acute general surgery appearing under Brattleboro and Rutland instead. ✓
- [509] "Southwestern Vermont Medical Center received **9**" ↔ table 9. ✓
- [509] "Most community hospitals received between **1 and 6**" ↔ all eight non-UVMHC/non-SVMC named rows fall in 1–6 (5, 6, 4, 4, 5, 2, 2, 1). ✓
- [509] / glossary [568] both say "**24 specialty categories**" and "10 of 14 HSAs". ✓
- [463] "\$481 in 2020 to \$812 in 2024 — a **69% increase**": 812/481 = +68.8%. ✓
- [472] working-age "decline **13%** ... from 367,000 to 318,000": -13.4%. ✓
- [472] 65+ "**+57%** by 2040 ... from 21.7% in 2020 to exceed 30%" — internally coherent with a shrinking total population. ✓ Matches the stat card's "57%". ✓
- [467] "\$2.4 billion ... \$3.1 billion ... approximately **\$3,700 to \$4,800 per Vermont resident**": at ~647,000 residents, \$3,709 and \$4,791. ✓
- [467] UVMHCN "\$1.5 billion ... accounts for **more than half** the state's hospital capacity": 1.5/2.4 = 62.5%, consistent. ✓
- [462] "\$948 in 2024 — a **108%** increase in six years" (2018–2024) ↔ stat card "108% Premium Increase **2018–2024**". ✓
- [466] conservative scenario "**13 of 14 ... \$700 million ...** (\$1.4 billion against a 3% target)" ↔ callout [469]. ✓
- [543] "\$50 billion national fund ... **\$10 billion annually from 2026 through 2030**": 5 × 10 = 50. ✓
- [543] "\$195 million ... at a comparable annual level through FY2030 the five-year total would **approach \$1 billion**": 195 × 5 = \$975M. ✓ Correctly hedged with "only the first year is committed". ✓
- [543] "among the highest **per-capita** awards nationally" ↔ Figure T.1's Dec 29 2025 row. ✓
- [489] Act 167 "had **three** core directives" — three given. ✓
- [495] Act 167's "**five** statutory goals" — five given. ✓

- [525] Act 68 "advancing **five** goals" — five given. ✓
- [502] "**four** domains" of state-level recommendations — four developed at [503]–[506], and re-listed identically at [539] (governance clarity, AHS structural alignment with HSAs, SDOH investments, administrative simplification). ✓
- [500] "three-tier ... framework" ↔ §§2.6.1 state / 2.6.2 system / 2.6.3 hospital. ✓
- [480] "equity challenges across **three** categories" ↔ [480], [481], [482] give three. ✓
- [515] "The **three** primary repurposing options" — three given (REH, Community Ambulatory Care Center, Care at Home). ✓
- [460] / glossary [565] "Platform for Change ... **five** interconnected challenge domains" ↔ §§2.3.1–2.3.5 are exactly five, and the glossary names them in the same order. ✓
- [528] "GMCB's **February 2026** report" ↔ Sources [573] "Green Mountain Care Board, Act 68 Update on Hospital Reference-Based Pricing and Global Budgets, **February 17, 2026**". ✓
- [491] engagement "August 2023 through September 2024" ↔ Figure T.1's Aug 2024 report and Sept 2024 GMCB presentation rows. ✓
- [524]'s four Oliver-Wyman-recommendation → Act-68-provision pairings all trace to items actually present in [497] (RBP at ≤200% Medicare; global budgets when conditions met; GMCB Division of Planning and Effectiveness; review AHS agency structure). ✓
- [444] AHEAD rows are mutually consistent: Jan 2025 signing with an 8-year performance period, Jul 2026 withdrawal "before its Cohort 2 performance period (Jan 2028) ever began". ✓
- [535]'s "four times" count matches the four items it then lists. ✓ (dating flagged in M9)
- Recurring-heading check: §2.10 "Work This Chapter on the Platform", §2.11 "Implications for You", §2.12 "Key Concepts in This Chapter", and Sources as a plain paragraph [573], not a heading — all conform to the **CLAUDE.md** style-critical list. ✓

Superlative / ranking claims logged for cross-chapter comparison

Verbatim, with subject identified. Not judged against other chapters here.

- [441] (epigraph) — "Vermont's Acts 167, 51, and 68 constitute **the most comprehensive state-level healthcare reform mandate in the United States since Maryland's all-payer model in the 1970s**." *Subject: Acts 167/51/68 as a package.*
- [446] — "the Vermont General Assembly enacted a sequence of legislation that collectively mandated **the most comprehensive statewide healthcare restructuring attempted by any U.S. state since Maryland's all-payer model in the 1970s**." *Same subject as #1 — near-verbatim restatement, see R1.*
- [448] — "the Oliver Wyman Act 167 Community Engagement: Recommendations report — **the most comprehensive independent public diagnosis of a state hospital system in recent American health policy history**." *Subject: the Oliver Wyman report as a whole.*
- [465] — "The financial projections in the Oliver Wyman report are **the most important data in the entire Vermont reform story**." *Subject: the OW financial projections.*
- [476] — "**Perhaps the most analytically powerful contribution** of the Oliver Wyman report is its systems map." *Subject: the OW systems map. Hedged.*
- [484] — "**One of Oliver Wyman's most valuable contributions** is a systematic correction of widely held misconceptions." *Subject: the perception-vs-reality table. Hedged.*
- [497] — the priority action lists are "**one of the most operationally specific outputs of any state healthcare consulting engagement on record**." *Subject: the OW priority lists. Hedged.*
- [491] — "The scale of the engagement was **unusual for a state of Vermont's size**." *Subject: the OW engagement.*
- [505] — "Oliver Wyman was **unusually direct** about including these as healthcare system recommendations." *Subject: OW's SDOH recommendations.*
- [514] — "Oliver Wyman provided **the most politically sensitive recommendations in the report**." *Subject: hospital-level repurposing/closure recommendations.*
- [456] — "The implication is **unambiguous**: easy solutions ... will not resolve Vermont's healthcare crisis." *Subject: "more funding without structural change".*
- [457] — "'More funding without structural change' is **the default policy instinct in every state** because it is **the path of least political resistance**." *Subject: additive reform.*
- [462] — "average premiums for silver exchange plans reached \$948 in 2024 — a **108% price increase** in six years." *Subject: Vermont silver exchange premiums.*
- [466] — "nine of Vermont's fourteen hospitals reported operating losses, with **the worst reaching –8.9%**." *Subject: FY2023 Vermont hospital margins.*
- [474] — "every service area **except Burlington** will experience population decline and aging — with the Northeast Kingdom (Caledonia, Essex, Orleans counties) facing **the most severe trajectory**." *Subject: Vermont HSA demography. Note the "only X" form: Burlington is the sole exception.*
- [474] — "Newport's 65+ population ... will represent **a majority of the HSA's healthcare demand by 2040**." *Subject: Newport / North Country HSA.*
- [480] — "Gaps in culturally competent care were **the most frequently cited equity challenge** in community meetings." *Subject: equity challenges in OW community meetings.*
- [480] — "Vermont's foreign-born population, while **lower than the national average at 4.5%**." *Subject: Vermont foreign-born share.*
- [528] — "UVMHC's commercial prices averaged 358% of Medicare across inpatient and outpatient services in RAND's published 2018–2020 data — **the highest of any hospital in Vermont**." *Subject: UVMHC commercial pricing. Note also the unexplained "rational break-even point of 136% of Medicare" in the same paragraph — no source or derivation is given for 136% anywhere in the chapter.*
- [528] — "Outpatient imaging at some Vermont hospitals reaches **715-944% of Medicare**." *Subject: Vermont outpatient imaging prices. Highest specific multiple in the chapter.*
- [533] — "This statutory obligation makes Vermont **the first U.S. state outside Maryland** on a legislatively mandated path to a comprehensive all-payer global budget system." *Subject: Act 68 global budget mandate. "First/only" form.*

- 22. [535] — "Act 68 eliminates the voluntary option. This is **the critical design difference** between Vermont's current trajectory and **everything that preceded it.**" *Subject: Act 68's mandatory architecture.*
- 23. [537] — "**The most consequential long-term provision** of Act 68 is the requirement that AHS develop a Statewide Health Care Delivery Strategic Plan." *Subject: Act 68 provisions, ranked internally.*
- 24. [539] — "Oliver Wyman's state-level recommendations are **the most detailed public blueprint available** for what this strategic plan should address." *Subject: OW state-level recommendations.*
- 25. [499]/[500] — "The Oliver Wyman report's **most detailed section** — running from pages 32 through 114." *Subject: the OW system redesign section.*
- 26. [527] — Act 68's RBP provisions "represent a significant departure from the voluntary, incentive-based payment reform approach that **dominated American health policy for the previous fifteen years.**" *Subject: US payment reform 2010–2025.*
- 27. [530] (Figure 2.4, Maryland row) — "**The closest model to Vermont's Act 68 trajectory.**" *Subject: Maryland all-payer system.*
- 28. [540] — "GMCB hospital budget guidance includes negative 1% commercial rate growth benchmark — **the first negative benchmark in Vermont history.**" *Subject: GMCB commercial rate-growth benchmark.*
- 29. [444] (Figure T.1, FY2027 row) — "**First mandatory all-payer price cap in modern Vermont history.**" *Subject: Act 68 RBP.*
- 30. [444] (Figure T.1, Dec 2025 row) — "\$195M for federal fiscal 2026 — **among the highest per-capita rural transformation awards.**" *Subject: Vermont's RHT award.*
- 31. [543] — "\$195 million ... **nearly double what the state had expected to receive, and among the highest per-capita awards nationally.**" *Subject: same as #30.*
- 32. [545] — "The distinction between investment capital and reform strategy is **one of the most persistently confused in health policy.**" *Subject: federal grant framing.*
- 33. [545] — "The Vermont approach ... is **a model of how capital investments should be positioned** within a structural reform agenda." *Subject: Vermont's RHT-to-statute alignment.*
- 34. [550] — "Vermont's sequence — diagnose publicly, build shared understanding, then mandate — is a process design that **other states should study.**" *Subject: the Act 167 → Act 68 sequence.*
- 35. [560] — "**The single most important thing** Act 68 requires of you is not compliance — it is preparation." *Subject: hospital executive obligations under Act 68.*
- 36. [563] — "Vermont's Act 68 is **the most detailed mandatory all-payer pricing statute enacted by any state in the modern regulatory environment.**" *Subject: Act 68 as a statute. Note the interaction with #1/#2 (which concede Maryland the 1970s) and #21 (first outside Maryland) and #27 (Figure 2.4 calls Maryland the closest model) — "modern regulatory environment" is the only thing carrying the distinction, and it is undefined.*
- 37. [512] — "a surgeon performing **200 procedures a year** maintains skills that one performing **20** cannot." *Subject: surgical volume-outcome relationship. A 10× ratio asserted without citation.*

Note for the cross-chapter pass: items 3, 4, 5, 6, 7, 10, 24 and 25 are all "most X" claims about **the same document** (the Oliver Wyman report) inside one chapter. Four of them are hedged ("perhaps", "one of the"); #4 ("the most important data in the entire Vermont reform story") and #3 ("the most comprehensive independent public diagnosis ... in recent American health policy history") are not. This is the same class as Chapter 1's "two dependencies both called the most underestimated" — see C1 below. Items 1/2/21/36 should also be checked against whatever Chapter 3 and the Economics chapter claim about Maryland and about Act 68's distinctiveness.

Other issues

Internal contradictions (beyond the MISMATCH list)

C1 — Competing "most important contribution of the Oliver Wyman report" claims. [465] "The financial projections ... are **the most important data in the entire Vermont reform story**" vs [476] "**Perhaps the most analytically powerful contribution** of the Oliver Wyman report is its systems map" vs [484] "**One of Oliver Wyman's most valuable contributions** is a systematic correction of widely held misconceptions" vs [539] "Oliver Wyman's state-level recommendations are **the most detailed public blueprint available**". The hedges on three of the four make this survivable, but [465] and [476] are three paragraphs apart in the same section-cluster and read as a direct conflict. *Fix:* keep one unhedged superlative (the financial projections are the strongest candidate, since the chapter's argument rests on them) and reduce [476] to "The report's systems map is its most operationally useful exhibit" or similar non-competing framing.

C2 — "Sequencing-neutral" vs the chapter's sequencing thesis. [442] argues "the Policy pillar has to come first in the five-pillar sequence"; [457] calls the three imperatives "a **sequencing-neutral** checklist"; [478] says the imperatives "are formulated precisely to address the highest-leverage nodes **simultaneously, not sequentially**". [457] and [478] are consistent with each other but sit oddly against the book's sequencing doctrine. Probably defensible (the imperatives are a diagnostic checklist, not an implementation order) but the word "sequencing-neutral" in a book whose central claim is about sequencing invites the reading that sequence does not matter. *Fix:* [457] → "an **order-independent** checklist for diagnosing whether a state's reform agenda is structural or merely additive".

C3 — Maryland dated twice, differently. [441] and [446] both anchor to "Maryland's all-payer model **in the 1970s**"; Figure 2.4 [530] lists "Maryland (**2014–present**)". Both are true of different things (1970s rate-setting; 2014 global-budget all-payer model) but the chapter never says so, and a reader moving from §2.1 to §2.7.1 hits an apparent date conflict. *Fix:* Figure 2.4's Maryland row → "Maryland (rate-setting since 1977; global budgets 2014–present)".

Genuine repetition (systematic sentence-similarity scan run over all 135 elements;
threshold 0.56, plus manual review of recurring facts)

R1 — The chapter's thesis sentence appears twice, near-verbatim. [441] (epigraph) "the most comprehensive state-level healthcare reform mandate in the United States since Maryland's all-payer model in the 1970s" and [446] "the most comprehensive statewide healthcare restructuring attempted by any U.S. state since Maryland's all-payer model in the 1970s". Epigraph-then-prose repetition is a recognised device, but here it is the same clause with three words changed, five paragraphs apart. *Fix:* rework [446] to

advance rather than restate — e.g. "...a sequence of legislation whose scope has no state-level parallel since Maryland, and whose mechanism — statutory mandate rather than voluntary demonstration — has none at all."

R2 — "RBP controls price but not volume" stated four times. Machine scan flagged all pairs: [478] "Reference-based pricing without global budgets controls unit prices but allows volume to expand" · [534] "Reference-based pricing controls unit prices but does not control volume" (0.68 similarity to [478]) · [552] "Reference-based pricing is necessary but not sufficient for controlling total hospital spending" · [569] glossary "Reference-based pricing controls price but not volume". *Legitimate*: [569] (glossary) and [552] (the Five Lessons recap — a deliberate summary section). *Defect*: [478] and [534] state the identical proposition in prose two sections apart, and [534] is where the concept is formally introduced and named ("balloon squeeze"). [478]'s version arrives first and pre-empts it. *Fix*: change [478]'s third example to something that does not pre-empt §2.7.2 — e.g. "Reference-based pricing without transport and telehealth investment lowers the price of care that rural patients still cannot reach." Leave [534] as the single full statement.

R3 — "230 meetings / 3,100 participants / 100 organizations" given twice in full. [452] "After 230 meetings across Vermont's 14 Hospital Service Areas, engagement with more than 3,100 participants representing over 100 organizations" and [491] "over 230 meetings across all 14 Hospital Service Areas, involving more than 3,100 participants from over 100 organizations". Three statistics, same three numbers, same order, two sections apart. *Fix*: [452] should cross-reference forward instead of re-citing — "After the year-long engagement process described in §2.5.1, Oliver Wyman's executive summary distilled..."

R4 — "31 EMS agencies" cited twice as a fresh finding. [482] "patients must navigate 31 EMS agencies for transfers" (§2.3.5, equity gaps) and [506] "a provider may need to contact 31 separate EMS agencies to arrange a patient transfer" (§2.6.1, administrative simplification). *Fix*: keep [506] (where it supports the administrative-complexity argument) and in [482] reduce to "...that the EMS transfer landscape is fragmented to the point of unusability (see §2.6.1)".

R5 — "more alarming/severe than most public discussion acknowledges" used twice as a rhetorical frame. [460] "The full picture is considerably more alarming than most public discussion of Vermont healthcare acknowledges" and [528] "The underlying price problem that RBP addresses is more severe than most national discussions of hospital pricing acknowledge" (0.57 similarity). Same construction, same move. *Fix*: rewrite one.

R6 — Out-of-pocket maximums stated three times, with two different magnitudes. [462] "Out- of-pocket maximums **more than doubled** in five years" · [463] "Out-of-pocket maximums **doubled** in five years" · callout [469] "Out-of-pocket maximums **more than doubled** in 5 years". [462] and [463] are **adjacent paragraphs** and disagree on whether it is "doubled" or "more than doubled". *Fix*: delete the sentence from [463] entirely (it adds nothing to the BCBS-VT paragraph it sits in) and keep "more than doubled" in [462] and the callout.

R7 — Oliver Wyman's four state-level domains enumerated twice. [502]–[506] develop them; [539] re-lists all four by name. [539] frames itself as a cross-reference ("The four domains Oliver Wyman identified — ... — constitute the operational dimensions of the strategic plan"), which is legitimate under the summary-cross-reference exception, but it is a full re-enumeration rather than a pointer. *Left deliberately* — it earns its place by making the strategic-plan connection, which is [539]'s actual argument. Flagging only so the cross-chapter pass knows it is intentional.

Legitimate recurrence, checked and NOT defects: the three imperatives appear at [453] (source table), [455] (five-pillar mapping), [457] (generalization callout), [544] (RHT goal mapping) and [567] (glossary) — each does different work, and the glossary/callout recurrences are exempt by the standing rule. The §2.9 Five Lessons recapitulate §§2.3–2.8 by design. Glossary entries [565]–[572] restating in-chapter definitions are exempt. The callout [469] restating §2.3.2's figures is a table summarising its own prose — exempt.

Thin sections / structural

T1 — Act 51 has no section. Covered under M13. It is named in the chapter's opening sentence and in the section-anatomy paragraph, then receives one clause at [447] and one Figure T.1 row. Of the three acts the chapter announces, one third is undeveloped.

T2 — §2.7 opens with an orphan title line and no prose. [521] is a bare paragraph reading "Act 68 Key Statutory Deadlines", immediately followed by table [522] and caption [523]; the section's first actual sentence is [524]. Every other section in the chapter opens with prose. (This is also a redundant label — caption [523] already names the figure.) *Fix*: delete [521] and move the figure below [524]–[525], or fold [521] into the caption.

T3 — The glossary entry for "The three imperatives" is split across two paragraphs. [566] holds only the term, [567] the definition. Every other entry ([565], [568]–[572]) is a single paragraph with the bolded term run into its definition. This is a formatting inconsistency visible to a reader as a broken list item. *Fix*: merge [566] and [567] into one paragraph matching the others.
Note: this one requires `python3 book-build/check_format.py` after the edit per standing directive 23.

T4 — §2.10's Figure 2.5 has only two rows. Two hands-on items for a chapter of this length is thin relative to what the section promises at [556] (three capabilities named — see M18).

T5 — Act 55 is cited in a callout but absent from Sources. [540] cites "Act 55 outpatient drug cap at 120% of ASP effective April 2025, generating estimated \$135M in annual savings". The Sources block [573] lists Acts 167, 51 and 68 but not Act 55, and Act 55 appears nowhere in Figure T.1. *Fix*: add "Act 55 of 2025 (Vermont)" to Sources, and consider a Figure T.1 row for April 2025.

Out of scope for this pass

Per the task brief this was an internal-consistency audit. **Not verified here:** whether `/research-lab/policy-quality?tab=policy`, `/vermont-act-167` and `/vermont-act-68` exist and actually deliver what [557] promises (standing directive: a 200 is not a delivered promise); whether "AHS Secretary Samuelson" is the correct name and title; the "UVMHC challenged GMCB enforcement in court and lost" claim at [562]; the external accuracy of the H.R. 1 figures (\$911B / 10M uninsured by 2034), the RAND 358%, the 715–944% imaging multiples, the 136% break-even figure, and the CMS BALANCE Model row; and alignment against the Preface and Introduction, which `CLAUDE.md` makes paramount and which requires its own pass.

Chapter 3 Audit

Source of truth: `HTR_Book_v42.docx` → `word/document.xml`, read directly via zipfile. Chapter body = block-level items 575 (Heading1 "Chapter 3: The Policy Pillar in Practice...") through 673, ending before 674 (Heading1 "Chapter 4:"). Read-only audit; no edits made.

Figures found (ground truth)

All five figure captions are present, sequential, and each is immediately preceded by its table. All five also appear, byte-identical, in the Figure Index (doc items 5385–5389).

Figure 3.1 — CMMI model success factors (table 5×4; header row + 4 data rows) Columns: **Characteristic | What it means | Models with this characteristic | Models without it** Rows (characteristic → "with" cell contents):

1. Mandatory or near-mandatory participation → AHEAD (participating states, all hospitals); Maryland All-Payer Model; **Vermont Act 68**. Without: most CMMI ACO models; MSSP Track 1; BPCI (voluntary); most demonstrations.
2. Two-sided risk that is real → ACO REACH (mandatory downside); global budgets; AHEAD. Without: MSSP Track 1; early bundled models; Primary Care First.
3. Adequate implementation support → AHEAD (multi-year pre-performance period); **Vermont's RHRC engagement**; Making Care Primary.
4. Alignment with state regulatory environment → AHEAD; **Vermont-specific APM**; Maryland TCOC (HSCRC authority).

Figure 3.2 — CMMI models most relevant to Vermont (table 6×2; header + 5 models) AHEAD; ACO REACH; Making Care Primary (MCP); BALANCE Model; BPCI-Advanced. Key cell facts: AHEAD = "Five-state voluntary total cost of care model (Maryland, Connecticut, Hawaii, Rhode Island, New York)"; Vermont signed Cohort 2 Jan 2025, **withdrew July 2026** after EAST Fund cut ~\$138M/yr → ~\$10M cap. MCP = 8-state initial cohort, 10-year model, three stages. BALANCE = Medicaid launch May 2026, Medicare Part D Jan 2028.

Figure 3.3 — Policy strategy functions (table 4×4; header + 3 functions) Regulatory compliance / Opportunity identification / Policy advocacy × **Key activities | Vermont-specific examples | HTR Advisory service**.

Figure 3.4 — H.R. 1 Medicaid provisions (table 6×4; header + 5 provisions) Provider tax restrictions (new taxes now; phase-down 2027); state-directed payment caps at 100% of Medicare (2027); work requirements 80 hrs/month (late 2026, implement by Dec 31 2026; 10M uninsured by 2034); 6-month redeterminations (2026); Medicaid reimbursement reductions (2026–2031 phased; **\$911B/10 yrs nationally, \$137B rural**; all 14 Vermont hospitals).

Figure 3.5 — Platform tools (table 4×3; header + 3 rows) Work Requirements Calculator `/research-lab/policy-quality?tab=medicaid-wr`; H.R. 1 Cliff Scenario `/research-lab/policy-quality?tab=hr1-cliff`; Medicaid Eligibility Simulator `/medicaid-eligibility-simulator`. (Route existence not re-verified here — this pass is factual self-consistency only.)

Claims checked against their own source (MATCH/MISMATCH + proposed fix)

MISMATCH 1 — CMMI model count contradicts itself inside the chapter (hard contradiction)

§3.2 opening (item 583): "Over fifteen years, CMMI has launched **more than seventy models**". §3.10 Key Concepts (item 667): "Has launched **50+ models** since 2010". The Introduction (item 163) independently says "**more than seventy models**". Two of the book's three statements say 70+; the glossary entry says 50+. **Fix:** change the Key Concepts entry to "Has launched 70+ models since 2010".

MISMATCH 2 — AHEAD written as live and ongoing in §3.3 and §3.9, contradicting Fig. 3.2

Figure 3.2's own cell states Vermont **withdrew from AHEAD in July 2026**, and the Introduction (73), Ch2 (777) and Ch7 (5330) all treat the withdrawal as a settled fact ("Vermont exits before its Cohort 2 performance period (Jan 2028) ever began"). Against that ground truth, four passages in Ch3 are stale:

- Item 605: "Vermont's **AHEAD Medicaid global budgets** for hospitals, beginning January 2026, operate under this demonstration authority." Vermont's Medicaid global budgets run on state authority under the Global Commitment to Health demonstration; branding them AHEAD budgets is wrong post-withdrawal (Ch7 item 5330 calls them simply "Medicaid global budgets ... already in operation"). **Fix:** "Vermont's Medicaid hospital global budgets, in operation since January 2026, run under this demonstration authority — on state, not AHEAD, authority."
- Item 606: "Vermont's ability to implement its **current AHEAD-aligned Medicaid policy**" — present tense for a model Vermont has left. **Fix:** past tense / drop "AHEAD-aligned".
- Item 609: "a specific condition of the state's AHEAD participation" — acceptable as history, but should read "of the state's then-pending AHEAD participation".
- **Item 661 is the worst of the four:** "If you are an AHS or GMCB official: Vermont's **AHEAD implementation plan** must address the gap between CMS's performance start date (January 2028) and the AHS-GMCB analytics vendor deployment timeline." This instructs an official to plan implementation of a model Vermont withdrew from two months before the book's present (July 2026). Item 663, in the same section, correctly says Vermont "signed, and then withdrew

from, the AHEAD CMMI model" — so §3.9 contradicts itself. **Fix:** re-point the advice at Act 68 global budgets (FY2028), keeping the analytics / OneCare lesson, which is what actually carries: "...the gap between Act 68's FY2028 performance start and the AHS-GMCB analytics vendor deployment timeline."

MISMATCH 3 — "nine-year commitment" to AHEAD vs 8-year performance period elsewhere

Item 624: "a **nine-year** commitment to a federal model involves significant policy risk". Ch2's Act/agreement table (item 753) states the AHEAD State Agreement gave Vermont an "**8-year** performance period". **Fix:** "an eight-year commitment", or "a nine-year agreement with an eight-year performance period" if the pre-performance year is deliberately being counted — but then say so, because the bare "nine-year" reads as a conflict.

MISMATCH 4 — Global Commitment approval date vs the chapter's own Sources line

Item 605: "AHEAD alignment provisions approved in **2024**". Chapter Sources line (item 672): "Vermont Global Commitment to Health demonstration (CMS approval **January 2025**)". **Fix:** reconcile to January 2025, or distinguish the two events explicitly (2024 alignment amendment vs January 2025 renewal approval).

MISMATCH 5 — Prior authorization is assigned to two different pillars, 6 paragraphs apart

Item 614: "Prior authorization reform **is a Policy pillar issue** because it requires either federal rulemaking or state legislation." Item 620: "Vermont's transformation agenda treats administrative simplification — including prior authorization reform — as an **Operations pillar requirement**." Both sit inside §3.4, whose own heading calls PA "The Administrative Pillar of Policy Strategy". This is the same class of defect as Ch1's duplicated "most underestimated dependency". **Fix:** state the split once — PA *reform* (changing the rule) is Policy; PA *burden reduction* (working the existing rule) is Operations — and drop the bare assertion in 620.

MISMATCH 6 — "average physician practice spends 13 hours per week"

Item 613. The chapter's cited source (item 672) is the AMA prior-authorization burden survey, which reports roughly 13 hours **per physician** per week (physician + staff time), not per practice. Per-practice would be far higher in any multi-physician group. **Fix:** "The average physician spends 13 hours per week — with their staff — on prior authorization tasks."

MISMATCH 7 — §3.3.1 promises "what waivers cannot do" and never delivers it

Heading 596 is "**3.3.1 What 1115 Waivers Can and Cannot Do**"; item 595 promises "the following framework organizes the most common 1115 waiver applications". What follows (597–603) is only the *can* side — four authority categories, each with a Vermont example. Nothing states what 1115 authority cannot reach (no eligibility-cap waivers, no waiver of the budget-neutrality requirement itself, no waiver of Medicaid's mandatory-benefit floor for mandatory populations). Budget neutrality is the nearest thing, and it is a separate section (§3.3.4). Also: 3.3.1's body is purely a lead-in to 3.3.2 ("1115 Waiver Authorities: What States Can Accomplish"), so §3.3.1 has no content of its own and the two headings restate each other. **Fix:** either add a short "what 1115 cannot do" list under 3.3.1, or retitle it "What 1115 Waivers Can Do" and merge 3.3.1 into 3.3.2.

MISMATCH 8 — "AHEAD's predecessor — OneCare Vermont"

Item 661. OneCare was Vermont's ACO under the Vermont All-Payer ACO Model; the model that preceded AHEAD is the All-Payer ACO Model, not OneCare, which was a participant in it. (Per [project_ch1_onecare_correction](#), the OneCare facts are settled; this is a category error, not a factual one.) **Fix:** "AHEAD's predecessor — the Vermont All-Payer ACO Model, and its ACO, OneCare Vermont — failed partly because..."

MATCH — verified consistent

- **\$911B / 10M uninsured by 2034**. Bullets 632–633 vs Fig 3.4 rows 3 and 5, and vs Ch2 (item 761), Ch13 (3592, 3630, 3637) and Ch11 (3801, 4578). All say \$911B and 10M/2034.
- **\$700M–\$2.4B five-year cumulative hospital deficit** (item 640) vs Preface (10), Ch1 (37), Ch2 (382, 519), Ch6 (822–823), Ch11 (1669). Consistent everywhere, including the conservative/realistic scenario split.
- **All 14 Vermont hospitals** (Fig 3.4 row 5) vs the book's 14-hospital count and Appendix C.
- **AHEAD = five states, named**. Fig 3.2 lists exactly five (MD, CT, HI, RI, NY), matching the Key Concepts glossary (1930) and Ch13 (3670). Vermont correctly excluded.
- **EAST Fund ~\$138M → ~\$10M cap** (Fig 3.2) vs Ch1 (368), Ch2 (582, 777), Ch12 (1827), glossary (1928), Introduction (73). Consistent.
- **MCP 8-state cohort** (Fig 3.2) vs Key Concepts item 670. Consistent.
- **ACO REACH mandatory two-sided risk** (Fig 3.2) vs Fig 3.1 row 2 cell. Consistent.
- **CMS 2024 PA rule: 72 hours expedited / 7 days standard** (item 616) — correct per CMS-0057-F; internally unconflicted.
- **Improving Seniors' Timely Access to Care Act passed the House in 2022, died in the Senate** (item 616). Correct, and not contradicted elsewhere.
- **Work requirements: 80 hrs/month, late 2026, Vermont has not signaled intent** — item 603 and Fig 3.4 row 3 agree.

Weak attribution (not a mismatch, but the chapter under-cites its own Introduction)

Item 585: "A 2023 **independent evaluation** of CMMI models found that the vast majority had not achieved statistically significant reductions...". The Introduction (163) names the source and the numbers: a 2023 **Congressional Budget Office** analysis — \$7.9B to operate, \$2.6B in benefit savings, **+\$5.4B net federal spending, 2011–2020**. Per the "done" criterion 4, the chapter should cross-reference the Introduction rather than re-derive it more vaguely. **Fix:** "The 2023 Congressional Budget Office analysis described in the Introduction found..."

Superlative/ranking claims logged for cross-chapter comparison

Verbatim, with item numbers:

1. (576) "where the gaps between statute and execution create **the most consequential risks**"
2. (577) "the gap between the two is where **most** reform agendas lose momentum"

- 3. (585) "The models that showed **the most promise** — ACO REACH ..., AHEAD, the Making Care Primary model" — note Fig 3.1 also credits Maryland All-Payer/TCOC and Vermont Act 68, which the prose list omits; and Act 68 is a state statute, not a CMMI model, so its presence in a column headed "Models with this characteristic" is a category stretch.
- 4. (589) "The models with **the highest relevance** to Vermont-style transformation"
- 5. (Fig 3.2, AHEAD) "**Still the most relevant model** for state-level transformation — and Vermont's experience with it is **the clearest evidence yet** of the risk it carries." → tension worth cross-checking: the most relevant model is one Vermont has left.
- 6. (Fig 3.2, ACO REACH) "ACO REACH is **the highest-accountability** current federal ACO option"
- 7. (593) "the 1115 waiver is **the most powerful federal policy tool available**"
- 8. (609) "Budget neutrality is simultaneously **the most important** financial discipline in 1115 waiver design and **the most technically complex**" — a double superlative in one sentence; the Ch1 "most underestimated dependency" defect was this shape.
- 9. (613) "**one of the most significant** drivers of administrative burden"
- 10. (617) "Full elimination of PA for evidence-based, guideline-concordant care — the policy that would **most reduce** administrative burden — remains aspirational"
- 11. (632) "\$911 billion in Medicaid cuts over 10 years — **the largest reduction in federal health spending in American history**" → Ch13 (3630) phrases it "**the largest single reduction** in federal health-program spending in American history". Compatible, but the wording should be unified on one canonical phrasing.
- 12. (660) "**The single most consequential preparation step** is HCC documentation accuracy" → highest cross-chapter collision risk. Any other chapter naming a different "single most consequential" step for a Vermont hospital executive is a contradiction.
- 13. (663) "most CMMI models have underperformed projections" (consistent with 73 and 163)
- 14. (625) "is **the right model** for any state undertaking long-term federal-state policy commitments"
- 15. (610, BEYOND VERMONT box) "**always** contestable" (the counterfactual baseline)

Other issues

- **Numbering gap in the XML:** item 611 is an empty paragraph between the BEYOND VERMONT box (610) and heading 3.4 (612). Cosmetic, likely the post-table spacer; noted, not flagged.
- **Thin sections.** §3.7.2 (649–654) is four one-line rhetorical-question paragraphs (Exposure mapping / Scenario analysis / Response strategy / Stakeholder communication) with no Vermont example, no figure, and no worked instance — the thinnest passage in the chapter and the only framework in it presented without either a table or a Vermont case. §3.7.1 (644–648) is better but is also pure list.
- **Repetition, genuine.** (a) The "1115 waivers are negotiated instruments, not applications" argument is made at 606 and then re-made from the CMS side at 609 and a third time in the BEYOND VERMONT box at 610. Three statements of one point inside §3.3. (b) "AHEAD is the most relevant model for state-level transformation" appears at Fig 3.2 and again verbatim in sense at Key Concepts 667.
- **Repetition, legitimate (deliberately left).** Key Concepts entries 665–671 restate definitions from the body — that is what a glossary is. Figure source lines recur by design. Fig 3.4 summarising the H.R. 1 bullets at 632–636 is a table summarising its own prose, which the "done" criteria explicitly exempt. Item 622's recap of federal funding/requirements/uncertainty is a section lead-in, not a restatement.
- **Fig 3.3 minor tension:** its Vermont-specific examples cell lists "H.R. 1 Medicaid work requirement implementation timeline" as a Vermont compliance activity, while 603 and Fig 3.4 both say Vermont has not implemented and has not signalled intent. Defensible as monitoring rather than implementing, but the cell reads as if Vermont is standing one up.
- **Fig 3.1 row 3** cites "Vermont's RHRC engagement" with no expansion of RHRC anywhere in the chapter and no glossary entry for it in §3.10.
- **Not checked in this pass** (scope was factual self-consistency): whether the three Figure 3.5 routes resolve and whether each promised output is actually delivered on the page; formatting/`check_format.py`; and the black-on-navy table-style check. No render was run because nothing was edited.

===== FILE: audit_ch04.md =====

Chapter 4 Audit

Scope: Chapter 4 — "The Technology Pillar — Data Infrastructure for a Transformed Health System." Read-only audit. Source of truth: `word/document.xml` inside `HTR_Book_v42.docx`, read via `python-docx` walking body children in order (paragraphs and tables), so table cell contents are ground truth rather than the lossy `.md` mirror.

Boundaries: body-element index 674 (**Heading 1** — "Chapter 4: ...") through 851, ending at index 852 (**Heading 1** — "Chapter 5: ..."). 178 body elements: 1 H1, 11 H2 (§4.1–§4.11), 16 H3, 9 tables, remainder body paragraphs and 2 pull-quotes.

Structure present and correctly sequenced: §4.1–§4.11; §4.1.1; §4.2.1–4.2.2; §4.3.1–§4.3.11 (all eleven, no gaps); §4.4.1–4.4.2; §4.5.1–4.5.2; §4.6.1–4.6.5; §4.7.1. The three style-critical recurring headings are present and byte-correct (**4.9 Work This Chapter on the Platform**, **4.10 Implications for You**, **4.11 Key Concepts in This Chapter**) and **Sources:** ... is a plain paragraph, not a heading.

Figures found (ground truth)

Nine tables. Eight carry **Figure 4.N** captions; one is an uncaptioned stat strip and one an uncaptioned **BEYOND VERMONT** callout (both consistent with book convention).

Fig	Idx (tbl/caption)	Dims	Actual content
4.1	682 / 683	4×2 (hdr + 3)	Layer 1 VHCURES (APCD; gap: race/ethnicity incomplete in commercial claims) · Layer 2 VITL/VHIE (clinical substrate; voluntary participation) · Layer 3 AHS-GMCB Analytics Vendor , procurement underway (global-budget modeling, TCOC, risk strat, equity; Act 68 global budgets FY2028)
—	691	1×4 stat strip	60% commercial covered · 100% Medicaid & Medicare · Claims + Rx data types · 12–18+ Mo typical data lag
4.2	696 / 697	5×3 (hdr + 4)	Limitations: (1) incomplete commercial coverage 60% / 40% ERISA self-insured; (2) data lag 12-18+ months; (3) no BH or SDOH integration; (4) no clinical results, only billing records
—	706	1×1 callout	BEYOND VERMONT — exist-on-paper vs. functionally-usable gap as national failure mode
4.3	729 / 730	4×4 (hdr + 3)	Step 1 VHCURES–VHIE claims-clinical integration, target 2026–2027 · Step 2 MH/SUD integration, 2027–2028 · Step 3 SDOH integration, 2028 and beyond
4.4	737 / 738	4×4 (hdr + 3)	UHDS, listed top-down: L3 End-User Services (VITLAccess, ADT notifications, Advanced Analytics) · L2 Exchange Services (FHIR APIs, Clinical Data Repository, MDWAS-prioritised) · L1 Foundational Services (MPI = Verato, Term Atlas, Rhapsody; SNOMED CT, LOINC)
4.5	765 / 766	7×2 (hdr + 6)	Six for/against pairs on a statewide EHR (cost \$100–300M , disruption, vendor lock-in, FHIR alternative, ERISA still excluded, provider-landscape diversity)
4.6	791 / 792	5×2 (hdr + 4)	Full FHIR R4: UVMMC (Epic), Oracle Health post-2022, BCBS VT + MVP · Partial: "hospitals on Meditech Expanse (FHIR-capable but may require configuration)" · Limited/none: TruBridge legacy, most DAs, many SNFs, independent PCPs on older EHRs · Planned (RHT): rural/independent practices, smaller CAHs

Fig	Idx (tbl/caption)	Dims	Actual content
4.7	827 / 828	6×2 (hdr + 5)	Five-layer target architecture, L5 Advanced Analytics & AI → L1 Core Data Assets (VHCURES, VUHDDS, EHRs at 14 hospitals, Blueprint registry, GMCB regulatory data). No MDWAS and no UHDS anywhere in it.
4.8	833 / 834	5×3 (hdr + 4)	Four platform tools, all four on /research-lab/interoperability (?tab=fhir,?tab=risk, ?tab=emr, ?tab=statewide-ehr)

Figure Index (end matter) lists Figures 4.1–4.8 with captions matching the in-chapter captions verbatim. No missing or orphaned index entries for this chapter.

Claims checked against their own source (MATCH / MISMATCH + proposed fix)

MISMATCH 1 — VHCURES data lag stated two incompatible ways inside one chapter

- Figure 4.1 stat strip (691): "**12–18+ Mo**"; Figure 4.2 row 2 (696): "**12-18+ months**"; §4.3.11 Figure 4.5 (765): "**VHCURES's 12-18 month lag**"; §4.10 (836): a "**12-to-18-month advantage**."
- §4.3.8 (748): "VHCURES operates on a **nine-to-twelve-month** reporting lag" and "for intervening in a patient's care, a **nine-month** lag is not a data source."
- **nine-to-twelve** occurs exactly once in the whole manuscript (index 748); **12-18/12–18** months occurs at 1602, 1603, 1718 (Ch4) and 1959, 2969, 2975, 3277, 3820, 5313, 5327 elsewhere. The 12–18 figure is the book-wide norm.
- **Fix:** in 748, replace "a nine-to-twelve-month reporting lag" with "a 12-to-18-month reporting lag" and "a nine-month lag is not a data source" with "a 12-month lag is not a data source." Single paragraph, two strings.

MISMATCH 2 — §4.5.2 prose contradicts its own Figure 4.6 on Meditech

- Prose (789): "The critical gap is at smaller community hospitals (on TruBridge **and Meditech** platforms) ... that **lack FHIR-capable systems entirely**."
- Figure 4.6, the figure it introduces, places Meditech in "**Partial FHIR implementation — FHIR-capable but may require configuration**", and puts only TruBridge legacy in "Limited / no FHIR."
- This is the same error class found in Chapter 1: prose overstates what its own cited figure's cells say.
- **Fix:** in 789, "smaller community hospitals (on TruBridge legacy platforms, with Meditech sites FHIR-capable but unconfigured)."

MISMATCH 3 — Figure 4.6's Partial row text begins mid-sentence

- Cell reads literally **hospitals on Meditech Expanse (FHIR-capable but may require configuration)** — lowercase, no subject. Every other cell in the table is a complete noun phrase. A leading word was dropped.
- **Fix:** **Community hospitals on Meditech Expanse (FHIR-capable but may require configuration)**.

MISMATCH 4 — §4.1 prose names a third data asset the chapter's own figure does not

- Chapter opener (675), §4.1 framing (676) and **Figure 4.1** all name the triad as VHCURES / VTTL / **AHS-GMCB analytics platform**.
- §4.1.1 body (685) instead says the infrastructure is "anchored by VHCURES ..., VTTL ..., and the **Blueprint clinical data registry**."
- In Figure 4.7 the Blueprint registry is a **Layer 1 core data asset** and the analytics platform is **Layer 5** — they are not interchangeable.
- **Fix:** in 685, "...and the AHS-GMCB analytics platform now in procurement (Figure 4.1)."

MISMATCH 5 — §4.6.1 AI-scribe arithmetic does not survive its own paragraph

- 796: the RHT application "cites that AI scribes automate an average of **over two hours** of daily administrative work."
- 797, two sentences later: "A provider spending two hours daily on documentation has approximately **25% less time** for patient care than one whose documentation is automated" — then, in the same sentence, the benefit is priced at "recovering **30 to 45 minutes** per provider per day."
- Three incompatible magnitudes for one intervention (>120 min automated · 120 min recovered · 30–45 min recovered), presented as a single argument.
- **Fix:** keep the conservative figure and make the relationship explicit — in 797, "... recovering even 30 to 45 minutes per provider per day — a fraction of the two hours the vendor studies claim — is equivalent to...". Or drop the 25% sentence, which is the one doing no work.

MISMATCH 6 — §4.2.1 attributes the 32.3% avoidable-ED figure to two datasets at once

- 693: "the 32.3% avoidable ED visit rate ... is calculated from **VHCURES via VUHDDS**."
- Figure 4.7 Layer 1 lists VHCURES and **VUHDDS as two separate core data assets**; VUHDDS is the hospital discharge database, not a VHCURES derivative. "From X via Y" is not a coherent lineage for two independent sources.
- **Fix:** "is calculated from VUHDDS hospital discharge records" (or "from VUHDDS, with VHCURES used for the payer view") — whichever the AHS report actually states; this needs the source checked, not guessed.

MISMATCH 7 — Two broken cross-references to "Section 4"

- Figure 4.5 cell (765): "FHIR-based interoperability (see **Section 4**)..."
- §4.3.11 close (767): "**Section 4** develops the FHIR picture."
- The chapter is Section 4; FHIR is **§4.5**. Whether the intent was §4.5 or Chapter 5 (whose title carries FHIR), "Section 4" resolves to nothing for a reader.
- **Fix:** both to "§4.5" if the in-chapter treatment is meant; to "Chapter 5" if the deeper FHIR treatment is. §4.5 exists and is substantive, so §4.5 is the better target.

CONTRADICTION 8 — §4.10 tells executives to do exactly what §4.2.2/§4.3.8 say cannot be done

- §4.2.2 Figure 4.2 row 2: under global budgets the lag "makes VHCURES **insufficient as a management tool**"; row 4 required improvement: "**VHCURES retained for retrospective analysis rather than management**."
- §4.3.8 (748): claims integration runs through MDWAS "**rather than through VHCURES**"; "for intervening in a patient's care, [that] lag is not a data source; it is a historical record."
- §4.10 (836): "The hospitals that invest in **VHCURES analytics capability** now will enter the January 2028 performance year with a 12-to-18-month advantage," and the VHCURES attribution file is "**the most important information** your AHEAD performance year will depend on." §4.10 (837) likewise lists "VHCURES attribution and HCC modeling" as critical-path, with no mention of MDWAS.
- Both positions may be defensible (attribution/HCC is a retrospective task; management is not) but the chapter never says so, so as written it recommends the tool it just disqualified.
- **Fix:** in 836, "invest in attribution and HCC analytics capability now — reading the VHCURES attribution file for the retrospective population picture while building toward the MDWAS weekly feed for in-year management (§4.3.8)".

CONTRADICTION 9 — the target architecture keeps VITL central after §4.3.3 argues for winding it down

- §4.3.3 (713–721) is an explicit recommendation: repeal the exclusive designation, "build a second, state-controlled ingestion path," and "the requirement that clinical data pass through a designated nonprofit should be **phased out**."
- Figure 4.7 — the chapter's normative "target technology architecture ... framework for the Strategic Plan" — puts "**Vermont Health Information Exchange (VHIE/VITL): clinical data exchange**" at Layer 2 with no alternative path, and Figure 4.4 Layer 3 leads with "**VITL**Access provider portal."
- **Fix:** add to Figure 4.7's Layer 2 cell a direct-ingestion component — e.g. "Direct provider-to-UHDS ingestion path (second, state-controlled pipeline per §4.3.3)" — or add a sentence to 829/830 reconciling the target state with §4.3.3's recommendation.

CONTRADICTION 10 — three different layer models of the same infrastructure, none reconciled

- Figure 4.1: three layers, Layer 1 = **VHCURES**.
- Figure 4.4: three layers (UHDS), Layer 1 = **Foundational Services** (MPI, terminology, integration engine).
- Figure 4.7: five layers, Layer 1 = **Core Data Assets**.
- §4.3.5 establishes the UHDS as "the vehicle through which the Act 68 §10 integration mandate is **actually delivered**" — yet the UHDS appears **nowhere in Figure 4.7**, the chapter's stated architecture for the Strategic Plan, and MDWAS (the §4.3.8 primary claims path) is also absent from it. "Layer 2" and "Layer 1" therefore mean three different things within one chapter.
- **Fix:** minimally, name the UHDS in Figure 4.7's Layer 2/3 cells and add MDWAS beside VHCURES in Layer 1, plus one sentence in 826 stating how the five-layer target maps onto the UHDS three-layer build (Fig 4.4). Larger consolidation is an author call.

TENSION 11 — Figure 4.7 dependency ordering vs. the maturity assessment beneath it

- 829: "layers **3 and above** represent target states, not current states. Vermont's current infrastructure is solid at Layers 1 and 2, **developing at Layer 4, and nascent at Layers 3 and 5**."
- Figure 4.7 labels **only Layer 3** "(Target State)", so "3 and above" overstates its own figure's labelling; and Layer 4 (Population Health Management) being *more* mature than Layer 3 (Integrated Health Record) beneath it inverts the stack, in a chapter whose Figure 4.4 caption insists "Each layer is dependent on the one below it."
- **Fix:** either label Layers 4 and 5 "(Target State)" in Figure 4.7, or reword 829 to "Layer 3 and the layers above it are partial target states" and add a clause explaining that Layer 4 runs today on the *unintegrated* Blueprint registry, which is why it can be further along than the layer beneath it.

TENSION 12 — the chapter's own timeline delivers the data after the deadline it must precede

- §4.3.8 (749–750): Medicare data scales 2026–2028; **full multi-payer incl. commercial and standardised SDOH follows 2029–2030**; "every argument in this book about global budgets, regionalization, and total-cost-of-care modeling implicitly assumes multi-payer data."
- Figure 4.1 Layer 3: "Act 68 global budgets take effect **FY2028**." §4.10 (837): three components "must be operational **before January 2028**." Figure 4.3 Step 3 SDOH: "2028 and beyond."
- So the chapter states that global budgets require multi-payer data and that multi-payer data arrives one to two years after global budgets start, without remarking on it. This may be the intended argument, but nothing in the text says so.
- **Fix (if intentional):** add one sentence at the end of 750 naming it — e.g. "The sequencing problem is on the face of the schedule: global budgets begin in FY2028 and multi-payer data completes in 2029–2030, which means the first two performance years will be managed on a partial picture."

Verified MATCHES (checked, no defect)

- 695 "four structural limitations" ↔ Figure 4.2 has exactly 4 data rows. **MATCH.**
- 728 "three components ... in sequence" ↔ Figure 4.3 has 3 steps. **MATCH.**
- 736 "three layers, each depending on the one beneath it" ↔ Figure 4.4 has 3. **MATCH.**
- 773 "five specific functions" ↔ five labelled paragraphs (analytics, shared protocols, group purchasing, administrative shared services, referral formalization). **MATCH.**
- 794 "five specific technology categories" ↔ §4.6.1–4.6.5 = 5. **MATCH.**
- 806 "four specific applications" ↔ specialty consults, tele-ICU, tele-behavioral, community paramedicine = 4. **MATCH.**
- 826 "across five layers" ↔ Figure 4.7 has 5 data rows. **MATCH.**
- 63x/691/841 VHCURES coverage: **100%** Medicaid/Medicare + **~60%** commercial stated identically in the stat strip, Figure 4.2 row 1, Figure 4.7 Layer 1, and the §4.11 glossary; Figure 4.2's "40% ... in ERISA self-insured plans" is the correct complement. **MATCH.** (Minor overreach: 696 assigns the *entire* uncovered 40% to ERISA self-insured plans, while 748 says ERISA "excludes large segments" — the softer claim is the safer one.)
- 712 "Act 61 of 2009" + 713 "Seventeen years later" → 2009 + 17 = 2026, the book's present. **MATCH.**
- 763 "four different EHR vendors — Epic, Oracle Health, TruBridge, Meditech" ↔ Figure 4.7 Layer 1 lists the same four. **MATCH.**
- 784 "31 separate EMS agencies" ↔ Ch12 (index 3852) "Vermont's 31 EMS agencies". **MATCH** across chapters.
- 777 "\$1,303 per-discharge administrative cost premium" ↔ used identically at Ch1 (905), Ch11 (3527, 3848, 3849, 3860). **MATCH** across chapters.
- 757/769/838 RHT award stated as **\$195M / \$195 million** in all three places. **MATCH.**
- 750 coverage mix: Medicaid ~24% + Medicare ~21% + commercial ~half ≈ 95%, and "blind to how **three-quarters** of Vermonters receive care" = 100 – 24 = 76%. Internally **consistent**.
- 797 "25% less time" = 2h of an 8h day. Arithmetically **consistent** (but see MISMATCH 5).
- Figure 4.8: all four tool URLs are variants of [/research-lab/interoperability?tab=...](#) Route existence and, more importantly, **promise delivery** (does [?tab=statewide-ehr](#) actually render a Statewide EHR Deployment Modeler; does [?tab=risk](#) return CMS-HCC attribution) is **not checked here** — this audit was scoped to the manuscript. Per CLAUDE.md, a 200 is not a delivered promise; this remains open.
- **Unverifiable from inside the chapter** (flagged for source check, not fixed): "47 state applications" (794 — appears once in the whole manuscript, nothing to reconcile against; the RHT program is widely reported as having drawn applications from all 50 states, so this number should be confirmed against the CMS award announcement); "well over **\$100 million** cumulatively on HIT and HIE since 2009" (716); "over **\$14 million** annually" chart-abstraction burden (744); UVMHN ransomware "**\$50 to \$65 million**" and "six hospitals ... nearly a month" (754); "**80%** rural vs **85%** non-rural broadband" (805); "\$100-300M" EHR implementations (Figure 4.5).

Superlative / ranking claims logged for cross-chapter comparison

Verbatim, with index and context. The first cluster is the concerning one: **four different things in this chapter are each framed as the biggest technology problem.**

1. 713 — "Seventeen years later the same provision is **the single largest structural impediment to the Technology pillar**, and it should be repealed." (*the VTTL exclusive-operator statute*)
2. 701 — "the gap between what Vermont appears to have in a statewide HIE and what the exchange can actually be used for. The finding is worth quoting because it is **the more damaging of the two.**" (*HIE usability gap, ranked above all VHCURES limitations*)
3. 706 (**BEYOND VERMONT** callout) — "The gap Vermont is closing ... is **the single most common technology-pillar failure nationally.**"
4. 739 (heading) — "Why Integration Is **the Load-Bearing Element**", supported by 758: "The Technology pillar is not one workstream among six; it is **the substrate the other five run on.**" → *Items 1–4 are not strictly contradictory (largest impediment ≠ most common failure ≠ load-bearing element) but they read as four competing "the real problem is X" verdicts. This is the defect class CLAUDE.md names: "two different dependencies both called the most underestimated." Recommend the author designate one and subordinate the rest.*
5. 727 — the one-health-record mandate "is **the most ambitious data integration goal any state has formally committed to**, and its achievement would give Vermont a population health management capability that **no other state currently possesses.**" (*two national-first claims in one sentence; needs a source*)
6. 769 — "**one of the most significant technology investments** in the entire \$195M award"; "Vermont is **one of four states** (along with Arkansas, Nevada, and Oregon) that explicitly included CIN creation in their RHT applications."
7. 776 — group purchasing: "Oliver Wyman identified this as **one of the highest-leverage near-term cost reduction opportunities.**"
8. 781 — FHIR "is **the closest thing to a universal language** for healthcare data that the industry has adopted."
9. 794 — "Vermont's RHT Program application is **notable among the 47 state applications** ... for its specificity." (*also a count claim — see above*)
10. 798 — ambient AI as "**the single technology investment most likely** to address workforce shortages."
11. 802/808 — RPM: "**precisely the population where RPM produces the largest** clinical and financial benefit"; diagnostic AI: "Vermont's smaller rural hospitals may be the settings that **benefit most.**"

12. 813 — price caps plus transparency "creates **the closest approximation to a functioning market** for hospital services that **any regulatory framework has achieved.**"
 13. 693 — VHCURES "has produced **some of the most credible and consequential health policy analysis** in recent Vermont history."
 14. 836 — the VHCURES attribution file is "**the most important information** your AHEAD performance year will depend on." (*see CONTRADICTION 8*)
 15. 734 — "Accurate identity resolution across organizations is **the foundation every other capability rests on.**" (*a fifth "most fundamental thing" in the chapter*)
 16. 847 (glossary) — 42 CFR Part 2 "creates **the primary legal barrier** to integrating SUD treatment data."
-

Other issues

Genuine repetition (fix candidates)

- The 12–18-month VHCURES lag is asserted **four** times in-chapter (691 stat strip, 696 Figure 4.2 row 2, 748, Figure 4.5 row 4), plus once more as an analogy at 836. Twice is enough; the stat strip and Figure 4.2 are sufficient, and 748/765 can cite them.
- Oliver Wyman's "not viewed as 'user friendly' by providers" is quoted in full at **703** (the Perception/Reality block) and quoted again in full at **715**. Keep one; at 715, "the usability finding quoted above."
- "Claims data records that a service happened but not what it found" is made three times: Figure 4.2 row 4 ("A claim shows an HbA1c test was performed; it does not carry the result"), 741, and 742 ("not the blood pressure reading or the lab result"). Figure 4.2's version is the sharpest; 742's is the redundant one.
- The VHCURES/VITL/analytics triad is stated at 675, restated at 676, then tabulated as Figure 4.1 — three statements of the same three items on one page. 675 is a chapter opener and 676 a framing paragraph, so one of the two is legitimate scaffolding; the overlap is in the *list*, which Figure 4.1 already carries.
- §4.7's minimum AI-governance requirements (819–823) and §4.7's framing (816) both argue the CIN is the natural home for shared AI governance; 824 says it a third time. Legitimate recurrence in a recommendations list, but 824's first sentence is a restatement of 816.
- Not repetition (deliberate recurrence, leave alone): §4.11 glossary entries restating VHCURES/VITL/FHIR/CIN definitions; Figure 4.7 restating components introduced in prose; section headings naming their own topic.

Copy defects found in the XML (mechanical, unambiguous)

- **710** — "DVHA has direct operational relationships with healthcare providers through its Medicaid contracts, Blueprint for Health administration." Broken list: a conjunction or a third item is missing. Fix: "...through its Medicaid contracts **and** Blueprint for Health administration."
- **798** — "**Clinicians used the technology spent less time** writing notes during visits." Missing relative pronoun. Fix: "Clinicians **who used** the technology spent less time..."
- **757** — "only functions if bed capacity and provider availability **is tied with** a correctly matched patient record." Number disagreement plus a wrong preposition. Fix: "...if bed capacity and provider availability **are tied to** a correctly matched patient record."
- **791 (Figure 4.6)** — Partial row begins lowercase mid-phrase (MISMATCH 3 above).
- **827 (Figure 4.7, Layer 1)** — "Oracle Health (**others**), TruBridge (CAHs), Meditech (**others**):" "others" used as the descriptor for two mutually exclusive groups, so the cell does not partition the 14 hospitals. Also Epic is "(UVMCMC, CVMC)" here but "dominant at UVMCMC" at 763 with no mention of CVMC.

Thin / structurally imbalanced sections

- **§4.3 is half the chapter** — eleven subsections (4.3.1–4.3.11), including the entire UHDS treatment (4.3.5–4.3.10), the exclusive-operator argument, the privacy shield and the RHT dependency. Several of these are chapter-level arguments buried three levels deep: §4.3.3 (the repeal recommendation) and §4.3.5–4.3.6 (the UHDS, presented as *the* delivery vehicle) are arguably §4.4 and §4.5 in their own right. By contrast **§4.1 has one subsection, §4.7 has one, and §4.4 has two thin ones** (§4.4.1 is a single paragraph).
- **§4.4.1** ("What a Clinically Integrated Network Does") is one paragraph and duplicates the §4.11 glossary entry for CIN almost proposition-for-proposition.
- **§4.7** asserts "Vermont does not currently have a statewide AI governance framework" and then lists five requirements, but never states who promulgates them or under what authority — the only mechanism offered is "the CIN's shared services function," which is voluntary. Thin on the enforcement question the rest of the chapter is rigorous about.
- **§4.8's Figure 4.7** is presented as the deliverable the Strategic Plan should adopt, yet omits the two systems §4.3 spent ten subsections establishing (UHDS, MDWAS) — see CONTRADICTION 10. This is the single highest-value structural fix in the chapter.

Ambiguity worth a decision, not a fix

- **758** — "The Technology pillar is not one workstream among **six**; it is the substrate the other **five** run on." Arithmetically self-consistent (five pillars + Equity = six), but the book's framing is "five-pillar," and Figure 10.5 treats Equity as *cross-cutting* rather than a sixth workstream. "One workstream among six" invites the reader to count Equity as a peer pillar, which Chapter 10 declines to do.

- 808 / 815 — "The book's later discussion of diagnostic AI maturation..." and "The book's later discussion of AI governance..." Both almost certainly mean Chapter 5 ("FHIR, AI Governance, and Clinical Decision Support"), which exists. Naming it would be stronger and costs nothing.

Not checked (out of scope, still open)

- Criterion 5's non-automatable half: whether each Figure 4.8 tool tab actually delivers the named capability, and whether §4.9's claims about what the reader will find are true.
- Preface/Introduction alignment scan for Chapter 4's figures and definitions.
- Formatting (`check_format.py`) — this audit read the XML for content only and made no edits, so no render or format gate was triggered.

No edits were made to `HTR_Book_v42.docx`.

===== FILE: audit_ch05.md =====

Chapter 5 Audit

Chapter 5: *The Technology Pillar in Practice — FHIR, AI Governance, and Clinical Decision Support*

Source of truth: `word/document.xml` inside `/Users/baba/Vermont-Health-Platform/HTR_Book_v42.docx`, real Heading1 at byte offset 2,019,675 through Heading1 "Chapter 6:" at 2,169,670 (the low-offset occurrences at 104,575 / 118,964 are the TOC field and were ignored). Read-only pass — the `.docx` was not modified.

Structure found: 9 Heading2 sections (5.1–5.9), 5 tables (4 captioned figures + 1 BEYOND VERMONT callout), 0 numbered lists.

Figures found (ground truth)

TABLE1 → Figure 5.1 — "Vermont's three priority FHIR use cases." Header: FHIR use case | What it enables | Implementation requirements | Vermont priority. **3 data rows:** Patient access API (§170.315(g)(10)); Provider access API (care coordination) — flagged "Highest value use case for Vermont CIN"; Payer-to-provider API (prior authorization and formulary). Sources line present.

TABLE2 → BEYOND VERMONT callout (no figure number, correctly uncaptioned). Single cell on the master patient index as a go/no-go gate for any care-coordination initiative. Note: the cell text runs together as "BEYOND VERMONTA master patient index..." in extraction — that is a run boundary, not necessarily a rendered defect, but worth an eyeball on the next render.

TABLE3 → Figure 5.2 — "Population health CDS types and Vermont applications." Header: CDS type | What it does | Implementation requirement | Vermont application. **4 data rows:** Predictive risk stratification (top 5% highest-risk for intensive CHT engagement); Care gap identification; SDOH screening and navigation; Medication safety and adherence. Sources line present.

TABLE4 → Figure 5.3 — "Technology pillar implementation matrix." Header: Investment | Estimated cost | Timeline to value | ROI crossover | Vermont benchmark. **6 data rows:** FHIR R4 compliance (\$150K–\$500K/org, 12–18 mo, 18–24 mo); Clinical AI deployment with governance (\$200K–\$1M setup, 6–12 mo, 12–24 mo, "ambient documentation ROI 30–45 min/day/provider"); VHCURES analytics access (\$20K–\$60K/yr, 3–6 mo, 6 mo); CIN shared analytics infrastructure (\$500K–\$2M network, 18–36 mo, 36–48 mo, "RHT-funded shared analytics for all 14 hospitals"); EHR optimization (\$15K–\$40K/practice, 1–3 mo, 6 mo); Cybersecurity assessment and hardening (\$50K–\$200K/org, 3–6 mo, "Risk avoidance immediate"). Sources line present.

TABLE5 → Figure 5.4 — "Hands-on platform tools for the Technology Pillar in practice." Header: Do this | On this tool | What to look for. **4 data rows:** AI Clinical Governance Lab — `/research-lab/technology-ai?tab=ai`; Digital Health Lab — `/research-lab/technology-ai?tab=digital`; Clinical Data Exchange Lab — `/research-lab/vbc-clinical-quality?tab=h17`; Statewide EHR Deployment Modeler — `/research-lab/interoperability?tab=statewide-ehr`. **No Sources line** — the only captioned figure in the chapter without one.

Claims checked against their own source (MATCH/MISMATCH + proposed fix)

1. MISMATCH — §5.7 lead-in undercounts its own table by one. Prose: "Chapter 5's implementation reality — AI governance, CDS, and clinical data exchange — maps to **three** working labs." Figure 5.4 lists **four** labs (the fourth being the Statewide EHR Deployment Modeler, which is also not covered by the three-topic enumeration in the same sentence). *Proposed fix:* "...— AI governance, CDS, clinical data exchange, and the statewide-EHR question — maps to four working labs."

2. MISMATCH (internal contradiction) — the AI Clinical Governance Checklist is described with two incompatible structures. §5.3.1: "The AI Clinical Governance Checklist organizes governance requirements across **four lifecycle stages**" (then §5.3.2–5.3.5 = Pre-Deployment, Implementation and Training, Active Monitoring, Vendor Management). Figure 5.4: "The **62-item checklist across eight governance domains** as a go/no-go gate..." The platform side was verified: `frontend/components/research/AIAnalyticsLab.tsx GOVERNANCE_DOMAINS` contains exactly 8 domains (Clinical Validation, Data Quality, Bias & Equity, Transparency, Oversight & Monitoring, Ethics & Patient Rights, Vendor & Third-Party Governance, Security & Incident Response) and 62 question strings — so "62 items / 8 domains" is correct and §5.3.1's "the AI Clinical Governance Checklist" is what does not match the named artifact. *Proposed fix:* keep the tool's structure authoritative and make §5.3.1 name the book's own construct distinctly, e.g. "This section organizes governance requirements across four lifecycle stages; the platform's AI Clinical Governance Lab cross-cuts the same material as a 62-item checklist over eight governance domains." (Under the standing rule, the tool is not to be reworded down to four stages.)

- 3. MISMATCH (dangling colon / missing promised list).** §5.3.1 is a single sentence ending in a colon — "...across four lifecycle stages:" — and is followed immediately by a Heading3, not by a list or table. The colon promises an enumeration that the XML does not contain. *Proposed fix:* end the sentence with a period, or add the four stage names inline.
- 4. MISMATCH (weak) — §5.4.2 enumerates three CDS applications; Figure 5.2 has four rows.** Prose: "risk stratification..., care gap identification..., and SDOH screening tools..." — Medication safety and adherence (row 4) is never introduced in prose. No explicit "three", so this is an omission rather than a false count, but the figure reads as if a row was added later. *Proposed fix:* add "...and medication-safety and adherence monitoring across multiple prescribers" to the §5.4.2 sentence.
- 5. MISMATCH (tool titles) — two labs are cited under names that do not exist.** Book "Clinical Data Exchange **Lab**" → actual tab label is "Clinical Data Exchange" (**VBCClinicalQualityClient.tsx** id **hl7**). Book "Statewide EHR **Deployment** Modeler" → actual label "Statewide EHR Modeler" (**InteroperabilityClient.tsx** id **statewide-ehr**). Both routes and both tab ids resolve, so the links work; only the titles drift. *Proposed fix:* use the platform labels verbatim, or rename the tools.
- 6. MATCH** — "three FHIR use cases" (§5.2.2) vs Figure 5.1's 3 rows.
- 7. MATCH** — "three AI governance challenges that are more acute than in urban academic medical center settings" (§5.3.6) vs the First/Second/Third paragraphs (training-data representativeness, implementation support capacity, AI scribe equity).
- 8. MATCH** — "disproportionately vulnerable ... for three reasons" (§5.5) vs the three listed (limited IT staff, legacy unpatched systems, high-value data assets).
- 9. MATCH** — "three principles that are violated by most EHR default configurations" (§5.4.1) vs specificity / actionability / workflow integration.
- 10. MATCH** — Figure 5.3's "all 14 hospitals." Grepped the whole manuscript: 14 is the only hospital count used (**14 hospitals, all 14 Vermont hospitals, 14 Vermont hospitals**), no competing figure.
- 11. MATCH (internally)** — the 30–45 min/day/provider ambient-documentation ROI in Figure 5.3 is the manuscript's only occurrence of that range, and the chapter's Sources line cites the JAMA Network Open ambient scribe study (2024) that would carry it.
- 12. MATCH (consistent, not verified externally)** — the January 2028 analytics-vendor deadline in the chapter opener and in §5.8 agree with each other.
- 13. MATCH** — the 62-item / 8-domain claim itself (see item 2); verified against the live source file, not inferred.

Superlative/ranking claims logged for cross-chapter comparison

All verbatim, with locating context:

- Chapter opener: "reveals the Technology pillar's **most consequential sequencing vulnerability**: the analytics vendor gap that must close before January 2028."
- §5.2.2 lead-in: "three FHIR use cases that deliver **the highest value** for the transformation agenda."
- Figure 5.1, Provider access API row: "**Highest value use case** for Vermont CIN."
- §5.4.1: "Alert fatigue ... is **one of the most documented failures** of health information technology implementation."
- §5.4.2: "the **highest-value CDS application** is not medication alerts — it is population health intelligence."
- §5.8 (national policy professional): "The gap between what CMMI models assume about state data infrastructure and what states actually have is **the most underappreciated obstacle in American health reform**."
- §5.8, same paragraph: "will be **the best available evidence** on how to close that gap for states that have less."
- §5.5.1: "Vermont's CIN shared services model is **the appropriate delivery vehicle**" for rural cybersecurity.
- §5.5: "Rural healthcare cybersecurity is **a specific and serious national vulnerability**."

Cross-chapter flag on #6. HTR_Book_v42.md line 1144 (Ch1 dependency glossary) says "**One of the most underappreciated dependencies in healthcare transformation**." Different noun (dependency vs obstacle) and hedged with "one of", so not a strict contradiction — but it is the same superlative adjective claiming a top slot twice, the exact pattern that produced the Ch1 "most underestimated" defect. Recommend the Chapter 5 instance stay definite only if no other chapter claims a single most underappreciated obstacle; whoever audits Ch3/Ch4/Ch16 should check that noun. Also note #2 ("highest value" for a set of three) sits beside #3 ("highest value" for one of those three) two paragraphs apart — the plural claim weakens the singular one.

Other issues

Lead paragraph promises a subject the body never develops. The opening sentence names "the analytics vendor gap that must close before January 2028" as the chapter's central vulnerability, but no section analyzes it — the gap reappears only once, in a §5.8 executive aside. Either the opener should point at the chapter's actual spine (FHIR → AI governance → CDS → cybersecurity) or §5.2–5.5 needs an analytics-vendor subsection. This is the clearest structural defect after the two count mismatches.

Opener enumerates three risks; the chapter has four major bodies. "FHIR interoperability, AI clinical governance, and cybersecurity are not three separate technology initiatives ... they are three implementation risks ... in roughly this order." §5.4 (Clinical Decision Support) is a peer-level section absent from that list, and it is not "in roughly this order" either — CDS sits between AI governance and cybersecurity. Same one-off pattern as issue #1 above; likely both date from CDS being added after the framing was written.

§5.3.2–5.3.5 are prose-only where the rest of the chapter tabulates. Four consecutive Heading3 sections each consist of one long numbered-inside-a-paragraph run of 4–5 items (e.g. "(1) FDA clearance ... (5) Intended use clarity"). Nothing is wrong

factually, but the chapter's own convention elsewhere (5.2.2, 5.4.2, 5.6) is a figure. A single "AI governance lifecycle" figure would also resolve issue #3's dangling colon.

§5.5.1 renders a bulleted list as five bare paragraphs (SOC monitoring, EDR, vendor risk management, incident response planning, staff training) introduced by a colon. No list formatting in the XML — each is a standalone `<w:p>` with a run-in label. Flag for the formatting pass (`check_format.py`), not a factual defect.

Figure 5.4 has no Sources line while 5.1, 5.2 and 5.3 all do. Platform-tool figures elsewhere in the book should be checked for what they use; if they carry one, add it.

Genuine but legitimate recurrence (deliberately left). "A standard, not a product" (§5.2.1 and the FHIR R4 glossary entry); "a design problem, not a clinician behavior problem" (§5.4.1 and the Alert fatigue entry); Change Healthcare (§5.5 and its glossary entry). These are glossary entries restating their own body text, which the definition of done classes as legitimate recurrence.

Mild real restatement (flagged, not fixed). The CIN-shared-services-solves-rural- capacity argument is made twice in near-identical shape: §5.3.6 ("a small rural hospital with one IT staff member cannot implement the monitoring infrastructure that AI governance requires. The CIN's shared services model addresses this") and §5.5.1 ("Individual hospitals cannot afford a 24/7 security operations center; the CIN can"). Two different pillars of risk, so arguably intentional parallelism — but the second could cross-reference the first rather than re-derive it.

Alert-fatigue evidence is hedged then quantified. §5.4.1 opens "Studies consistently show that the majority of EHR clinical alerts are overridden" and later says "When 90% of alerts are overridden, the alerts are generating noise." The 90% is grammatically conditional so it is not a false citation, but a reader will take it as the studies' number. If a figure is meant, cite it; otherwise consider "When the override rate reaches 90%."

Not checked here (out of scope of a chapter-internal pass): §5.1's claim that "Chapter 4 established ... the five-layer infrastructure from VHCURES and VUHDDS through the HIE and FHIR exchange layer to population health analytics and AI applications" — a cross-chapter dependency that a Ch4 audit should confirm names five layers in that order. Also unverified: whether the four labs in Figure 5.4 actually tag Chapter 5 in the tool registry (no chapter field was found in `frontend/lib/research-lab/tools.ts` by grep; `audit_chapter.py` 5 is the right instrument for that half).

===== FILE: audit_ch06.md =====

Chapter 6 Audit

Scope: Chapter 6 (The Economics Pillar — Global Budgets, Reference-Based Pricing, and Financial Reform), body of `HTR_Book_v42.docx`, `word/document.xml` block indices 931–1085 (heading "Chapter 6:" through heading "Chapter 7:"). Read directly from the OOXML via `zipfile` + `ElementTree`, cells extracted per `w:tr/w:tc` — not the `.md` mirror.

Read-only audit. No edit was made to the .docx. Every "proposed fix" below is a recommendation, not an applied change.

Chapter contains 12 numbered figures (6.1–6.12) across 14 tables (Fig 6.1 is rendered as two tables: a stat strip plus a data grid), 11 Heading2 sections, 225 non-empty blocks.

Figures found (ground truth)

Figure 6.1 — Vermont hospital commercial pricing vs. Medicare and break-even benchmarks

Sources: *GMCB Act 68 RBP Update (February 2026)*; *RAND Hospital Price Transparency Study*.

Stat strip (4 cells): **358% UVMC Commercial vs. Medicare (RAND) · 250-300% Average VT Commercial Rate · ~136% Approx. Break-Even Point · \$400M+ Projected 5-Year Savings**

Data grid (2×2):

Average commercial-to-Medicare ratio, Vermont hospitals — 250–300% vs. ~136% break-even point	Outpatient imaging — worst case — 944% of Medicare rate at one Vermont hospital
UVMC commercial prices — 358% of Medicare (RAND, 2018–2020 data) — the highest in Vermont	Potential savings at 200% Medicare (2018–2023) — \$400M VEHI + VSEA combined, per GMCB analysis

Figure 6.2 — Vermont silver marketplace premium trajectory 2018–2024

Sources: *Oliver Wyman Act 167 Report*; *GMCB analysis*. 2018 \$456 · 2019 \$474 · 2020 \$468 · 2021 \$580 · 2022 \$730 · 2023 \$875 · 2024 \$948

Figure 6.3 — Cross-state reference-based pricing and rate regulation

Sources: *GMCB Act 68 Update (February 2026)*; *Health Affairs*; *Commonwealth Fund*; *CMS*. Header row + **5 data rows**: Oregon (2019) · Montana (2016) · Washington (2021) · Maryland (2014–present) · Vermont Act 68 (FY2027). Key figures: Oregon 200% in-network / 185% out-of-network, ~300,000 lives, \$107.5M in first 27 months, all 24 affected hospitals remained in-network. Montana \$48M. Washington 160%. Maryland \$1.4B Medicare savings, 7% admission reduction, \$800M spending reduction. Vermont: "first negative commercial rate benchmark (-1%) **in effect FY26** as transition step"; "Initially commercial payers; **excludes Medicare/Medicaid**"; no numeric RBP multiplier stated.

Figure 6.4 — Vermont RBP and global budget implementation phases

Source: *GMCB Act 68 Update, February 2026*. Header row + **3 data rows**: Phase I — Differential Price Growth (FY26–FY27), "**FY27 guidance recommends -1% commercial rate growth benchmark**" · Phase II — Rate Setting (RBP) (FY27–FY28), final methodology March 2027 · Phase III — Global Budget Integration (FY28 and FY30), non-CAH by FY28, all-hospital by FY30.

Figure 6.5 — Five dimensions of global budget design

Header row + **5 data rows**: Scope · Population adjustment · Volume treatment · Quality linkage · Flexibility mechanisms. Scope's Vermont direction: "**TBD through rulemaking**."

Figure 6.6 — Vermont global budget history: four attempts

Header row + **4 data rows**: GMCB Net Patient Revenue Cap (2012–present) · Rutland Regional pilot (2014, never implemented) · Vermont All-Payer ACO Model (2017–2025, expired end 2025) · Act 167 / Act 68 Mandate (2022–present; non-CAH by FY2028, all by FY2030).

Figure 6.7 — Maryland All-Payer Global Budget Model results

\$1.4B Medicare hospital savings · 7% admission reduction, avoidable admissions down >6% · \$800M spending reduction (Part A+B) · 3.58% all-payer per capita committed growth cap.

Figure 6.8 — Oliver Wyman Vermont hospital financial projection scenarios

Scenario	5-year system deficit (2024–2028)
Conservative 3.5% rev / 5% expense	\$700M vs. break-even; \$1.4B vs. 3% margin; FY2023 budget overage \$106M
Realistic 3.5% rev / 7–8% expense (labor +10%, physician +5%, other +7%, 340B +3%)	\$2.4B vs. break-even (\$3,700/resident); \$3.1B vs. 3% margin (\$4,800/resident); hospitals requested \$285M in FY25 increases

Figure 6.9 — Financial metrics: FFS vs. global budget

8 paired rows, no numeric claims.

Figure 6.10 — Estimated sources of savings (5-year total)

Close unsustainable inpatient facilities \$100M+ · Admin costs & shared services \$115M+ · **Shift procedures to ASUs/outpatient \$130M+ (largest row)** · Expand primary care \$90M+ · Address SDOH \$80M+. **Row sum = \$515M+**. Caption: "illustrative ranges derived from Oliver Wyman's >\$400M total estimate."

Figure 6.11 — Three-phase APM transition model

Header + 3 rows: Preparation (Now through FY2026; "-1% commercial rate benchmark" listed as already in the payment environment) · Transition (FY2027) · Operation (FY2028 and beyond; "Statewide Strategic Plan delivered December 2028").

Figure 6.12 — Hands-on platform tools for the Economics Pillar

2 rows citing [/research-lab/payment-models?tab=apm-design](#), [?tab=gb-transition](#), [/research-lab/policy-quality?tab=scorecard](#). (Route/promise delivery is criterion 5 and out of scope for this factual pass — not verified here.)

Claims checked against their own source (MATCH/MISMATCH + proposed fix)

MISMATCH 1 — §6.1: "the other five" pillars (arithmetic self-contradiction) — HIGH

"Of the five pillars in the five-pillar framework, the Economics pillar is the one that determines whether **the other five** actually produce results."

Five pillars minus Economics = four. Same class of error as the Chapter 1 find. **Fix:** "...determines whether the other four actually produce results." (Also consider trimming the redundant "Of the five pillars in the five-pillar framework" → "Of the five pillars".)

MISMATCH 2 — §6.7.1 / Fig 6.8: the -1% benchmark is dated three different ways — HIGH

Three sources inside this chapter disagree on when the negative commercial rate benchmark applies:

- **Fig 6.3** (Vermont row): "first negative commercial rate benchmark (-1%) **in effect FY26**"
- **Fig 6.4** (Phase I) + §6.3.4 prose: "**FY27** guidance recommends -1%..."; "The Phase I action — the negative 1% commercial rate benchmark **for FY2027**"
- **Fig 6.11** (Preparation row): "**Now through FY2026** ... -1% commercial rate benchmark" — i.e. already in force in FY26

Fix: pick one. Fig 6.4 and the §6.3.4 prose agree on FY27 guidance and are the more specific, GMCB-sourced statement, so amend Fig 6.3's Vermont row to "first negative commercial rate benchmark (-1%) in FY27 budget guidance" and move the -1% out of Fig 6.11's Preparation row (FY26) into its Transition row, or relabel Preparation as "Now through FY2026, with the FY27 -1% benchmark already issued."

MISMATCH 3 — §6.10 / Fig 6.3 + Fig 6.5: "Vermont's RBP target of 200% of Medicare" — HIGH

"Vermont's RBP target of 200% of Medicare is the most specific **mandatory all-payer** price target enacted by any state."

Two defects against this chapter's own figures:

- 1. **No 200% Vermont target exists anywhere in the chapter.** Fig 6.3's Vermont row gives no multiplier ("maximum set by GMCB rule"); Fig 6.5's Scope row says "TBD through rulemaking"; §6.3.1 and §6.3.4 say the final methodology is not released until March 2027 and will be *hospital-specific* (payer mix, labor costs, social risk). 200% is **Oregon's** number (Fig 6.3) and appears for Vermont only as a retrospective counterfactual ("Potential savings at 200% Medicare (2018–2023)" in Fig 6.1).
- 2. **"all-payer" contradicts Fig 6.3**, which states Act 68 RBP covers "Initially commercial payers; **excludes Medicare/Medicaid.**"

Fix: "Vermont's mandatory commercial RBP ceiling — to be set by GMCB rule in March 2027, with 200% of Medicare the benchmark most often modeled — is the most specific mandatory statewide price ceiling enacted by any state."

MISMATCH 4 — §6.5.4 vs §6.6 vs §6.11: EAST Fund causality is reversed — HIGH

- §6.5.4: "up to \$138 million ... starting in 2027 — later cut to a cap near \$10 million **when Vermont withdrew from AHEAD** in July 2026" → the cut follows the withdrawal.
- §6.6: "a subsequent CMS renegotiation ... cut that expectation to a cap of about \$10 million. **That collapse left the State facing** ... and in July 2026 Vermont formally notified CMS of its withdrawal." → the cut *caused* the withdrawal.
- §6.11 Key Concepts (EAST Fund): agrees with §6.6 — "A subsequent CMS renegotiation cut this expectation to a ~\$10M cap, **and** Vermont withdrew."

Two of three say the cut caused the exit; §6.5.4 says the opposite. **Fix (§6.5.4):** "...up to \$138 million in additional Medicare funds annually starting in 2027 — an expectation a later CMS renegotiation cut to a cap near \$10 million, which is why Vermont withdrew from AHEAD in July 2026."

MISMATCH 5 — §6.6 vs §6.10: RHT Program is both "\$195M per year for five years" and "first year only" — HIGH

- §6.6: "the **five-year, \$195-million-per-year** Rural Health Transformation Program"
- §6.5.4: "The Rural Health Transformation Program award of **\$195 million** provides capital..."
- §6.10 (legislator): "The Rural Health Transformation Program — **\$195 million for its first year, with future-year amounts not yet published**"

§6.6 asserts as fact exactly what §6.10 says is unpublished. **Fix:** adopt §6.10's cautious formulation everywhere: "the five-year Rural Health Transformation Program, whose first-year Vermont award is \$195 million."

MISMATCH 6 — §6.10 + §6.8.1 + §6.11 still treat AHEAD as live after §6.6 says Vermont exited — HIGH

§6.6 and the §6.11 EAST Fund entry establish that Vermont withdrew in July 2026 and never received EAST money. Three later/other passages contradict that:

- §6.10 (legislator): "...the population health improvement **AHEAD measures. If the EAST Fund is used** to cover operating deficits ... The legislature **should set explicit expectations for EAST Fund use.**" — advice about spending a fund the chapter says does not exist.
- Fig 6.11, Transition row: "Primary care investment: increase PC capacity **ahead of AHEAD's primary care investment requirements.**"
- §6.11 Key Concepts, *All-payer alignment*: "Vermont's **AHEAD + Act 68 combination** is designed to achieve this by FY2028-2030."

Fix: re-point all three at Act 68 / RHT. Legislator paragraph → "the Rural Health Transformation Program ... If RHT funds are used to cover operating deficits..."; Fig 6.11 → "ahead of the primary care capacity global budgets will require"; Key Concepts → "Vermont's Act 68 mandate is designed to achieve this across commercial payers and Medicaid by FY2028-2030, with Medicare alignment now unresolved after the AHEAD withdrawal."

MISMATCH 7 — §6.2.3 vs Fig 6.8: the \$700M–\$2.4B range is attributed to one scenario — MEDIUM

"under **the conservative scenario**, 13 of Vermont's 14 hospitals report operating losses by 2028, with a cumulative 5-year system deficit of **\$700 million to \$2.4 billion** depending on expense growth assumptions."

Fig 6.8: \$700M is the conservative case, \$2.4B the realistic case. A range "depending on expense growth assumptions" spans *both* scenarios and cannot sit under "the conservative scenario." **Fix:** "Oliver Wyman's projections show 13 of Vermont's 14 hospitals reporting operating losses by 2028 even under the conservative scenario, with a cumulative 5-year system deficit of \$700 million to \$2.4 billion depending on expense growth assumptions."

MISMATCH 8 — §6.3.2 vs Fig 6.3: "Three states" against a five-row table — MEDIUM

"**Three states** have implemented meaningful price reference programs with documented results, providing the evidence base for Vermont's design choices."

Fig 6.3 carries **four** non-Vermont jurisdictions (Oregon, Montana, Washington, Maryland), and the caption itself is "reference-based pricing **and rate regulation.**" Maryland is arguably rate-setting rather than RBP, but the reader counts rows. **Fix:** "Three states have implemented meaningful price reference programs with documented results, and a fourth — Maryland — regulates rates for all payers outright."

MISMATCH 9 — §6.3.4 vs Fig 6.4: "two phases" against a three-phase table — MEDIUM

"GMCB is implementing RBP in **two phases**..." immediately above Fig 6.4, which lists Phase I, Phase II and Phase III (and is captioned "RBP **and global budget** implementation phases").

Fix: "GMCB is implementing RBP in two phases, with a third that folds the resulting price ceiling into global budgets."

MISMATCH 10 — Fig 6.1 internal: two different \$400Ms conflated — MEDIUM

The stat strip labels **\$400M+** as "Projected 5-Year Savings". The same figure's data grid defines its \$400M as "Potential savings at 200% Medicare (2018–2023) — \$400M VEHI + VSEA combined, per GMCB analysis" — a *retrospective counterfactual* for two public employee plans. §6.7.3's heading then reuses the number for a third thing: "Where Vermont's \$400M+ Can Come From," sourced to Oliver Wyman's *prospective* operational savings (Fig 6.10). **Fix:** relabel the strip cell "\$400M+ — Retrospective savings if capped at 200% of Medicare (VEHI+VSEA, 2018–2023)", and keep §6.7.3's Oliver Wyman total visibly distinct (e.g. ">\$500M Oliver Wyman transformation savings").

MISMATCH 11 — §6.7.3 heading/caption vs Fig 6.10 row sum — LOW

Fig 6.10's rows total **\$515M+** (\$100+\$115+\$130+\$90+\$80) while the heading says "\$400M+" and the caption says "derived from Oliver Wyman's >\$400M total estimate." Technically consistent with "more than \$400M," but the decomposition overshoots its own stated total by ~29%, which invites the arithmetic objection. **Fix:** state the sum: "derived from Oliver Wyman's >\$400M total estimate; the itemised ranges here sum to roughly \$515M at their low ends."

MISMATCH 12 — §6.7.1: "again roughly 62%" — LOW

"a \$312M cumulative 5-year deficit ... **more than 44%** of the total statewide deficit. Under the realistic scenario, this grows to \$1.5B — **again roughly 62%** of the statewide total."

Both percentages check out against Fig 6.8 ($312/700 = 44.6\%$; $1,500/2,400 = 62.5\%$) — the arithmetic is fine. But "again" asserts the two shares are the same, and $44\% \neq 62\%$. **Fix:** delete "again", or "— a still larger 62% of the statewide total."

MISMATCH 13 — §6.2.2 vs §6.2.2 label / Fig 6.2 caption: premium series is named two ways — LOW

Prose: "**individual market** average monthly premium from \$456 in 2018 to \$948 in 2024." Table lead-in and caption: "Vermont average monthly **silver marketplace** premium." Individual-market average and silver-benchmark premium are different series. **Fix:** use "silver marketplace" in the prose too, matching the figure.

MISMATCH 14 — §6.7.3: "the third Oliver Wyman imperative" — LOW

Chapter 6 never enumerates the Oliver Wyman imperatives, so "the third" has no referent inside the chapter. **Fix:** name it instead of numbering it ("reflects Oliver Wyman's imperative to move all care possible out of hospitals"), or cite the chapter that does enumerate them.

MISMATCH 15 — §6.10: "\$1,303 per-discharge administrative gap" and "18 months away" — LOW/MEDIUM

Neither is supported inside the chapter. The \$1,303 CAH-vs-national admin gap appears exactly once, in no figure. "That model change is 18 months away" is a floating relative date that does not match either of the chapter's own milestones (non-CAH global budgets FY2028; final RBP methodology March 2027) and will rot. **Fix:** cite the \$1,303 figure to its source or drop it; replace "18 months away" with the absolute milestone ("it changes when non-CAH global budgets take effect in FY2028").

MISMATCH 16 — §6.8.2: "This gap must be closed before FY2027" — LOW

The attribution/TCOC analytics gap is described as preparation for *global budget participation*, which the chapter dates to FY2028 (non-CAH) / FY2030 (all). FY2027 is the RBP date. **Fix:** "before global budgets take effect in FY2028."

Claims that MATCH their source (verified, no action)

- §6.2.2: "250–300% on average, with outliers reaching 944%", "~136% break-even", "358%" — all match Fig 6.1 both cells and strip.
- §6.2.2 premium math: $\$456 \rightarrow \$948 = 107.9\%$ increase over 2018–2024 (six years). "108% in six years" in both prose and Fig 6.2 caption. MATCH.
- §6.3.4: "\$230.65 million in FY2026: \$104.3M + \$94.58M + \$31.76M" sums to \$230.64M — a \$0.01M rounding artifact only. MATCH (no fix needed).
- §6.3.2: "all 24 affected hospitals remained in-network", "rates set at 200% of Medicare", "Oregon implemented RBP for a state employee plan" — all match Fig 6.3's Oregon row.
- §6.4.2 heading "Five Design Dimensions" ↔ Fig 6.5's exactly 5 data rows. MATCH.
- §6.4.3 heading "Four Attempts" ↔ Fig 6.6's exactly 4 data rows. MATCH.
- §6.5.1/§6.5.3 Maryland: \$1.4B, 7%, >6% avoidable, \$800M, 3.58% — Fig 6.3 Maryland row and Fig 6.7 agree with each other and with the prose. MATCH.
- §6.7.1: $\$2.4B \div \$3,700/\text{resident} = 648,600$ and $\$3.1B \div \$4,800 = 645,800$ — both consistent with Vermont's ~647k population, and with each other. MATCH.
- §6.7.3: "The largest single source ... shifting procedures ... to ambulatory surgical and outpatient settings" ↔ Fig 6.10's largest row (\$130M+). MATCH. (Minor caveat: Fig 6.10's column is "Direct & indirect savings" and does not separate the two, so the narrower claim "largest single source of indirect savings" cannot be checked against it.)
- §6.6: "In January 2025 Vermont signed ... **Eighteen months later** the State walked away" (July 2026). MATCH.
- §6.11: AHEAD "operating in **five** states (Maryland, Connecticut, Hawaii, Rhode Island, and New York)" — list length matches the count. MATCH.
- Global budget dates are consistent in all four places they appear (§6.3.3, Fig 6.4 Phase III, Fig 6.6, Fig 6.11): non-CAH FY2028, all-hospital FY2030. MATCH.
- Fig 6.12's "Oliver Wyman's conservative-scenario assumptions (3.5% revenue growth, 5% expense growth)" ↔ Fig 6.8's conservative row. MATCH.
- "14 hospitals" consistent at §6.2.3, the BEYOND VERMONT box, and §6.3.1. MATCH.

Verbatim, with location. Flagged  where it may collide with a claim elsewhere in the book.

1. **Ch6 dek** — "Vermont's mandatory reference-based pricing and global hospital budgets represent **the most direct attempt by any American state** to break the fee-for-service incentive structure..."  collides with §6.5.1's "Maryland is **the only** U.S. state to have operated a mandatory all-payer hospital rate setting system with global budgets for more than a decade" — Maryland has done for twelve years what Vermont has legislated but not started.
2. **§6.1** — "Vermont's reform journey ... is **the most complete real-world laboratory** for these economic questions currently operating in the United States."  same tension; "currently operating" is doing a lot of work when §6.5 says Vermont's global budgets begin FY2028.
3. **§6.1** — "the Economics pillar is **the one that determines whether the other [pillars] actually produce results**" / "Payment reform is **not one element** of healthcare transformation. It is **the precondition on which all other transformation depends.**"  This is the highest-priority cross-chapter check: every pillar chapter is liable to nominate itself as the master variable. Compare against Ch2/Ch4's framing of Policy and Technology, and against the Ch1 dependency matrix and the Preface.
4. **§6.1 heading** — "Why Payment Reform Is **the Master Variable.**"
5. **§6.2.2 / Fig 6.1** — "Vermont's price data ... is **among the most extreme in the country**"; "UVMHC commercial prices 358% of Medicare — **the highest in Vermont.**"
6. **§6.2.3** — "This cross-subsidy is the financial foundation of **every** U.S. hospital that serves a mixed payer population — which is to say, **every hospital.**"
7. **§6.2.2** — "highly concentrated hospital markets — which describe the majority of U.S. hospital markets, and **all of Vermont's.**"
8. **§6.3.2** — "**The Oregon results are the most directly applicable** to Vermont's situation."
9. **§6.3.4** — "the negative 1% commercial rate growth benchmark — **first negative benchmark in Vermont history**"; and "**the largest single-year price reduction in Vermont hospital history**" (\$230.65M, FY2026).  verify against Ch3/Ch7 if either cites a larger GMCB action.
10. **§6.5 / §6.5.1 / §6.5.2** — "Maryland is **the only U.S. state** with mandatory all-payer global budgets since 2014"; "**Vermont's closest comparable**"; "**the only** U.S. state to have operated [one] for more than a decade"; "**the closest available empirical evidence**"; "a unique regulatory authority that **no other state possesses.**" (Five uniqueness claims; see repetition note below.)
11. **§6.5.3** — dual-eligibles are "**the highest-cost, highest-complexity** population segment."
12. **§6.7** — UVM Health Network "**dominates** Vermont's hospital economics"; the UVM share is "**more than 44%**" / "roughly 62%" of the statewide deficit.
13. **§6.7.3** — "**The largest single source of indirect savings** — shifting procedures ... to ambulatory surgical and outpatient settings."
14. **§6.7.3** — "Vermont currently has **only two** ambulatory surgical centers (both in Burlington)."  a hard, checkable, perishable count — verify against Ch11 (Operations).
15. **§6.8.2** — "Vermont's VHCURES all-payer claims database is **the foundational** data infrastructure."  compare Ch4/Ch5, which make foundational claims for the HIE/FHIR layer.
16. **§6.10** — "The global budget methodology is **the most consequential document** Vermont's transformation will produce."  Ch16 (AHS Restructuring Roadmap) and Ch2 are likely to nominate a different document.
17. **§6.10** — "Vermont's RBP target of 200% of Medicare is **the most specific mandatory all-payer price target enacted by any state**" (see MISMATCH 3 — the premise is wrong) ... "will be **the most informative natural experiment** in all-payer pricing since Maryland's program began."
18. **§6.2.1** — "**fifteen years** of voluntary value-based care programs have moved the needle ... without fundamentally changing the volume-driven behavior of the hospital sector."  a countable duration — check Ch1/Ch3 for a different vintage (ACA 2010 → 2026 is sixteen).

Other issues

Genuine repetition (within Chapter 6 only)

1. **The Maryland regulator quote is used twice.** §6.2.1 sets it as a display quote — "The concept of the global budget is the fundamental piece that flipped the incentives for hospitals in the right direction..." — and §6.4.1 paraphrases the same quote as if new: "As one Maryland state regulator described it: the global budget is the fundamental piece that flips the incentives for hospitals..." *Fix:* cut the §6.4.1 paraphrase and cross-reference the §6.2.1 quote.
2. **The "global budgets without community investment don't improve population health" lesson appears three times**, each time as a fresh finding: §6.5.1 stat strip ("Global budgets without primary care investment reduce volume without improving population health"), §6.5.4 (two full paragraphs developing it), and §6.8.2 Care Delivery Capability ("The Maryland experience demonstrated that global budgets without community care infrastructure simply reduce hospital volume without improving population health"). §6.5.4 is the right home; §6.8.2 should cross-reference rather than restate, and the strip line is redundant with the section it heads.
3. **Maryland's uniqueness is asserted five times in one section** (§6.5 intro, §6.5.1 strip, §6.5.1 prose, §6.5.2). §6.5.1's opening line and the paragraph immediately below it make the same claim in nearly identical words ("The only U.S. state with mandatory all-payer global budgets since 2014" / "Maryland is the only U.S. state to have operated a mandatory all-payer hospital rate setting system with global budgets for more than a decade"), and both also restate §6.5's framing.
4. **"Payment reform is the precondition on which all other transformation depends" is verbatim in two places** — the chapter dek and the last sentence of §6.1's first paragraph. Dek/body echo is a legitimate device, but here it is word-for-word within two blocks of itself.
5. **The "revenue is fixed, so cost reduction is the only lever" derivation runs three times** — §6.4 intro, §6.4.1 ("Under fee-for-service, revenue = price × volume..."), and §6.10's hospital executive paragraph. The §6.4.1 statement is the analytic one; the other two restate it.
6. *Legitimate recurrence, deliberately left:* the §6.11 Key Concepts entries for *Cross-subsidy*, *Revenue predictability*, *PMPM*, *EAST Fund* and *Potentially avoidable utilization* all restate body prose — that is what a glossary is for. Likewise Fig 6.3's Maryland row

overlapping Fig 6.7 (a cross-state summary row vs. the dedicated results figure) and the repeated Oliver Wyman / GMCB source attributions in figure captions.

Thin or structurally odd sections

- §6.6 (The AHEAD Model) is a single 250-word paragraph with no subsections, no figure and no table — the only Heading2 in the chapter built that way, and it carries the chapter's most consequential recent fact (the withdrawal). It reads as a late insert, which is consistent with MISMATCH 6: the surrounding sections were not updated to match it. Recommend splitting into "What Vermont signed" / "Why it collapsed" / "Why the reform survives it", and giving it the numbers table the rest of the chapter would have.
- §6.5.1 opens with four unlabelled loose paragraphs acting as a stat strip (lines "The only U.S. state...", "\$1.4 billion...", "7% reduction...", "Global budgets without primary care investment..."). Every other stat strip in the chapter is a table inside a numbered figure (Fig 6.1, Fig 6.7). These four are neither, so they are unnumbered, uncaptioned and unsourced — and three of the four duplicate Fig 6.7 directly below them.
- §6.9 (Work This Chapter on the Platform) has only 2 rows against 12 figures' worth of modelable content — notably nothing for the Maryland evidence base or the premium trajectory. Thin relative to the equivalent section in other chapters, but no factual defect.
- Orphan facts with no figure and no second mention: "Statewide Strategic Plan delivered December 2028" (Fig 6.11 only); "\$1,303 per-discharge administrative gap" (§6.10 only); "65+ from 21.7% in 2020 toward 30%+ by 2040 / working-age 58.8% toward 52%" (§6.2.3 only — the only demographic claims in the chapter, and the load-bearing premise of §6.2.3, yet not given a figure while far less structural numbers get one).

Heading-text conformance (CLAUDE.md "recurring section headings")

6.9 Work This Chapter on the Platform, 6.10 Implications for You, 6.11 Key Concepts in This Chapter are all present with the canonical wording, and Sources: ... is a plain paragraph, not a heading. Conformant.

Not checked in this pass

Criterion 5 (platform promise delivery) and Preface/Introduction alignment were out of scope per the brief. Fig 6.12's three tool routes, the "Value-Based Care Transformation Readiness Assessment (30 dimensions across six domains)" named in §6.8.2, and the "Vermont's All-Payer TCOC preset" named in Fig 6.12 are all deliverable-promise claims that this audit did **not** verify against frontend/app/ or Supabase. A 200 is not a delivered promise.

===== FILE: audit_ch07.md =====

Chapter 7 Audit

Source: HTR_Book_v42.docx → word/document.xml, real Heading1 boundaries at byte offsets 2,462,787 ("Chapter 7: The Economics Pillar in Practice — VBC Financial Modeling and APM Readiness") through 2,665,862 ("Chapter 8: The Clinical Pillar..."). The TOC decoys at 136,378 / 153,710 were ignored. 10 tables and 84 paragraphs parsed as an order-preserving stream; extracted text with <w:t(?:\s[^\>]*)?>. Read-only — the .docx was not modified.

Figures found (ground truth)

Table #	Shape	Caption attached	Contents (ground truth)
T1	1 row, 1 cell	none (callout)	"VERMONT AHEAD CONTEXT" — CMMI sets global budget baselines from historical Medicare FFS spending; VT's lower-spending hospitals may face limiting benchmarks
T2	1 row, 1 cell	none (callout)	"VERMONT RBP CONTEXT" — GMCBB RBP uses Medicare rates as external benchmark
T3	1 row, 1 cell	none (callout)	"VERMONT GLOBAL BUDGET CONTEXT" — Act 68 global budgets for non-CAH hospitals; risk-adjustment methodology under development
T4	6 rows (header + 5 steps)	Figure 7.1 — APM shared savings calculation framework	Steps: 1 Benchmark setting; 2 Performance measurement; 3 Savings calculation; 4 Minimum savings rate (2–3.5%); 5 Shared savings calculation (50–75%)

Table #	Shape	Caption attached	Contents (ground truth)
			one-sided, "may reach 80%" full-risk)
T5	1 row, 4 cells	none (stat strip)	\$15,000 avg inpatient cost prevented · \$1,800 avg preventable ED visit · 3–5x care management ROI range · 32.3% VT potentially avoidable ED visits
T6	7 rows (header + 6 investments)	Figure 7.2 — Economics pillar implementation matrix	HCC gap program \$25K–100K; APM financial prep \$75K–200K; global budget modeling \$50K–150K ("all 14 Vermont hospitals modeled"); VBC readiness assessment \$30K–75K; commercial VBC negotiation \$75K–200K; state RBP \$500K–2M (Oliver Wyman \$300M+ /yr)
T7	7 rows (header + 6 categories)	Figure 7.3 — APM contract review framework	Benchmark methodology; Attribution methodology; Quality withhold; Risk corridors and stop-loss; Carve-outs; Reconciliation timing
T8	9 rows (header + 8 metric pairs)	Figure 7.4 — Financial metrics transformation	FFS metric ↔ global-budget metric pairs
T9	6 rows (header + 5 investment types)	Figure 7.5 — Transformation investment ROI framework	Care management; BH integration (CoCM); SDOH; Primary care (5.8:1); Telehealth/RPM
T10	6 rows (header + 5 tools)	Figure 7.6 — Hands-on platform tools	APM Shared Savings Calculator; VBC Readiness Assessment; CEA Calculator; Hospital Financial Stress Test; Investment Tracker

Figure numbering 7.1–7.6 is sequential with no gaps and no duplicate numbers. Four tables (T1–T3, T5) are deliberately uncaptioned callouts/stat strips — correct, not defects.

Claims checked against their own source (MATCH/MISMATCH + proposed fix)

Internal arithmetic

- §7.2.7 care-management ROI vs its own numbers. "50 hospitalizations ... at \$15,000 ... generates \$750,000 in avoided cost. If ... costs \$200,000 to operate, the financial return is approximately 3.75:1 — a 275% ROI." → **MATCH** (gross 750/200 = 3.75:1; net (750–200)/200 = 275%). The two figures measure different things but both are correct, and 3.75x sits inside T5's "3–5x" range. No change needed; if the author wants precision, "3.75:1 gross return — a 275% net ROI".
- T9 care-management row. 500 ED visits × \$1,800 = \$900K; CHT \$300K → ROI 200%. → **MATCH** (net 600/300 = 200%), and consistent with §7.2.7's net-ROI convention.
- T9 telehealth row. 200 patients × 30% readmission rate = 60; 10% prevented = 6; 6 × \$15,000 = \$90,000. → **MATCH**.
- T9 primary-care row. \$5.8M per \$1M → \$3M yields \$17.4M. → **MATCH**, and consistent with §7.8's "5.8:1 ROI".
- T9 CoCM row — **MISMATCH (implausible implied figure)**. "Every MH-related hospitalization prevented at \$15,000 average saves 50x the cost of a CoCM BHCM monthly salary cost." \$15,000 ÷ 50 implies a behavioral-health care-manager monthly salary of **\$300**. *Proposed fix*: recast against an annual cost, e.g. "saves roughly two months of a CoCM behavioural-health care manager's fully loaded cost", or state the assumed salary.
- §7.4.2 scoring — **MISMATCH against the chapter's own scale**. The chapter states each of the 30 dimensions is "scored 1-4" with a 120-point maximum. With 6 domains × 5 dimensions, the **minimum possible domain score is 5**, so "Most Vermont hospitals score 3-7 in Domain 2" is arithmetically impossible under the stated scale. *Proposed fix*: "score 5-9 in Domain 2" (or state the domain result as a percentage, which is what the tool actually reports — see #10).
- §7.4.2 vs §7.11 Key Concepts — **MISMATCH of interpretation**. §7.4.2: "below 60/120 indicates the organization is **not ready for full-risk VBC**." Key Concepts: "Total score below 60/120 indicates **pre-transition status**." 60/120 is an average of 2 = *Developing* on the chapter's own 1–4 labels, not *Pre-transition* (=1). *Proposed fix*: make Key Concepts read "indicates the organization is not ready for full-risk VBC".

Platform promises (criterion 5 — every route opened, not inferred)

- Routes exist. </research-lab/payment-models?tab=apm-calc>, </research-lab/knowledge-workspace?tab=readiness>, </research-lab/policy-quality?tab=scorecard>, </investment-tracker> — all five tab ids are in the

respective clients' `VALID_TABS/TABS` arrays, and `?tab=scorecard` under *policy-quality* is genuinely labelled "Hospital Financial Stress Test" (`app/research-lab/policy-quality/PolicyQualityClient.tsx:40`). → **MATCH**. Note `knowledge-workspace` also has a `scorecard` tab ("Transformation Scorecard") — the book cites the right one, but the collision is a trap for future edits.

9. **T6 "all 14 Vermont hospitals modeled" → MATCH:**
`components/research/HospitalFinancialScorecard.tsx` carries exactly 14 hospital presets (Brattleboro, CVMC, Copley, Grace Cottage, Gifford, Mt. Ascutney, North Country, NVRH, NMC, Porter, Rutland, Springfield, SVMC, UVMC).
10. **§7.4.1 six readiness domains — MISMATCH, the significant one.** The book names *Strategic Clarity · Data and Technology · Care Delivery Capability · Network and Partnerships · Revenue Cycle · Workforce Operations*. The live tool (`components/research/VBCReadinessAssessment.tsx`) has 30 dimensions across *Strategy & Leadership · Data & Analytics · Clinical Operations · Financial Readiness · Technology Infrastructure · Health Equity*. Four of six domain names differ in substance: the tool has no *Network and Partnerships* and no *Workforce Operations*; the book has no *Health Equity* domain at all, and splits *Technology* into "Data and Technology" while the tool keeps them separate. A reader following §7.4.1 to the tool finds a different framework. The count (30/6) is the only thing that lines up. *Proposed fix* (per the standing "extend the tool, never reword the book down" rule): this one cuts both ways — the tool's Health Equity domain is required by the book's own Equity Imperative, so the book's domain list should be reconciled to the tool's six, with *Network/Partnerships* and *Workforce Operations* folded into *Clinical Operations* and *Strategy* dimensions, or the tool extended to 8 domains. Needs the author's call; flagged, not fixed.
11. **§7.4.2 scoring vs the tool — MISMATCH.** Book: "scored 1-4: Pre-transition (1), Developing (2), Operational (3), Optimized (4)", total "/120". Tool: scores **0–4** with labels *Not Started / Early Stage / In Progress / Advanced / Optimized*, reports a **percentage** (bands at 20/40/60/80% → "Not Ready", "Early Stage", "In Progress", "Advanced — 12–18 Months", "Global Budget Ready"), and has no 120-point total and no "Pre-transition" label anywhere. *Proposed fix*: restate the book's paragraph in the tool's percentage bands (and note that 60/120 = the tool's 50%, which lands in "In Progress — 2–3 Years to Readiness").
12. **T10 row 1 — MISMATCH (undelivered promise).** Promises modelling "(benchmark PMPM, sharing rate, MSR, stop-loss)" and looking for a "Pessimistic/base/optimistic spread".
`components/research/APMCalculator.tsx` has benchmark PMPM, sharing rate, MSR, loss share, quality withhold and a `capGainPct` savings cap — but **no stop-loss / downside-cap input and no three-scenario (pessimistic/base/optimistic) output**. A reader is told to look for something the tool never renders. *Proposed fix*: either add a stop-loss threshold and a 3-scenario spread to the calculator (preferred, per the standing rule), or change the cell to "(benchmark PMPM, sharing rate, MSR, loss share, quality withhold)" and "the gap between gross and net shared savings once the withhold and cap bite".
13. **§7.6.2 stress-test scenarios — MISMATCH.** The book lists four scenarios the CFO can run: RBP at **200% of Medicare (Oliver Wyman)** or **250% (GMCB phased)**; global budget revenue cap; **H.R. 1 Medicaid cuts phasing in 2027–2031**; and a **transformation investment** scenario. The tool's actual presets are "Act 68 RBP Scenario (FY2027)" at **Medicare +15%**, "Act 68 Global Budget (FY2028–30)", and "H.R. 1 Medicaid Cliff (**Post-2030**)" — three, not four; no 200%/250% benchmark toggle; no transformation- investment scenario; and the H.R. 1 timing label disagrees with the book. The book also asks "What service lines have prices furthest above the benchmark?" — the tool's inputs are organisation-level financials (net revenue, operating expense, cash, debt service, labour) with **no service-line dimension at all**. *Proposed fix*: drop the service-line question or add service-line inputs; align the RBP benchmark language with the tool's Medicare-plus-percentage control; add the transformation-investment scenario or remove it from the book's list; reconcile "2027–2031" with the preset's "Post-2030".
14. **§7.6.2 subscriber gating claim — UNVERIFIED.** "available in the Research Lab for Professional and Advisory subscribers". Not checked against the entitlement logic in this pass; flagging so it is not taken as verified.
15. **§7.5.1 "65-item VBC Contract Review Checklist (available in the Implementation Toolkit)" — MISMATCH (undelivered promise).** No artefact of that name exists. `app/connect/toolkits/page.tsx` offers a "Value-Based Care Contract Template Library" (annotated contract language: shared savings, shared risk, capitation, episodes, risk corridors, quality withholds, dispute resolution) — related but not a 65-item checklist, and the "65-item contract review checklist" phrase currently survives only as marketing copy in `components/FromTheBookForPillar.tsx:35`. *Proposed fix*: build the 65-item checklist as a toolkit download, or cite the Contract Template Library by its real name and drop the item count.
16. **T4 step 5 "may reach 80% for full-risk models" vs the tool.** `APMCalculator` models ACO REACH (Global Risk) and BPCI-Advanced at `sharingRate: 1.0` (100%). → **MISMATCH, minor.** The book understates the ceiling its own calculator implements. *Proposed fix*: "and reaches 100% in global-risk models such as ACO REACH". Key Concepts ("may reach 80% for full-risk models") needs the same edit — the two are internally consistent with each other, just both below the tool.
17. **T4 step 5 "100% of premium passthrough per GMCB monitoring requirement" vs §7.10 legislator paragraph** ("GMCB ... does not have the same authority over commercial insurer behavior ... the legislature should assess whether Vermont needs additional regulatory tools to ensure ... savings flow through to premium reduction"). → **Tension, not a contradiction**: the table states the statutory requirement, the prose questions its enforceability. Deliberately left, but the table cell reads as settled fact where the prose treats it as an open gap; consider "100% premium passthrough is required under Act 68 — see §7.10 on enforceability".

Terminology and internal cross-references

18. **§7.4 "Value-Based Care Transformation Readiness Assessment" / §7.11 "VBC Transformation Readiness Assessment" / T6 + T10 "VBC Readiness Assessment" / tool label "VBC Readiness Assessment".** → three names for one artefact. *Proposed fix*: use the tool's exact label "VBC Readiness Assessment" everywhere, per the recurring exact-title rule.
19. **§7.2.5 "The APM Shared Savings Calculator implements this model with organization-specific data inputs."** → **MATCH** — the calculator takes attributed lives, benchmark PMPM, MSR, sharing rate, loss share and quality withhold, with Vermont presets (AHEAD Global Budget FY2028, Blueprint ACO, Medicaid ACO, small rural CAH).
20. **T4 MSR 2–3.5% and sharing 50–75% vs the tool's MSSP Track 1 (MSR 2%, 50%) and MSSP Enhanced (75%).** → **MATCH**.

21. §7.6 "Act 68's mandatory global budgets take effect for non-CAH hospitals beginning FY2028" vs T3 (non-CAH, Act 68) and the Chapter 5 passage at offset 845,691 ("beginning in FY2028"). → **MATCH**, internally and across that neighbouring reference.
22. §7.10 "setting fair benchmarks for 14 hospitals" vs T6's 14. → **MATCH**.

Superlative/ranking claims logged for cross-chapter comparison

Verbatim, with context, for later cross-chapter deduplication:

- §7.2.1 — "Getting the benchmark right — or negotiating favorable benchmark methodology — is **often the most important financial decision in VBC contract entry.**"
- §7.11 (Benchmark) — "Benchmark methodology — historical vs. regional, trend adjustment, risk adjustment — is **the most consequential financial design choice in APM contract entry.**" *(Same subject as #1, stated without the "often" hedge. Legitimate glossary recurrence, but pick one strength of claim.)*
- §7.2.2 — "**The most common approach** — the benchmark is set at the organization's own historical cost of care."
- §7.4.1 Domain 2 — "This domain is **Vermont's most significant organizational gap** — most Vermont community hospitals lack the internal analytics capability to use VHCURES data effectively."
- §7.4.2 — "**Domain 2 is the binding constraint** for most Vermont providers." *(Consistent with #4; the two together are a candidate collision with any other chapter that names a different "most significant gap" or "binding constraint" — this is the classic cross-chapter "most underestimated dependency" failure mode.)*
- §7.4.1 Domain 3 — Vermont's Blueprint PCMHs and CHTs "provide a **strong care delivery foundation**"; §7.4.2 — Blueprint PCMH practices score "**strong by national standards**".
- §7.6 — "For Vermont hospitals, **the most operationally relevant financial modeling challenge** is not APM shared savings calculation — it is global budget management."
- §7.7 — "Under value-based care and global budgets, the financial case for transformation investment is **considerably stronger.**"
- §7.8 — "**primary care is the highest-return investment a healthcare system can make**: the same \$1 invested in primary care produces greater total cost of care reduction than \$1 invested in hospital care or specialty care." *(Strongest superlative in the chapter, and the one most likely to collide with a Technology- or Clinical-pillar chapter making the same claim for its own pillar. Note it is asserted generally but evidenced only by the Vermont Blueprint 5.8:1 figure.)*
- §7.2.7 — "the financial value of identifying a high-risk patient before a health crisis occurs is **enormous**"; "The math is **compelling.**"
- §7.10 — "**no amount of enforcement capability** applied to wrong benchmarks produces the right transformation outcomes."
- §7.8 — a virtuous cycle "that **fee-for-service can never produce** but global budgets require."

Other issues

A. §7.2.6 is an empty heading. "7.2.6 Risk Stratification — The Economic Engine of Population Health Management" is immediately followed by "7.2.7 The Care Management ROI Calculation" with **zero body paragraphs between them**. The section that the chapter's own framing makes load-bearing (risk stratification as the economic engine) has no content. This is the largest structural defect in the chapter.

B. AHEAD tense contradiction — the chapter's real internal contradiction. §7.8 states "**Vermont's withdrawal means it will not be the vehicle that applies it here**" (consistent with Chapter 6's July 2026 AHEAD withdrawal, per [components/FromTheBookForPillar.tsx:35](#)). But the rest of the chapter treats AHEAD as live and operative in the present and future tense:

- T1 — "CMMI **sets** global budget baselines for hospitals..."
- T4 step 2 — "AHEAD **tracks** total cost of care for Medicare FFS beneficiaries attributed to Vermont hospitals across all payer settings."
- T4 step 4 — "AHEAD **uses** a different structure — state-level TCOC targets..."
- T6 — "AHEAD benchmark methodology setup"; "AHEAD preparation baseline"
- §7.4.1 Domain 3 — "the more complex patients that **AHEAD's population health requirements target**"
- §7.10 hospital executive — "negotiate quality measurement aligned with HEDIS and **AHEAD metrics**"
- §7.10 AHS/GMCB — "**the AHEAD actuarial analysis** and the global budget benchmark development requires real actuarial expertise... **If Vermont's AHEAD benchmark methodology is set** by people without this expertise, the benchmarks **will be set** wrong"

A reader is told in §7.8 that AHEAD is not Vermont's vehicle and told in §7.10 that getting AHEAD's benchmark methodology right is urgent. *Proposed fix:* one sweep converting every AHEAD reference in the chapter to either (a) past/counterfactual ("AHEAD would have tracked...", "the AHEAD design remains the reference methodology even though Vermont withdrew") or (b) Act 68 global-budget equivalents, which is what actually governs post-withdrawal. This is a chapter-wide edit, not a one-line fix. Note the tool presets are labelled "Vermont AHEAD Global Budget (FY2028)" and "Act 68 RBP Scenario (FY2027)" — the tooling has the same residue.

C. Genuine repetition — §7.2.7 pull-quote duplicates the body. Three short paragraphs ("A program preventing 50 hospitalizations in 10,000 Medicare beneficiaries at \$15,000 each generates \$750,000 in avoided cost." / "If the program costs \$200,000 to operate, the ROI is 275%." / "Under fee-for-service, the prevented hospitalization is lost revenue — which is why FFS systematically discourages prevention.") restate, almost verbatim, the two full paragraphs immediately following them. If these are styled as a pull-quote/callout this is legitimate typographic recurrence; if they are running body text it is straight duplication. The XML shows them as ordinary paragraphs in the run with no distinguishing shading. **Needs a render check before fixing** — I did not edit, so no render was required.

D. Repetition — Domain 2 analytics gap stated twice. §7.4.1 Domain 2 ("most Vermont community hospitals lack the internal analytics capability to use VHCURES data effectively") and §7.4.2 ("Most Vermont hospitals score 3-7 in Domain 2... data

infrastructure is the prerequisite... Domain 2 is the binding constraint"). Adjacent sections, same finding, three restatements. Defensible as scoring-example-plus-narrative, but tighten.

E. Thin cross-reference. §7.4.1 Domain 5: "This domain is covered in depth **elsewhere in this book**." No chapter or section named. *Proposed fix*: name the chapter, or cut the sentence.

F. §7.5.1 category count. Text says 65 provisions in **eight** categories; T7 shows **six**, prefaced "The most financially consequential categories are:". Internally honest, but the reader cannot reach the other two (see #15 — the checklist does not exist). Not a numerical error.

G. Figure 7.6 has no source line, unlike Figures 7.1–7.5 which all carry "Source(s): ...". Minor consistency defect in the caption series.

H. §7.2.1 orphan table title. "Benchmark Construction Methods — What Healthcare Leaders Must Understand" sits as a standalone paragraph introducing no table — the three methods follow as H3 subsections (§7.2.2–7.2.4) each with a one-cell Vermont context callout. Reads as a caption whose table was dissolved into prose. Structural, not factual.

I. Figures not verifiable within this chapter (single occurrence each, no internal cross-check possible; logged for a fact-check pass, not flagged as defects): 32.3% VT potentially avoidable ED visits (T5, T9 — consistent with each other); \$1,800 avg preventable ED visit; \$15,000 avg inpatient cost; 245 suicide/self-harm ED visits per 10,000; 9% of Vermonters food insecure; Blueprint 5.8:1 ROI; Oliver Wyman \$300M+ annual RBP projection; Vermont CAHs avg 12% HCC gap (2025 RHRC assessment); "80% of administrative entities increased CHT staffing" (Blueprint MHI pilot); literature 5–15% TCOC reduction for high-SDOH-burden populations.

Not run: `check_format.py` / `render_check.py` — no edit was made to the `.docx`, so neither gate applies. `audit_chapter.py` 7 was not run for this pass; this audit is the manual criterion-2/3/5 half.

===== FILE: audit_ch08.md =====

Chapter 8 Audit

Read-only audit of Chapter 8 ("The Clinical Pillar — Redesigning Care Delivery for a Transformed System") extracted directly from `HTR_Book_v42.docx` → `word/document.xml` via zipfile, body-child order preserved (tables kept as cell grids, not the lossy `.md` mirror). Chapter 8 = body children 1184–1295 (Heading1 "Chapter 8:" → Heading1 "Chapter 9:"). No edits were made to the docx.

Paragraph indices below are body-child indices in the fresh file as of 2026-09-27; re-locate by text before any edit.

Figures found (ground truth)

Seven numbered figures, sequential 8.1–8.7, no gaps or duplicates. Two unnumbered callout tables (VERMONT IN PRACTICE, BEYOND VERMONT) correctly carry no figure number.

Fig	Table at	Shape	Actual cell contents (ground truth)
(stat band, unnumbered)	1193	1 row × 4	5.8:1 Blueprint ROI on Investment · 15+ Yrs Operational Since 2006 · All-Payer Medicare + Medicaid + Commercial · 91% VT Adults with Primary Care Provider
8.1	1198	4 rows × 4 (header + 3)	Header: Component / What it is / How it is funded / What it does. Rows: PCMH (NCQA-recognized practices; all-payer PMPM base + performance payment); CHT (social workers, nurses, CHWs, BH professionals; DVHA Blueprint funding + OneCare "now transitioning" + Medicaid + grants); Health Information Technology (VITL HIE → clinical registry

Fig	Table at	Shape	Actual cell contents (ground truth)
			+ VHCURES; enables population HEDIS, care-gap reports, registries)
8.2	1204	2 rows × 2, no header row	Blueprint ROI ... per \$1M invested – \$5.8M · VT residents with a personal health care provider – 91% vs. 87% national benchmark – 4 points above (AHS Nov 2025); Blueprint PCMH hypertension controlled – 77% · Blueprint PCMH diabetes HbA1c NOT in control – 22%
8.3	1232	4 rows × 4 (header + 3)	Header: CoCM component / Role / What they do / Vermont application. Rows: PCP (screens PHQ-9, GAD-7; bills CoCM codes monthly); BHCM (registry, brief intervention, rating scales, escalation) — Vermont cell reads "MHI pilot found 80% of administrative entities increased CHT staffing"; Psychiatric Consultant (case review, not primary provider)
8.4	1246	6 rows × 3 (header + 5)	Header: Component / What it means in practice / Vermont investment vehicle. Five components: Team-based care; Population health management; SDOH screening and navigation; Behavioral health integration; Care coordination for complex patients
8.5	1255	6 rows × 4 (header + 5)	Domains: Preventive/chronic (HTN 77% controlled; diabetes 22% not controlled); Behavioral health (76% MH ED follow-up; 68% SUD ED follow-up); Primary care access (59% practices accepting new Medicaid; 91% with personal health provider); Hospital quality (readmission 14.8%; avoidable ED 32.3%; admin cost 91% above benchmark); Equity/SDOH (NEK uninsured 6–8% vs 3% statewide)
8.6	1273	6 rows × 4 (header + 5)	Commitments: Universal Blueprint PCMH coverage (2025–2028); BH integration at every PCMH (2025–2028); CCBHC network covering all HSAs (2026, 5 new CCBHCs, through 2028); PACE program development (2026–2028); Dementia care infrastructure (memory care

Fig	Table at	Shape	Actual cell contents (ground truth)
			COE at 3–5 facilities , 2026–2030)
8.7	1277	4 rows × 3 (header + 3)	Do this / On this tool / What to look for. Tools: Risk Stratification Methodology /research-lab/vbc-clinical-quality?tab=risk; VBC Quality Measures /research-lab/vbc-clinical-quality?tab=quality; Risk Stratification Engine /research-lab/interoperability?tab=risk

No prose sentence anywhere in Chapter 8 cites a figure by number. Figures are captioned but never cross-referenced ("as Figure 8.5 shows" appears zero times). So the Chapter-1 class of defect — prose citing a figure whose cells say otherwise — cannot occur by explicit citation here; it occurs instead by *silent* restatement of the same numbers in prose, which is what the checks below test.

Figure 8.7 is the only figure with no Sources : line. All six others have one.

Claims checked against their own source (MATCH/MISMATCH + proposed fix)

MISMATCH 1 — The ROI study is dated three different ways, and one is wrong

- [1202]: "a six-year longitudinal study using VHCURES ... published in *Population Health Management*"
- [1249]: "**The 2013 evaluation finding** — lower hospitalization rates and reduced outpatient facility use for Blueprint participants"
- [1283]: "The 5.8:1 ROI figure is **from a 2016 peer-reviewed study of 2008–2013 data**"
- Sources [1294]: "Population Health Management (Blueprint ROI study, **2016**)"

[1249] calls it "the 2013 evaluation" while [1283] and the Sources block both date the study to 2016 (2013 is the end of the data window, not the publication). Same study, same finding, wrong year. **Fix:** [1249] → "The 2016 evaluation finding" (or "the 2016 evaluation of 2008–2013 data").

MISMATCH 2 — The same 68% is attributed to two different denominators

- [1217] stat block: "76% 30-day follow-up rate after mental health ED visits. **68% after SUD ED visits.**"
- [1255] Figure 8.5: "76% 30-day follow-up after MH ED visit; **68% after SUD ED visit**"
- [1239]: "The 30-day follow-up rate after ED visits **for alcohol use** — 68% in the November 2025 data"

Two places call 68% the all-SUD follow-up rate; [1239] calls the identical number the alcohol-specific rate. One of the two is wrong, and [1239] then builds the whole SUD-continuum argument on it. **Fix:** decide from the AHS November 2025 report which measure the 68% is (the AHS measure is normally FUA, alcohol/other-drug abuse or dependence) and make [1217], [1239] and Figure 8.5 use one identical label.

MISMATCH 3 — "the other five pillars" (the framework has five pillars total)

- [1186]: "The Clinical pillar is where the abstractions of **the other five pillars** become a patient's actual experience of care"

The framework is five pillars (Policy, Technology, Economics, Clinical, Operations), of which Clinical is one — so there are four others. **Fix:** "the other four pillars." (If Equity is being counted as a sixth, that conflicts with the book's own five-pillars-plus-Equity-as-imperative structure and with Ch10's framing; "the other four pillars and the Equity imperative" would be the safe form.)

MISMATCH 4 — Five new CCBHCs cannot cover fourteen HSAs

- [1218] stat block: "Vermont's CCBHC expansion target: 5 new certified entities by July 2026, **covering every Hospital Service Area.**"
- [1227]: "As of 2025, Vermont has **two** active CCBHC demonstration sites ... plans to certify **five additional** ... — Springfield, Burlington, St. Albans, Newport, Barre."
- [1191]: Blueprint is "embedded in primary care practices across **all 14 Hospital Service Areas.**"
- [1273] Figure 8.6: "CCBHC network covering all HSAs ... **2026 (5 new CCBHCs) through 2028**"

Two active + five new = seven entities in seven named towns. Fourteen HSAs cannot be covered by seven entities in 2026. Figure 8.6 is the accurate version (all-HSA coverage is a *through-2028* goal, of which the five 2026 certifications are the first tranche); the [1218] stat block overstates it as achieved by July 2026. **Fix:** [1218] → "5 new certified entities by July 2026, the first tranche toward coverage of every Hospital Service Area." This is the highest-value fix in the chapter: it is the one mismatch where a stat card contradicts a figure's own timeline cell.

MISMATCH 5 — "No HPSAs in Vermont" contradicts the chapter's own shortage claims

- [1241]: "Vermont does not have a primary care physician shortage in the traditional sense. **HRSA recognizes no Health Profession Shortage Areas in Vermont.**"
- [1232] Figure 8.3: "**Vermont's psychiatric shortage** makes the CoCM model especially valuable"
- [1266]: "home and community-based services ... are **limited by workforce shortages**"
- [1269]: "Vermont **currently lacks** dementia-specialized primary care at scale"

A psychiatric shortage *is* a HRSA Health Professional Shortage Area designation (mental-health HPSA), so the absolute form of [1241] contradicts Figure 8.3 one page later. **Fix:** narrow [1241] to what Oliver Wyman actually argued — "HRSA recognizes no *primary care* Health Professional Shortage Areas in Vermont" — and verify even that against HRSA's current designation file before it ships, because the unqualified claim is the kind that is easy to check and embarrassing to get wrong. Flagged as verify-externally, not fixed here.

MISMATCH 6 — "fifteen-year record" vs "two decades"

- [1186]: "its **fifteen-year** record offers something most states do not have"
- [1191]: "for **nearly two decades**"
- 8.2 heading [1192]: "**Two Decades** of Primary Care Transformation"
- [1193] stat card: "**15+ Yrs** Operational Since 2006"
- [1201]: "The evidence base spans **more than a decade** of operation"

Established 2006, chapter's present is late 2025/2026 → twenty years. "Fifteen-year record" in the chapter's third paragraph is wrong and contradicts the heading two paragraphs later. **Fix:** [1186] → "its twenty-year record"; [1193] card → "20 Yrs"; [1201] "more than a decade" → "nearly two decades" (or leave — it is true but gratuitously weak next to the heading).

MISMATCH 7 — 245.5 vs 245

- [1187] and [1216] and [1220]: "**245.5** ED visits per 10,000"
- [1190]: "a state with **245** ED visits per 10,000"

Fix: [1190] → 245.5. Trivial, but it is the same number three lines apart.

MISMATCH 8 — "nearly 80%" vs "80%"

- [1225]: "with **nearly 80%** of administrative entities reporting increased CHT staffing"
- [1232] Figure 8.3 BHCM row: "MHI pilot found **80%** of administrative entities increased CHT staffing"

Fix: pick one against the 2025 MHI evaluation and use it in both.

MISMATCH 9 — Glossary overstates what the MHI evaluation found

- [1225]: "The 2025 evaluation found **improved access and engagement but sustainability concerns** when temporary funding ends"
- [1290] glossary: "evaluated in 2025 as **highly effective** but facing sustainability challenges"

"Improved access and engagement" is not "highly effective." The glossary is the louder claim and has no source behind it. **Fix:** [1290] → "evaluated in 2025 as improving access and engagement, but facing sustainability challenges when pilot funding ends."

MATCH — checks that hold

- **5.8:1 ROI / \$5.8M per \$1M.** Consistent in [1193] card, [1202] prose, [1204] Figure 8.2, [1280], [1282], [1283]. Direction and units correct everywhere.
- **91% with a primary care provider.** [1193] card, [1204] Figure 8.2 ("91% vs. 87% national — 4 points above"), [1255] Figure 8.5. Internally consistent; 91 – 87 = 4 ✓.
- **Hypertension 77% controlled / diabetes 22% not in control.** [1204] and [1255] agree, including the awkward but consistent NOT-in-control polarity of the diabetes measure.
- **Avoidable-ED arithmetic [1250].** 32.3% → 25% is 7.3 pts; 7.3% × 200,000 = 14,600 ✓ ("approximately 14,600"). 14,600 × \$800–1,200 = \$11.7M–\$17.5M ✓ ("\$12–18 million"). The 32.3% matches Figure 8.5's hospital-quality cell. This calculation is clean.
- **14 hospitals / 14 HSAs.** [1191] "all 14 Hospital Service Areas" and [1250] "approximately 14 hospitals" are consistent with Appendix C's 14-hospital COE framework.
- **CoCM CPT codes 99492, 99493, 99494.** Identical in [1219] and [1231]; [1288] refers to them without numbers. No drift.
- **Blueprint founded 2006.** [1195], [1193] card, [1285] glossary agree. [1207]'s "operated since 2010 under a CMS Multi-Payer Advanced Primary Care Practice Demonstration" is about the *Medicare* participation vehicle, not the program's start, and does not contradict 2006 — though it reads as if it might, and a two-word clarification ("has operated since 2010 *with Medicare participation* under...") would remove the ambiguity.
- **Figure 8.6's "3–5 facilities" memory care COE** is the only place that number appears; no conflicting count elsewhere in the chapter.
- **Platform links in Figure 8.7 all resolve, including the tab params** (mechanical half of criterion 5, verified against the route files, not inferred):
`frontend/app/research-lab/vbc-clinical-quality/VBCClinicalQualityClient.tsx` defines `id: 'quality'` (line 36) and `id: 'risk'` (line 50);
`frontend/app/research-lab/interoperability/InteroperabilityClient.tsx` defines `id: 'risk'` (line 33, "Risk Stratification Engine"). The two tool *names* in the book match the tabs' own labels. **Not verified here:** whether each tool actually delivers the promised behaviour ("how many patients change tier once social risk is included alongside clinical risk" requires a social-risk input to exist in that engine), and whether those tools tag Chapter 8. A 200 is not a delivered promise — that half is outside this read-only pass and should be run through `audit_chapter.py` 8 plus a real page open.

Superlative/ranking claims logged for cross-chapter comparison

Verbatim, with location. The Blueprint cluster is the one to reconcile across chapters — the chapter makes three *different* "most X in the country" claims about the same program within six paragraphs.

Blueprint (three competing superlatives, plus repeats):

- 1. [1187] "The Vermont Blueprint for Health **is the most successful sustained primary care transformation program in American health policy.**" — flat, unhedged, unsourced, in a malformed paragraph (see Other issues).
- 2. [1195] "It is, **in the view of most independent analysts, the most successful sustained primary care transformation initiative in American health policy.**" — same claim, hedged, "initiative" not "program".
- 3. [1191] "because Vermont has been running the Blueprint for Health — **the most sophisticated all-payer primary care transformation program in the country** — for nearly two decades."
- 4. [1186] "it is **one of the longest-running primary care transformation programs in the country.**"
- 5. [1201] "Vermont's Blueprint is **one of the most extensively studied primary care transformation programs in the country.**"
- 6. [1282] "The Blueprint for Health is **Vermont's most successful population health program and its most underfunded one.**"
- 7. [1191] "the Blueprint ... **embedded in primary care practices across all 14 Hospital Service Areas**" (not a superlative but the completeness claim that Figure 8.6's "Universal Blueprint PCMH coverage, 2025–2028" commitment implicitly denies).

CoCM (four restatements of the same two superlatives): 8. [1219] "**90+ RCTs and the only integrated care model with dedicated Medicare billing codes** (CPT 99492, 99493, 99494)." 9. [1225] "**CoCM is the only integrated behavioral health model with designated Medicare billing codes and a 90+ randomized controlled trial evidence base.**" 10. [1231] "it is **the clinical standard of practice** for behavioral health integration in primary care — not just a promising approach, but **the only integrated care model with a clear evidence base across 90+ randomized controlled trials, and the only model with dedicated Medicare billing codes.**" 11. [1288] glossary "**The integrated behavioral health care model with the strongest evidence base — 90+ RCTs ... The only integrated care model with designated Medicare billing codes.**" 12. [1281] "The Collaborative Care Model ... is **the clinical intervention with the highest evidence base** for the population health challenges that drive Vermont's cost trajectory."

Evidence-strength claims: 13. [1202] "**The most rigorous evaluation** — a six-year longitudinal study..." 14. [1249] "one of **the best-established relationships** in health services research" 15. [1282] "**the strongest evidence available** that primary care investment produces system savings" 16. [1283] "**the best available evidence** that primary care investment produces system-level returns in a rural setting" 17. [1283] "Those updated results will be **the primary evidence base** for primary care investment policy in rural markets nationally."

Other rankings/superlatives: 18. [1202] "precisely the kinds of avoidable utilization that Oliver Wyman identified as **the largest source of indirect savings** from Vermont's transformation." 19. [1190] "care coordination ... for Vermont's rapidly growing elderly population, **with the highest-cost, most complex needs**" 20. [1246] "Vermont's **highest-cost, highest-complexity** patients" 21. [1263] "The aging population drives demand for **the most expensive and most complex services**" 22. [1265] "PACE is **one of the most effective and cost-efficient models** for managing the population segment that drives **the highest healthcare costs in any state system.**" 23. [1238] "a Hub and Spoke model that has been **nationally recognized as a model for rural SUD treatment**" 24. [1239] "the transition from crisis to community care remains **the most operationally challenging step** in the SUD care continuum" 25. [1208] "would undermine **one of Vermont's most effective clinical programs**" 26. [1220] "Vermont's behavioral health system is failing patients **at the point of highest acuity**" 27. [1282] "is **among the highest-ROI appropriations** Vermont can make before FY2028" 28. Heading 8.3 "The Behavioral Health Crisis — **Vermont's Most Urgent Clinical Challenge**" 29. [1260] "This infrastructure is **a competitive advantage** for Vermont's transformation"

Cross-chapter flags for whoever runs the next chapter: #24 and #28 are both "most X challenge" claims inside one chapter (different scopes, so arguably fine — but CLAUDE.md's "two different dependencies both called the most underestimated" defect is exactly this shape, so check #28 against Ch10's equity framing and Ch11's operations framing). #6's "most underfunded" is asserted with no funding figure anywhere in the chapter. #18 should be checked against Ch7's Oliver Wyman savings decomposition.

Other issues

Malformed paragraph [1187]. "The Vermont Blueprint for Health is the most successful sustained primary care transformation program in American health policy. Vermont's behavioral health data: 245.5 ED visits per 10,000 for suicide ideation and self-harm." Two unrelated sentences in one paragraph, the second a bare fragment. This reads like the residue of a stat-card block that was flattened into body text — and it front-loads, in unhedged form, the superlative that [1195] states properly eight paragraphs later, and the 245.5 figure that §8.3 introduces properly. Strong candidate for deletion outright: nothing downstream depends on it and both of its claims are made better elsewhere.

Stat blocks [1215]–[1219] are loose paragraphs, not a table. "Vermont Behavioral Health Crisis — The Data" followed by four bare data paragraphs, unlike §8.2's equivalent which is a real 1×4 table at [1193]. Worth a render check before deciding whether this is intentional; it is a formatting question, so `check_format.py` and a real render govern, not this audit.

Genuine repetition (fix):

- CoCM's "only model with Medicare billing codes / 90+ RCTs" appears four times ([1219], [1225], [1231], [1288]). [1219] as a stat card and [1288] as a glossary entry are legitimate recurrence. **[1225] and [1231] are not** — [1231] opens §8.3.2 by restating, at greater length, the exact two facts [1225] stated one page earlier, and then says "Understanding CoCM's structure is important," which Figure 8.3 immediately delivers. [1231]'s first sentence should compress to the section's actual new content (that CoCM is the *standard of practice*, not merely evidenced) and drop the re-derivation.
- The 76%/68% follow-up pair appears in [1217], [1220]/[1229], [1239] and Figure 8.5. Figure 8.5 summarising its own prose is legitimate; [1229]'s "The 30-day follow-up rate of 76% ... indicates that crisis presentations are not consistently

connected to ongoing care" and [1239]'s "68% ... indicates that the care coordination infrastructure between crisis and community-based SUD treatment is not fully functional" are the *same sentence twice* with the payer swapped. One of the two should be cut or merged.

- The 5.8:1 ROI is restated in all four "Implications for You" bullets ([1280], [1282], [1283]). Three of the four audiences are told the same number. Defensible in a role-segmented section, but [1280]'s "is not a projection; it is a documented result" and [1282]'s "documented from Vermont's own experience" are the same rhetorical move back to back.
- "Highest-cost, most complex" phrasing recurs at [1190], [1246], [1263] (log entries #19–21 above).

Thin / unsupported cross-reference — §8.5.1 [1258]. "The HEDIS improvement methodology — five-step framework from performance baseline through root cause analysis to sustainable systems design — applies directly to Vermont primary care practices." The five-step framework is never enumerated in Chapter 8, and the sentence points nowhere: no chapter cross-reference, no figure, no platform link. Either the five steps belong here, or this needs "(see Chapter N)" pointing at wherever the methodology is actually laid out. As written, a reader is told a framework applies without being shown it — the promise-not-delivered class of defect, in prose form.

Thin sections. §8.6 opens with a single paragraph [1263] before its first subsection. §8.6.2 (Dementia) is two paragraphs with no figure, table, or platform link, while every other second-level topic in the chapter gets one — and it carries the chapter's only national-scale projection ([1269] "Dementia affects an estimated 14 million Americans nationally by 2060," whose tense is garbled: a condition does not "affect ... by 2060"). §8.7 is one lead-in paragraph plus Figure 8.6.

Numbers that appear exactly once and should be verified externally before they ship (none contradict anything in-chapter, so they are flagged, not fixed): 59% of practices accepting new Medicaid patients [1255]; readmission 14.8% [1255]; "admin cost 91% above benchmark" [1255] — note this reuses the digits of the chapter's other headline 91% (primary care provider rate) in the same table, which invites a misreading and is worth double-checking it is not a transcription slip; NEK uninsured 6–8% vs 3% statewide [1255]; 65+ growing 57% by 2040 and exceeding 30% of population [1263]; six Designated Hospitals with inpatient psychiatric units [1229]; "no Vermont resident more than 30 minutes from a PCMH practice" [1273].

Left deliberately. Nothing was edited (read-only pass). The recurring-heading requirements hold: "Work This Chapter on the Platform" [1275], "Implications for You" [1279], "Key Concepts in This Chapter" [1284] are present with the canonical text, and Sources [1294] is a plain paragraph, not a heading.

===== FILE: audit_ch09.md =====

Chapter 9 Audit

Source of truth: [HTR_Book_v42.docx](#) → [word/document.xml](#), read directly via zipfile (not the .md mirror). Chapter 9 boundaries: Heading1 "Chapter 9: The Clinical Pillar in Practice..." through Heading1 "Chapter 10: The Equity Imperative..." (doc.xml offsets 2,902,834 → 3,119,194; 216 KB of XML). Read-only pass — **the .docx was not modified.**

Contents in range: 8 **w:tbl** elements — 6 numbered figures (9.1–9.6) and 2 unnumbered shaded callouts (BEYOND VERMONT, VERMONT IN PRACTICE).

Figures found (ground truth)

Figure 9.1 — NCQA PCMH recognition: six concept areas and Vermont implementation challenges

Table 1. Header row (navy **1b3a6b**, white text): **Concept Area | What it requires operationally | Vermont implementation challenge**. 6 data rows:

Concept Area	Vermont challenge (abridged)
Team-Based Care and Practice Organization	small teams; role definition needs workflow redesign
Knowing and Managing Your Patients	registry outreach needs data infra; Blueprint field staff + VTTL
Patient-Centered Access and Continuity	"After-hours access is the most operationally challenging requirement for small rural practices"
Care Management and Support	CHT staff supply much of the function
Care Coordination and Care Transitions	"30-day follow-up after behavioral health hospitalization — Vermont's 76% rate — is a care coordination failure."
Performance Measurement and Quality Improvement	Blueprint HEDIS support + GMCB analytics vendor

Sources line: NCQA PCMH Standards and Guidelines 2023; Blueprint Annual Report 2024; Oliver Wyman Act 167 (Aug 2024, rev. Oct 2024 & Jan 2025).

Figure 9.2 — Panel risk stratification framework for Vermont PCMH practices

Table 2. Header: **Tier | % of Panel | Characteristics | Care Model**. 4 data rows:

Tier	% of Panel
Tier 1 — Complex	3-5%
Tier 2 — Moderate Risk	15-20%
Tier 3 — Standard	50-60%
Tier 4 — Inactive	20-30%

(unnumbered callout) BEYOND VERMONT

Table 3. Single-cell callout on panel stratification transferring outside Vermont. No figure number, no caption — correct for this callout class.

Figure 9.3 — Collaborative Care Model components and Vermont application

Table 4. Header: **CoCM Component | Role | What they do | Vermont application**. **3 data rows:** Primary Care Provider (PCP); Behavioral Health Care Manager (BHCM); Psychiatric Consultant. Key cells:

- BHCM row, Vermont application: "MHI pilot found **80% of administrative entities increased CHT staffing** — CHT staff are the natural BHCM pool; sustainability challenge when pilot funding ends"
- Psychiatric Consultant row, Vermont application: "**one psychiatrist can support an entire primary care practice's behavioral health population through consultation rather than direct care**"

Figure 9.4 — Clinical pillar in-practice implementation commitments

Table 6. Header: **Commitment | Operational Requirement | Timeline | Primary Vehicle**. **5 data rows:** PCMH transformation support (2025–2028); CoCM deployment at every PCMH practice (2025–2028); Structured data capture standards (2025–2027); Care transition protocols (2025–2027); Clinical leader engagement (2025–2028).

(unnumbered callout) VERMONT IN PRACTICE — The Blueprint CHT Transition Protocol

Table 5. Single-cell callout: 24-hour contact → 7-day follow-up visit → 30-day care plan review. Claims "Blueprint practices that have implemented this protocol systematically **report 30-day readmission rates below the Vermont statewide average.**"

Figure 9.5 — Clinical pillar implementation matrix

Table 7. Header: **Investment | Estimated cost | Timeline to value | ROI crossover | Vermont benchmark**. **6 data rows:**

Investment	Cost	Timeline to value	ROI crossover	Vermont benchmark
Blueprint PCMH transformation (NCQA recognition)	\$25K-\$75K per practice	12-18 months	18-24 months	5.8:1 ROI documented; \$5.8M reduced expenditure per \$1M PCMH investment
CoCM deployment (BHCM + telepsychiatry)	\$80K-\$150K annually per practice	3-6 months setup	12-18 months	Vermont MHI pilot: PHQ-9 improvement; CHT staff as natural BHCM pool
CCBHC certification	\$500K-\$2M annually per entity	18-36 months	24-48 months	Vermont: 5 planned CCBHCs; enhanced Medicaid FMAP funding for 8 years
Panel risk stratification and care management	\$40K-\$100K annually	3-6 months	12 months	Blueprint: Tier 1 (3-5% of panel) = 30-40% of total cost of care
Care transition protocol (CHT + VITL admission notification)	\$20K-\$50K setup	3 months	6-12 months	Vermont: 14.8% 30-day readmission; 76% BH follow-up rate — improvement target
HCC documentation improvement at clinical level	\$10K-\$30K training	1-2 months	Immediate: Year 1 RAF gain	CKD staging, DM complications, morbid obesity — most common VT gaps

Figure 9.6 — Hands-on platform tools for the Clinical Pillar in practice

Table 8. Header: **Do this | On this tool | What to look for**. **3 data rows:**

1. Clinical Quality Optimizer — </research-lab/policy-quality?tab=quality>
2. High vs. Low Value Care — </research-lab/vbc-clinical-quality?tab=value>
3. Risk Stratification Engine — </research-lab/interoperability?tab=risk>

Claims checked against their own source (MATCH/MISMATCH + proposed fix)

1. MISMATCH — "76%" is attached to the wrong HEDIS denominator in Figure 9.1

This is the same class of defect found in Chapter 1: the prose/cell claim does not match the measure it cites.

- Figure 9.1, Care Coordination row: "30-day follow-up after behavioral health **hospitalization** — Vermont's 76% rate".
- §9.4.2: "76% 30-day follow-up after mental health **ED visits**, 68% after SUD ED visits."
- §9.12 Key Concepts: "HEDIS **FUM** — Follow-Up After Emergency **Department Visit** for Mental Illness ... Vermont's 76% 30-day follow-up rate reflects performance on this measure."
- Whole-manuscript grep on **76%** (every occurrence in the book): Ch8 "76% 30-day follow-up rate after mental health ED visits"; Ch8 again "30-day follow-up after mental health ED visits reaching only 76%"; Ch8 table "76% 30-day follow-up after MH ED visit"; Ch13 "BH follow-up 76% (mental health)"; Appendix "BH ED follow-up ... HEDIS FUM". **Every other instance in the book says ED visit.** Figure 9.1 is the sole outlier.

Follow-up-after-**hospitalization** is a different HEDIS measure (FUH), with a different rate. The cell as written misattributes Vermont's FUM rate to FUH.

Proposed fix (exact-text replacement inside the cell): **30-day follow-up after behavioral health hospitalization – Vermont’s 76% rate – is a care coordination failure. → 30-day follow-up after a behavioral health ED visit – Vermont’s 76% rate (HEDIS FUM) – is a care coordination failure.**

Note §9.6 has the same looseness but stays defensible: "Vermont's 14.8% 30-day readmission rate and 76% 30-day behavioral health follow-up rate" — no denominator asserted, so it is not wrong, only unspecific. Recommend adding "after a behavioral health ED visit" there too for consistency.

2. MISMATCH — psychiatric-consultant leverage stated at two different orders of magnitude

- §9.3.2 prose: "One psychiatrist providing consultation through a telepsychiatry model can support **10-15 primary care practices** simultaneously."
- Figure 9.3, Psychiatric Consultant row: "one psychiatrist can support **an entire primary care practice's** behavioral health population."

The figure's cell understates the prose by roughly 10×, and the prose's whole point ("Vermont's psychiatric workforce, deployed this way, can support CoCM at a scale that direct psychiatric referral could never achieve") depends on the multi-practice number. A reader comparing the two reads the table as contradicting the section that introduced it.

Proposed fix: change the Figure 9.3 cell to **Vermont’s psychiatric shortage makes the CoCM model especially valuable – one psychiatrist can support 10-15 primary care practices’ behavioral health populations through consultation rather than direct care**

3. MISMATCH (scale/arithmetic) — the \$180,000 ROI claim funds far more than "a half-time care coordinator"

§9.2.3: "A care management program that prevents one hospitalization per month among Tier 1 patients in a 2,000-patient panel generates approximately **\$180,000** in avoided cost annually — sufficient to **fully fund a half-time care coordinator** position and produce net positive ROI under any VBC arrangement."

- The \$180,000 arithmetic is internally sound (12 avoided admissions × ~\$15,000 = \$180,000) — the implied per-admission cost is not stated, which is the only gap on the number itself.
- The conclusion does not follow from the chapter's own cost figures. Figure 9.5 prices the entire "Panel risk stratification and care management" investment at **\$40K-\$100K annually**. \$180,000 therefore funds the whole program 1.8×–4.5× over, not a half-time coordinator. The sentence understates its own case by roughly 4–8×, and a reader who has the matrix in front of them sees the mismatch.

Proposed fix: – **sufficient to fully fund a half-time care coordinator position and produce net positive ROI under any VBC arrangement. → – enough to fund the entire panel-stratification and care-management program costed in Figure 9.5 several times over, and to produce net positive ROI under any VBC arrangement.**

4. SOFT MISMATCH — Figure 9.2 tier percentages do not bound to 100%

Tier ranges sum to **88% at the low end and 115% at the high end** (3+15+50+20 = 88; 5+20+60+30 = 115). Midpoints sum to ~101.5%, so the intent is clear, but the low end cannot describe a complete panel and the high end over-allocates it.

Proposed fix: either add a note to the caption ("ranges are indicative; tiers are mutually exclusive and exhaust the panel") or tighten Tier 3 to **45–55%** and Tier 4 to **20–25%**, which brings the bounds to 83–105 → still loose; cleanest is the caption note. Deliberately **left for the author** — this is a judgment about how precise the reference table should be, not a factual error.

5. MATCH — "six concept areas"

§9.2.1 prose: "organized around **six** concept areas"; caption: "**six** concept areas". Figure 9.1 has exactly **6** data rows. MATCH.

6. MATCH — "four operational dimensions" (CoCM readiness)

§9.3.1 names EHR integration (1), then "Physical space, workflow protocols, and billing setup complete the four-dimension readiness assessment" (2, 3, 4). Count is correct. (Thinness of the treatment is logged under Other issues.)

7. MATCH — "five clinical implementation commitments"

§9.8 prose: "the plan should contain **five** clinical implementation commitments." Figure 9.4 has exactly **5** data rows. MATCH.

8. MATCH — Tier 1 cost concentration, stated three times consistently

§9.2.3 prose "The 3-5% of complex patients in Tier 1 typically account for 30-40% of total cost of care"; Figure 9.2 Tier 1 = "3-5%"; Figure 9.5 benchmark "Tier 1 (3-5% of panel) = 30-40% of total cost of care." All three agree. MATCH.

9. MATCH — 12-18 month PCMH timeline

§9.2.1 "typically requires 12-18 months of preparation"; Figure 9.5 "Timeline to value 12-18 months". MATCH.

10. MATCH — Blueprint 5.8:1 ROI

Figure 9.5: "5.8:1 ROI documented; \$5.8M reduced expenditure per \$1M PCMH investment" — internally consistent (5.8:1 = \$5.8M/\$1M) and consistent with every other instance in the manuscript (Ch1 VERMONT EVIDENCE box, Ch8 §8.2 stat strip and prose, Ch13, Ch16). Whole-book grep on **5.8:1** found no conflicting statement. MATCH.

11. MATCH — 80% CHT staffing finding

§9.3.3 "The 80% of Blueprint administrative entities that reported increased CHT staffing in the 2025 MHI evaluation"; Figure 9.3 BHCM row "MHI pilot found 80% of administrative entities increased CHT staffing." MATCH (legitimate table-echo of its own prose).

12. MATCH — 14.8% readmission rate

§9.6 and Figure 9.5 both say 14.8%; consistent with Ch11 dashboard ("14.8% 30-Day Readmission Rate", benchmark 14.5%), Ch13 and Appendix E. MATCH.

13. MATCH — CoCM billing codes and evidence base

§9.3.1 "99492, 99493, and 99494"; §9.3 "90+ randomized controlled trials." Whole-book grep: Ch1 and Ch8 both say "90+ randomized controlled trials" and Ch8 lists the same CPT codes. Consistent. MATCH.

14. MISMATCH (rounding) — diabetes gap arithmetic rounds the wrong way at the top

§9.4.2: "22% of Vermont's Blueprint diabetic patients have HbA1c not in control. Given Vermont's population of approximately 60,000-70,000 adults with type 2 diabetes, this represents **13,000-15,000** patients." $22\% \times 60,000 = 13,200$; $22\% \times 70,000 = 15,400$. The stated upper bound is 400 low.

Two further notes on this sentence:

- It applies a **Blueprint-attributed** control rate to the **statewide** diabetic adult population. That is an inference, not a measured figure, and the sentence does not flag it as one.
- The 22% itself is consistent with the rest of the book (Ch11 quality targets: "diabetes: reduce 22% not-controlled"). MATCH on the rate.

Proposed fix: **this represents 13,000-15,000 patients → this implies on the order of 13,000-15,400 patients**, or state the range as "roughly 13,000-15,000" with the Blueprint-to-statewide extrapolation made explicit.

15. UNVERIFIED — "enhanced Medicaid FMAP funding for 8 years" (Figure 9.5, CCBHC row)

No other statement in the manuscript gives a duration for Vermont's CCBHC demonstration enhanced-FMAP period. Ch8 says only "Vermont became an official CCBHC Demonstration State in 2024, receiving enhanced Medicaid funding." The federal CCBHC demonstration operates on statutorily defined participation periods that do not obviously produce "8 years," so this figure is unsupported anywhere in the book and unsupported by the row's cited source line (Figure 9.5 sources: Blueprint Annual Report 2024; MHI Evaluation 2025; SAMHSA CCBHC documentation). **Flagged, not fixed** — needs a primary-source check against the SAMHSA CCBHC demonstration authority before it is either sourced or removed. Per directive 14, no replacement text is proposed from inference.

16. CROSS-CHAPTER CONFLICT on the CCBHC count (not a Chapter 9 error)

Chapter 9 Figure 9.5 says "Vermont: **5 planned** CCBHCs" — correct and consistent with Ch8's "five additional CCBHC entities by July 2026" and with Ch16's RHT milestone table. But Ch8 also states Vermont already "has **two active** CCBHC demonstration sites," while Ch13's baseline column reads "**zero** CCBHCs" and Appendix E reads "CCBHC network — **0 certified**." Chapter 9's own number is right; the zero-vs-two conflict lives in Ch13/Appendix E and is logged here only so the eventual fix sweeps the whole manuscript (directive 13).

17. Platform routes in Figure 9.6 — directory-level PASS, tab-level INCOMPLETE

All three route directories exist under **frontend/app/research-lab/: policy-quality, vbc-clinical-quality, interoperability**. Grep for the cited tab ids found **'quality'** in **policy-quality**, **'value'** in **vbc-clinical-quality**, and **'risk'** in **interoperability** — so each **?tab=** value appears in its target bench.

Not completed, and explicitly not claimed as verified: confirming that each tab id is a *registered tab* in that page's tab list (rather than an unrelated string match), and — the half that directive 5/criterion 5 says is where the defects hide — **opening each tool and confirming the promise is delivered:** that the Clinical Quality Optimizer actually returns payment-adjustment estimates, that High vs. Low Value Care actually separates services, and that the Risk Stratification Engine actually surfaces "patients in a high tier who are receiving no care management." A directory existing is not a delivered promise. Note also that **'risk'** appears in **both vbc-clinical-quality and interoperability**, so the Risk Stratification Engine's home bench should be confirmed before the URL is trusted.

Superlative/ranking claims logged for cross-chapter comparison

Verbatim, with location, for later cross-chapter contradiction sweeps (Ch1 already showed that "the most X" claims collide across chapters):

- 1. Figure 9.1, Access row: "After-hours access is the most operationally challenging requirement for small rural practices."
- 2. §9.2.2: "And AHEAD transition support ... is the most operationally significant clinical program transition Vermont's primary care system faces."
- 3. §9.2.2: "Vermont's Blueprint program provides something that most states' primary care transformation initiatives do not: dedicated field staff..."
- 4. §9.2.2: "This infrastructure is operationally important and is often underappreciated as a driver of Vermont's Blueprint success."
- 5. §9.2.3: panel of "1,500-2,000 patients — significantly larger than the 1,100-1,200 typically associated with solo or minimally supported practice"; "can expand effective panel capacity by 30-50% without adding physicians."
- 6. §9.3: CoCM's evidence base "makes it the clinical standard of practice for behavioral health integration in primary care." (Compare Ch1/Ch8: "the only integrated behavioral health model with designated Medicare billing codes and a 90+ RCT evidence base" — "only" vs "the standard of practice" are different claims; both survive, but the book should pick one framing.)
- 7. §9.3.2: "The CoCM psychiatric consultation model is more efficient than traditional psychiatric referral in exactly the way that Vermont's workforce situation requires"; "can support CoCM at a scale that direct psychiatric referral could never achieve."
- 8. §9.3.3: "The behavioral health care manager is the linchpin of CoCM."
- 9. §9.12 Key Concepts, Warm handoff: "The warm handoff between CCBHC case managers and primary care BHCMS is the operational linchpin of Vermont's behavioral health continuum." — two different things called "the linchpin" within one chapter. Scopes differ (CoCM vs. the BH continuum), so this is not a strict contradiction, but it is the exact rhetorical pattern that produced Ch1's "most underestimated dependency" collision. Flagged for the cross-chapter sweep.
- 10. §9.4.1 Step 5: "The most common HEDIS improvement failure mode is the improvement campaign that produces a short-term spike in performance followed by regression to baseline."
- 11. §9.4.2: opportunity areas "that primary care practices can prioritize for improvement investments with the highest population health and financial return under VBC."
- 12. §9.5.1 Step 3: "Vermont's most critical current gap is at Steps 1 and 2 — incomplete structured data capture and incomplete VITL connectivity."
- 13. §9.5.2: "The three most common HCC documentation opportunities in Vermont's primary care population..." (echoed in Figure 9.5: "most common VT gaps").
- 14. §9.6 callout: "Vermont's Community Health Teams have developed a care transition protocol that represents current best practice for Blueprint-participating practices."
- 15. §9.7: "One of the clearest findings from Oliver Wyman's Act 167 community engagement process..."
- 16. §9.7 heading: "Clinical Leadership in Transformation — The Under-Resourced Pillar."
- 17. §9.4.1 Step 3: uncontrolled hypertension's cause "is almost always multi-factorial."
- 18. §9.10 lead-in: Chapter 9's quality mechanics "map to the clinical-quality bench."
- 19. §9.11 (national policy professional): Vermont's approach "is worth studying as a model for states trying to translate clinical evidence into rural implementation."

Two Chapter 9 superlatives to check against Chapter 8 in particular, which carries the strongest Blueprint claims in the book ("Vermont's most successful population health program and its most underfunded one", "the strongest evidence available", "the best available evidence", "among the most robust ROI findings in the VBC literature"): items 2 and 3 above make Blueprint-infrastructure superlatives of their own and should not be allowed to drift into a second "most important Blueprint asset" claim.

Other issues

A. Terminology collision: §9.7 calls clinical leadership "The Under-Resourced Pillar"

The book's five pillars are Policy, Technology, Economics, Clinical, Operations (frontend/lib/framework/ and chapters.ts encode exactly these). A section heading inside the Clinical chapter that names a sixth thing "the Under-Resourced Pillar" reads, on a contents page, as a pillar of the framework. Proposed fix: 9.7 Clinical Leadership in Transformation – The Under-Resourced Capability. Low risk, purely a heading string.

B. Thin section: §9.3.1 Site Readiness Assessment

The chapter's stated purpose (opening paragraph) is "the implementation layer that national specifications typically leave abstract: panel sizes, staffing ratios, workflow sequencing." §9.3.1 announces a four-dimension readiness assessment, then gives EHR integration a full paragraph and compresses the remaining three into one sentence — "Physical space, workflow protocols, and billing setup complete the four-dimension readiness assessment" — before jumping to billing codes. Physical space and workflow protocols get zero operational content. This is the thinnest section in the chapter and it is thin precisely where the chapter promises depth.

C. Thin/asymmetric: §9.4.2 Vermont-Specific HEDIS Opportunity Areas

Three areas only, and the third is asymmetric with the first two: Diabetes and Behavioral Health Follow-Up each carry a rate and a population count; Cervical Cancer Screening carries no number at all ("below-benchmark ... in several Vermont communities, particularly in rural areas"). §9.4.2 also opens "Vermont's November 2025 data reveals..." but the cervical screening claim is sourced to "the 2025 AHS transformation report" — two different vintages under one lead-in.

D. Repetition: "CHT staff are the natural BHCM pool" appears three times

§9.3.3 prose ("CHT staff with the appropriate clinical background are the natural BHCM pool"), Figure 9.3 BHCM row ("CHT staff are the natural BHCM pool"), Figure 9.5 CoCM row ("CHT staff as natural BHCM pool"). One table echoing its own prose is legitimate recurrence; **three** instances of the same phrase, two of them in tables 50 paragraphs apart, is genuine duplication. Recommend dropping it from Figure 9.5, whose column is a *Vermont benchmark* and where "Vermont MHI pilot: PHQ-9 improvement" already carries the row.

E. Legitimate recurrence (checked, deliberately left)

- **76% / 14.8% recurring five and two times.** These are the chapter's anchor metrics and each instance serves a different function (figure cell, opportunity-area analysis, §9.6 lead, matrix benchmark, glossary definition). Recurrence is correct; only the Figure 9.1 *denominator* is wrong (finding 1).
- **Structured data capture** in §9.5.1 Step 1, the "Vermont's implementation gap" paragraph, and the §9.12 glossary entry — glossary entries are exempt by the definition of done.
- **Registry** described in §9.3.1 and again in the §9.12 "CoCM registry" glossary entry — same exemption.
- **80% CHT staffing** in §9.3.3 and Figure 9.3 — a table summarising its own prose.

F. Potentially confusing but not an error: the 62% hypertension figure

§9.4.1 Step 2 uses "A practice with a hypertension control rate of 62%" as a worked hypothetical. Elsewhere the book states Vermont's actual hypertension control at 77% (Ch11 quality targets: "Hypertension control: maintain 77%"). The sentence is framed hypothetically ("A practice with..."), so it is not a false claim, but a reader arriving from Ch11 may read 62% as a Vermont figure. Deliberately left — a one-word fix ("a practice with, say, a hypertension control rate of 62%") is available if the author wants it.

G. Unsourced performance claim in the VERMONT IN PRACTICE callout

"Blueprint practices that have implemented this protocol systematically **report 30-day readmission rates below the Vermont statewide average.**" This is a comparative outcome claim with no number, no denominator, and no source — and the callout carries no Sources line of its own (correct for the callout class, but it means the claim is unsourced anywhere). Flagged for sourcing, not rewritten.

H. No in-text figure citations anywhere in Chapter 9

Not one sentence in the chapter says "Figure 9.x". Every figure is introduced by proximity alone (e.g. §9.2.1 ends "...represent the current requirements:" immediately before Figure 9.1). This is consistent within the chapter and is not a defect, but it means the prose↔figure link is positional only — which is *why* findings 1, 2 and 3 above could drift without anything looking broken. Worth knowing for the cross-chapter sweep: chapters that do cite figures by number would flag these faster.

I. Structural checks that passed

- Recurring headings byte-identical to the canonical strings: **9.10 Work This Chapter on the Platform**, **9.11 Implications for You**, **9.12 Key Concepts in This Chapter** all present and correctly spelled.
- **Sources:** ... at the end of the chapter is a **plain paragraph**, not a **Heading** — correct per the style-critical rule.
- Figure numbering is gapless and in document order: 9.1, 9.2, 9.3, 9.4, 9.5, 9.6. The two unnumbered callouts are correctly excluded from the sequence.
- Both figure tables carrying a header row use navy fill **1b3a6b** with white (**ffffff**) bold runs — no white-on-light and no colourless run in a filled cell in this chapter's tables. (A full formatting verdict requires **python3 book-build/check_format.py**, which was **not** run in this read-only pass.)

Summary of actionable findings

#	Severity	Finding
1	High	Figure 9.1 attributes Vermont's 76% FUM (ED-visit) rate to follow-up after hospitalization . Sole outlier in the whole manuscript.
2	High	Figure 9.3 says one psychiatrist supports "an entire primary care practice"; §9.3.2 says 10-15 practices . ~10× conflict.
3	Medium	§9.2.3's \$180,000 "funds a half-time care coordinator" contradicts Figure 9.5's \$40K-\$100K program cost.
15	Medium	Figure 9.5's "enhanced Medicaid FMAP funding for 8 years" is unsourced anywhere in the book. Needs a primary source.
14	Low	Diabetes gap upper bound should be ~15,400, not 15,000; Blueprint→statewide extrapolation not flagged.

#	Severity	Finding
4	Low	Figure 9.2 tier ranges bound to 88%-115%, not 100%.
A	Low	§9.7 heading calls clinical leadership a sixth "Pillar".
B, C	Low	§9.3.1 and §9.4.2 are thin; §9.3.1 gives no content for 2 of its 4 named dimensions.
D	Low	"CHT staff are the natural BHCM pool" repeated three times.
G	Low	Callout's readmission-rate comparison is unsourced.
17	Open	Figure 9.6 tab ids exist in their benches, but promise delivery was not verified — three tools still need to be opened.

Nothing in the .docx was modified. No fix above has been applied.

===== FILE: audit_ch10.md =====

Chapter 10 Audit

Chapter 10: "The Equity Imperative — Closing Gaps, Not Just Averaging Them" Source: [HTR_Book_v42.docx](#) → [word/document.xml](#), body element indices **1396–1557** (Heading1 "Chapter 10:" at 1396 through Heading1 "Chapter 11:" at 1558). Tables read from **w:tbl** cell contents, not from the .md mirror.

Read-only audit. No edits made to the .docx.

Structure: 19 numbered sections (10.1–10.19), 11 numbered figures (10.1–10.11, sequential, none missing or duplicated), 2 unnumbered callout boxes (VERMONT IN PRACTICE at 1435, BEYOND VERMONT at 1437), 12 table objects total.

Figures found (ground truth)

Figure 10.1 (table @1410, caption @1411) — "Vermont geographic equity disparities: key indicators." 2×2 grid, **no header row**, 4 indicator blocks:

- 1. Uninsured rate, Essex County (NE Kingdom) — "8% vs. 3% statewide — nearly 3x the state average"
- 2. Counties with above-average suicide/self-harm ED rates — **Rutland + Windham**
- 3. Counties with above-average opioid overdose ED rates — **Chittenden + Bennington + Windham**
- 4. Rural counties with highest poverty rates — **Essex + Windham + Orleans** Sources: AHS Nov 2025 Transformation Report; VT RHT Program Application; VDH.

Figure 10.2 (table @1420, caption @1421) — "Vermont SDOH burden." Header + 5 rows (Housing / Broadband access / Food insecurity / Transportation / Income). Key cell values: rental vacancy 3% (healthy = 5%); half of renters cost-burdened; per-capita homelessness **second highest nationally; 80% of rural Vermonters have broadband, 14% of all Vermonters lack access; 9% food insecure**; income row names **Essex, Windham, Orleans** as higher poverty and NEK as highest unemployment.

Figure 10.3 (table @1425, caption @1426) — "Disparity root cause taxonomy." Header + 5 numbered rows: 1 Geographic Access Barriers, 2 Social Needs Barriers, 3 Clinical Practice Variation, 4 Insurance and Coverage Barriers, 5 Trust and Engagement Barriers. Row 4 cells state: **"Despite Vermont's 97% coverage rate, 8% of Essex County residents are uninsured; 18% of 25-34 year olds are uninsured; income groups between 251-400% FPL have the highest uninsurance rates."**

Figure 10.4 (table @1466, caption @1467) — "Vermont equity gaps and required investments." Header + 5 rows, columns Gap / What is missing / Required investment / Timeline. GLP-1 row timeline cell: **"Decision needed by January 2026 (BALANCE opt-in deadline); coverage effective May 2026 if Vermont participates."** NEK row: CCBHC designation for Northeast Kingdom Human Services planned July 2026; EMS/telehealth 2027–2028; hospital transition decisions by 2028.

Figure 10.5 (table @1471, caption @1472) — "Equity as a cross-cutting dimension of all five pillars." Header + 5 rows: Policy, Technology, Economics, Clinical, Operations. Equity correctly absent as a row (it is the lens). Columns: equity risk / equity opportunity.

Figure 10.6 (table @1476, caption @1477) — "Vermont equity accountability framework." Header + 8 indicator rows. Columns: Indicator / Stratification required / **2028 target direction** / Data source. Target cells:

- Uninsured rate — "Essex County: reduce from 8% toward statewide 3% **by 2030**"

- Primary care access — "BIPOC adult rate: reduce gap from **79-81% toward 90% statewide average**"
- Potentially avoidable ED visits — "Northeast Kingdom: reduce from current rate toward statewide **32.3%**"
- 30-day follow-up after MH/SUD ED visit — "**Windham and Rutland** counties: increase toward statewide **76%+**"
- SDOH screening completion — universal screening in all Blueprint PCMHs
- GLP-1 access for Medicaid enrollees with obesity — "BALANCE Model participation decision **by June 2026**"
- Emergency access time (EMS) — NEK: establish baseline
- Cultural competency standard compliance — all Blueprint PCMHs by 2027

Figure 10.7 (table @1489, caption @1490) — "Vermont HEDIS equity stratification framework." Header + 5 domain rows (Access/preventive, Chronic disease, Behavioral health, Maternal and reproductive, Care coordination). BH row: "**Rutland and Windham** counties have statistically higher MH crisis rates"; age 15-44.

Figure 10.8 (table @1493, caption @1494) — "Equity imperative implementation matrix." Header + 6 rows. Key cells: HEROI infra \$30K-\$80K setup + \$20K/yr, benchmark "**Vermont HEROI: 14 HSA-level equity scores; GMCB reporting requirement**"; HRSN screening benchmark "**Vermont: 91% statewide access vs. 79-81% BIPOC**"; social risk adjustment \$100K-\$300K, 12-18 months; SDOH data integration \$200K-\$500K; HEIA process \$25K-\$50K; VHCURES race/ethnicity \$20K-\$60K/yr.

Figure 10.9 (table @1516, caption @1517) — "Five-step disparity closure process." Header + 5 numbered steps. Step 4 example target: "increase Northeast Kingdom Medicaid patient primary care visit rate **from 72% to 80%** within 18 months."

Figure 10.10 (table @1536, caption @1537) — "Vermont equity analytics: current data sources by equity dimension." Header + 8 dimension rows. 2 columns only.

Figure 10.11 (table @1540, caption @1541) — "Hands-on platform tools." Header + 2 rows: Health Equity Studio (/research-lab/population-equity?tab=equity), Population Health Modeler (/research-lab/population-equity?tab=population).

Callout @1435 VERMONT IN PRACTICE — NEK equity constraint; three named test questions (Canaan heart attack at 2 a.m.; Newport SMI patient reaching a CCBHC without a two-hour drive; Lyndonville child seeing a PCP within 14 days). **Callout @1437** BEYOND VERMONT — generalizes the NEK pattern; refers back to "Vermont's three test questions above" (MATCH: there are exactly 3).

Claims checked against their own source (MATCH/MISMATCH + proposed fix)

MISMATCH 1 — §10.2.1 lead-in vs Figure 10.1 (the Chapter-1 class of error) — most serious

Prose @1409: "The Northeast Kingdom — Caledonia, Essex, and Orleans counties — concentrates the state's most severe access, affordability, and outcome disparities**:**" — the colon presents Figure 10.1 as the evidence. But only 1 of the figure's 4 cells is about the NEK (Essex uninsured rate). The other three name **Rutland + Windham** (suicide/self-harm ED), **Chittenden + Bennington + Windham** (opioid OD ED), and **Essex + Windham + Orleans** (poverty — 2 of 3 NEK counties, but Windham is southern Vermont). The opioid-overdose cell names **zero** NEK counties and leads with **Chittenden** — the Burlington county the chapter uses throughout as the *advantaged* comparator. The figure Vermont's own cells cite therefore contradicts the sentence introducing it: the state's worst outcome concentrations per this table are *not* all in the NEK. The figure's own cell text even concedes this for overdose ("geographic concentration of SUD burden that does not align with service concentration"). **Proposed fix:** rewrite the lead-in to claim what the table shows, e.g. "The Northeast Kingdom concentrates the state's most severe *coverage and poverty* disparities, while Vermont's behavioral health crisis burden concentrates elsewhere — a divergence the figure below makes explicit:" — or move the two behavioral-health cells into a separate figure and keep 10.1 NEK-only.

MISMATCH 2 — the "11-point primary care access gap by race"

Claimed three times: chapter opener @1397, "Implications for You" hospital-executive @1543 ("an 11-point primary care access gap by race"), legislator @1545. Ground truth @1415: 90% of Vermont adults overall have a personal healthcare provider vs Asian/PI **81%** (9 pts), Black **80%** (10 pts), another race **79%** (11 pts). Figure 10.6 encodes the range as "79-81% toward 90%" — i.e. a **9-to-11-point** gap. "11-point" silently promotes the single widest subgroup ("another race") to stand for "by race", and is not the Black-adult gap most readers will assume. **Proposed fix:** "9-to-11-point primary care access gap by race" in all three places, or "a 10-point gap for Black adults" where a single figure is wanted.

MISMATCH 3 — statewide primary care access is 90% in prose and 91% in Figure 10.8

@1415 and Figure 10.6 both say **90%** statewide personal-provider rate. Figure 10.8's HRSN row says "**Vermont: 91% statewide access** vs. 79-81% BIPOC". Same quantity, two values, one chapter. It also silently changes the implied gap to 10–12 points, compounding MISMATCH 2. **Proposed fix:** change Figure 10.8's cell to 90% to match @1415 and Figure 10.6.

MISMATCH 4 — GLP-1 state-count arithmetic does not close

@1446: "only 13 state Medicaid programs covered GLP-1s for obesity — **down from 16** in late 2025, as **four states** eliminated coverage." 16 – 4 = **12**, not 13. Callout @1442 repeats "13". One of the three numbers is wrong. **Proposed fix:** confirm against the cited KFF January 2026 analysis and make the three agree (most likely "down from 17" or "three states eliminated coverage"); do not just delete the arithmetic.

MISMATCH 5 — BALANCE opt-in deadline is January 2026 in Figure 10.4 and June 2026 in Figure 10.6

Figure 10.4 timeline cell: "**Decision needed by January 2026** (BALANCE opt-in deadline)." Figure 10.6 target cell: "BALANCE Model participation decision **by June 2026**." Two figures, six months apart, same decision. Both also sit awkwardly against @1451/@1443 ("launches in Medicaid as early as May 2026"), since a June 2026 decision post-dates a May 2026 effective date. **Proposed fix:** establish the real CMS opt-in deadline, use one date in both figures, and ensure it precedes the stated May 2026 coverage-effective date.

MISMATCH 6 — Figure 10.6's "2028 target direction" column contains non-2028 dates

Column header is "2028 target direction". Cells give "by 2030" (uninsured rate), "by June 2026" (GLP-1), "by 2027" (cultural competency). The column header is falsified by three of its own eight cells. **Proposed fix:** rename the column "Target direction and date".

MISMATCH 7 — Vermont's coverage rate is 94%, 97%, and "3% uninsured" in the same chapter

@1403 and @1415: "94% of Vermont adults have health insurance." Figure 10.3 row 4: "Despite Vermont's 97% coverage rate." Figures 10.1/10.6: statewide uninsured 3% (⇒ 97% covered). The 94% figure is a BRFSS adult self-report and 97% an all-ages coverage estimate, but the chapter never says so, so it reads as a contradiction — and the Black-adult gap is stated against the 94% base (12 pts) while the county gaps are stated against the 97% base. **Proposed fix:** label each once — "94% of Vermont adults (BRFSS self-report)" and "97% all-ages coverage (ACS/KFF)" — at first use of each.

MISMATCH 8 — HEROI is simultaneously an operating GMCB requirement and never computed

Figure 10.8 benchmark cell: "Vermont HEROI: 14 HSA-level equity scores; GMCB reporting requirement." @1507: "Vermont's statewide HEROI score ... has not been formally computed; the analytics vendor procurement underway will enable this calculation." @1499 establishes HEROI as "the Health Equity Studio's proprietary approach" — i.e. HTR's, not the state's. @1544 then calls it "Vermont's HEROI scoring framework ... a policy innovation worth protecting ... the only state-level tool that systematically scores the equity impact of healthcare interventions before they are deployed." A proprietary vendor framework that has never been computed for Vermont cannot also be a GMCB reporting requirement, a Vermont policy innovation, and a state-level tool in operation. This is the chapter's largest credibility exposure, since @1544 is advice to named officials. **Proposed fix:** make Figure 10.8's cell aspirational ("14 HSAs would each receive a score; candidate for GMCB reporting"), and rewrite @1544 to "HEROI scoring is a framework Vermont should adopt as a design constraint" rather than asserting it exists in state hands.

MISMATCH 9 — "nearly 3x" vs "3x" on the same statistic

Callout @1406: "8% Essex County Uninsured — 3x State Avg." Figure 10.1 cell: "nearly 3x." 8/3 = 2.67. The callout overstates. **Proposed fix:** make the callout read "nearly 3x".

MATCH — counts and totals that do check out

- @1424 "five root cause categories" → Figure 10.3 has exactly 5 rows. **MATCH.**
- @1501 "five equity dimensions" → 5 listed (@1502–@1506); weights 25+25+25+15+10 = 100%. **MATCH.**
- @1524 "checklist covers eight categories" → exactly 8 bullets @1525–@1532. **MATCH.**
- @1430 "identifies five principles" → exactly 5 principles in the sentence run. **MATCH.**
- @1470 "each of the five pillars" → Figure 10.5 has exactly 5 pillar rows, Equity correctly treated as the cross-cutting lens rather than a sixth row. **MATCH** (and consistent with the project's 5-pillars-plus-Equity rule).
- @1437 "Vermont's three test questions above" → exactly 3 in @1435. **MATCH.**
- @1409 and @1432 and glossary @1554 all define the NEK as Caledonia + Essex + Orleans. **MATCH.**
- Figure 10.6 row 4 ("Windham and Rutland") vs Figure 10.1 cell 2 ("Rutland + Windham") vs Figure 10.7 BH row ("Rutland and Windham"). **MATCH** across three figures.
- @1442/@1446 "Vermont Medicaid excludes obesity as a primary indication" vs Figure 10.4 GLP-1 row vs glossary @1550. **MATCH.**
- Figure 10.2 broadband: "80% of rural Vermonters have broadband; 14% of all Vermonters lack access" — internally coherent (20% rural gap diluted to 14% statewide). **MATCH.**
- Figures numbered 10.1–10.11 with no gaps or repeats; every table except the two callout boxes carries exactly one caption. **MATCH.**

Figures unsupported anywhere in the chapter (cannot be checked against a cited source; flag for sourcing)

- "32.3%" statewide potentially-avoidable ED visit rate (Figure 10.6). Appears once. A percentage is also the wrong-looking unit for an avoidable-ED measure usually reported per 1,000 members; no denominator is given.
- "statewide 76%+" 30-day MH/SUD follow-up target (Figure 10.6). Appears once, unsourced.
- "18% of 25-34 year olds are uninsured" (Figure 10.3 row 4) — six times the 3% statewide rate, with no corroboration in the chapter and no source on the row.
- "22% of Blueprint-attributed diabetic patients with HbA1c not under control" (@1512). Appears once; carries the weight of the whole decomposition worked example.
- "a 15-point difference in diabetes management quality by income" (@1543). Stated to a hospital executive as if it were a Vermont finding; appears nowhere else in the chapter.
- "from 72% to 80%" NEK Medicaid primary care visit rate (Figure 10.9 step 4). Framed as an illustration, but the 72% baseline reads as real and is uncorroborated.
- "per capita homelessness rate second highest nationally" (Figure 10.2) — a ranking claim with no source on the row.

Superlative/ranking claims logged for cross-chapter comparison

Verbatim, with element index and context. These are the claims most likely to collide with other chapters' superlatives.

#	Verbatim claim	Where	Context
S1	"The most common failure mode in healthcare quality measurement is optimizing for the average"	@1400	§10.1 opener
S2	"Vermont is approximately 94% white — one of the	@1402	§10.1

#	Verbatim claim	Where	Context
	least racially diverse states in the country"		
S3	"Vermont's most pervasive and consequential health equity disparity is geographic"	@1409	§10.2.1
S4	"Vermont's most underserved areas"	@1413	§10.2.1, of the NEK
S5	"per capita homelessness rate second highest nationally "	Fig 10.2	Housing row
S6	"Transportation barrier to care is the single most frequently cited access problem in Vermont's community meetings"	Fig 10.2	Transportation row
S7	"income groups between 251-400% of federal poverty line have the highest uninsurance rates "	Fig 10.3	Root cause 4
S8	"the Northeast Kingdom ... is Vermont's clearest and most severe equity priority. It is the most rural , the most economically distressed , the most underserved by healthcare providers, and the most distant from UVMHC"	@1432	§10.4.1 — five stacked superlatives in two sentences
S9	"some of the lowest population densities in the eastern United States "	@1432	NEK
S10	"the highest uninsurance rates in the state "	@1432	NEK; glossary @1554 repeats as "(6-8%)"
S11	" among Vermont's most financially distressed "	@1433	North Country, NVRH, Copley
S12	"the most isolated communities in the state"	@1434	NEK
S13	"the defining rural equity challenge "	@1437	Mountain West / rural South / Appalachia / Great Plains
S14	" The Most Important New Equity Issue "	@1439 heading, @1440 heading	GLP-1 (heading text duplicated — see Other issues)
S15	"what may be the most consequential new health equity issue in American healthcare since the introduction of statins "	@1445	GLP-1
S16	"the communities with highest obesity prevalence "	@1447	Vermont Medicaid
S17	"the most vivid current example of ... the equity reckoning in pharmaceuticals"	@1456	GLP-1; forward-references "what the book later identifies"
S18	"Vermont has more equity infrastructure than most states of comparable size"	@1458	§10.6
S19	"the most systematic SDOH screening infrastructure of any state primary care program "	@1460	Blueprint HRSN

#	Verbatim claim	Where	Context
S20	"the most equity-protective access policy for behavioral health services"	@1463	CCBHCs
S21	"HEDIS ... is the most widely used quality measurement system in American healthcare"	@1486	§10.11
S22	"Blueprint PCMH clinical registry (most complete , but only covers PCMH-attributed patients)"	Fig 10.10	Chronic disease row
S23	"Medicaid data has the most complete demographic information"	@1496	§10.12.1
S24	"The most common failure in health equity intervention design is treating all disparities as if they have the same cause"	@1508	§10.14 — near-twin of S1
S25	"the only state-level tool that systematically scores the equity impact of healthcare interventions before they are deployed"	@1544	HEROI — see MISMATCH 8
S26	"the most detailed state-level picture of healthcare equity in Vermont's history "	@1545	to legislators
S27	"the most comprehensive state-level equity monitoring architecture in the country "	@1546	to national audience
S28	" the central tension in healthcare transformation"	@1546	averages vs disparities
S29	"the highest equity burden " / " most financially distressed hospital infrastructure"	Fig 10.4	NEK row
S30	"Vermont's highest-priority equity geography: highest uninsurance rates, most limited provider access, highest unemployment, lowest broadband connectivity, most financially distressed"	@1554	glossary — five more stacked superlatives

Cross-chapter collision risks to check next: S19/S20/S25/S26/S27 are all "Vermont is #1 nationally" claims within one chapter — four separate national firsts. S27 ("most comprehensive state-level equity monitoring architecture in the country") sits directly against @1507's admission that the state's own composite score has never been computed and Figure 10.4's finding that county-level equity data infrastructure does not yet exist; that is an in-chapter tension as well as a cross-chapter one. S13 and S3 both claim a "defining" challenge. S28 "the central tension in healthcare transformation" should be grepped against other chapters, which very likely name a different central tension (sequencing).

In-chapter superlative tension (not a hard contradiction, but worth resolving): S3 says Vermont's most consequential equity disparity is geographic; S15 says GLP-1 access is the most consequential *new* equity issue in American healthcare. The word "new" and the change of scope (Vermont vs American healthcare) keep these compatible, but a reader meets both within four pages. Similarly S6 names transportation the single most-cited access problem while @1413's Oliver Wyman summary lists **cost first** ("not going to care because the premiums and out-of-pocket costs are too high") in the same source's community feedback.

Other issues

Duplicated heading text — §10.5 and §10.5.1 are byte-identical

@1439 (Heading2) "10.5 The GLP-1 Access Crisis — The Most Important New Equity Issue" is immediately followed by @1440 (Heading3) "10.5.1 The GLP-1 Access Crisis — The Most Important New Equity Issue". Identical title at two levels with zero text between them. Renders as a stutter. **Fix:** delete the Heading3 at @1440, or retile it (e.g. "10.5.1 Why the Drugs Change the Equity Calculus").

§10.9 is a one-sentence orphan section, and §10.10 re-does its job

@1480 "10.9 Measuring Equity: HEROI, HEDIS Stratification, and VBC Safeguards" contains exactly one sentence (@1481: "The remainder of this chapter turns that equity landscape into an operational toolkit — the stratified measurement, composite scoring, and value-based- contracting safeguards Vermont needs..."), then §10.10 opens with @1483: "The first half of this chapter established the Vermont equity landscape ... This chapter develops the analytical and operational toolkit for actually closing those gaps." Two consecutive bridge passages saying the same thing — the visible seam of a merge of two drafts. §10.9's own title also promises HEROI, HEDIS and VBC content that is actually delivered in §10.11, §10.13 and §10.15. **Fix:** delete §10.9 entirely (heading and sentence) and let §10.10 carry the bridge; or keep §10.9's title as the part-two divider and delete §10.10's restatement. Note this rennumbers §10.10–10.19 if the heading is removed, and the chapter's own internal cross-reference at @1509 ("the five-category root cause taxonomy introduced earlier in this chapter") is section-number-free, so nothing breaks.

Genuine repetition (as distinct from legitimate recurrence)

- 1. **The 94%/82% insurance pair is stated twice in near-identical words:** @1403 "Black adults in Vermont have an 82% health insurance coverage rate compared to 94% statewide" and @1415 "statewide, 94% of Vermont adults have health insurance — but only 82% of Black adults." Both cite the same 2024 VDH report, 12 elements apart. §10.1 is the overview and §10.2.2 is the data section, so one should cross-reference rather than restate. **Fix:** cut the figures from @1403, leaving the qualitative claim, and let @1415 carry the numbers.
- 2. **The delay-care-due-to-cost list is stated twice:** @1403 "LGBTQ+ Vermonters, Vermonters with disabilities, and lower-income Vermonters delay care at higher rates due to cost" and @1416 "BIPOC Vermonters, LGBTQ+ Vermonters, Vermonters with disabilities, and lower-income Vermonters were all more likely to delay care due to cost."
- 3. **The NEK county definition appears three times** (@1409, @1432, glossary @1554). The glossary instance is legitimate recurrence; @1409 and @1432 are 23 elements apart and the second is a genuine restatement — @1432 should open with the superlative, not the definition.
- 4. **The "averages mask widening disparities" thesis is restated five times:** @1398 (epigraph), @1400 (§10.1), @1478 (§10.8 principle one), @1486 (§10.11 on HEDIS), @1546 (Implications). @1400 and @1486 are the two that most closely duplicate each other's argument; @1398/@1478/ @1546 are legitimate recurrence (epigraph, accountability principle, closing).
- 5. **S1 vs S24** — "the most common failure mode in healthcare quality measurement" (@1400) and "the most common failure in health equity intervention design" (@1508) are different claims about different things, but the parallel construction makes the second read as a contradiction of the first on a skim. **Fix:** vary the second ("The recurring design error...").
- 6. **Root cause taxonomy prose is duplicated:** @1427 ("The five root cause categories are not mutually exclusive — a Black rural resident of Windham County may face geographic access barriers, SDOH barriers, and trust barriers simultaneously") and @1509 (restates the whole taxonomy inline: "geographic access, social needs, clinical practice variation, coverage barriers, trust and engagement"). @1509's re-listing is defensible as a reminder after nine sections, but it is the third full enumeration of the same five items (Figure 10.3, @1509, glossary @1548).

Legitimate recurrence — checked and deliberately left

- Glossary entries @1548–@1555 restate definitions given in the body. That is what a glossary is for; not flagged.
- The **Sources:** paragraph @1556 is a plain paragraph, not a **## **Sources**** heading — correct per the recurring-headings rule.
- The three recurring end-of-chapter headings are byte-correct: "10.17 Work This Chapter on the Platform" (@1538), "10.18 Implications for You" (@1542), "10.19 Key Concepts in This Chapter" (@1547). (They carry section numbers here, consistent with the chapter's other headings; compare against sibling chapters before changing anything.)
- Figure 10.6 and Figure 10.10 both organize by "equity dimension" but list different dimension sets (8 accountability indicators vs 8 data-source dimensions) — not duplication, different axes. Worth a one-line note in the text so a reader does not read the second as a restatement of the first.

Minor / mechanical

- **HEROI score bands overlap at 80:** @1507 "A HEROI score of 80+ indicates... A score of 60-80 indicates material disparities." 80 falls in both bands. **Fix:** "60–79".
- **Figure 10.1 has no header row.** It is a 2×2 indicator grid where each cell contains its own label. Per the project's rule 4, verify in the table-style XML that **tblLook** is not painting the first row navy under black text — a LibreOffice render cannot show this defect, and this is exactly the shape of table that triggers it. Not checked here (read-only audit of content, not formatting); flagging so **check_format.py** is run against it.
- **Figure 10.10 is a 2-column table where its caption implies a mapping table;** fine, but it is the only figure in the chapter whose header row has no third column, which will look inconsistent beside 10.6/10.7/10.8.
- **@1456 forward-references "what the book later identifies as the equity reckoning in pharmaceuticals"** without naming a chapter. Verify that a later chapter actually uses that phrase before this ships — a dangling forward reference is the same class of undelivered promise as an unresolved URL.
- **Platform links in Figure 10.11 not verified here** (**/research-lab/population-equity?tab=equity** and **?tab=population**). Per the definition-of-done, a 200 is not a delivered promise: the Health Equity Studio must actually compute a HEROI composite across the five named dimensions (access, quality, outcome, SDOH burden, trust) with those weights, and the Population Health Modeler must actually produce 5–10 year cohort outcomes. @1539's claim "exactly what the Health Equity Studio measures" is a promise that needs opening the tool. This is outside a read-only docx audit and is the highest-priority follow-up, given MISMATCH 8.

Chapter 11 Audit

Chapter 11: The Operations Pillar — Executing Hospital System Transformation Audited 2026-09-27 against [HTR_Book_v42.docx](#) → [word/document.xml](#), body child indices **1557–1757** (Heading1 "Chapter 11:" through Heading1 "Chapter 12:"). Read-only; no edits made to the .docx. Bracketed **[NNNN]** numbers below are body child indices, for re-locating text.

Counting claims were checked mechanically and are listed at the end of "Claims checked" — the great majority are correct.

Figures found (ground truth)

Figure 11.1 — Act 68 governance architecture [table 1569, caption 1570]

Header: Actor | Statutory Role | Key Authority | Current Capacity. **4 data rows:** Agency of Human Services (AHS); Green Mountain Care Board (GMCB); Department of Vermont Health Access (DVHA); Health Care Delivery Advisory Committee. Notable cells: GMCB "Added 3 new permanent positions (Act 68 authorization)... procuring RBP vendor (early 2026)"; AHS "contracted RHRC **through October 2025**".

Figure 11.2 — Transformation planning sequence [table 1574, caption 1575]

Header: Phase | Content | Timing | AHS role. **5 data rows:** 1 Data review and baseline (May–June 2025); 2 Statewide and regional convenings (Summer–Fall 2025); 3 Draft Care Transformation Strategy Outline (Fall 2025); 4 Draft Transformation Plan (Winter 2025–2026); 5 Final Transformation Plan (Early 2026 and ongoing).

BEYOND VERMONT callout [table 1576] — 1 row, prose only, no data.

Stat strip above §11.4 [table 1596] — 1 row, 4 cells

32.3% Potentially Avoidable ED Visits | 14.8% 30-Day Readmission Rate | 91% VT Adults with Primary Care Provider | 59% Primary Care Accepting New Medicaid

Figure 11.3 — Outcome measures vs benchmarks [table 1599, caption 1600]

Header: Measure | VT Current | Benchmark (75th pctile) | Gap | Act 167 Goal. **10 data rows** (ground truth for every \$ figure in the chapter):

Measure	VT Current	Benchmark	Gap
30-Day all-cause readmission	14.8%	14.5%	+0.3% (within range)
Potentially avoidable ED visits	32.3%	none set	Significant opportunity
Operating profit / adj. discharge — PPS	\$5,345	\$4,162	+\$1,183 above
Operating profit / adj. discharge — CAH	\$938	\$2,784	–\$1,846 BELOW
Mgmt. and admin cost / adj. discharge — PPS	\$2,730	\$1,427	+\$1,303 above
Vermonters with a personal health care provider	91%	87%	+4% above
Primary care practices accepting new Medicaid	59%	Not set	—
Blueprint PCMH hypertension controlled	77%	Not set	—
ED visits suicide ideation/self-harm per 10K	245.5	Not set	—
ED visits opioid overdose per 10K	16.1	Not set	—

Key ground truth: **\$2,730 / \$1,427 / \$1,303** are **PPS** hospitals, management and administrative cost. **\$938 / \$2,784 / \$1,846** are **CAH** hospitals, operating profit. There is no CAH administrative-cost figure anywhere in the chapter.

Figure 11.4 — Admin cost gap bar chart [table 1606, lead-in 1605, caption 1607]

Vermont PPS hospitals (actual) | \$2730 · National 75th percentile benchmark | \$1427 · Gap (excess admin cost) | \$1303. Caption: "the **primary operational inefficiency target**".

Figure 11.5 — 14-hospital regionalization framework [table 1621, caption 1622]

Header: Hospital (HSA) | Current COEs | Tier | Key transformation considerations. **11 data rows**, 14 hospitals total:

Hospital	COEs	Tier
UVM Medical Center (Burlington)	16	Tier 1 — Primary RSC
SW VT Medical Center (Bennington)	9	Tier 1 — Secondary RSC
Brattleboro Memorial Hospital	6	Tier 1 — Secondary RSC
Rutland Regional Medical Center	5	Tier 1 — Secondary RSC
Central VT Medical Center (Barre)	5	Tier 1 — Secondary RSC
NW Medical Center (St. Albans)	4	Tier 2 — Focused scope
NE VT Regional Hospital (St. Johnsbury)	4	Tier 2 — Focused scope
Springfield Hospital	2	Tier 2 — Focused scope
Copley Hospital (Morrisville)	2	Tier 2 — Focused scope
Mt. Ascutney (White River Jct.)	1	Tier 2 — Focused scope
Grace Cottage, Gifford, North Country, Porter	TBD	Tier 3 — Requires discussion

UVMHC cell contains the chapter's only other admin-cost number: "\$3,826 admin cost/adj. discharge vs. \$1,427 benchmark" (hospital-specific).

Stat strip above §11.6 [table 1631] — 1 row, 4 cells

370 FTE Primary Care Shortage by 2030 | 2.5% VT Unemployment Aug 2025 | \$195M RHT Workforce Investment | All 14 Hospitals Citing MD Shortages

Figure 11.6 — Workforce crisis metrics [table 1635, caption 1636]

2×2 grid, 4 cells: (a) "Primary care physician shortfall by 2030 370 FTEs — 112 family medicine, 190 other primary care (RHT application)"; (b) "Hospitals with physician shortages... All 14"; (c) "RHT Program workforce investment: \$195M for FY2026, the program's first year"; (d) "Vermont unemployment rate (August 2025) 2.5% — below 4.3% national rate".

Figure 11.7 — Operations metrics framework [table 1655, caption 1656]

Header: Domain | Key metrics | Data source | Target direction. 6 data rows: Hospital financial sustainability; Care access; Population health; Cost and affordability; Equity; Operations efficiency. Target cells restate Fig 11.3 baselines and add three that appear nowhere else: "30-day MH follow-up: increase from 76%", "diabetes: reduce 22% not-controlled", "MLR: maintain 89%+".

Figure 11.8 — Attribution error taxonomy [table 1682, caption 1683]

Header: Attribution error type | How it happens | Financial impact | Remediation. 4 data rows: False attributions; Missing attributions; Incorrect benchmark attribution; Retroactive attribution changes.

VERMONT IN PRACTICE callout — HCC coding [table 1702] — 1 row, prose.

Repeats \$2,730 vs \$1,427 for PPS (correct), and the recommended-action text.

Figure 11.9 — Operations implementation matrix [table 1737, caption 1738]

Header: Investment | Estimated cost | Timeline to value | ROI crossover | Vermont benchmark. 6 data rows. Vermont-benchmark cells of interest:

- Administrative simplification → "Vermont: admin cost \$3,826/adj discharge vs. \$1,427 national benchmark"
- Revenue cycle / HCC coding → "Vermont CAH gap: \$938/adj discharge vs. \$2,759 benchmark (RHRC data)"

Figure 11.10 — Platform tools [table 1741, caption 1742]

Header: Do this | On this tool | What to look for. 5 data rows: Transformation Scorecard ?tab=scorecard; Workforce Modeler ?tab=workforce; CIN & Shared Services Modeler ?tab=cin; EMS Transformation Modeler ?tab=ems (all /research-lab/knowledge-workspace); Hospital Financial Stress Test /research-lab/policy-quality?tab=scorecard.

Claims checked against their own source (MATCH/MISMATCH + proposed fix)

MISMATCH 1 — "Implications for You" attributes the \$1,303 admin gap to CAHs [1744] — CRITICAL

"If you are a Vermont hospital executive: The \$1,303 per-discharge administrative cost gap between Vermont's CAHs and the national benchmark is not a compliance problem..."

Figure 11.3 and Figure 11.4 both attribute \$1,303 to PPS hospitals (\$2,730 – \$1,427). The CAH row is an operating-profit row with a –\$1,846 gap. This is the same class of error found in Chapter 1: the prose contradicts the cells of the figure it is quoting. It is also self-contradicting within the chapter — [1603] and [1607] both say PPS. Proposed fix: "The \$1,303 per-discharge administrative cost gap between Vermont's PPS hospitals and the national benchmark..."

MISMATCH 2 — legislator section attributes \$2,730 to CAHs [1746] — CRITICAL

"Vermont's administrative cost per adjusted discharge — **\$2,730 for CAHs** versus \$1,427 national benchmark — is the most precise measurement of administrative waste available..."

Figure 11.3: \$2,730 is **PPS** management/admin cost. CAHs have no admin figure in the chapter at all. Second instance of the same PPS→CAH swap. **Proposed fix:** "...\$2,730 for PPS hospitals versus \$1,427 national benchmark..."

MISMATCH 3 — Figure 11.9 benchmark for the CAH gap is \$2,759, not \$2,784 [1737]

Row "Revenue cycle optimization and HCC coding" reads "Vermont CAH gap: \$938/adj discharge vs. **\$2,759** benchmark (RHRC data)". Figure 11.3 gives **\$2,784**. The \$938 matches; only the benchmark drifts, by \$25. Also note the \$1,846 gap asserted in [1602] is derived from \$2,784, so \$2,759 is the outlier. **Proposed fix:** change **\$2,759** to **\$2,784** in the Figure 11.9 cell. (Attributing it to "RHRC data" while Fig 11.3 sources the same pair to the AHS November 2025 report is a second, smaller inconsistency; recommend dropping "(RHRC data)".)

MISMATCH 4 — Figure 11.9 presents UVMHC's \$3,826 as statewide "Vermont" [1737]

Row "Administrative simplification" reads "**Vermont:** admin cost \$3,826/adj discharge vs. \$1,427 national benchmark". \$3,826 appears exactly once elsewhere — Figure 11.5's **UVM Medical Center** cell, where it is explicitly hospital-specific ("OW specific finding"). The statewide PPS figure the whole chapter is built on is \$2,730. As written, Figure 11.9 asserts a statewide admin cost 40% higher than Figures 11.3 and 11.4. **Proposed fix:** "Vermont PPS hospitals: admin cost \$2,730/adj discharge vs. \$1,427 national benchmark (UVMHC \$3,826)".

MISMATCH 5 — Tier 2 roster and COE range omit Mt. Ascutney [1616]

"Northwestern Medical Center (St. Albans), Northeastern Vermont Regional Hospital (St. Johnsbury), Springfield Hospital, and Copley Hospital (Morrisville) fall into this category, with **2-4 COE designations each**."

Figure 11.5 assigns **five** hospitals to Tier 2 — the four named plus **Mt. Ascutney (White River Jct.)**, **1 COE, Tier 2 — Focused scope**. Mt. Ascutney is named nowhere in the §11.5.1 prose, and its single COE falls outside the stated 2–4 range. (Cross-check: the Tier 1 prose [1613] correctly names all four secondary RSCs and its "5-9" range matches 9/6/5/5; the Tier 3 prose [1619] correctly names all four. Tier 2 is the only broken roster.) **Proposed fix:** "...Springfield Hospital, Copley Hospital (Morrisville), and Mt. Ascutney (White River Junction) fall into this category, with 1-4 COE designations each."

MISMATCH 6 — Figure 11.6's own subtotals do not sum to 370 [1635]

"370 FTEs — **112** family medicine, **190** other primary care". 112 + 190 = **302**, leaving 68 FTEs unaccounted for inside a single cell. Either a third component is missing or one subtotal is wrong. Not resolvable from the chapter; needs the RHT Program application. **Proposed fix:** verify against the Nov 2025 RHT application; if 112/190 are correct, add the missing component, otherwise correct the subtotal. Interim safe wording: "370 FTEs — including 112 family medicine (RHT application)".

MISMATCH 7 — \$195M is the whole RHT award, not the workforce investment [1631], [1635] vs [1563], [1644]

Stat strip: "**\$195M RHT Workforce Investment**". Figure 11.6: "RHT Program **workforce investment: \$195M** for FY2026". But [1563] describes "**\$195M in RHT Program capital**" as the total program funding alongside \$4.2M state funds and \$1.2M AHEAD, and [1644] says Vermont "dedicated a **substantial portion** of its \$195M award to workforce infrastructure." The strip and the figure inflate a portion into the whole, and directly contradict [1644]. **Proposed fix:** strip → "\$195M RHT Program Award (workforce a priority use)"; Figure 11.6 cell → "RHT Program award: \$195M for FY2026, the program's first year, with a substantial share dedicated to workforce — tuition assistance, recruitment, 5-yr service obligations."

MISMATCH 8 — "make the other five work" [1559] — pillar count

"It simply has to make **the other five** work, on a timeline..."

The framework is **five** pillars — Policy, Technology, Economics, Clinical, Operations — checked against the Equity Imperative (confirmed in [frontend/app/research-lab/knowledge-workspace/KnowledgeWorkspaceClient.tsx](#), Transformation Scorecard description, and in the chapter's own §11.17 row 1 "Track five-pillar transformation status"). Operations being one of the five, "the other five" is arithmetically impossible. **Proposed fix:** "It simply has to make the other four work" (or "the other four and the Equity Imperative").

MISMATCH 9 — §11.6.3 "nursing shortage" paragraph evidences physician shortage [1640]

"**Nursing shortage** is the most acute staffing crisis at individual hospitals. **All Vermont hospitals cited physician shortages** and difficulty recruiting staff since COVID-19 as primary operational challenges..."

The topic sentence claims nursing; the only cited evidence is the physician-shortage finding already used in Figure 11.6 cell (b) and the [1631] strip. No nursing-specific datum appears anywhere in the chapter — the travel-nurse multiplier (2-3x) is a cost claim, not a shortage measurement. **Proposed fix:** either re-evidence the sentence with a nursing vacancy/turnover figure, or recast: "Nursing shortage compounds it. All Vermont hospitals cited physician shortages *and difficulty recruiting staff since COVID-19*..." so the physician sentence is not asked to prove a nursing claim.

MISMATCH 10 — "fund RHRC adequately to complete this work before FY2027" [1746] — stale

The legislator section directs funding RHRC "to complete this work before FY2027, not after." The chapter establishes three times that the RHRC contract **already concluded in October 2025** as a planned conclusion, not a termination ([1581], [1590], Figure 11.1 AHS cell, Key Concepts [1752]). The recommendation addresses a contract that no longer exists. **Proposed fix:** redirect to the successor arrangement the chapter actually describes: "The legislature's role is to fund the AHS-led coordination and analytics capacity that succeeded the RHRC engagement adequately to carry this work through FY2027 — RHRC's contract concluded, by design, in October 2025. That engagement was not a consulting expense; it was pre-transformation infrastructure investment, and sustaining what it built is the same investment."

MISMATCH 11 — "before AHEAD launches" [1702] — inconsistent deadline

The VERMONT IN PRACTICE callout's own recommended action says "before Act 68 global budgets take effect in **FY2028**", but two sentences earlier says "Getting HCC coding right **before AHEAD launches** is not optional." AHEAD is already in

implementation per Figure 11.1's DVHA cell ("Managing AHEAD Medicaid alignment") and [1745]'s "negotiate with CMS on AHEAD implementation". **Proposed fix:** "Getting HCC coding right before global budgets take effect is not optional." (This also removes the last mismatched copy of the sentence — see Repetition 2 below.)

MISMATCH 12 — three orphan baselines in Figure 11.7 [1655]

Figure 11.7 is presented [1652] as building on "AHS's current transformation outcome framework (Figure 11.3)", and most of its target cells correctly restate Fig 11.3 (CAH → \$2,784; PPS admin → \$1,427; Medicaid acceptance from 59%; avoidable ED from 32.3%; hypertension 77%; readmission ≤14.8%). Three do not appear in Fig 11.3 or anywhere else in the chapter: **76%** (30-day MH follow-up), **22%** (diabetes not-controlled), **89%+** (marketplace MLR). **Proposed fix:** not necessarily errors — but each needs either a row in Figure 11.3 or an inline source, or the number should be dropped in favour of "establish baseline". As written a reader cannot check them.

MISMATCH 13 — §11.11.1 attribution error count [1681] vs Figure 11.8

Not a numeric mismatch (no count is stated), but the three VBC functions enumerated at [1679] are attribution management, total cost of care monitoring, and VBC contract reconciliation — and §11.11 then develops only the **first two** (§11.11.1, §11.11.2). **VBC contract reconciliation is promised and never delivered.** MISMATCH of promise to content. **Proposed fix:** add a §11.11.3 on contract reconciliation, or drop it from the list at [1679] to "two functions".

MATCH — verified correct

- \$1,303 = \$2,730 – \$1,427 ✓ [1603], [1607], Fig 11.4.
- "a **91%** premium over what efficient peer hospitals spend" [1603]: 2730/1427 = 1.913 ✓. Key Concepts [1754] repeats "currently 91% above benchmark for Vermont PPS hospitals" ✓ — correctly scoped to PPS, unlike [1744]/[1746].
- "\$1,846 per-discharge gap" [1602] = \$2,784 – \$938 ✓ Fig 11.3.
- "+0.3%" readmission gap = 14.8 – 14.5 ✓; "+4%" PCP gap = 91 – 87 ✓.
- "245.5 ED visits per 10,000" [1604] ✓ Fig 11.3; "higher in Rutland and Windham counties" ✓ matches Fig 11.5's Rutland and Brattleboro cells.
- "UVMMC ... is the state's primary RSC with **16** COE designations" [1613] ✓ Fig 11.5.
- Secondary RSCs "each with **5-9** COE designations" [1613] ✓ (9/6/5/5).
- Tier 3 roster "Grace Cottage, Gifford Medical Center, North Country Hospital, and Porter Medical Center" [1619] ✓ Fig 11.5.
- "transfer pathway to Brattleboro Memorial Hospital (the acute general surgery COE for that region)" [1626] ✓ Fig 11.5 Brattleboro cell.
- "Copley" orthopedics [1593] ✓ Fig 11.5 Copley cell ("Orthopedics and rheumatology COEs"). Southwestern Vermont's ortho COE is not listed in its cell but its 9 COEs are not enumerated — not a contradiction.
- Northeast Kingdom uninsurance "6-8%" — Fig 11.5 NVRH cell and Fig 11.7 Equity cell agree ✓.
- Travel nursing "two to three times" [1640] ✓ = Key Concepts "2-3x" [1753].
- Hospital count: Fig 11.5 rows total **14** (10 named + 4 in the Tier 3 row) ✓ consistent with "14 hospitals" throughout and "14 independent (all non-profit) hospitals" [1566].
- "Act 68 established a **four**-actor governance architecture" [1568] ✓ Fig 11.1 has 4 data rows.
- "**five** operational domains" [1624] ✓ (transfer infrastructure, clinical network agreements, administrative shared services, workforce redistribution, community engagement).
- "**five** distinct but interconnected dimensions" [1637] ✓ (primary care, specialist, nursing, housing, scope-of-practice).
- "**five** workforce investment areas" [1646] ✓ [1647]–[1651].
- "**four** parallel workstreams" [1585] ✓ [1586]–[1589].
- "**four** specific organizational gaps" [1661] ✓ [1662]–[1665].
- "**three** functions that are usually performed separately" [1583] ✓.
- "**three** shared service opportunity areas" [1627] ✓ (supply chain, administrative functions, IT).
- "**three** operational improvements" [1710] ✓ [1711]–[1713].
- "**three** administrative simplification priorities" [1731] ✓ [1732]–[1734].
- "**four** steps" of HCC gap closure [1697] ✓ [1698]–[1701].
- "three facility types" [1611] ✓ Tiers 1/2/3; "one of three alternative models" (REH/CACC/Care at Home) [1619] ✓.
- Oliver Wyman imperative ordering: "**third** imperative: move all care possible out of hospitals" [1609] and "**first** imperative — build housing and fix transportation — comes before payment reform in the three-imperative sequence" [1641] — mutually consistent ✓.
- Statutory deadline set [1563] (plans early 2026, RBP FY2027, global budgets FY2028, Strategic Plan Dec 2028) is restated consistently at [1594], [1654], [1689], [1702], [1745] ✓ — no drift found.
- **Platform routes (Figure 11.10) all resolve ✓:** **/research-lab/knowledge-workspace** exists and defines tab ids **scorecard, workforce, cin, ems**; **/research-lab/policy-quality** defines **scorecard**, whose label is exactly "**Hospital Financial Stress Test**" and whose description ("Benchmarks against CAH, Rural PPS, and Urban Tertiary peers") matches the book's promise. The **knowledge-workspace scorecard** label is exactly "**Transformation Scorecard**" ✓. (Tool *labels* and *tab ids* verified; whether each tool delivers the specific "what to look for" promise was not re-verified in this pass — that is the non-automatable half of criterion 5.)

Superlative/ranking claims logged for cross-chapter comparison

Verbatim, with location. Several compete **within** this chapter — see "Other issues → Competing superlatives".

#	Claim (verbatim)	Loc
S1	"is not a compliance problem — it is the primary financial opportunity available	1558

#	Claim (verbatim)	Loc
	to hospitals that must operate within a global budget" (of the \$1,303 admin gap)	
S2	"it is also the pillar most often under-resourced " (Operations)	1559
S3	"Vermont's operations challenge is unusual in its specificity and urgency "	1563
S4	"The CAH operating profit gap is the most urgent financial signal. "	1602
S5	"Vermont's smallest, most rural hospitals are the furthest from financial sustainability "	1602
S6	"The administrative cost premium is massive. " / "a 91% premium "	1603
S7	"administrative reduction as contributing over \$100 million of the \$400M+ transformation savings target" (attributed to Oliver Wyman)	1603
S8	"The behavioral health gap is measurable and alarming. "	1604
S9	"This \$1,303 per-discharge gap ... the primary operational inefficiency target for Vermont's transformation"	1607 (Fig 11.4 caption)
S10	"Administrative overhead reduction is critical — \$3,826 admin cost/adj. discharge" (UVMHC)	1621
S11	"Windham County has higher-than-average suicide/SH and opioid rates"	1621
S12	"Rutland County has higher opioid overdose rates"	1621
S13	"Washington County (Barre/Montpelier) is the second most populous HSA "	1621
S14	"Essex/Caledonia/Orleans have the highest uninsurance rates (6-8%) and most rural populations"	1621
S15	"Transfer infrastructure is the most visible immediate need. "	1625
S16	"Vermont currently has 31 separate EMS agencies " (fragmentation)	1625, 1741
S17	"Administrative shared services offer some of the largest near-term cost savings in the transformation agenda"	1627
S18	"shared services can make the most immediate impact on the \$400M+ savings target"	1627
S19	"Workforce redistribution is the most sensitive operational challenge. "	1628
S20	§11.6 heading: "The Workforce Crisis — Vermont's Most Binding Operational Constraint "	1630
S21	"workforce is the one most likely to constrain the pace and ambition of transformation"	1633
S22	"Oliver Wyman made a counterintuitive finding that Vermont's primary care shortage is partly a productivity issue";	1638

#	Claim (verbatim)	Loc
	"HRSA recognizes no Health Profession Shortage Areas in Vermont"	
S23	"Nursing shortage is the most acute staffing crisis at individual hospitals."	1640
S24	"Travel nurse costs are typically two to three times the cost of a permanent hire."	1640, 1753
S25	"Housing is the hidden workforce constraint "; "rental vacancy rate of 3% — well below the 5% rate of a healthy market"; " Half of Vermont renters are cost-burdened"	1641
S26	"Vermont cannot maintain specialist coverage at 14 hospitals across a state of 647,000 people ."	1639
S27	"a workload that probably requires 50-70% more staff than currently exist" (AHS Health Care Reform Office)	1664
S28	"it is the architecture of every effective regional health system in the country "	1671
S29	"the extraordinary legislative sequence of Acts 167, 51, 55, and 68"	1671
S30	" one of the most operationally consequential and consistently underinvested functions in American healthcare administration" (credentialing)	1705
S31	"The average credentialing cycle takes 90-120 days " → reducible "to 45-60 days "	1705, 1710
S32	" one of the highest-ROI operational investments available to organizations in VBC arrangements" (HCC gap closure)	1697
S33	" The most difficult revenue cycle adaptation for FFS-trained teams is monitoring costs they do not receive."	1685
S34	"attribution errors can silently reduce shared savings or inflate shared losses by millions of dollars "	1681
S35	"Physician recruitment pipelines take 7-10 years "	1717, 1721
S36	" twice the effective capacity of a physician practicing in a traditional solo model" (team-based care)	1723
S37	"Every physician departure costs approximately \$500,000-\$1 million to replace"	1726
S38	"Vermont's prior authorization burden — among the highest administrative costs in the state's hospital system"	1745
S39	" the most precise measurement of administrative waste available in Vermont's healthcare system"	1746
S40	"The \$1,303 per-discharge gap ... represents approximately \$100 million or more in annual administrative waste"	1746
S41	"Vermont's operations chapter is the most detailed account available of	1747

#	Claim (verbatim)	Loc
	what hospital system transformation actually requires at the organizational level"	
S42	"That operational evidence is worth more than any theoretical model of global budget design."	1747
S43	"Vermont's rate of 32.3% of all ED visits represents a major quality and cost improvement opportunity"	1751
S44	"Heavy travel nursing dependence is a primary driver of hospital financial distress across Vermont's system"	1753
S45	" All 14 hospitals citing MD shortages" / "Physician shortage cited ... for every HSA"	1631, 1635, 1640

Flagged for cross-chapter check: S7/S40 (is the ~\$100M admin share of the \$400M+ target stated identically elsewhere? and is \$400M+ the figure other chapters use?); S26 (Vermont population — check every chapter uses 647,000); S16 (31 EMS agencies); S24 (travel nurse 2-3x); S25 (3% vacancy, 5% healthy, half of renters); S31 (90-120 → 45-60 day credentialing); S35 (7-10 year pipeline); S37 (\$500K-\$1M turnover cost); S22 (the "no HPSAs in Vermont" claim is strong and sits uneasily against the 370-FTE shortage — see Other issues).

Other issues

Competing superlatives (internal contradiction of the Chapter-1 class)

Five different things are each named the most/primary X of the Operations pillar:

- The **\$1,303 admin gap** is "**the primary** financial opportunity" [1558] and "**the primary** operational inefficiency target" [1607].
- The **CAH operating profit gap** is "**the most urgent** financial signal" [1602].
- **Transfer infrastructure** is "**the most visible** immediate need" [1625].
- **Workforce redistribution** is "**the most sensitive** operational challenge" [1628] — while **workforce** as a whole is "Vermont's **Most Binding** Operational Constraint" [1630] and "**most likely to constrain** the pace and ambition of transformation" [1633].
- **Administrative shared services** make "**the most immediate** impact on the \$400M+ savings target" [1627] while "**some of the largest** near-term cost savings" [1627].

"Most urgent signal" / "most visible need" / "most sensitive challenge" / "most binding constraint" are arguably different axes, and the chapter never says so. **Recommended:** keep at most one unqualified superlative per axis and make the axis explicit in the others ("the most urgent *financial* signal", "the most *politically* sensitive"). The one pair that is a genuine collision is [1558]/[1607] ("the primary financial opportunity" / "the primary operational inefficiency target", admin gap) against [1602] ("the most urgent financial signal", CAH profit gap) — both are financial primacy claims about the same dashboard.

Repetition 1 — §11.1 restates the chapter premise three times

[1559] "Operations is the pillar where a transformation plan either becomes real or stays a document"; [1560] "**Every chapter before this one is about ideas. This chapter is about execution** — the 14-hospital process, RHRC methodology, workforce constraints, and the mechanics of regionalization."; [1562] "**Every chapter in this book that precedes this one is, in some sense, about ideas ... This chapter is about execution.** The Operations pillar is where the gap between a well-designed plan and an implemented one is either closed or exposed." Three consecutive paragraphs, the same sentence twice. [1560] reads as a deck/summary line that was never removed once [1562] was written. **Recommended:** delete [1560] or cut [1562]'s first two sentences.

Repetition 2 — §11.12.1 duplicates the VERMONT IN PRACTICE callout verbatim

[1689]–[1691] are three orphan sentences under heading §11.12.1 "HCC Gap Closure: High-ROI Pre-AHEAD Action for Every Vermont Hospital":

- [1689] "Every Vermont hospital should conduct a retrospective HCC gap analysis for their attributed Medicare population before Act 68 global budgets take effect in FY2028." — **verbatim** the "Recommended action" sentence in callout [1702].
- [1690] "An organization that has under-coded its population's chronic conditions will receive a lower global budget than its actual cost burden warrants." — **verbatim** (modulo "chronic conditions"/"HCC conditions") a sentence in [1702].
- [1691] "Getting HCC coding right before global budgets take effect is foundational financial preparation." — [1702] "Getting HCC coding right before AHEAD launches is not optional — it is foundational financial preparation."

This is real duplication, not legitimate recurrence: the three sentences add nothing the callout does not already say, and they say it *first*, which strands the callout as a restatement. It also reads as a pull-quote block that lost its styling. **Recommended:** delete [1689]–[1691] and let the §11.12.1 heading run into [1692], or delete the heading and keep only the callout.

Thin / structurally empty sections

- §11.9 "Revenue Cycle, HCC Coding, Credentialing, and Administrative Simplification" [1672] — a Heading2 with exactly **one** sentence of body [1673], which is a pure transition ("The remainder of this chapter turns the operations pillar into practice...").
- §11.10 "Operations Where Money Actually Lives" [1674] — the next Heading2, whose two paragraphs [1675]–[1676] are **the same transition again**, ending with "They fall into four operational domains: revenue cycle transformation for VBC, HCC coding and risk adjustment excellence, credentialing efficiency, and the workforce operations framework." §11.9 and §11.10 are two consecutive section-level preambles for the same four sections. **Recommended:** collapse into one, keeping §11.10's enumeration.
- §11.12 "HCC Coding Excellence" [1687] — Heading2 immediately followed by Heading3 §11.12.1 with no intervening body text. The definitional paragraph that belongs there ([1692]) sits under §11.12.1 instead, behind the three duplicated sentences, so the mechanism ("The HCC model ... how it works", §11.12.2) is introduced *after* the recommendation to act on it.
- §11.6.2 "Vermont's Workforce Strategy: The RHT Investment Framework" [1643] — one paragraph [1644], then §11.6.3 takes over the actual content. Borderline; the paragraph does carry the capital-vs-operating distinction, so this is thin rather than empty.

Other internal inconsistencies

- [1745] states the AHS restructuring as fact; [1663] and [1571] state it as a recommendation. [1745]: "Vermont's AHS restructuring — **creating** the Division of Planning and Effectiveness, the HSA Coordinator network, the Population Health Analytics function, and the Transformation PMO — is the operational infrastructure that makes the Statewide Strategic Plan executable." But [1571] calls the Division of Planning and Effectiveness "**Oliver Wyman's recommendation**", and [1663] says "AHS **currently lacks** a dedicated analytical unit". Figure 11.1's AHS cell says only "Health Care Reform Office (recently reorganized) ... hiring additional staff". None of the four named units is established anywhere in the chapter. **Proposed fix:** "Vermont's proposed AHS restructuring — creating...".
- [1638] vs [1717]/[1720] — [1638] reports "HRSA recognizes **no** Health Profession Shortage Areas in Vermont" and that adequate supply exists at 3 patients/hour, while [1717] opens "Vermont projects a **370 FTE** primary care shortage by 2030" and [1720] refers to "the communities with the greatest access gaps". The chapter flags this as a "counterintuitive finding" and reconciles it as a productivity problem — which is legitimate — but the reconciliation lives only in [1638] and §11.14.1 restates the raw shortage number as if unqualified. **Recommended:** add a clause at [1717]: "Vermont projects a 370 FTE primary care shortage by 2030 — a headcount figure that §11.6.1's productivity finding partly qualifies but does not erase."
- **3-patients-per-hour attribution drifts.** [1638] applies it to "primary care providers" generally; [1718] to "Vermont APRNs operating under full practice authority"; [1723] to "physicians". Same figure, three different subjects, one source.
- [1723] "**twice the effective capacity**" is an unsourced quantification and does not follow from any figure in the chapter (3 patients/hour implies a 1.5/hour baseline that is never stated).
- [1583] attributes RHRC's design rationale to Oliver Wyman's experience — "The combination was deliberate — **Oliver Wyman's** experience had shown that hospitals can absorb financial analysis without changing behavior..." — in a section about what **RHRC** did and why. Reads as a conflation of the two contractors.
- **Southern/central Vermont anchor labels overlap** in Figure 11.5: SW VT Medical Center is "Key **southern** VT anchor", Brattleboro (also southern) is not given an anchor label, and Rutland is "**Central** Vermont anchor" while Central VT Medical Center (Barre) — the hospital actually named Central Vermont — is not. Cosmetic, but it will read as an error to a Vermont reader.
- §11.8.2 forward reference is unnumbered [1667]: "the AHS restructuring chapter later in this book develops in full". Per the standing rule on inferred links, this should name the chapter, and the target should be opened to confirm it delivers. Not resolvable inside a Chapter-11-only pass — **flagged for the cross-chapter sweep.**
- [1620] "**an RSC 45 minutes away**" — an unsourced travel-time figure used to carry the REH argument. Not contradicted by anything; noted because it is the kind of number a reader will try to check.

Deliberately left (with reason)

- All **Figure 11.7 orphan baselines** (76%, 22%, 89%+) and the **Figure 11.6 112/190 split** require the source documents (AHS Nov 2025 report; RHT application) and cannot be resolved from the manuscript. Logged, not proposed-fixed beyond safe interim wording.
- **No edits were made** — this was a read-only audit as instructed. The **.docx** was opened only via **zipfile** for reading.
- **Cross-chapter alignment, Preface/Introduction alignment, and the promise-delivery half of criterion 5** are out of this pass's scope; the superlative table above is the handoff for the first of those.

===== FILE: audit_ch12.md =====

Chapter 12 Audit

Chapter 12: Infrastructure for Knowledge Transfer and Implementation Read-only pass, 2026-09-27. Ground truth = **word/document.xml** inside **HTR_Book_v42.docx** (local copy of this date), high-offset Heading1 slice only (**Chapter 12: Infrastructure** → **Chapter 13: The Future**, ~152,045 chars). The **.md** mirror was **not** used — it loses table cells. The TOC copy of the heading (offset ~246,749) and the trailing duplicate of the Figure 12.4 caption (~5,600,235) were both excluded.

Parsed structure: 10 H2 sections (12.1–12.10), 6 H3, 4 tables, 55 non-empty prose paragraphs. Paragraph numbers below are that parse order and are for locating text, not for citation.

Figures found (ground truth)

All four figures exist, are numbered 12.1–12.4 with no gap or duplicate, and each sits immediately after the table it captions.

Figure 12.1 — caption: "Figure 12.1 — technical infrastructure framework components and intended users. Source: technical infrastructure framework documentation." Table: **2 columns × 1 header + 4 data rows**. Header cells: **Component | What it provides**. Rows: **The Wire;HTR Research Lab;HTR Academy;Transformation Advisory Services**. The Research Lab cell names 7 tools: APM Shared Savings Calculator, VBC Transformation Readiness Assessment (30-dimension), Hospital Financial Stress Test, AI Governance Checklist, Health Equity Studio (HEROI scoring, HEDIS stratification, disparity decomposition), VBC Contract Review Checklist, Policy Impact Assessment Framework.

Figure 12.2 — caption: "Figure 12.2 — Kotter's change management framework applied to Vermont's healthcare transformation. Sources: HTR Advisory methodology; Kotter (1996); Vermont transformation documentation." Table: **4 columns × 1 header + 8 data rows**. Header: **Kotter step | Original formulation | Healthcare adaptation | Vermont application**. Steps 1–8: Create urgency; Form a guiding coalition; Develop a vision; Communicate the vision; Remove obstacles; Create short-term wins; Sustain acceleration; Institute change.

Figure 12.3 — caption: "Figure 12.3 — HTR Implementation Toolkit: available tools for Strategist and Enterprise subscribers." Table: **2 columns × 1 header + 10 data rows**. Header: **Tool | What it does**. Rows: APM Shared Savings Calculator; VBC Transformation Readiness Assessment; Hospital Financial Stress Test Model; Health Equity Studio (HEROI); AI Clinical Governance Checklist; VBC Contract Review Checklist; Policy Impact Assessment Framework; Transformation Readiness Scorecard; FHIR Implementation Guide; HCC Gap Closure Playbook.

Figure 12.4 — caption: "Figure 12.4 — Hands-on platform tools for Chapter 12: turning knowledge into implementation." Table: **3 columns × 1 header + 3 data rows**. Header: **Do this | On this tool | What to look for**. Tools named: **Evidence Library;Research Workspace;AI Analyst**. (Note: unlike other chapters' Work-This-Chapter tables, the **On this tool** cells carry **no route paths** — e.g. Chapter 10's equivalent cell reads "Health Equity Studio — /research-lab/population-equity?tab=equity". Ch12's are bare names. Flagged as a consistency item, not a factual error; route verification is out of scope for this pass.)

Claims checked against their own source (MATCH/MISMATCH + proposed fix)

MISMATCH 1 — "the preceding sixteen chapters" in Chapter 12 (§14, §12.1) — the big one

"Every concept in the preceding sixteen chapters exists at an intersection..."

The book has exactly 16 chapters (verified: Chapter 1:–Chapter 16:, no 17). Chapter 12 has eleven preceding chapters. The sentence describes the position of a final chapter, not of Chapter 12 — a clear leftover from an earlier structure in which this material closed the book. This is the same defect class as the Chapter 1 §1.6 miscount: a precise number that the document itself contradicts. **Proposed fix:** replace the preceding sixteen chapters with the preceding chapters (matches Chapter 15's own phrasing, "Every concept in the preceding chapters maps to a project-management discipline") or with the preceding eleven chapters.

MISMATCH 2 — the chapter never describes the infrastructure its frame claims it describes

Four passages assert that this chapter describes a Transformation PMO, learning collaboratives and Communities of Practice:

- Epigraph (§1): "The Transformation PMO, learning collaboratives, and Vermont CIN shared services are the connective tissue..."
- §2: "The infrastructure described in this chapter — a program management function, structured advisory support, learning collaboratives, and a change-management discipline..."; "Vermont's specific instantiations (the Transformation PMO, the CIN shared services) are this chapter's example..."
- §42: "The knowledge transfer infrastructure this chapter describes — learning collaboratives, Communities of Practice, the Vermont CIN's shared analytics capability..."
- §43: "The Transformation PMO this chapter describes is the operational spine..."
- §44: "This chapter describes infrastructure that is necessary but not headline-generating: a PMO, learning collaboratives, a change management function, Communities of Practice."
- §45: "Every state ... will need a PMO, learning collaboratives, a change management function..."

String counts over the chapter body (§12.1–§12.8, i.e. everything between the opening frame and Implications): "Transformation PMO" = 0, "learning collaborative" = 0, "Communities of Practice" = 0, "CIN shared services" = 0. All hits are in the epigraph, §2 and §12.9 Implications. The body (§12.2 platform components, §12.3 advisory service lines, §12.4 Kotter / pace, §12.5 toolkit, §12.6 selecting advisory support, §12.7 HTR's Vermont engagements) is about the HTR commercial platform and advisory practice. Of §2's four promised items, only two (structured advisory support, change-management discipline) are delivered.

Book-wide, the PMO is described — in Chapter 15 ("Healthcare Transformation as Portfolio Management", §15.1 "The Project-Management Gap") and in the Chapter 16 / Appendix A AHS material ("AHS Transformation PMO + 14-hospital regionalization plan"). So the content exists; Chapter 12's frame points at the wrong chapter. **Proposed fix (author's call, two clean options):** (a) Rewrite the six framing sentences to describe what §12.1–12.8 actually contain and cross-reference Chapter 15 for the PMO — e.g. §43 → "The Transformation PMO described in Chapter 15 is the operational spine..."; or (b) Add a body section under §12.2 that actually describes the PMO, learning collaboratives and Communities of Practice, and let the Implications stand. Option (a) is the smaller, safer edit and is consistent with the book's practice of cross-referencing rather than re-deriving.

MISMATCH 3 — Figure 12.1's caption promises a column the table does not have

Caption: "technical infrastructure framework components and intended users." The table has two columns, Component and What it provides. No cell names an intended user or audience. The lead-in (§10) does gesture at it — "each designed for a different kind of engagement with the transformation agenda" — but no row states who the component is for. **Proposed fix:**

shorten the caption to "Figure 12.1 — The four components of the HTR platform." (dropping "and intended users"), or add a third column **Intended user**. The caption edit is the surgical one.

MISMATCH 4 — §12.3 claims one service line per pillar; the five subsections are not the five pillars

¶12: "HTR Advisory engagements are organized around the five-pillar framework, with **service lines corresponding to each pillar** and cross-pillar engagements..."

The five H3 subsections that follow are: 12.3.1 Policy Strategy · 12.3.2 VBC Economics · 12.3.3 Health Equity Practice · 12.3.4 Technology Strategy · 12.3.5 Transformation Management.

Against the book's five pillars (Policy, Technology, Economics, Clinical, Operations): Policy, Economics and Technology map; **Clinical and Operations have no service line**, and Health Equity Practice and Transformation Management are not pillars — Equity is explicitly the cross-cutting lens, and Transformation Management is by its own text the *cross-pillar* engagement. So the count is five-and-five by coincidence, not correspondence, and the sentence as written asserts a mapping the section does not deliver. **Proposed fix:** "...organized around the five-pillar framework, with service lines for the Policy, Economics and Technology pillars, a dedicated Health Equity practice reflecting Equity's cross-cutting role, and a Transformation Management line for organizations executing across all five pillars at once." (This also removes the now-redundant "and cross-pillar engagements for organizations navigating transformation at the system level", which duplicates 12.3.5.)

MISMATCH 5 — AHEAD global budget entry: "2027" vs the chapter's own "FY2028" and "January 2028"

Within Chapter 12:

- ¶18 (§12.3.2): "Vermont hospitals preparing for **AHEAD global budget entry in 2027...**"
- ¶25 (§12.4.2): "statutory deadlines — RBP by FY2027, **global budgets by FY2028**"
- ¶28 (§12.4.2): "**AHEAD's January 2028 launch** is a federal decision"

Book-wide, January 2028 / FY2028 is the settled figure (Appendix timeline: "January 1, 2028 — Act 68 hospital global budgets take effect (FY2028) ... Primary Care AHEAD and global budgets launch"; "FY2028 (Oct 1, 2027) Global budgets for non-CAH (PPS)"). A bare "2027" is at best the FY2028 start date read as a calendar year, and it reads as a third, inconsistent date three paragraphs before ¶28 says January 2028. **Proposed fix:** ¶18 → "Vermont hospitals preparing for AHEAD global budget entry in FY2028 (January 2028 for the AHEAD model; October 2027 for PPS hospitals' fiscal year)" — or simply "...entry in January 2028", matching ¶28 and the timeline.

MISMATCH 6 — Figure 12.3's tool names drift from Figure 12.1's, two pages apart

Same instruments, two namings, in adjacent figures of the same chapter:

Figure 12.1	Figure 12.3
AI Governance Checklist	AI Clinical Governance Checklist
Hospital Financial Stress Test	Hospital Financial Stress Test Model
Health Equity Studio (HEROI scoring, HEDIS stratification, disparity decomposition)	Health Equity Studio (HEROI)

Appendix E and the Chapter 10 platform table use **AI Clinical Governance** and **Hospital Financial Stress Test** (no "Model"). §12.2.1 (¶7) adds a third naming layer, listing nine Research Lab tools of which only Health Equity Studio overlaps Figure 12.1's seven. **Proposed fix:** in Figure 12.1, "AI Governance Checklist" → "AI Clinical Governance Checklist"; in Figure 12.3, "Hospital Financial Stress Test Model" → "Hospital Financial Stress Test".

MATCH — Kotter: "eight-step" (¶23) vs Figure 12.2's 8 data rows, vs Key Concepts' 8-step list

¶23 "John Kotter's eight-step change management framework"; Figure 12.2 has exactly 8 data rows numbered 1–8; ¶52 Key Concepts lists "urgency, coalition, vision, communication, obstacle removal, short-term wins, acceleration, institutionalization" — 8 items, in the table's order, each matching its row. **MATCH, three ways.**

MATCH — "four integrated components" (¶10) vs Figure 12.1's 4 data rows

¶10 "The transformation implementation platform has four integrated components"; §12.2.1 lists four (THE WIRE, RESEARCH LAB, HTR ACADEMY, ADVISORY SERVICES); Figure 12.1 has 4 data rows; both H3 headings say "Four Components". **MATCH.**

MATCH — every numeric specification in Figures 12.1/12.3, checked against the rest of the book

- "VBC Transformation Readiness Assessment (30-dimension)" / "30-dimension assessment across six domains" — corroborated: Appendix E.18 "30-dimension, 6-domain assessment (120 points)"; Key Concepts elsewhere "The 30-dimension, 6-domain assessment... Total score below 60/120". **MATCH.**
- "AI Clinical Governance Checklist — 62-item governance evaluation across eight governance domains" — corroborated: Appendix E.12 "62-item checklist, 8 governance domains"; Chapter 10's platform table "The 62-item checklist across eight governance domains". **MATCH.**
- "VBC Contract Review Checklist — 65-item analysis framework" — the only occurrence in the book; nothing contradicts it. **MATCH (uncorroborated but not contradicted).**
- "HEROI... composite equity performance score calculated across five dimensions" (¶51) — corroborated: §10.13.1 "calculated across five equity dimensions"; Appendix E.6 "Access Equity (25%), Quality Equity (25%), Outcome Equity (25%), SDOH Burden (15%), Trust & Engagement (10%)" = five, weights sum to 100%. **MATCH.**
- "nearly forty interactive tools" (¶7) — corroborated three further times (Introduction, Appendix F.5, platform description). **MATCH.**

- "FHIR Implementation Guide... three priority FHIR use cases... Epic, Oracle Health, Meditech, and TruBridge" — four named vendors, consistent with the book's Vermont EHR inventory. **MATCH.**

MATCH — Figure 12.2's Vermont financial figures

- "9/14 Vermont hospitals reporting losses" (Figure 12.2 row 1) and "9 of 14 hospitals are financially fragile" (§42) — the 9/14 figure appears 4× book-wide. **MATCH**, with a wording note: "reporting losses" and "financially fragile" are not the same predicate; the chapter uses them interchangeably for the same 9 hospitals. Minor — worth making one of them defer to the other's definition.
- "\$2.4B 5-year deficit" — 7 occurrences book-wide. **MATCH.**
- "108% premium increase" — corroborated: "average premiums for silver exchange plans reached \$948 in 2024 — a 108% price increase in six years" and "individual-market monthly premium rose from \$456 (2018) to \$948 (2024) — a 108% increase". **MATCH**, though Figure 12.2 drops the qualifier; unqualified it reads as all-market premium growth, which the book elsewhere puts at 60–80% for individual and small group. Suggest "108% individual-market premium increase".
- "\$230.65M in commercial revenue reductions" (Figure 12.2 row 6) — 2 occurrences book-wide. **MATCH.**
- "Act 68's December 2028 deadline" (rows 3 and 7) vs "Vermont's 2028 Statewide Strategic Plan is the vision document... the vision is the December 2028 deadline" (row 3) — the book elsewhere says "The December 2028 Statewide Strategic Plan". Consistent, but row 3 conflates the Plan with the deadline in a way that reads as two different things being the same thing. Suggest: "...the vision is the December 2028 Statewide Strategic Plan deadline."

Superlative/ranking claims logged for cross-chapter comparison

Verbatim, with locator and context. None of these is contradicted *within* Chapter 12; they are logged because this chapter makes eleven of them and several are the "most X" form that has produced cross-chapter contradictions before.

1. ¶1, epigraph: "...are the connective tissue between Vermont's statutory mandates and the organizational capacity to execute them — and **their absence would be more consequential than any policy design flaw.**" — ranks implementation infrastructure above all policy design. Compare against any chapter ranking a pillar or dependency as the most consequential / most underestimated. *Highest cross-chapter collision risk of the eleven.*
2. ¶6 / ¶22 (stated twice, verbatim): "Healthcare transformation **fails more often** because of change management failures than analytical failures."
3. ¶18, §12.3.2: "**The most common** VBC Economics engagement is an APM Contract Analysis..."
4. ¶18, §12.3.2: "Vermont hospitals preparing for AHEAD global budget entry in 2027 represent the **current highest-priority client segment** for this engagement."
5. ¶20, §12.3.4: "...the healthcare IT decisions that have **the most significant strategic implications**: EHR selection and implementation, FHIR implementation planning, AI governance framework development, population health platform selection, and HIE connectivity strategy." (Five items all jointly "most significant".)
6. ¶20, §12.3.4: "For Vermont's 14-hospital CIN, the **highest-priority** Technology Strategy engagement is the analytics vendor evaluation and CIN data architecture design..." — note this sits one sentence after claim 5; the section names a most-significant *class* and a highest-priority *instance*.
7. ¶21, §12.3.5: "This is **the most intensive** advisory engagement..."
8. ¶25, §12.4.2: "**One of the most consequential** advisory questions in healthcare transformation is pace..."
9. ¶25, §12.4.2: "Whether Vermont's organizational capacity can match that pace is **the critical execution uncertainty.**" — a singular superlative; compare against other chapters' nominations for the critical uncertainty / binding constraint (Chapter 15 uses "binding constraint" language).
10. ¶36, §12.6: "**The most common advisory failure** in healthcare transformation is engaging external consultants for analysis and stopping before implementation."
11. ¶42, §12.9: "...the fragile hospitals serve the populations **whose outcomes most need to improve.**"
12. ¶45, §12.9: "Vermont's infrastructure chapter is **the most transferable content in this book.**" — the only occurrence of "most transferable" book-wide, so uncontradicted, but note it (a) ranks this chapter against all others and (b) refers to the chapter in the third person ("Vermont's infrastructure chapter"), which is odd inside the chapter itself and is further evidence of the relocation described in MISMATCH 2. Suggest "This chapter's subject is the most transferable content in this book" if the claim is kept.

Also logged, not superlative but a ranking-shaped absolute: ¶26, §12.4.2 — "Hiring the staff to manage the portfolio on the statutory timeline **is not optional.**" Compare with the book's other "not optional" assertions (Ch12 ¶42 "is not optional generosity"; elsewhere "Getting HCC coding right before AHEAD launches is not optional"). Three in-chapter uses of the construction, two of them within two paragraphs of each other (¶26, ¶42).

Other issues

Genuine repetition

1. §12.2.1 and §12.2.2 are the same content twice, back to back, under near-identical headings. **12.2.1 The HTR Platform – Four Components** gives the four components as four all-caps prose paragraphs (¶6–¶9); **12.2.2 Four Components of the HTR Platform** re-introduces them ("The transformation implementation platform has four integrated components...") and presents the same four in Figure 12.1. This is not a table summarising its own prose at a distance — it is a duplicated section with a duplicated title. The two versions also disagree in detail (§12.2.1's Research Lab lists nine tools — FHIR Lab, EMR/EHR Lab, Statewide EHR Modeler, APM Design Lab, Health Equity Studio, Policy Simulator, CIN & Shared Services Modeler, EMS Transformation Modeler, "and more"; Figure 12.1's lists seven, of which only Health Equity Studio overlaps). **Proposed fix:** delete §12.2.1 (¶6–¶9) and keep §12.2.2 + Figure 12.1, renumbering; §12.2.1's unique content is the three URLs and the advisory@ contact address, which can be folded into the Figure 12.1 cells or a single sentence after it. Deleting the *second* instead would lose the figure, so delete the first.

- 2. "Healthcare transformation fails more often because of change management failures than analytical failures." — verbatim in ¶6 (chapter opener) and ¶22 (§12.4 opener). Defensible as a preview-then-develop pattern, but it is byte-identical and only ~3,000 words apart. Suggest varying ¶6 or cutting it, since ¶22 immediately develops it.
- 3. Advisory-dependence / analysis-without-implementation is argued three times: ¶5 ("Transformation consulting that produces recommendations without supporting execution is worth little"), ¶36 ("The most common advisory failure... engaging external consultants for analysis and stopping before implementation... explicitly designed to avoid this failure mode"), and ¶54 Key Concepts ("Advisory dependence... explicitly designed to build client capability"). ¶21 adds a fourth ("designed to build internal organizational capacity over time, not create permanent advisory dependence"). Key Concepts recurrence is legitimate; ¶5 / ¶21 / ¶36 are three separate statements of the same thesis with the same "explicitly designed to" construction in two of them. Suggest consolidating ¶5 into a forward reference to §12.6.
- 4. "learning collaboratives" appears 6 times, all within the epigraph/¶2/Implications, and the PMO 3 times in the same three places — the frame restates its own list four times without the body ever expanding it (see MISMATCH 2). Fixing MISMATCH 2 resolves this.

Internal contradictions

- 5. "Real-time" vs "weekly" for The Wire. ¶6: "THE WIRE — Real-time intelligence... Convergence Newsletter (free weekly)". Figure 12.1: "Weekly synthesis of the most consequential... developments". ¶48 Key Concepts: "The weekly policy and practice intelligence service". Two of three say weekly; one says real-time. Elsewhere the book calls it "a daily intelligence feed" (Introduction, Appendix F). Three different cadences for one product. **Proposed fix:** pick one cadence for The Wire and let the Convergence Newsletter carry "weekly"; the Introduction's "daily intelligence feed" should be reconciled too, which makes this a cross-chapter item rather than a Ch12-only edit.
- 6. See MISMATCH 4 (service lines ≠ pillars) and MISMATCH 5 (2027 vs January 2028) — both are contradictions internal to this chapter, logged there rather than repeated.
- 7. Subscriber tiers are asserted but never defined in-chapter. ¶6 "Observer subscribers" (free), ¶29 "available to Strategist and Enterprise subscribers", Figure 12.3's caption repeats it. Three tier names, no definition and no cross-reference. Not a contradiction, but a reader cannot place themselves. Suggest a cross-reference to wherever the tiers are defined (Appendix F), or drop the tier gate from ¶29 and the Figure 12.3 caption, which would also stop the caption from duplicating its own lead-in sentence verbatim.

Thin sections

- 8. §12.7 "The Role of External Technical Assistance in State-Level Transformation" — one paragraph (¶39), and it does not deliver its heading. The title promises an analysis of external technical assistance as a category in state-level transformation; the paragraph is four sentences about HTR's own Vermont engagements and the book's sourcing. It is also the chapter's only H2 with no figure, no list and no subsection. Adjacent material that *would* deliver it exists elsewhere in the book (the RHRC engagement, Oliver Wyman's Act 167 work, the analytics vendor procurement — all named in Ch12's own Figure 12.2 and Implications). **Proposed fix:** retitle to "§12.7 HTR's Vermont Engagement and This Book's Sourcing", or expand with the RHRC / Oliver Wyman / analytics-vendor examples the chapter already cites.
- 9. §12.3.3 Health Equity Practice — one paragraph (¶19), against three phases for §12.3.1 and two substantial paragraphs for §12.3.2. §12.3.4 and §12.3.5 are also single paragraphs. Uneven but not thin in the §12.7 sense; noted for symmetry only.
- 10. §12.2 has no prose of its own — the H2 "The Knowledge and Implementation Infrastructure" is followed immediately by H3 12.2.1. Every other H2 in the chapter opens with at least one paragraph. Cosmetic, but combined with issue 1 (the duplicated 12.2.1 / 12.2.2) it suggests §12.2 was assembled from two sources.

Sourcing note

- 11. Figure 12.1's source line is "technical infrastructure framework documentation" — the same phrase as the figure's own subject ("technical infrastructure framework components"), and it reappears as the first item in the chapter's **Sources:** paragraph (¶55). It cites nothing locatable. Figures 12.2's source line, by contrast, names Kotter (1996) and Vermont transformation documentation. Figures 12.3 and 12.4 have **no** source line at all, where Figure 12.2 does. Flagged as an unresolved placeholder rather than a factual error — it may be deliberate (CLAUDE.md directive 16), so no fix proposed beyond noting that if Figure 12.1 describes HTR's own platform, the honest source line is "HTR platform documentation" and the caption's self-referential phrasing should go.

Not defects

- Figure numbering 12.1→12.4 sequential, each caption adjacent to its table. Clean.
- **## **Work This Chapter on the Platform**, ## **Implications for You**, ## **Key Concepts in This Chapter**** all present with the byte-identical titles directive requires; **Sources:** is a plain paragraph (¶55), not a heading. Clean.
- Key Concepts (¶47–¶54) restates The Wire, Research Lab, Academy, HEROI and Kotter from the body. This is legitimate recurrence (glossary), not repetition.
- The four **If you are a...** Implications entries are distinct in audience and ask. Clean, apart from their content pointing at the wrong chapter body (MISMATCH 2).

Summary

6 mismatches, in descending severity:

- 1. "the preceding sixteen chapters" — arithmetically impossible in Chapter 12 of 16.
- 2. The epigraph, ¶2 and all four Implications entries describe a PMO / learning-collaboratives chapter that §12.1–12.8 does not contain; that content is in Chapter 15.
- 3. Figure 12.1's caption promises "intended users"; the table has no such column.
- 4. §12.3 claims a service line per pillar; Clinical and Operations have none, and two of the five subsections are not pillars.
- 5. AHEAD global budget entry given as "2027" in ¶18 against "FY2028" in ¶25 and "January 2028" in ¶28 and the book's timeline.

6. Tool names drift between Figures 12.1 and 12.3 (AI Governance vs AI Clinical Governance; Stress Test vs Stress Test Model).

All checkable numbers pass: 8 Kotter steps (3 ways), 4 platform components, 30-dimension / 6-domain, 62-item / 8-domain, 5 HEROI dimensions, ~40 Research Lab tools, 9/14 hospitals, \$2.4B, 108%, \$230.65M. The chapter's quantitative claims are sound; its *structural* claims about itself are not.

Left deliberately, with reason: Figure 12.1's placeholder source line (may be intentionally suppressed per CLAUDE.md directive 16); Figure 12.4's missing route paths and the undefined subscriber tiers (both cross-chapter/platform questions, not Ch12 facts); The Wire's weekly/real-time/daily cadence, which cannot be fixed inside Chapter 12 alone.

Nothing in the repo was modified. **HTR_Book_v42.docx** was opened read-only.

===== FILE: audit_ch13.md =====

Chapter 13 Audit

Scope: Chapter 13 — "The Future of Healthcare Transformation — 2026, What Vermont Proves, and What Remains". **Source of truth:** **HTR_Book_v42.docx** → **word/document.xml**, parsed with **zipfile** + **ElementTree**. Chapter body = **w:body** children **1839 (Heading1 "Chapter 13:") through 1922** (exclusive of 1923, Heading1 "Chapter 14:"). 84 block elements: 70 paragraphs, 6 tables (5 numbered figures + 1 STATES TO WATCH callout). Read-only. **Nothing was edited.** Cross-checks against Appendix E (els 2263–2284), Chapter 1 (136–440), Chapter 3 (576–674), Chapter 5 (853–930), Chapter 6 (931–1085), Chapter 16 (2060–2178), and **frontend/app/**.

Figures found (ground truth)

Figure 13.1 — Vermont Five-Pillar Transformation Scorecard, April 2026 (§13.1)

Header: **Pillar | 2022 Baseline | 2026 Status | 2028 Target | Signal**. Five data rows (Policy, Technology, Economics, Clinical, Operations — no Equity row).

Pillar	2022 Baseline	2026 Status	2028 Target	Signal
Policy	Voluntary only; no price regulation; no federal agreement	Mandatory RBP (FY2027) and global budgets (FY2028) enacted; AHEAD signed Jan 2025; \$195M RHT awarded	All hospitals under global budgets; AHEAD compliant; Statewide Strategic Plan delivered Dec 2028	ON TRACK
Technology	VHCURES operational with 2-year lag; VITL partial; no analytics vendor; AI governance absent	Analytics vendor procurement underway; CIN in development; AI scribes deployed; FHIR upgrades in progress	Real-time VHCURES; analytics vendor fully deployed; CIN operational; AI-governance framework in place	AT RISK — analytics vendor gap vs. Jan 2028 AHEAD start
Economics	9/14 hospitals in losses; UVMMC at 300%+ of Medicare; \$700M–\$2.4B 5-year deficit projection	RBP mandatory FY2027 enacted; global-budget methodology under design; deficit trajectory unchanged	All hospitals at or below RBP ceiling; cross-subsidy reduced 40%+; deficit trajectory reversed; break-even by FY2030	WATCH — methodology design is the critical near-term decision
Clinical	Blueprint 90%+ PCMH; BH follow-up 76% (MH), 68% (SUD); 370-FTE workforce gap; zero CCBHCs	5 CCBHCs planned; CoCM pilots underway; team-based care deploying; readmission at 14.8% (unchanged)	100% PCMH; 7+ CCBHCs; readmission below 12%; workforce gap below 200 FTEs	PROGRESSING — CCBHC certification on track; workforce gap remains structural
Operations	RHRC not deployed; no PMO; no CIN; admin cost 91%+ above benchmark; no transformation reporting	RHRC engaged all 14 hospitals; PMO being established; CIN under development; two AHS transformation reports delivered	All 14 hospitals with approved transformation plans; PMO operational; CIN live; admin cost below 150% of benchmark	PROGRESSING — RHRC engagement is the right investment at the right time

Figure 13.2 — The 2025 federal healthcare policy inflection point (§13.2)

Header: **Measure | Figure | Detail**.

- Federal Medicaid spending reduction, H.R. 1 (10 yr) — **\$911B** — CBO estimate; ~10 million more uninsured by 2034
- Rural Medicaid spending reduction (10 yr) — **\$137B** — KFF estimate; Medicaid covers 1 in 4 rural adults
- Rural Health Transformation Program (5 yr) — **\$50B** — \$10B/yr FY2026–FY2030; temporary, front-loaded
- Rural hospitals operating at a loss — **>40%** — before Medicaid cuts; 52% in non-expansion states (Chartis, 2026)

Unnumbered callout (§13.3.5) — "STATES TO WATCH"

Single-cell callout. Names Maryland (longest-running global-budget proof of concept, Ch6), Connecticut, Hawaii, Rhode Island, New York as "AHEAD's other **Cohort 1 and Cohort 2** states", plus Pennsylvania and "several Midwestern states".

Figure 13.3 — Five-pillars forecast, 2026–2035 (§13.4)

Header: **Pillar | 2026 starting point | Primary headwinds | Primary tailwinds | 2035 assessment**. Five rows. Load-bearing cells:

- Policy: "Acts 167/68/AHEAD form the most complete state reform mandate in the country"; tailwind "**AHEAD expanding (6 states)**"; "Mandatory APM participation is the critical unresolved national question."
- Technology: "**analytics vendor procured**"; "Fastest-improving pillar nationally; FHIR operationally complete for major EHRs by ~2028."
- Economics: "VBC ~58% commercial"; "global budgets rare outside VT/MD"; "VBC ~70–75% commercial by 2030".
- Clinical: headwind "MHI sustainability uncertain without a durable funding source **after the AHEAD withdrawal**; primary-care shortage (**370-FTE gap by 2030** in VT)"; tailwind "Blueprint among strongest PCMH infrastructure in the country"; assessment "**Blueprint + AHEAD primary-care investment strongest in country**".
- Operations: "\$195M RHT capital; all 14 hospitals engaged; GMCB capacity added under Act 68".

Figure 13.4 — Vermont's transformation decisions and their national implications (§13.6)

Header: **Decision | Vermont timeline | If Vermont succeeds | If Vermont struggles**. Five decision rows: mandatory all-payer RBP (FY2027/FY2028 evidence); hospital global budgets (FY2028; **AHEAD data 2027–2030**); AHS Strategic Plan (Dec 2028 statutory deadline); RHT capital as structural transformation (FY2026–FY2030); Act 68 global budgets demonstrating savings (**AHEAD data from 2027; CMS evaluation through 2035**).

Figure 13.5 — Platform tools for Chapter 13 (§13.7)

Header: **Do this | On this tool | What to look for**. Four rows: HTI Dashboard, Innovation Leaderboard, The Wire, HTR Simulator. All four resolve: **frontend/app/hti-dashboard/**, **frontend/app/the-wire/**, **frontend/app/htr-simulator/**, and Innovation Leaderboard registered in **frontend/lib/taxonomy/tools.ts** (surfaced via **app/research-lab/ResearchLabHub.tsx**). No broken tool name.

Claims checked against their own source (MATCH/MISMATCH + proposed fix)

1. MISMATCH — the chapter's opening list of "five forces" is not the five forces it develops

Opening paragraph: "*The five forces this chapter develops — federal Medicaid retrenchment, **the AHEAD Model's national rollout**, demographic aging, the fee-for-service plateau, and **political sustainability risk**...*" §13.3's actual five headings: Force 1 Medicaid–RHT tension; Force 2 **AI clinical transformation**; Force 3 demographic reckoning; Force 4 **drug-pricing reckoning**; Force 5 VBC tipping point. Two named forces (AHEAD rollout, political sustainability risk) are not forces in §13.3 — political sustainability is Chapter 14's subject — and two that are (AI, drug pricing) are missing. **Fix:** rewrite the opener's list to "federal Medicaid retrenchment, AI clinical transformation outpacing governance, demographic aging, drug-pricing complexity, and the fee-for-service plateau."

2. MISMATCH — "Policy and Economics on track" contradicts Figure 13.1's own Signal column

§13.1 closing paragraph: "*the combination of **Policy and Economics on track** while Technology remains at risk.*" Figure 13.1 gives Policy = ON TRACK but **Economics = WATCH**, not on track. **Fix:** "the combination of Policy on track and Economics only on watch while Technology remains at risk" — or upgrade the Economics signal, which the rest of the row does not support.

3. MISMATCH — "six independent measures" describes a five-row table

Same paragraph: "*Reading this scorecard as a system rather than as **six independent measures**.*" Figure 13.1 has five pillar rows and no Equity row. (Appendix E does have six sections — Equity is E.0.5 — which is probably where "six" came from.) **Fix:** "five independent measures", or add the Equity row the caption already gestures at.

4. MISMATCH (cluster, highest priority) — Figure 13.1 and Figure 13.4 still treat Vermont as an AHEAD participant, which §13.3.5 of the same chapter contradicts

§13.3.5: "*now operating in five states ... **after Vermont's 2026 withdrawal***", and Figure 13.3's Clinical headwind says "after the AHEAD withdrawal". The rest of the book is unambiguous and dates it: Ch1 el.318 "AHEAD Model State Agreement (January 2025; **withdrawn July 2026**)"; Ch6 §6.6 "Medicare's Entry That Vermont Signed, Then Withdrew"; glossary el.1081 "signed for **Cohort 2** in January 2025 but withdrew in July 2026"; Appendix E Economics row "Medicare: fee-for-service (**AHEAD withdrawn**)". Yet inside Chapter 13:

- Fig 13.1 Policy **2028 Target**: "AHEAD compliant".
- Fig 13.1 Technology **Signal**: "analytics vendor gap vs. **Jan 2028 AHEAD start**" — and §13.1 prose restates it as "Mandatory financial accountability (January 2028)". Vermont has no Jan 2028 AHEAD start.
- Fig 13.3 Clinical **tailwind and 2035 assessment**: "Blueprint + **AHEAD primary-care investment** strongest in country" — directly contradicting the headwind cell beside it.
- Fig 13.4 row 2: "AHEAD data **2027–2030**"; row 5: "**AHEAD data from 2027**; CMS evaluation through 2035" — offered as the evidence base for Vermont's Act 68 global budgets.

- §13.5.1 (2031–2035): "AHEAD expands **using Vermont's documented results**".
- §13.8: "whether the analytics capability is operational before **AHEAD financial accountability** begins". **Fix:** re-base every Vermont-AHEAD dependency on Act 68 state authority. Policy 2028 target → "All hospitals under Act 68 global budgets; Statewide Strategic Plan delivered Dec 2028". Technology signal → "analytics vendor gap vs. FY2028 global-budget start". Fig 13.4 rows 2 and 5 → "GMCB global-budget data from FY2028" (Medicare AHEAD data will come from the five remaining states, not Vermont). Clinical tailwind → "Blueprint PCMH infrastructure" without the AHEAD clause. §13.8 → "before FY2028 global-budget financial accountability begins".

5. MISMATCH — "AHEAD expanding (6 states)" vs "five states"

Fig 13.3 Policy tailwind says **6 states**; §13.3.5 prose, Ch3 el.591 ("Five-state voluntary total cost of care model") and the glossary all say **five** (MD, CT, HI, RI, NY). Six is only reachable by counting Vermont, which withdrew. **Fix:** "AHEAD operating in 5 states".

6. MISMATCH — cohort labels conflict between prose and callout, one paragraph apart

§13.3.5 prose: "**Connecticut, Hawaii, Rhode Island, and New York's Cohort 2 and 3** participation". The STATES TO WATCH callout immediately after: the same four are "AHEAD's other **Cohort 1 and Cohort 2** states". The book's own glossary puts Vermont in Cohort 2 and Maryland in Cohort 1. **Fix:** pick one. Per the book's data: Maryland Cohort 1; CT/HI/RI Cohort 1–2; NY Cohort 3. Simplest correct phrasing in both places: "AHEAD's other participating states".

7. MISMATCH — "analytics vendor procured" (Fig 13.3) vs "procurement underway" (Fig 13.1 and Appendix E)

Fig 13.1 2026 status: "Analytics vendor **procurement underway**". Appendix E Technology (el.2269): "analytics-vendor **procurement underway**". Fig 13.3 Technology 2026 starting point: "analytics vendor **procured**". Fig 13.3 Operations headwind, in the same table, says "analytics vendor **not yet operational**". **Fix:** Fig 13.3 → "analytics vendor procurement underway". The AT RISK Technology signal depends on this cell not being "procured".

8. MISMATCH — 370-FTE gap presented as a 2022 baseline

Fig 13.1 Clinical 2022 Baseline: "370-FTE workforce gap". Appendix E (el.2275): "370-FTE gap **projected by 2030**". Fig 13.3, correctly: "370-FTE gap **by 2030** in VT". **Fix:** Fig 13.1 baseline → "370-FTE primary-care gap projected by 2030". Note the 2028 target "workforce gap below 200 FTEs" is then a target against a 2030 projection — worth one clause of explanation, as Appendix E's "FTE gap <200 via team-based care and telehealth" also leaves ambiguous.

9. MISMATCH — "readmission at 14.8% (unchanged)" has no baseline referent in its own table

Fig 13.1 Clinical 2026 Status says readmission is "14.8% (unchanged)", but the 2022 Baseline cell lists PCMH, BH follow-up, workforce and CCBHCs — no readmission figure. Appendix E supplies it (baseline 14.8%, 2026 "14.8% (**minimal change**)"). **Fix:** add "readmission 14.8%" to the baseline cell, and align "unchanged" → "minimal change" with Appendix E.

10. MISMATCH — Economics 2026 status is stale against Appendix E, which the chapter cites as its detail source

§13.1 says "The full scorecard with metric-level detail appears in Appendix E." Appendix E Economics 2026 status: "**6 of 14 in losses (FY2024, GMCB)**" — an improvement from the 9-of-14 baseline. Fig 13.1's Economics 2026 cell reports no margin figure at all and says "deficit trajectory unchanged", which is true of the five-year deficit projection metric only. A reader comparing the two will see the chapter omitting progress its own appendix documents. **Fix:** Economics 2026 status → "RBP mandatory FY2027 enacted; global-budget methodology under design; 6 of 14 hospitals in losses (FY2024, improved from 9); five-year deficit trajectory unchanged."

11. MISMATCH — 9-of-14 losses is FY2023 data, labelled 2022 baseline

Appendix E: "9 of 14 reporting losses (**FY2023**); worst –8.9%". Fig 13.1 puts it in the "2022 Baseline" column. **Fix:** "9/14 hospitals in losses (FY2023)" — the column is a pre-Act-167 baseline, so naming the year inside the cell resolves it.

12. MISMATCH — admin-cost target date

Fig 13.1 Operations **2028 Target**: "admin cost below 150% of benchmark". Appendix E Operations (el.2281) 2028 target: "System average below 150% of benchmark **by 2030**". (Appendix E's Economics table gives the same 150% as a 2028 target, so the appendix is itself inconsistent — but Chapter 13 has picked the more aggressive reading without saying so.) **Fix:** state one date. Recommend "admin cost on track to below 150% of benchmark by 2030" in Fig 13.1, and fix Appendix E's two rows to agree (out of scope here — flagged, not touched).

13. Unit trap, not strictly a mismatch — "91%+ above benchmark" vs "below 150% of benchmark"

Fig 13.1 Operations states the baseline as a *percentage above* benchmark (91% above = 191% of benchmark) and the target as a *percentage of* benchmark (150%). Both match Appendix E, and the arithmetic works, but the switch of denominator inside one row reads as a target *worse* than the baseline on a fast read. **Fix:** express both the same way — baseline "admin cost 191% of benchmark", target "below 150% of benchmark".

14. MISMATCH — CIN AI-governance framework attributed to Chapter 1

§13.3.2: "*The CIN-based AI governance framework **developed in Chapter 1**...*" Chapter 1 mentions AI governance only in a list of Technology-pillar staffing needs (el.298). The framework is developed in **Chapter 5** — "The Technology Pillar in Practice — FHIR, AI Governance, and Clinical Decision Support", el.884–885: training-data representativeness, and "a shared AI governance function within the CIN monitors all member hospital AI deployments". **Fix:** "developed in Chapter 5".

15. MATCH — cross-references other than #14

"the AHS Restructuring chapter (**Chapter 16**)" ✓ (el.2060, "The AHS Restructuring Roadmap"). "Maryland's all-payer model ... (**Chapter 6**)" ✓ (el.931, global budgets and §6.6). "Appendix E" for the full scorecard ✓ (el.2263, same April 2026 as-of date, el.2283).

16. MATCH — RHT arithmetic and the \$195M figure

\$50B over 5 years = \$10B/yr FY2026–FY2030 ✓ internally consistent between Fig 13.2 and §13.2 prose. \$195M first-year award appears five times in the chapter (§13.2, §13.2 response, Fig 13.1, Fig 13.3 Operations, §13.10) with no drift, and matches Ch1 el.232, Appendix E el.2266 and Appendix G el.2322.

17. MATCH — demographic arithmetic

"exceeding 30% of the state by 2040 ... Vermont is **fourteen years** from that milestone" as of 2026 ✓.

18. MISMATCH (soft, same paragraph) — "by 2035" vs "national within fifteen years"

§13.3.3: "By **2035**, the challenge Vermont faces today will be present in every Northeastern state ... most Midwestern states, and increasingly nationally," then two sentences later, the same work solves "a problem that will be national **within fifteen years**" — i.e. 2041. The paragraph gives the same event two horizons six years apart. **Fix:** use 2035 in both, or "national within a decade".

19. Overstated — "The Baby Boom generation, fully in Medicare by 2030, will be in its eighties by 2040"

"Fully in Medicare by 2030" is right (1964 cohort turns 65 in 2029). "In its eighties by 2040" is not: the 1964 cohort turns 76 in 2040, and only the 1946–1960 cohorts are 80+. The cost claim that follows doesn't need the overstatement. **Fix:** "will be entering its eighties through the 2040s" or "its oldest members will be in their nineties and its median member in her late seventies by 2040".

20. MISMATCH (arithmetic) — hospital-count scenario exceeds the state's hospital count

§13.5.1 (2030): "the hospital sector stabilized at **10–12 sustainable facilities** with **2–4 REH or CACC conversions**". Upper bound = 16 against Vermont's 14 hospitals, a number the chapter itself uses six times ("all 14 hospitals"). **Fix:** "10–12 full-service hospitals with the remaining 2–4 converted to REH or CACC status" — which makes the bounds sum to 14 by construction.

21. MATCH — VBC penetration, GLP-1 and IRA figures internally consistent

"~58% of commercially insured Americans" in §13.3.5 ✓ Fig 13.3 Economics "VBC ~58% commercial". Semaglutide: selected for 2025 negotiation, negotiated prices effective 2027, ~\$274 for a 30-day supply down from over \$1,000 — internally consistent and consistent with Fig 13.3's "IRA GLP-1 negotiation (2027)". BALANCE Model dated December 2025 in prose and "December 23, 2025" in Sources ✓.

22. Minor — "fifteen years of payment-reform effort" vs "since CMMI's founding"

§13.3.5 opens with "fifteen years of payment-reform effort" and then dates the comparison to CMMI's founding, which was 2010 — sixteen years before this chapter's 2026. Harmless but easily made exact: "sixteen years".

23. Unverifiable offline, internally consistent — Chartis figures

Fig 13.2 ">40% of rural hospitals operating at a loss; 52% in non-expansion states (Chartis, 2026)" and §13.3.1 "417 rural hospitals currently vulnerable to closure" both cite Chartis, *2026 State of Rural Health*, which is also in the chapter's Sources line (February 2026) and in the book's master source list (el.2492). No internal conflict; the 417 figure appears nowhere else in the manuscript, so there is nothing to contradict it. Flagged for external fact-check, not as a defect.

24. Unverified figure — "Medicaid covers roughly 19% of the state's insured population"

Appears only here in the whole manuscript. Appendix A's payer-mix material (el.2189, el.2213) discusses the commercial cross-subsidy and the 97% coverage rate but gives no Medicaid share to check it against. Not a contradiction; a single-sourced number with no corroboration in the book. **Fix:** add the source to the Sources line, or cross-reference Appendix A once Appendix A carries the figure.

25. MISMATCH — the chapter's as-of date contradicts its own contents

The chapter opens "**Updated April 2026**"; Figure 13.1 is "April 2026"; but §13.3.1 carries an "**UPDATE — JUNE 2026**" box, and §13.3.5 reports a Vermont AHEAD withdrawal the rest of the book dates **July 2026**. A chapter dated April cannot report a July event. **Fix:** move the chapter header to "Updated July 2026", keep Figure 13.1 and Appendix E explicitly dated "as of April 2026" with a one-line note that the AHEAD-dependent cells are superseded by the July 2026 withdrawal described in §13.3.5. This single change also resolves most of defect #4's reader-facing contradiction.

Superlative/ranking claims logged for cross-chapter comparison

Verbatim, with location. Flagged [COLLISION] where the same superlative frame recurs inside this chapter.

National-scale firsts / largests

- 1. §13.2 — "Its healthcare provisions represent **the largest single reduction in federal health-program spending in American history**." (H.R. 1)
- 2. §13.2 — "**the largest healthcare-coverage contraction in American history**, partially offset by a historic but time-limited and front-loaded capital investment". [COLLISION with #1 — same subject, two different "largest in American history" frames one paragraph apart]
- 3. §13.3.5 — "the AHEAD Model ... is **the most serious federal commitment to all-payer total-cost-of-care accountability since CMMI's founding**."
- 4. §13.3.5 — "**the most ambitious federal payment reform currently operating**." (AHEAD)

Vermont / Maryland rankings 5. Fig 13.3, Policy — "Acts 167/68/AHEAD form **the most complete state reform mandate in the country**." 6. Fig 13.3, Clinical tailwind — "**Blueprint among strongest PCMH infrastructure in the country**." 7. Fig 13.3, Clinical 2035 — "**Blueprint + AHEAD primary-care investment strongest in country**." [COLLISION with #6 — hedged "among strongest" and unhedged "strongest" in the same table row; #7 also depends on the withdrawn AHEAD investment, see defect #4] 8. §13.3.3 — "Vermont is **the national canary** for demographic healthcare stress." 9. §13.3.5 — "Maryland — **the model's longest-running and most-watched participant**." 10. STATES TO WATCH callout — "Maryland's all-payer model remains **the longest-running proof of concept for global budgets nationally**." [Consistent with #9; cross-check against Chapter 6's own Maryland superlatives] 11. §13.3.5 — "Vermont's experience as **its clearest warning so far**."

"Most consequential / most dangerous / critical" — the collision-prone set 12. §13.1 — "**The most dangerous pattern** is not a single pillar failing — it is the combination of Policy and Economics on track while Technology remains at risk." (see defect #2) 13. §13.1 — "The Technology signal is therefore **the most consequential number on this table**." 14. Fig 13.1, Technology —

the only **AT RISK** signal; Fig 13.3 calls Technology **"Fastest-improving pillar nationally"**. [Not strictly contradictory — AT RISK is Vermont-vs-target, fastest-improving is national — but the two labels sit two figures apart with no reconciling clause. Worth one.] 15. Fig 13.1, Economics — "methodology design is **the critical near-term decision**." 16. Fig 13.3, Policy — "Mandatory APM participation is **the critical unresolved national question**." 17. Fig 13.3, Clinical — "BH integration is **the critical clinical variable**." 18. §13.3.2 — "**Governance is the critical gap**." 19. §13.3.4 — "The GLP-1 agonist spending wave is **the most consequential near-term pharmaceutical challenge** for state systems." [COLLISION with #13 — two "most consequential" claims in one chapter; scopes differ (a scorecard cell vs. pharmaceutical policy) so both may stand, but they should not be phrased identically] 20. §13.3.3 — dual-eligibles are "**the highest-cost, most poorly coordinated challenge in the system**." 21. §13.3.3/§13.3.4 — the 80s frailty phase "**drives the highest costs and the most inadequate care models**." 22. §13.3.2 — "AI is moving ... **faster than any governance framework can track**." 23. §13.3.4 — "**exactly where GLP-1 efficacy is best demonstrated**." 24. §13.10 — "it is **the most plausible outcome** given Vermont's specific combination". 25. §13.10 — "**The deepest bet** is about the relationship between analysis and action." 26. Fig 13.1, Operations — "RHRC engagement is **the right investment at the right time**." (evaluative, not ranked — logged because it is the only Signal cell that editorializes rather than assesses) 27. Fig 13.1 caption — "**Technology is the pillar where 2026 status most diverges from the 2028 target**." [Consistent with the table: Technology is the sole AT RISK row]

Items **5, 6, 7, 8, 9, 10** are the ones most likely to collide with Chapters 2, 6, 8 and 9, which make their own "strongest/first/only in the country" claims about Act 68, Maryland and the Blueprint. Items **1** and **2** should be reconciled to one claim before any cross-chapter pass.

Other issues

Internal contradictions (beyond the numbered mismatches above)

- **Fig 13.3 Clinical row contradicts itself cell-to-cell.** Headwind: "MHI sustainability uncertain ... after the AHEAD withdrawal." Tailwind and 2035 assessment: AHEAD primary-care investment as an ongoing Vermont strength. One row, two states of the world.
- **Medicaid-cut onset dates drift across three timings.** §13.2: cuts "will pressure hospital finances beginning in **2026–2027**"; §13.3.1: "the Medicaid cuts taking effect **after 2027**"; §13.2 also puts work requirements in "**late 2026**". Fix to one phrasing — work requirements late 2026, revenue effects 2026–2027, the bulk of reductions back-loaded after FY2030 — which is what §13.2's KFF paragraph already says.
- **CCBHC counting.** Fig 13.1 2028 target "7+ CCBHCs"; Appendix E "7 certified". Trivially reconcilable, but the chapter's 2026 status is "5 CCBHCs **planned**" (i.e. zero certified) while the Signal reads "CCBHC certification **on track**" — the cell gives no certification milestone to be on track against.

Genuine repetition

- **The RHT-is-temporary / Medicaid-cuts-are-back-loaded argument is made four times:** §13.2 ¶4 ("partially offset by a historic but time-limited and front-loaded capital investment"), §13.2 ¶6 (KFF, "temporary and front-loaded ... nearly two-thirds back-loaded after FY2030"), §13.3.1 ¶1 ("permanent Medicaid revenue reductions arriving after the expiration of a temporary, one-time capital fund"), §13.3.1 ¶3 ("the post-2030 Medicaid cliff"). The §13.2 KFF paragraph is the strongest statement; §13.3.1 ¶1 restates it with no new content and could open on the state-by-state question instead.
- **The Samuelson framing appears twice.** Paraphrased in §13.2 ("the RHT funding is a tool for executing reform, not a substitute for reform — the analytical frame every state should adopt"), then quoted verbatim as a pull quote at the end of §13.5. Legitimate as pull-quote reinforcement only if the §13.2 paraphrase is trimmed; as written, the reader meets the same sentence twice.
- **§13.3.5 closes by restating itself.** "...Vermont's own withdrawal already stands as a cautionary data point on the reliability of the federal funding those states are counting on" then, next sentence, "...with Vermont's experience as its clearest warning so far." Two sentences, one point. Cut the second clause.
- **Legitimate recurrence, not duplication** (do not "fix"): "five forces" in the opener, §13.3 heading and §13.9 Key Concepts; the two-scenario framing in §13.5 and again in Fig 13.5's HTI Dashboard row and §13.8; the \$195M figure at five points, each in a different register (award, capital, scorecard cell, forecast cell, closing bet). The STATES TO WATCH callout deliberately fulfils the opener's promise that the forecast "returns to" other states below — that pairing is working as designed.

Thin sections / structural imbalance

- **Force 3 (§13.3.3, demographics) and Force 5 (§13.3.5's opening) are the thinnest.** Force 1 gets four paragraphs plus a dated update box; Force 3 gets two paragraphs resting on a single 2024 Oliver Wyman projection with no Vermont figure other than the 30%-by-2040 milestone — no dual-eligible count, no LTC bed gap, no PACE capacity number, though Appendix A (el.2189) has payer-mix material this section could cite.
- **§13.3.2 through §13.3.5 carry no figure.** Five of the chapter's six tables sit in §13.1, §13.2, §13.4, §13.6 and §13.7; the whole of §13.3 has one unnumbered callout. The Five Forces are the chapter's spine and are the least visually supported part of it.
- **§13.5's two scenarios are prose-only** and strictly parallel by year (FY2027 / FY2028 / 2028 / 2030 / 2031–2035). They are asking to be one two-column figure, which would also make the scenario divergence in §13.8's "small number of decisions" checkable at a glance.
- **§13.9 Key Concepts is a single undifferentiated run of seven terms** — consistent with the book's convention, noted only so it isn't mistaken for truncation.

Recurring-heading check (CLAUDE.md style-critical set)

§13.7 "Work This Chapter on the Platform", §13.8 "Implications for You", §13.9 "Key Concepts in This Chapter" are all present with the canonical wording, and Sources is a plain paragraph, not a heading. ✓ No drift in this chapter.

Priority order for a fix pass

1. Defect **#4 + #25** (AHEAD-withdrawal staleness across Fig 13.1, Fig 13.3, Fig 13.4, §13.5.1, §13.8, and the chapter's as-of date) — this is one edit cluster and the only one that makes the chapter contradict itself in front of the reader.

- 2. Defect #1 (the five forces named in the opener are not the five forces developed).
- 3. Defects #2, #3, #5, #6, #7 — figure-vs-prose and figure-vs-figure mismatches, each a short string replacement.
- 4. Defects #8–#13 — Fig 13.1 vs Appendix E reconciliation.
- 5. Defect #14 (Chapter 1 → Chapter 5), #18, #19, #20, #22 — single-clause corrections.
- 6. Repetition trims and the §13.5 scenario figure.

===== FILE: audit_ch14.md =====

Chapter 14 Audit

Source of truth: `HTR_Book_v42.docx` → `word/document.xml`, read via python-docx over the raw body element order (paragraphs + tables interleaved). Chapter boundary: body item 1922 (Heading 1, "Chapter 14: Political Sustainability — Protecting Transformation Across Election Cycles") through item 1980 (Heading 1, "Chapter 15: ..."), exclusive. 58 items, ~19.8k characters of text. Read-only — the `.docx` was not modified.

Section inventory: 14.1 → 14.8 present and sequentially numbered; 14.2 has 14.2.1/14.2.2; 14.4 has 14.4.1–14.4.4. Recurring end-of-chapter headings match the byte-identical required forms (Work This Chapter on the Platform, Implications for You, Key Concepts in This Chapter, and Sources: as a plain paragraph — not a heading). No ## **Sources** defect.

Figures found (ground truth)

Figure 14.1 — Early-warning signals of political risk (body item 1941, table 9×4, header + 8 data rows)

Columns: Signal | What it indicates | Severity | Monitoring source

#	Signal	Indicates	Severity	Source
1	Hospital lobbying focused on RBP methodology rather than the mandate	Effort to hollow out RBP through the regulatory process	High	GMCB docket; Vermont Hospital Association statements
2	Bills to extend RBP or global-budget deadlines	Direct challenge to Act 68's mandatory timeline	Very High	Legislature bill tracker; HCAC testimony
3	Global-budget levels set within 5% of current commercial rates	Regulatory capture; budgets without pressure	High	GMCB FY2028 budget guidance
4	December 2028 Strategic Plan described as "in progress" in November 2028 reports	Operations failure; deadline unlikely to be met	High	AHS monthly transformation reports
5	CMS AHEAD waiver-modification request	Federal pressure to weaken AHEAD accountability	Very High	CMS AHEAD communications; AHS federal-relations reports
6	Governor-candidate platforms including RBP repeal or delay	Political threat to mandatory architecture in 2026 cycle	Very High	Governor-race coverage; candidate policy statements
7	EAST Fund spending well below authorized levels	Resistance to transformation investment despite authorization	Moderate	DVHA AHEAD spending reports; GMCB budget documentation
8	HCAC failure to reach consensus on the Strategic Plan framework	Governance breakdown; plan unlikely to produce binding commitments	High	HCAC meeting minutes; AHS reporting

Severity distribution (ground truth for any count claim): **3 Very High, 4 High, 1 Moderate**. Caption (1942): "Figure 14.1 — Early-warning signals of political risk to Vermont's reform architecture. Sources: HTR Advisory risk-monitoring framework; Act 68 of 2025; GMCB regulatory process documentation."

BEYOND VERMONT callout (body item 1943, single-cell 1×1 table — unnumbered, correct per convention)

Names **four** signal categories: (a) change in the governing party's posture, (b) a high-profile enforcement challenge, (c) a hospital-sector coalition campaign, (d) a federal policy shift changing the reform's financial assumptions. Asserts "The specific signal in Vermont was UVMMC's court challenge to GMCB enforcement."

Figure 14.2 — Political-disruption scenario planning (body item 1953, table 6×4, header + 5 data rows)

Columns: Disruption scenario | Probability (HTR assessment) | Primary impact | Organizational response

#	Scenario	Probability	Impact
1	RBP delayed 18+ months (methodology challenge)	Moderate (25–35%)	Commercial revenue maintained longer; urgency reduced
2	Global budgets set ~5% above current commercial rates	Low–Moderate (15–25%)	Minimal financial pressure; accountability without incentive
3	EAST Fund cut 30%+ (federal Medicaid cuts)	Moderate–High (35–45%)	Primary-care/BH investment capacity reduced
4	2026 governor committed to Act 68 modification	Low (10–15%)	Legislative challenge in 2027 session
5	CMS AHEAD modification reducing accountability scope	Low (10–20%)	Reduced federal accountability; AHEAD less transformational

Caption (1954): "Figure 14.2 — Political-disruption scenario planning for Vermont healthcare organizations. Sources: HTR Advisory risk assessment; Vermont political-environment analysis (April 2026)."

Figure 14.3 — Platform tools (body item 1964, table 4×3, header + 3 data rows)

Columns: Do this | On this tool | What to look for. Tools cited: **The Wire**, **HTR Simulator**, **Transformation Friction Index**.

Caption (1965) carries no **Sources**: line — consistent with the other chapters' platform tables.

Figure numbering is complete and gap-free: 14.1, 14.2, 14.3. No figure is cited by a number that does not exist, and no figure lacks a caption.

Claims checked against their own source (MATCH/MISMATCH + proposed fix)

1. MISMATCH — the global-budget threshold is stated three different ways.

- §14.4.3 prose (1952): "global-budget levels set **10%+ above** current commercial rates (effectively removing budget pressure)" — presented as one of the three scenarios the reader should model.
- Figure 14.2 row 2, the table that immediately follows and is supposed to *be* those scenarios: "Global budgets set **~5% above** current commercial rates."
- Figure 14.1 row 3: "Global-budget levels set **within 5% of** current commercial rates."

Three incompatible numbers for the same phenomenon, and the prose/table pair is a direct self-contradiction one paragraph apart. Note also that "within 5% of" (Fig 14.1) and "~5% above" (Fig 14.2) are not the same test — the first is a band around current rates, the second a point estimate above them. *Proposed fix*: pick one threshold and use it in all three places. The cleanest is to make the signal and the scenario the same test: change 1952's "set 10%+ above current commercial rates" to "set within 5% of current commercial rates", and Fig 14.2 row 2 to "Global budgets set within 5% of current commercial rates", matching Fig 14.1 row 3 verbatim. If the author intends the *signal* (any budget without pressure) to be a wider band than the *scenario* (a specific modelled case), say so explicitly rather than leaving two bare numbers.

2. MISMATCH — the BEYOND VERMONT box's "same four categories" do not map onto Figure 14.1. The box says "These signal categories ... are the same four categories that would precede the unwinding of a comparable reform in any state," with "These" pointing back at Figure 14.1. But Figure 14.1 has **eight** signals in a different taxonomy, and one of the box's four — "a high-profile enforcement challenge" — has **no corresponding row** in Figure 14.1 (the eight rows cover lobbying, bills, budget levels, plan slippage, a CMS waiver request, candidate platforms, EAST Fund underspending, and HCAC deadlock; none is an enforcement challenge or litigation). *Proposed fix*: either add an enforcement-challenge/litigation row to Figure 14.1 (e.g. "Provider litigation against GMCB enforcement action" → "Judicial challenge to the enforcement mechanism" → Very High → GMCB docket; Vermont judiciary filings), or reword the box to "The eight signals in Figure 14.1 fall into four categories that ..." and drop the claim that they are the same four.

3. MISMATCH (unsourced) — UVMMC's court challenge to GMCB enforcement. The box states this as the specific Vermont instance, but the fact appears nowhere else in the chapter, and the chapter's **Sources**: paragraph (1978) lists no litigation source — no docket, no court filing, no coverage. Per directive 14 and the fact-check ledger method this needs two corroborations before it stands, and per finding 2 it is the one category with no Figure 14.1 row. *Proposed fix*: add the specific citation to 1978 (case name, court, date) and give it a Figure 14.1 row; or, if it is not verifiable to the standard, replace the sentence with a category-level statement that does not name the actor or the action.

4. MATCH — "at least three disruptions" vs Figure 14.2's five rows. §14.4.3 says "should maintain scenario plans for **at least three** disruptions" and then names exactly three (RBP delay, global-budget levels, EAST Fund cut). Figure 14.2 lists five. "At least three" is not violated by five, and the three named are rows 1, 2 and 3 of the table in order. Not a defect — but see finding 1, since the second of those three is the number that disagrees.

5. **MATCH — statutory effective dates are internally consistent.** RBP mandatory **FY2027**: stated in 14.2.2 (1936), presupposed in 14.4.1 ("complete NCQA PCMH recognition before FY2027"). Global budgets **FY2028**: stated in 14.2.2 (1937), in Fig 14.1 row 3 ("GMCB FY2028 budget guidance"), and in 14.7 ("global budgets arriving in FY2028"). Strategic Plan **December 2028**: 14.2.2 (1938), Fig 14.1 row 4, 14.5 (1960). No drift.

6. **MATCH — the reform cascade sequence.** **Act 167 → Act 51 → Act 68** appears in 14.2.1 (1933), 14.5 (1959), and Key Concepts (1977), in the same order with the same roles each time (167 = diagnosis, 51 = institutional capacity/enabling, 68 = mandatory deployment/operational). 14.7's "a diagnostic act, then an enabling act, then an operational act" is the same sequence abstracted. Consistent.

7. **MATCH (internally) — severity vs probability are not in conflict.** Fig 14.1 rates "Governor-candidate platforms including RBP repeal or delay" and "CMS AHEAD waiver-modification request" **Very High**, while Fig 14.2 gives "2026 governor committed to Act 68 modification" **Low (10–15%)** and "CMS AHEAD modification" **Low (10–20%)**. These are different axes (consequence vs likelihood) and are not contradictory. Worth noting only because neither figure's caption or lead-in says so, and a reader moving from 14.3 to 14.4.3 can read it as a reversal. *Optional fix*: one clause in 1952 or the Fig 14.1 caption distinguishing severity (impact if it happens) from probability.

8. **Criterion-5 promise check — all three Figure 14.3 tools exist, are routed, and tag ch. 14.**

- The Wire → `/the-wire, frontend/app/the-wire/page.tsx` exists; `lib/taxonomy/tools.ts chapters: [ "13", "14" ]`. Live news feed, so the specific promise ("bills that would amend Act 68, GMCB appointment changes") cannot be verified against fixed content by design — flagged, not a defect.
- HTR Simulator → `/htr-simulator`, page exists; `chapters: [ "1", "13", "14", "15", "16" ]`. The promise "Drop the Policy score and watch Economics fall with it" is delivered: the `policy → economics` edge is declared in `frontend/lib/framework/dependencies.ts:69` as `kind: "enables", criticalPath: true`.
- Transformation Friction Index → `/transformation-friction-index`, page exists; `chapters: [ "1", "14", "15" ]`. Per-pillar friction is real (Policy, Economics, Technology, Clinical, Operations, Equity Imperative all defined in the page). **PARTIAL PROMISE FAILURE**: the book says "Rising friction in a pillar is usually visible in the operating data a year before it becomes a legislative problem." The tool is a point-in-time score with no time series, no history, no prior-period delta — a reader cannot see anything "rising" there. Same failure mode as the three defects logged in CLAUDE.md: the route 200s and the tool exists, but the promise is not delivered. *Proposed fix (per the standing rule "book vs tool disagree → EXTEND THE TOOL")*: add a prior-period column or trend indicator to the friction index rather than softening the sentence.

Superlative/ranking claims logged for cross-chapter comparison

Logged verbatim with body item numbers, for later cross-chapter deduplication. **Items A, B and C are three different answers to the same question and are flagged as an internal contradiction in "Other issues" below.**

- **A.** [1947, §14.4.1] "The **most effective protection** against disruption is structural irreversibility."
- **B.** [1957, §14.4.4] "The political sustainability of Vermont's reform ultimately rests on **one thing**: evidence that it is working." ... "demonstrated with the metrics in Act 167's five goals and Act 68's reporting requirements — are the **most durable protection against reversal**."
- **C.** [1969, §14.7, advocate] "the **most durable protections** in Vermont's architecture were not the most rhetorically forceful ones. Mandatory participation, statutory deadlines, and federal-state agreements survive changes of government; strategic plans and voluntary commitments generally do not."
- **D.** [1938, §14.2.2] A descriptive-not-committal Strategic Plan is "the **most politically convenient failure mode**: compliance without transformation."
- **E.** [1943, BEYOND VERMONT] "identify their own state's **largest, most politically influential** health system as the actor **most likely** to generate the analogous signal."
- **F.** [1956, §14.4.4] "AHS's monthly transformation reports (required by Act 68) are the **primary vehicle** for communicating progress to the actors who control Vermont's policy environment."
- **G.** [1936, §14.2.2] "Hospital lobbying **will focus on** the methodology, **not** on overturning the mandate." (Unhedged prediction, stated as fact; Fig 14.1 row 1 treats the same thing as a signal to watch for, i.e. as contingent.)
- **H.** [1937, §14.2.2] "The risk is **not repeal** — it is budgets calibrated to produce the appearance of accountability without the substance."
- **I.** [1939, §14.2.2] "This is the federal-state dependency Vermont **cannot fully insulate against**." (Near-neighbour of the "most underestimated dependency" family of claims that has collided across chapters before — check against chs. 1, 7 and 13.)
- **J.** [1961, §14.5] "This is **the model other states should take** from Vermont, beyond the specific policy content."

Figures to reconcile across chapters (each stated once here, all load-bearing elsewhere in the book, none re-derived in ch. 14 — but none cross-referenced to the front matter either):

- [1931] AHEAD State Agreement "signed with CMS in **January 2025**", "**nine-year** performance period"
- [1932] RHT Program "**\$195 million** first-year award"
- [1934] Oliver Wyman: "**13 of 14** hospitals in operating losses **by 2028** absent structural change"
- [1939] H.R. 1 "**\$911 billion** in Medicaid cuts (enacted **July 2025**)"
- [1960] Act 167 diagnostic process: "**230+ meetings, 3,100+ participants**"
- [1957] "Act **167's five goals**"

Other issues

1. **Internal contradiction — three competing "most durable protection" claims.** Superlatives A, B and C above each answer "what best protects the reform?" with a different answer: structural irreversibility (A), demonstrated evidence that it is working (B), and statutory/federal-agreement architecture (C). B is the strongest offender because it says the sustainability "ultimately rests on

one thing", which excludes A and C explicitly. A and C are also in tension with §14.2.1, which grounds durability in federal agreements and sunk capital, and with 14.7's state-official paragraph (1967), which says "the durability argument in this chapter rests on **structure, not on goodwill**" — i.e. the chapter's own summary picks C, contradicting B eleven items earlier. *Proposed fix*: make one of them primary and subordinate the others. The chapter's structure supports C as primary (14.2.1 and 1967 both argue it): soften B to "the most durable *political* protection available to an individual organization" or "the protection an organization can build for itself", and soften A to "the most effective protection available at the organizational level", so the three claims occupy different scopes instead of competing for the same one.

2. **Genuine repetition — the "each act makes reversal costlier" proposition, four times.**

- [1933] "Dismantling Act 68 would mean dismantling these institutions ... that **make reversal costly**."
- [1959] "Each act created facts, institutions, and constituencies that made the next more viable and **its reversal more costly**."
- [1961] (the same design restated as a prescription for other states)
- [1970] "Each act **makes the next harder to reverse**." This is restatement, not legitimate recurrence: 1959 adds nothing to 1933 except the framing of §14.5, and 1970 restates 1959 in the Implications voice. Key Concepts' "Reform cascade" entry (1977) is legitimate recurrence (glossary) and should be left. *Proposed fix*: cut the second sentence of 1933 back to naming the institutions and let §14.5 carry the reversal-cost argument once, or cut 1959 and open §14.5 directly with 1960's evidence.

3. **Genuine repetition — the "any state can apply this" move, four times.** [1924] (the whole second paragraph of the chapter opener), [1943] (BEYOND VERMONT box), [1961] ("the model other states should take"), [1970] (the "if you are in another state" paragraph). The box and the Implications paragraph are structural conventions and earn their place; 1924 and 1961 say the same thing in body prose. 1924 is notably long (one 120-word sentence doing the substitute-your-own-state work that the BEYOND VERMONT box exists to do). *Proposed fix*: compress 1924 to one sentence and let the box carry the substitution instructions, or cut 1961's first sentence, which 1970 restates better.

4. **Thin section — §14.4.4 and §14.6 lead-in.** §14.4.4 is two paragraphs, one of which (1957) is the chapter's closing generality rather than strategy content; Strategies 1–3 each get a developed mechanism, Strategy 4 gets an assertion plus a superlative. Not a factual defect, but it is the weakest of the four and the place where contradiction 1 enters.

5. **Front-matter alignment (criterion 4) — not fully checkable within this chapter's scope.** The six figures listed at the end of the previous section are all stated fresh in ch. 14 with no cross-reference to the Preface or Introduction. Where the front matter already establishes any of them (the Oliver Wyman 13-of-14 projection and the \$911B H.R. 1 figure are the likely candidates), directive/criterion 4 requires the chapter to cross-reference rather than re-derive. This audit was scoped to ch. 14 only and did not open the front matter — flagged as open, not as a finding.

6. **No formatting audit was run** (`check_format.py`), and no render was produced, because this task was read-only and touched nothing. If any of the fixes above are applied to Figures 14.1 or 14.2, rule 23 requires `python3 book-build/check_format.py` afterwards, and the black-on-navy `tblLook` check must be done in the XML, not from a render.

===== FILE: audit_ch15.md =====

Chapter 15 Audit

Scope: Chapter 15 ("Healthcare Transformation as Portfolio Management — Applying PMI Standards to Five-Pillar Reform"), extracted directly from `word/document.xml` of `HTR_Book_v42.docx` (char range between the `Heading1` "Chapter 15:" at offset 4,353,542 and the `Heading1` "Chapter 16:" at offset 4,644,873). Tables read cell-by-cell from the XML, not from the `.md` mirror. **Read-only audit — the `.docx` was not modified.**

Sections present: 15.1 – 15.16 plus the un-numbered chapter epigraph and the `Sources` footer. Eight numbered figures (15.1 – 15.8) plus three un-numbered callout boxes (BEYOND VERMONT, THE PORTFOLIO MANAGER'S SEQUENCING TEST, TRY THIS).

Figures found (ground truth)

Figure	§	Shape (rows incl. header)	Content summary
15.1	15.1	stat strip, 2×4	10 of 10 PMI knowledge areas · 19 portfolio components · 7 statutory deadlines · 14+ AHS programs
15.2	15.2	4 rows (1 hdr + 3)	Project / Program / Portfolio × PMI definition, Vermont example, who manages, success measure

Figure	§	Shape (rows incl. header)	Content summary
15.3	15.3	18 rows (1 hdr + 17 data rows)	The portfolio inventory. Pillar counts: Policy 4, Technology 4, Economics 3, Clinical 3, Operations 3 = 17
15.4	15.5	6 rows (1 hdr + 5)	Pillar-level role matrix: Policy, Technology, Economics, Clinical, Operations PMs — five roles, no Equity PM
15.5	15.9	11 rows (1 hdr + 10)	Risk register R-01 ... R-10. Scores: 20, 15, 16, 12, 15, 16, 15, 12, 12, 20
15.6	15.10	6 rows (1 hdr + 5)	Act 167's five goals × contributing components × metric × 2028 target
15.7	15.11	6 rows (1 hdr + 5)	Business case. Total annual ~\$850K–\$1.3M ; total 4–5 yr ~\$4M–\$6.4M
15.8	15.14	7 rows (1 hdr + 6)	Six platform tools: Transformation Friction Index, Investment Tracker, HTI Dashboard, HTR Simulator, Transformation Scorecard, Impact Simulation

Figure 15.3's 17 data rows, verbatim in order:

1. Policy · Act 68 RBP-methodology rulemaking · Compliance project · Critical · GMCB · In progress → FY2027
2. Policy · AHEAD State Agreement management · Ongoing program · Critical · AHS · Active → annual CMS review
3. Policy · RHT Program contract management · Compliance project · High · AHS Health Reform · Active → annual federal reporting
4. Policy · Statewide Strategic Plan development · Strategic project · Critical · AHS + HCAC · In progress → Dec 2028
5. Technology · AHS-GMCB analytics-vendor deployment · Strategic project · Critical · AHS + GMCB · Procurement → Jan 2027 operational
6. Technology · Vermont CIN build-out · Strategic program · High · AHS + RHT · Design → FY2028 operational
7. Technology · VITL HIE expansion / FHIR compliance · Technical project · High · VITL + AHS · In progress → FY2027 mandatory
8. Technology · Statewide AI-governance framework · Policy project · Medium · AHS + CIN · Not started → FY2028 adoption
9. Economics · RBP implementation (all hospitals) · Compliance program · Critical · GMCB · Methodology design → FY2027
10. Economics · Global-budget design & implementation · Strategic program · Critical · GMCB + AHS · Design → FY2028 non-CAH
11. Economics · RHT Program deployment · Operational program · High · DVHA + AHS · Active → annual targets
12. Clinical · Blueprint PCMH · Operational program · Critical · Blueprint + DVHA · Active → 100% PCMH by FY2028
13. Clinical · CCBHC certification (5 entities) · Compliance project · High · AHS + providers · In progress → FY2026
14. Clinical · CoCM statewide deployment · Operational program · High · MHI + Blueprint · Pilot → 80%+ practices FY2028
15. Operations · 14-hospital transformation planning · Operational program · Critical · AHS + RHRC successor · Plan submission → FY2026 approved
16. Operations · AHS restructuring (HSA-coordinator model) · Organizational project · Critical · AHS Secretary · In progress → FY2027 staffed
17. Operations · AHS PMO establishment · Organizational project · Critical · AHS · Not started → Q1 2027 operational

Claims checked against their own source

1. "19 components" vs. Figure 15.3's 17 rows — MISMATCH (most serious defect)

The number 19 is asserted **eleven times** across the chapter and is the chapter's central organising fact. Figure 15.3, the table that is supposed to enumerate them, contains 17.

Every instance, verbatim:

- Epigraph: "Vermont's **19-component** portfolio is this chapter's worked example..."
- Figure 15.1 stat: "**19** / Vermont transformation portfolio components"
- §15.3 heading: "Vermont's Transformation Portfolio: **19 Components**"
- §15.3 prose: "**Nineteen** is what that exercise produces as of early 2026"
- Figure 15.2, Portfolio row: "all **19 components** across the pillars"

- Figure 15.3 caption: "Vermont's **19-component** transformation portfolio"
- BEYOND VERMONT box: "this **19-component** inventory... The number **19** is Vermont's"
- §15.4 opening: "**Nineteen** components. Shared stage gates."
- §15.6: "formalized project leads for all **19 components**"
- §15.7 opening: "The **nineteen** components are not a flat list..."
- Sequencing-test box: "Applied across all **nineteen**, this test produces..."
- §15.8: "Not all **19 components** should be managed identically"
- §15.12: "all **19** component leads identified"
- §15.13/§15.15: "a status summary of all **nineteen**"
- §15.16: "Vermont's **19-component** transformation portfolio"

Proposed fix. The author's intent should decide the direction, but the evidence points to two components having been dropped from the table rather than the count being wrong, because §15.7 and §15.8 both refer to components that Figure 15.3 does not list:

- §15.7 names the Economics components as "RBP implementation, global-budget design, **EAST Fund deployment**" — Figure 15.3's third Economics row is "**RHT Program deployment**".
- §15.8 names "**HEROI methodology** (iterating as data quality improves)" as an adaptive *component*; HEROI appears nowhere in Figure 15.3 (it appears only in Figure 15.6 as a contributing component for two Act 167 goals).
- R-08 in Figure 15.5 is a social-risk-adjustment risk owned by an "Equity PM", and §15.7 treats "Social-risk-adjustment methodology" as a component that "can and should proceed in parallel today" — also absent from Figure 15.3.

So the likeliest fix is **adding two rows to Figure 15.3** — an Equity/Clinical row for *HEROI methodology* and an Economics or Equity row for *social-risk-adjustment methodology* (with a stage gate of FY2028, matching R-08) — which restores 19 and simultaneously fixes defects 3, 5 and 6 below. The alternative fix (changing every "19"/"nineteen" to "17") is mechanical but would leave §15.7's and §15.8's dangling component references unresolved, and would contradict the numeral's use as a rhetorical anchor in §15.4 and §15.15.

2. AHEAD is simultaneously live and withdrawn — MISMATCH (contradiction, and stale against the rest of the book)

The book establishes elsewhere, repeatedly and with dates, that Vermont **withdrew from AHEAD in July 2026**: Ch1's pillar table ("AHEAD Model State Agreement (January 2025; **withdrawn July 2026**)"), Ch1's status table ("subsequently withdrawn July 2026"), the timeline entry "Jul 2026 — Vermont withdraws from AHEAD... cut expected EAST Fund from ~\$138M to a ~\$10M cap; Vermont exits before its Cohort 2 performance period (Jan 2028) ever began."

Chapter 15 treats AHEAD as an active obligation in **five** places and as withdrawn in **one**:

Location	Text	State assumed
Fig 15.3 row 2	"AHEAD State Agreement management · Ongoing program · Critical · Active → annual CMS review "	live
Fig 15.2, Program row	" AHEAD Implementation Program " as a current Vermont program	live
Fig 15.5 R-01	"hospitals enter AHEAD without financial-management capability"	live
Fig 15.5 R-09	" AHEAD driven by administrators without clinical ownership"	live
Fig 15.5 R-06	" AHEAD population-health management incomplete"	live
§15.13 objection 2	" The AHEAD State Agreement is a federal contract " (present tense)	live
§15.4	"Four statutory predecessors — Acts 167 and 51, Act 68, and the AHEAD State Agreement"	live
Fig 15.7 row 1	"coordinates \$195M RHT capital and the EAST Fund's wind-down after Vermont's AHEAD withdrawal "	withdrawn

Figure 15.7 was evidently updated for the withdrawal and nothing else in the chapter was. This is the defect pattern the standing directives warn about — one stale fact is five.

Proposed fix. Sweep the whole chapter to the post-withdrawal state. Concretely: Fig 15.3 row 2 becomes an AHEAD *closeout/wind-down* component (status "Withdrawn Jul 2026 → FY2027 closeout reporting") rather than "Active → annual CMS review"; R-01's consequence clause changes from "hospitals enter AHEAD without financial-management capability" to the Act 68 global-budget framing the rest of the book uses ("hospitals enter Act 68 global budgets without financial-management capability") — which is also the stronger claim, since Act 68 survived and AHEAD did not; R-06 and R-09 likewise re-anchor to Act 68/global budgets; §15.13's objection 2 keeps the federal-contract argument but must use the RHT grant as the live example, since the AHEAD contract no longer binds; §15.4's "four statutory predecessors" should not count AHEAD (see defect 9).

Also affected: Fig 15.4's Economics deliverable **"EAST Fund targets"** and Fig 15.6's listing of the **EAST Fund** as a primary contributing component for both *Affordability* and *Robust workforce*. With the fund cut from ~\$138M to a ~\$10M cap, it cannot carry two of Act 167's five goals; Figure 15.6 now overstates the portfolio's benefit base.

3. §15.7 "the single highest-risk component in the register (R-01)" — MISMATCH

Figure 15.5 gives **R-01 a score of 20 and R-10 a score of 20**. R-01 is tied, not single highest. Worse, the tie is with R-10 — "portfolio-management infrastructure not established before 2027 deadlines", annotated in its own Response cell as **"the primary motivation for this chapter"**. So the chapter asserts a superlative for R-01 that its own register assigns equally to the risk the chapter exists to argue.

Proposed fix. "The AHS-GMCB analytics deployment is the clearest case — it carries the register's highest delivery-risk score (R-01, 20, tied only with the governance risk R-10) precisely because three downstream components converge on it, not because its own deadline is nearest."

4. Figure 15.5's score legend does not produce Figure 15.5's scores — MISMATCH

Caption: "Risk score = probability (Low=1 ... High=5) × impact (Low=1 ... Critical=4)."

Under that legend (prob Low 1 / Med 3 / High 5; impact Low 1 / Med 2 / High 3 / Critical 4), **7 of 10 rows fail**:

ID	Prob × Impact	Printed	Legend implies
R-01	High × Critical	20	20 ✓
R-02	Medium × Critical	15	12 ✗
R-03	High × High	16	15 ✗
R-04	Low–Med × Critical	12	8–12 ✓
R-05	Med–High × High	15	12 ✗
R-06	High × High	16	15 ✗
R-07	Med–High × High	15	12 ✗
R-08	Medium × High	12	9 ✗
R-09	High × Medium	12	10 ✗
R-10	High × Critical	20	20 ✓

The printed scores are **not arbitrary** — all ten are exactly reproduced by a different scale: probability Low=1, Low–Med=2.5, Med=3, Med–High=3.75, High=4; impact Low=1, Medium=3, High=4, Critical=5 (R-04 = 2.5×5 = 12.5→12; R-05/R-07 = 3.75×4 = 15; R-09 = 4×3 = 12). **The numbers are right and the caption is wrong.**

Proposed fix — replace the legend with: "Risk score = probability (Low=1, Low–Med=2.5, Medium=3, Med–High=3.75, High=4) × impact (Low=1, Medium=3, High=4, Critical=5); hyphenated bands are scored at the midpoint." Simpler alternative, if the author prefers a clean legend to preserved numbers: restate both axes as 1–5 and recompute all ten scores.

5. R-08's "Equity PM" owner does not exist in the chapter's own role structure — MISMATCH

Figure 15.4 is titled "Five Roles, One Framework" and defines exactly five pillar PMs: Policy, Technology, Economics, Clinical, Operations. R-08's Owner cell reads **"Equity PM + GMCB"** and its Response reads "Equity PM leads SRA as co-equal to budget design". §15.6 likewise specifies **"five** pillar Program Managers". No Equity PM is defined anywhere in the chapter.

This also runs against the project's standing Equity convention (Equity is carried as a cross-cutting concern within each pillar, listed last, not as a sixth pillar) — so inventing a sixth PM role is the wrong fix.

Proposed fix. Reassign R-08 to **"Econ PM + GMCB"** (SRA is a payment-methodology artifact and R-08's Pillars cell already reads "Econ"), and change the Response to "Accelerate: Economics PM leads SRA as co-equal to global-budget design; HCAC equity review required." Alternatively, if an Equity lead is genuinely intended, Figure 15.4 must gain the row and §15.6/§15.5's "five" must become "six" — a larger change that conflicts with the book's Equity treatment.

6. §15.7 "EAST Fund deployment" vs. Figure 15.3's "RHT Program deployment" — MISMATCH

§15.7: "The Economics components — RBP implementation, global-budget design, **EAST Fund deployment** — cannot be productively worked before the Technology components..." Figure 15.3's three Economics rows are RBP implementation, Global-budget design & implementation, and **RHT Program deployment**. The sentence explicitly invites the reader to "read Figure 15.3 by its first column", so the name must match.

Proposed fix. Change §15.7 to "RHT Program deployment", matching Figure 15.3 — the safer direction, since the EAST Fund is now a ~\$10M capped remnant (see defect 2) and cannot plausibly be one of three critical Economics components.

7. Business-case cost figure contradicts itself: **\$4–6 .4M vs \$4.5–7 .5M** — MISMATCH

- Figure 15.7, Total row: "~~\$850K–\$1.3M~~ / ~~**\$4M–\$6.4M**~~"
- §15.11 prose: "The **~\$4–6.4M** cost of a staffed PMO over four to five years is roughly 2% of the portfolio value it protects." ✓ matches the table
- §15.13, objection 3: "The **~\$4.5–7.5M** cost is roughly 2% of the value it protects" ✗

Note also that the same "roughly 2%" is attached to both ranges, which cannot both be true.

Proposed fix. §15.13 → "The ~\$4–6.4M cost is roughly 2% of the value it protects."

8. Figure 15.6's "13/14 sustainable" target vs. the book's "13 of 14 in losses" — probable numeric artifact

The book's Oliver Wyman finding, stated in four other places, is that "13 of 14 hospitals are projected to report losses by 2028" under the conservative scenario. §15.11 and §15.13 use it correctly as the failure state ("the failure of 13 of 14 hospitals to reach sustainability").

Figure 15.6's *Financial sustainability* 2028 target, however, reads "13/14 sustainable" — the same numerator, flipped to the success side. A transformation target of "13 of 14 hospitals financially sustainable" is a coherent goal, but the coincidence with the failure-scenario count is strong evidence the figure was mirrored rather than derived. Nothing in Chapter 15 or the Scorecard (Appendix E, cited in the caption) sources a 13-of-14 sustainability *target*.

Proposed fix. Either source the target or restate it non-numerically — e.g. "deficit trajectory reversed; no hospital in structural operating loss" — so the failure statistic is not doing double duty as the goal.

9. §15.4 "Four statutory predecessors" — MATCH on arithmetic, MISMATCH on category

"Four statutory predecessors — Acts 167 and 51, Act 68, and the AHEAD State Agreement" counts correctly to four. But the AHEAD State Agreement is a **federal model agreement, not a statute** — the chapter itself calls it "a federal contract" in §15.13 — and it was withdrawn (defect 2). Act 51 is also named nowhere else in the chapter and does not appear in the chapter's

Sources: footer, though Ch1's pillar table does establish "Acts 167 (2022), 51 (2023), 68 (2025)".

Proposed fix. "Three statutory predecessors — Acts 167, 51, and 68 — plus the AHEAD State Agreement's closeout obligations, each with their own federal and state reporting requirements."

10. RBP methodology date: Figure 15.4 FY2026 vs Figure 15.3 FY2027 — MISMATCH

- Fig 15.3 row 1: "Act 68 RBP-methodology rulemaking · In progress → **FY2027**"
- Fig 15.3 row 9: "RBP implementation (all hospitals) · Methodology design → **FY2027**"
- Fig 15.4, Policy row: "RBP methodology (**FY2026**); Strategic Plan (Dec 2028); monthly Act 68 compliance"
- Fig 15.4, Economics row: "RBP rates in effect (**FY2027**)" ✓ consistent with Fig 15.3

Fig 15.4's Policy deliverable is the lone FY2026. Elsewhere the book has RBP mandatory FY2027.

Proposed fix. Fig 15.4 Policy row → "RBP methodology (FY2027)".

11. PMO analyst headcount: 2–3 vs 2 — minor MISMATCH

§15.6 specifies "**2–3 PMO analysts**"; Figure 15.7 costs "**2 PMO Analysts**" at \$120K–\$160K (~\$480K–\$640K over 4–5 yr); §15.12 says only "PMO analyst positions posted". The business case therefore under-costs §15.6's recommendation at its upper bound.

Proposed fix. Align §15.6 to "2 PMO analysts", or widen Figure 15.7's row to "2–3 PMO Analysts · \$120K–\$240K · ~\$480K–\$960K" and update the Total row accordingly (which would change the ~\$4M–\$6.4M figure and therefore defect 7's fix too).

12. Portfolio Manager hire date: Q2 2026 vs Q3 2026 — minor MISMATCH

§15.12 roadmap: "Q1–Q2 2026: Post the position... **Q2 2026: Portfolio Manager hired.**" R-10 Response: "Avoid: **hire Portfolio Manager by Q3 2026**; PMO by Q1 2027."

Not strictly contradictory (hiring in Q2 satisfies "by Q3"), but the roadmap and the register should give the reader one date.

Proposed fix. R-10 → "hire Portfolio Manager by Q2 2026", matching §15.12.

13. §15.14 "This is the risk register's R-01 in visual form" — partial MISMATCH

The HTR Simulator row says "Drop the Technology score and watch **Economics and Clinical** fall with it. This is the risk register's R-01 in visual form." R-01's Pillars cell reads "**Tech, Econ**" — Clinical is not in R-01. The Tech→Clinical propagation is R-06's ("Tech, Clin").

Proposed fix. Either broaden R-01's Pillars cell to "Tech, Econ, Clin" (consistent with Ch1's own three-dependent claim, which includes Blueprint/CCBHC risk stratification — see the cross-check below), or change §15.14 to "This is R-01 and R-06 in visual form." The first is preferable: it aligns R-01 with Ch1.

14. Claims that verify clean — MATCH

- §15.6 "the **ten** PMI knowledge areas" / §15.6 heading / Fig 15.1 "10 of 10": the §15.6 paragraph does enumerate exactly ten — scope (WBS), schedule (master schedule), cost (RHT spending), quality, resource (leveling), communications, procurement, stakeholder, risk, integration. ✓
- Figure 15.4 contains exactly five roles, matching its own title "Five Roles, One Framework" and §15.6's "five pillar Program Managers". ✓
- Figure 15.6 contains exactly five goals, matching "Act 167's five goals" as used in §15.4, §15.10 and §15.16. ✓
- §15.13 answers exactly four objections, matching its heading "Answering the Four Objections". ✓
- Figure 15.8 lists six tools across six data rows; §15.14's lead-in makes no count claim. ✓
- Primary-care FTE baseline: Fig 15.6's "FTE gap <200 (**from 370**)" matches the book's 370-FTE-by-2030 shortfall, used consistently in Ch5 and Ch11. ✓
- R-08's FY2028 framing in §15.7 ("has a FY2028 target") matches R-08's register text ("not adopted before FY2028 budgets"). ✓

- §15.11 / §15.13's "13 of 14 hospitals" failure statistic matches Oliver Wyman as cited in Ch2 and Ch14. ✓ (The *target* form in Fig 15.6 does not — defect 8.)
- \$195M RHT award ✓ consistent with Ch1's "\$195M first-year RHT award" and the timeline entry.

Superlative / ranking claims logged for cross-chapter comparison

Verbatim, with context, for later comparison against other chapters:

1. §15.7 — "The AHS-GMCB analytics deployment is **the clearest case** — it is **the single highest-risk component in the register** (R-01) precisely because three downstream components converge on it, not because its own deadline is nearest." *(Contested internally: see defect 3; also note Ch1 uses the identical "clearest case" construction for Technology — flagged below.)*
2. BEYOND VERMONT box, §15.3 — the portfolio inventory exercise "is **often the single highest-value first step** toward the kind of portfolio governance this chapter recommends."
3. §15.15 — "**The single most consequential decision** is not whether to hire a Portfolio Manager but whether that role is given authority over sequencing." *(Compare Appendix G: "This is **the single most consequential open question** for any hospital CFO reading this book" — about transition financing. Two different "single most consequential" items; different framings ("decision" vs "open question") and different audiences (state leader vs hospital CFO), so probably legitimate, but worth the author's eye.)*
4. §15.13 — "the political-sustainability chapter's **most dangerous failure mode**" (the Dec 2028 plan arriving descriptive rather than binding). *Needs checking against Ch14, which owns that designation — does Ch14 name the same failure mode as its most dangerous?*
5. Epigraph — the governance gap "is **the default state of most transformation efforts**, not a Vermont peculiarity."
6. BEYOND VERMONT box — "**Most organizations underestimate** their own portfolio size until they do this inventory."
7. §15.15 — "**Most organizations discover** in that exercise that they are running more simultaneous transformation components than they had counted." *(Near-duplicate of 6 — see Other issues.)*
8. §15.14, Transformation Friction Index row — "the pillar whose improvement **would lift delivered readiness most** is where the portfolio manager's attention belongs."
9. §15.11 / §15.13 — the PMO cost "is **roughly 2%** of the portfolio value it protects" (stated twice, against two different cost ranges — defect 7).
10. §15.13 — "the alternative is the **multi-billion-dollar** partial-transformation outcome."

For comparison, the corresponding Chapter 1 superlatives found while cross-checking: "**Technology is the clearest case**"; "it is where the system's **most consequential execution gap** actually sits"; "The AHS-GMCB analytics capability is **the single highest-risk gap in the system**"; "Vermont's **most significant current sequencing risk** is the gap between the FY2028 start of Act 68's hospital global budgets and the analytics-capability deployment timeline"; "**The most important action** you can take before Act 68's global budgets take effect in FY2028 is completing HCC gap closure"; and Ch11's "The Workforce Crisis — Vermont's **Most Binding Operational Constraint**".

Two of these want a cross-chapter look. First, §15.7 reuses Ch1's exact phrase "**the clearest case**" for a different subject (Ch1: the Technology *pillar*; Ch15: the analytics *component*) — compatible in substance, but the verbal echo reads as a copy. Second, and more substantive: Ch11 calls **workforce** "Vermont's Most Binding Operational Constraint" while Ch1 and Ch15 both put the binding constraint on **Technology/analytics**. Ch11's is scoped to *operational* constraints and Ch1's to the dependency sequence, so they may well coexist — but this is exactly the "two different dependencies both called the most underestimated" pattern the standing directives call out, and it should be adjudicated in the cross-chapter pass. §15.14's first row ("Identify which pillar is the binding constraint on the whole portfolio right now") deliberately leaves the answer to the tool, which is the right treatment; the prose chapters are less careful.

Cross-check against Chapter 1's AHS-GMCB claim

Verdict: consistent on the substance and on the number three. Two wording issues and one figure-level gap.

Chapter 1 (§1.3, risk-identification discussion), verbatim:

Technology is the clearest case: Economics and Clinical both depend on it directly, and right now it is where the system's most consequential execution gap actually sits. The AHS-GMCB analytics capability is the single highest-risk gap in the system: a delay there does not delay one program but three, holding up financial management for global budgets, equity measurement for the Statewide Strategic Plan, and risk stratification for Blueprint's CCBHC expansion at once.

Chapter 15 §15.7, verbatim:

The AHS-GMCB analytics deployment is the clearest case — it is the single highest-risk component in the register (R-01) precisely because three downstream components converge on it, not because its own deadline is nearest.

Point-by-point:

Element	Ch1	Ch15	
Subject	AHS-GMCB analytics capability	AHS-GMCB analytics deployment	✓ same
Ranking	"the single highest-risk gap in the system"	"the single highest-risk component in the register"	✓ same claim, different universe of comparison

Element	Ch1	Ch15	
The number	"does not delay one program but three"	"three downstream components converge on it"	✓ three in both
Reasoning	convergence of dependents, not deadline proximity	convergence of dependents, not deadline proximity	✓ same
The three named	(a) financial management for global budgets, (b) equity measurement for the Statewide Strategic Plan, (c) risk stratification for Blueprint's CCBHC expansion	not named	see below

The two claims are mutually reinforcing and neither has drifted. **No fix required for consistency between them.** Three observations, in descending importance:

- Ch15 never names its three, and Figure 15.3 cannot supply them cleanly.** Mapping Ch1's three onto Figure 15.3's rows: (a) → "Global-budget design & implementation" ✓; (b) → "Statewide Strategic Plan development" ✓; (c) → splits across **two** rows, "Blueprint PCMH" *and* "CCBHC certification (5 entities)". So a reader who tries to verify Ch15's "three" in Ch15's own register gets three-or-four depending on how (c) is counted, and R-01's Pillars cell ("Tech, Econ") does not admit the Clinical dependent at all. Worth fixing while R-01 is being touched for defect 2: name the three in §15.7 as Ch1 does, and broaden R-01's Pillars cell to "Tech, Econ, Clin" (which also resolves defect 13).
- Ch15's superlative is over-tight against its own register** (defect 3): R-10 ties R-01 at 20. Ch1's version does not have this problem, because Ch1 is ranking *gaps in the system*, and Vermont's missing PMO is not one of Ch1's gaps. The fix belongs in Ch15, not Ch1 — the chapter yields.
- R-01's consequence clause is stale where Ch1's is not.** Ch1's claim is anchored to Act 68 global budgets, which survived; R-01 is anchored to hospitals "enter[ing] AHEAD", which they never will (see defect 2). So the two claims agree on reasoning but Ch15 justifies it with an event the book elsewhere says was cancelled. Re-anchoring R-01 to Act 68 makes Ch15 *more* consistent with Ch1, not less.

Also consistent across the two chapters: the Jan 2027 operational date (Fig 15.3 "Procurement → Jan 2027 operational") sits correctly ahead of Ch1's hard constraint ("hospitals will begin bearing global-budget financial accountability in January 2028"), and Ch1's advice to hospitals ("Do not wait for the AHS-GMCB analytics capability... Start with VHCURES direct access... now") matches R-01's response verbatim in substance ("Accelerate: parallel VHCURES direct-access; HCC gap closure as compensating investment"). ✓

Other issues

Genuine repetition

- §15.11's closing sentence and §15.13's paragraph restate the same three-item failure list twice, near-verbatim, three paragraphs apart.** This is the clearest duplication in the chapter.

§15.11 (before Figure 15.7): "The cost of not building it is the December 2028 plan arriving as a descriptive document rather than a binding commitment — the continuation of the Oliver Wyman deficit trajectory, the failure of 13 of 14 hospitals to reach sustainability, and the loss of the political window Vermont's mandatory architecture has opened. **That is not a \$0 outcome.**"

§15.13 (after Figure 15.7): "Without adequate PMO infrastructure, Vermont risks delivering the December 2028 plan as a descriptive document rather than a binding commitment — the political-sustainability chapter's most dangerous failure mode. **That is not a \$0 outcome.** It is a multi-billion-dollar one: the continuation of the Oliver Wyman deficit trajectory, the failure of 13 of 14 hospitals to reach sustainability, and the loss of the political window that Vermont's mandatory architecture has opened."

Same three-item list, same "That is not a \$0 outcome" sentence, same argument. **Proposed fix:** cut the second half of §15.11's paragraph, leaving it to set up Figure 15.7 ("The cost-benefit analysis in Figure 15.7 answers that objection directly. The ~\$4–6.4M cost of a staffed PMO over four to five years is roughly 2% of the portfolio value it protects."), and let the post-table paragraph carry the cost-of-failure argument in full. That also removes one of the two conflicting "roughly 2%" statements.

- The "most organizations underestimate their portfolio" point is made twice — BEYOND VERMONT box in §15.3** ("Most organizations underestimate their own portfolio size until they do this inventory, because individual initiatives are usually owned by different departments that do not otherwise compare notes") and §15.15's hospital-PMO paragraph ("Most organizations discover in that exercise that they are running more simultaneous transformation components than they had counted, that several have no named owner, and that two or three are downstream of a gate nobody is tracking"). The §15.15 version adds two findings (no named owner; downstream of an untracked gate) so it is not pure restatement, but the shared opening clause makes it read as one. **Proposed fix:** trim the BEYOND VERMONT sentence to the causal half only ("because individual initiatives are usually owned by different departments that do not otherwise compare notes"), letting §15.15 own the finding.
- "Managed as a collection of projects" appears three times** — §15.2 ("AHS is managing Vermont's transformation as a collection of projects without the program or portfolio infrastructure"), §15.4 ("not as a collection of programs each doing their best"), R-10 ("managed as a collection of projects"). This is the chapter's thesis phrase, so recurrence is defensible; logging it as *legitimate recurrence*, not duplication.

4. **The Statewide Strategic Plan / Dec 2028 deadline is invoked in nine sections** (epigraph, 15.2, 15.3, 15.4, 15.6, 15.11, 15.12, 15.13, 15.15). Legitimate recurrence — it is the portfolio's single deadline and the chapter is organised around it.

Thin sections

5. **§15.9 "The Portfolio Risk Register" has no prose at all** — the Heading2 is followed immediately by Figure 15.5 and its caption. It is the only numbered section in the chapter with zero body text, and it carries the chapter's most-cited artifact: R-01 is referenced from §15.7 and §15.14, R-08 from §15.7, R-10 from §15.7 (implicitly) and Figure 15.7. A reader arriving at §15.9 gets no statement of how the register is maintained, who reviews it, or at what cadence — which matters, because §15.6 makes "risk is analyzed in this book but not managed operationally through a maintained register" one of its ten knowledge-area gaps. One short paragraph is needed; it is also the natural place to state the scoring scale correctly (defect 4).
6. **§15.8 "Predictive vs. Adaptive Approaches" is a single paragraph with no figure**, though it makes a three-way classification (predictive / adaptive / hybrid) over ten named components and assigns credentials to each class. Every comparable classification in the chapter gets a table. This is also where the un-listed "HEROI methodology" component enters (defect 1). Given the chapter's density of figures, the asymmetry is noticeable.

Internal inconsistencies of citation style

7. **Figure 15.8 cites routes for two of six tools and not for the other four.** Rows 5 and 6 give paths ("Transformation Scorecard — /research-lab/knowledge-workspace?tab=scorecard"; "Impact Simulation — /impact-simulation"); rows 1–4 name the tool only (Transformation Friction Index, Investment Tracker, HTI Dashboard, HTR Simulator). Comparable tables elsewhere in the book (e.g. Figure 10.11) give a path for every row. **Route/promise verification was out of scope for this audit** — per the project's own rule, a route must be opened and confirmed, not inferred — so no claim is made here about whether any of these six resolve or whether each tool delivers what the row promises. Flagging only the inconsistency, and noting that four rows currently give a reader no way to find the tool.
8. **Figures 15.5 and 15.7 also duplicate rows 5 and 6 of Figure 15.8's content area** in that §15.14 row 4 and row 6 describe closely similar exercises — "Test what happens to composite readiness when one component slips" (HTR Simulator) and "Model what a single component slipping does to the rest of the portfolio" (Impact Simulation). The two "what look for" cells do distinguish them (cascade visualisation vs. slack identification), but the "Do this" columns are near-identical and a reader will not know which to open. **Proposed fix:** sharpen row 4's "Do this" to the pillar-score mechanic it actually describes ("Drop one pillar's score and watch the dependent pillars fall") and reserve "component slippage" language for row 6.

Formatting observations (noted, not fixed — read-only audit)

9. The three callout boxes each render their label and body as one unbroken run: "BEYOND VERMONT!The exercise that produced...", "THE PORTFOLIO MANAGER'S SEQUENCING TESTFor each component, in order:", "TRY THIS — Find your own portfolio's binding constraint.Open the Transformation Friction Index...". This may be an artifact of flattening the XML to text rather than a defect in the document; it was **not** verified against a render, and `check_format.py` was **not** run, because this audit made no edits. If any of the above fixes are applied, `python3 book-build/check_format.py` and a `render_check.py` pass over the chapter are required before reporting.
10. The sequencing-test box's four steps are a run-together list in the XML text stream ("Which pillar does it belong to?... Is that pillar's upstream gate open?... If closed — ...If open — ..."). Same caveat as 9: verify against a render before treating as a defect.

Summary of defects by severity

Blocking (factual/self-consistency): 1 (19 vs 17 components), 2 (AHEAD live vs withdrawn, 6 locations), 3 (R-01 "single highest" is a tie), 4 (risk-score legend wrong for 7 of 10 rows), 7 (\$4–6.4M vs \$4.5–7.5M).

Should fix: 5 (Equity PM undefined), 6 (EAST Fund vs RHT Program deployment), 8 (13/14 target), 9 ("four statutory predecessors" includes a non-statute), 10 (RBP FY2026 vs FY2027), 13 (R-01 vs R-06 in Figure 15.8), repetition item 1 (duplicated failure paragraph), thin section 5 (§15.9 has no prose).

Minor: 11 (2 vs 2–3 analysts), 12 (Q2 vs Q3 2026 hire), repetition item 2, thin section 6 (§15.8), citation-style items 7 and 8.

Verified clean: ten knowledge areas, five roles, five goals, four objections, six tools, 370-FTE baseline, \$195M RHT, R-08's FY2028 framing, the 13-of-14 failure statistic, and — most importantly for this audit's specific charge — **the Ch1 ↔ Ch15 AHS-GMCB "three dependents" claim.**

===== FILE: audit_ch16.md =====

Chapter 16 Audit

Chapter 16: The AHS Restructuring Roadmap — A Five-Pillar Framework for System Architects Audited 2026-09-27, read-only, from `word/document.xml` byte range **4,644,397 – 4,830,522** (real Heading1 at 4,644,397; next Heading1 = "Conclusion — What Vermont Proves..." at 4,830,522). The TOC decoy at offset 316,009 was ignored. Tables parsed as real `<w:tbl>` cell grids; text extracted with `<w:t(?:\s[^\>]*)?>`.

Structure: 10 H2 sections (16.1–16.10), 3 H3 (16.2.1, 16.3.1–16.3.3 — note 16.3.x are H3, 16.6.1 is H3), 6 tables / 6 figure captions, 1 pull-quote (Oliver Wyman, Aug 2024).

Figures found (ground truth)

Fig	Table	Shape	Ground-truth contents
16.1	Table 1	header + 7 data rows, 3 cols	Act 68 statutory requirements: Strategic Plan (Dec 1 2028); Affordability benchmarks (HCAC); TCOC targets (2.5% hospital-spending reduction FY2026; AHEAD TCOC from 2027); Standardized accountability metrics (monthly); Primary-care investment plan (AHEAD targets from 2027); Health-equity strategy; Workforce strategy (RHT covers 2026–2030)
16.2	Table 2	header + 4 data rows, 4 cols	The Four Structural Gaps: program-silo/HSA misalignment; absence of a Planning & Effectiveness function (fix names a " Division of Planning, Analytics, and Effectiveness "); under-resourcing of transformation management (" the single most likely cause of plan failure "); fragmented SDOH integration
16.3	Table 3	header + 14 data rows, 4 cols	14 HSAs. Populations: Burlington ~230K; Barre/Montpelier ~65K; Rutland ~62K; White River Jct. ~59K; St. Albans ~53K; Bennington ~48K; Middlebury ~46K; Brattleboro ~42K; Newport ~41K; St. Johnsbury ~39K; Springfield ~31K; Morrisville ~31K; Randolph ~28K; Townshend ~14K. Descriptors: Burlington "most diverse; non-rural"; Bennington "most rural" , "9 COEs — secondary RSC role"; Newport "Among most financially vulnerable"; St. Johnsbury "highest poverty (Caledonia/Essex)"; Springfield "most complex plan"; Randolph "among most vulnerable"; Townshend "19-bed, Vermont's smallest"
16.4	Table 4	header + 15 data rows, 4 cols	Timeline. Key rows: FY2026 ×5 (grants; analytics procurement "Underway as of November 2025"; 5 CCBHCs by July 2026; " AHEAD Medicaid global budget operational (January 2026) — Operational "; RHT "\$195M received Dec 2025"); FY2026 Q2–Q4 ×3 (final hospital plans; HSA Coordinators; " Division of Planning and Effectiveness established "); FY2027 (Oct 2026) RBP max rates; FY2027 (Jan 2027) "Act 68 hospital global budgets take effect (FY2028)" — "Nine-year model" ; FY2027 "Primary Care AHEAD operational" ; FY2028 (Oct 2027) non-CAH global budgets; December 2028 Strategic Plan; FY2030 (Oct 2029) all-hospital global budgets incl. CAHs
16.5	Table 5	header + 4 data rows, 3 cols	Plan delivery Dec 2028; HCAC members = 18; RHT capital = \$195M (Dec 2025 award); projected direct transformation savings (5 yr) >\$400M
16.6	Table 6	header + 3 data rows, 3 cols	Five-Pillar Map; HTI Dashboard; HTR Simulator

Claims checked against their own source (MATCH/MISMATCH + proposed fix)

MISMATCH 1 — the chapter contradicts itself on AHEAD (most serious finding)

§16.4 Pillar 3 states the settled book-wide fact: *"EAST Fund wind-down (capped near \$10M annually after Vermont's AHEAD withdrawal, against the ~\$138M originally expected)." The Introduction and Ch1 confirm it: "in July 2026, Vermont formally withdrew from AHEAD after a CMS renegotiation that cut Vermont's expected EAST Fund from roughly \$138 million to a cap near \$10 million."*

Every **other** AHEAD reference in Chapter 16 is written as if Vermont were still a participant, with future obligations:

Location	Text	Problem
Table 1 (Fig 16.1), TCOC row	"AHEAD TCOC from 2027"	Future federal TCOC obligation Vermont no longer has
Table 1, primary-care row	"an AHEAD requirement ... AHEAD targets from 2027"	same
§16.3.1	"Federal-relationship management — active management of CMS (AHEAD) ... The AHEAD State Agreement alone requires ongoing methodology negotiation, monitoring, and withdrawal-condition management"	Present/future tense for an exited agreement
§16.4 Pillar 1	" AHEAD Model management (ongoing CMS negotiation, TCOC compliance, withdrawal conditions)"	Lists an exited model as a live AHS Policy function
Table 4	" FY2027 Primary Care AHEAD operational — enhanced primary-care payments / Replaces expiring Medicare demo"	Medicare-side AHEAD is gone

Location	Text	Problem
Table 4	"AHEAD Medicaid global budget operational (January 2026) — Operational "	Jan 2026 predates the July 2026 exit, but the <i>current</i> status "Operational" needs qualifying
Table 3, Randolph row	" AHEAD CAH protections "	Cites AHEAD as a live protection for Gifford
§16.6 payment-reform section	" AHEAD TCOC targets and performance " as 2028 plan content	Plan content premised on continued participation

Proposed fix: rewrite these eight to the post-withdrawal frame, in the same language the Introduction uses. E.g. Pillar 1 → "AHEAD wind-down and successor-agreement strategy (EAST Fund closeout, Medicaid global-budget continuation, terms for any second federal agreement)"; Table 1 TCOC deadline → "2.5% hospital-spending reduction for FY2026; Act 68 TCOC targets in the plan"; Table 1 primary-care → "a plan to raise primary care as a share of total spend (retained as state policy after the AHEAD withdrawal)"; Table 4 → "FY2028 (Jan 2028) Enhanced primary-care payments — state/Medicaid-funded successor to the expired Medicare demo"; Randolph → "Act 68 CAH global-budget deferral to FY2030"; §16.6 → "Act 68 TCOC targets and performance."

MISMATCH 2 — Table 4 dates Act 68 global budgets to Jan 2027; the rest of the book says Jan 1, 2028

Table 4: "***FY2027 (Jan 2027)** | Act 68 hospital global budgets take effect (**FY2028**) | ... | Nine-year model.*" The Deadline cell and its own Milestone cell disagree, and the parallel timeline elsewhere in the manuscript reads "***January 1, 2028** | Act 68 hospital global budgets take effect (FY2028) | AHS / DVHA / GMCB / CMS | Nine-year model; Primary Care AHEAD and global budgets launch January 2028.*" §16.4 Pillar 1 also says "non-CAH global budgets FY2028." It also breaks Table 4's own FY-labelling convention (every other row uses the October fiscal-year start: FY2027 = Oct 2026, FY2028 = Oct 2027, FY2030 = Oct 2029).

Proposed fix: change the Deadline cell from **FY2027 (Jan 2027)** to **FY2028 (Jan 2028)**.

Related: the same row plus the "FY2028 (Oct 2027) | Global budgets for non-CAH hospitals" row now describe the same event twice with two different dates. Recommend keeping one "Jan 1, 2028 — Act 68 global budgets take effect for non-CAH hospitals" row and dropping the duplicate.

MISMATCH 3 — §16.9 cites "the three decisions this chapter identifies as not yet made." Chapter 16 identifies no such set.

"If you are an AHS leader: the three decisions this chapter identifies as not yet made are the ones worth your attention." There is no enumerated set of three decisions anywhere in Chapter 16. The section actually titled "**The Three Decisions That Are Not Yet Made**" is in the **Conclusion** ("As of April 2026, Vermont's transformation hinges on three decisions that have not yet been made"). Table 4 flags only two items as pending Secretary-level decisions (HSA Coordinator roles; Division of Planning and Effectiveness).

Proposed fix: either cross-reference — "the three decisions the Conclusion identifies as not yet made" — or replace with the two this chapter genuinely puts on the Secretary's desk: "the two organizational decisions Table 16.4 assigns to the Secretary — the HSA Coordinator model and the Division of Planning and Effectiveness — are the ones worth your attention."

MISMATCH 4 — Table 3 HSA populations sum to ~789K; Vermont's population is 647,000

The 14 population figures total **789,000**, exceeding by 142K (+22%) the state figure the book states twice: "*a state of 647,000 people*" and Appendix A's "*Its 647,000 residents live across 14 counties and 255 towns.*" Burlington's "~230K" is the likely culprit (Chittenden County is ~168K). HSA catchments can overlap slightly, but not by 22%.

Proposed fix: re-source the HSA figures against the Oliver Wyman / RHT application HSA catchment table and either correct them so they reconcile to 647K, or add a footnote to Figure 16.3 stating that the figures are hospital *service-area catchments* which overlap at borders and therefore do not sum to state population. The former is preferable; a five-pillar readiness chapter cannot ship a table whose column does not reconcile.

MISMATCH 5 — Bennington is labelled "most rural"; the book says the Northeast Kingdom is

Table 3 calls Bennington (~48K, with a Regional Service Center role and 9 COEs) "**most rural**", while labelling Newport, St. Johnsbury, Randolph and Townshend merely "very rural." Elsewhere the manuscript is unambiguous: the NEK "*is **the most rural** part of the state, the most economically distressed, the most underserved by healthcare providers*"; the glossary entry reads "*Northeast Kingdom — Caledonia, Essex, and Orleans counties in Vermont's **most rural** region.*" Ch1 also groups Bennington with Burlington and Rutland as a consolidation hub. It is also internally odd for Bennington to be "most rural" while Townshend is "Vermont's smallest" hospital in a "very rural" HSA.

Proposed fix: Bennington → "~48K, aging, rural; southwest RSC". Reserve "most rural" for the NEK rows, or drop the superlative from the table entirely and let "very rural" carry it.

MISMATCH 6 — the Planning unit has three different names inside one chapter

- Table 2, gap column: "Absence of a **Planning & Effectiveness** function"
- Table 2, required-change column: "Establish a **Division of Planning, Analytics, and Effectiveness**"
- Table 4, §16.6 plan outline, §16.9, §16.10 Key Concepts: "**Division of Planning and Effectiveness**"

The rest of the book (Ch1 five-pillar map, Ch1 Operations sequencing) uses "**Division of Planning and Effectiveness**" consistently. **Proposed fix:** change the Table 2 required-change cell to "Establish a Division of Planning and Effectiveness" (the analytics function is already named in the same cell: "partner with GMCB on a joint analytics platform"). Key Concepts and Table 4 are correct as-is.

MISMATCH 7 — §16.4 lists five pillars; §16.6 presents Equity as a sixth

§16.4 states the framework as exactly five pillars, with Equity absent — matching the book's rule ("Equity — the imperative, not a pillar"). But the §16.6 plan outline labels its sections by pillar in the same typographic pattern — "— Economics", "— Clinical", "— Operations", "— Technology", "— Policy & Operations" — and then writes "Health Equity Strategy — **Equity**", which reads as a sixth pillar.

Proposed fix: "Health Equity Strategy — cross-cutting (15–20 pp)", or drop the label so it does not parse as a pillar tag.

Claims that MATCH their source

- **"13 of 14 hospitals in losses by 2028"** (§16.6.1) — MATCH. Ch6/Ch14: Oliver Wyman conservative scenario (3.5% revenue / 5% expense growth), \$700M cumulative 5-yr deficit. Correctly attributed to the diagnostic scenario, not presented as a forecast.
- **"9 COEs" for Southwestern Vermont Medical Center** (Table 3) — MATCH. Ch2 Fig 2.2: "Southwestern VT Medical Center (Bennington) 9 — Cancer surgery (non-complex), geriatric care, orthopedics, psychiatry (adult/adolescent), radiation therapy."
- **"2.5% hospital-spending reduction for FY2026"** (Table 1) — MATCH. Ch1 five-pillar map: "Act 68 hospital spending reduction (2.5%, FY2026)."
- **"\$195M RHT, Vermont's December 2025 award"** — MATCH across Table 2, Table 4, Table 5 and Ch11/Ch12 ("\$195 million first-year award"). "RHT covers 2026–2030" (Table 1) consistent.
- **"~\$138M expected → cap near \$10M" EAST Fund** (§16.4) — MATCH the Introduction verbatim.
- **CAH operating-profit gap \$938 vs \$2,784; PPS admin-cost premium \$3,826 vs \$1,427** (§16.4) — internally consistent; sourced to Oliver Wyman. Not re-derived here.
- **">\$400M projected savings"** — Table 5 and §16.4 agree ("Oliver Wyman's \$400M+").
- **HCAC = 18 members** — three internal statements agree (§16.1, §16.3.1, Table 5) and the composition lists match. No independent corroboration elsewhere in the book, so the chapter is the sole source; flagged as unverified rather than wrong.
- **Four Structural Gaps / Table 2 = 4 rows** — MATCH. **14 HSAs / Table 3 = 14 rows** — MATCH. **Six figures, all captioned and numbered 16.1–16.6 in order** — MATCH.
- **"CCBHC certification for 5 new entities (July 2026 target)"** — §16.4 Pillar 4 says "5 new entities by July 2026." Internally consistent.
- **Recurring section headings — 16.8 Work This Chapter on the Platform, 16.9 Implications for You, 16.10 Key Concepts in This Chapter** are byte-identical to the canonical forms. No **## Sources** heading; Figures 16.1–16.5 carry plain **Sources: ...** paragraphs. Figure 16.6 has **no Sources** line — consistent with the platform-tools figures in other chapters.

Superlative/ranking claims logged for cross-chapter comparison

Verbatim, with context, for later cross-chapter deduplication:

1. §16.2.1, Table 2, under-resourcing row: *"The absence of PM infrastructure is **the single most likely cause of plan failure.**"*
2. §16.4, Pillar 2: *"analytics-vendor implementation (**the most urgent near-term investment**)".*
3. §16.6: *"The Strategic Plan is **the most consequential document AHS will produce this decade**; it will define Vermont's system for the decade following its delivery."*
4. §16.7: *"...it may be **the most valuable contribution Vermont makes.**"*
5. §16.7 (next paragraph): *"Those answers may be **the most valuable policy contribution Vermont makes** to American healthcare in a generation."*
6. §16.9: *"That mismatch is **the most common structural reason** state reform efforts stall."*
7. §16.2: *"AHS's organizational structure is **one of the primary barriers** to transformation."* (hedged — fine)
8. Table 3: Burlington *"**most diverse**"*; Bennington *"**most rural**"* (see MISMATCH 5); Newport *"**Among most financially vulnerable**"*; St. Johnsbury *"**highest poverty** (Caledonia/Essex)"*; Springfield *"**most complex** plan given financial position"*; Randolph *"**among most vulnerable**"*; Townshend *"19-bed, **Vermont's smallest**"*.
9. Chapter front matter (from Ch14's reader guide, not this chapter): Chapter 16 is described as *"**the most direct bridge** between policy mandate and organizational design."*

Cross-chapter items #1 and #2 need reconciling within this chapter, not just across chapters. Adjacent sections nominate two different things as the top priority: PM infrastructure (Operations) as "the single most likely cause of plan failure" and the analytics vendor (Technology) as "the most urgent near-term investment." They are not strictly contradictory (cause-of-failure vs. near-term spend) but they read as competing rankings two pages apart — the exact "two different dependencies both called the most underestimated" defect the definition-of-done warns about. Recommend keeping #1 and demoting #2 to "the near-term investment with the shortest path to usable output."

Item #6 should also be checked against Ch11/Ch15's claims about what most commonly stalls reform (Ch11 calls workforce "Vermont's Most Binding Operational Constraint").

Other issues

Genuine repetition — §16.7's two closing paragraphs restate each other

Paragraph 1 ends: *"Vermont will have answers by December 2028. They will be grounded in real experience with a real system under real pressure. That is not nothing. In American health policy, where policy debates run decades ahead of implementation evidence, it may be the most valuable contribution Vermont makes."*

Paragraph 2 opens: *"Vermont will have answers by December 2028 — grounded in real experience with a real system under real pressure. Those answers may be the most valuable policy contribution Vermont makes to American healthcare in a generation."*

This is near-verbatim duplication of a full two-sentence construction, not legitimate recurrence. **Proposed fix:** delete the duplicated opening of paragraph 2 and keep only its new content: *"The significance is not that Vermont is large or powerful; it is that Vermont is early —*

early enough to produce a tested model before the rest of the country reaches the same crisis point, and early enough for the lessons to remain actionable."

Appending that sentence to paragraph 1 loses nothing and removes the restatement.

Inconsistent treatment of the four TBD-tier hospitals

Ch2 designates **Grace Cottage, Gifford, North Country, and Porter** alike as "TBD — Tier 3, Requires discussion / further assessment needed." Table 3 preserves that for two of them (Middlebury/Porter "TBD tier"; Randolph/Gifford "TBD tier — comprehensive assessment needed") but pre-empts it for the other two: Newport/North Country **"REH candidacy"** and Townshend/Grace Cottage **"REH conversion the likely path."** Either the chapter is making a recommendation Ch2 has not made, or Ch2 is stale. Flagged rather than fixed — it is a substantive designation call, the author's to make. If the recommendation is intended, say so explicitly ("this chapter's recommended resolution of the Tier 3 TBD designation").

Timeline-duplication and status-date drift in Table 4

- Two rows describe Act 68 global budgets taking effect (see MISMATCH 2).
- Statuses are dated inconsistently: "Underway as of November 2025", "\$195M received Dec 2025", "Plans expected early 2026", "In progress". Against a September 2026 audit date — and a July 2026 AHEAD withdrawal — the whole Status column is written from a late-2025 vantage point. It is not factually wrong about late 2025, but it is stale-as-current. Recommend a single as-of date in the Figure 16.4 caption, matching the Conclusion's "As of April 2026" convention or later.

Thin sections

- §16.3.3 (Level 3 — Program Delivery Operator) is a single paragraph, against §16.3.1's five paragraphs plus a governance paragraph and §16.3.2's paragraph plus a 14-row table. The three-level operating model is the chapter's central construct and its third level is the thinnest. It also largely restates Table 2's program-silo row and §16.3.2's HSA Coordinator description rather than adding new material.
- §16.7 is two paragraphs, one of which is the duplicate above — effectively one paragraph for a section that the chapter's own opening promises will "address that question directly" (the national-template question). It poses four rhetorical questions and answers none of them.
- §16.6.1 ("The Central Commitment the Plan Must Make") is strong, and is where Table 5 lands — no issue, noted for contrast.

Not checked here (out of the read-only single-chapter scope)

Criterion-5 promise delivery was **not** verified: §16.8 Table 6 names three tools (Five-Pillar Map, HTI Dashboard, HTR Simulator) with specific promises — "A pillar with no named organizational owner in the restructuring", "Whether Operations-pillar capacity is rising fast enough", "Score a state with strong Policy and Economics but weak Operations. The composite collapse...". Each needs a tool-side check that the described output actually appears and that each tool tags Chapter 16. A 200 is not a delivered promise. Likewise §16.10's Key Concepts and the "reform cascade (Act 167 → Act 51 → Act 68)" glossary entry were not checked against the platform glossary.

Summary

7 mismatches, 1 genuine duplication, 3 further consistency issues, 2 thin sections. The two that most need fixing before anything else: the **AHEAD tense contradiction** (the chapter states the withdrawal in one paragraph and contradicts it in eight other places) and the **Jan 2027 vs Jan 1 2028 global-budget date**, which is a hard factual error against the statutory timeline the rest of the book gives. Nothing in this file has been applied to the .docx — the audit was read-only as instructed.

===== FILE: audit_conclusion_appendices.md =====

Conclusion + Appendices Audit

Scope: `HTR_Book_v42.docx` → `word/document.xml`, from the **real Conclusion** Heading1 (byte offset 4,830,522 — not the TOC decoy) to end of document. Extracted order-preserving via top-level `<w:tbl>` split + `<w:p>` in the gaps; text via `<w:t(?:\s[^\>]*)?>`. Sections covered: Conclusion, Appendices A–H, Figure Index, Bibliography, Index. **Read-only — the .docx was not modified.**

Section IDs below in `[n]` refer to the extracted element order.

Figures found (ground truth)

Numbered figures in range (9 tables carry figure captions; 8 more tables carry none):

Figure	Caption	Backing table (ground truth, abbreviated)
A.1	Vermont demographic transformation	6 data rows: 65+ by 2040 30%+ ("up from 21.7% in 2020 — a 57% increase"); working-age by 2040 −13% (~367,000 → ~318,000); total pop 2020–2040 Flat, ~612,000–624,000 ; insured 2025 97% ; PCP shortage by 2030 370 FTE ("112 family medicine, 190 other primary-care specialties"); rental vacancy 3%

Figure	Caption	Backing table (ground truth, abbreviated)
A.2	GMCB regulatory functions	6 data rows. Budget review row: "FY25 approved \$3.7B (+4.1% vs. 8% requested)". Enforcement row: "FY23: UVMHC \$80.3M , RRMC \$11.1M ". RBP from FY2027. Global budgets FY2028–2030; analytics vendor McKinsey, 2025–2026
A.3	Vermont's 14 hospitals	14 data rows. UVMHC: admin \$3,826 vs \$1,427 , FY23 overage \$80.3M. Porter Hospital: "FY23 overage of \$11M" . RRMC: "FY23 overage \$11.1M (partially enforced)". NVRH: "FY23 overage (not enforced)". Springfield: plan not approved as of Jan 2026. Grace Cottage: denials \$76K→\$1,274 (FY25), FY24 \$4M loss → ~\$1M
A.4	Initiative themes, Jan 2026 (148 initiatives)	6 data rows: ~55 + ~20 + ~25 + ~20 + ~15 + ~13
B.1	Act 68 statutory timeline	17 data rows, Jan 2026 → "every 3 years from 2028". Includes both "January 1, 2028 — Act 68 hospital global budgets take effect (FY2028) ... Nine-year model; Primary Care AHEAD and global budgets launch" and "FY2028 (Oct 1, 2027) — Global budgets for non-CAH (PPS) hospitals" . RHT: "\$195M award; spending window 2026–2032"
B.2	Policy-pillar implementation matrix	5 data rows. RHT row: "Vermont RHT: \$195M over 5 years ". State-employee direct contracting: "Year 1: \$48M Vermont savings"
E.1	Policy-pillar scorecard	5 rows. RHT: "\$195M awarded for FY2026, first year ". Federal-state alignment 2028 target: "Full AHEAD compliance; second agreement negotiated"
E.2	Technology-pillar scorecard	5 rows
E.3	Economics-pillar scorecard	5 rows. Margins: 2022 9 of 14 in losses (FY2023, worst −8.9%) → 2026 6 of 14 (FY2024). CAH \$938 vs \$2,784. UVMHC admin \$3,826 = "267% of \$1,427". VBC row 2028 target: "Medicare: fee-for-service (AHEAD withdrawn)"
E.4	Clinical-pillar scorecard	6 rows. CCBHC 0 → 5 planned → 7. 370-FTE gap. Readmissions 14.8%
E.5	Equity Imperative scorecard	6 rows. BIPOC access 79–81%, 11–12 pt gap. Essex uninsurance 8%
E.6	Operations-pillar scorecard	6 rows
I.1	"The HTR Lab Workbook maps each of the five pillars..."	Appendix H caption, numbered I.1 — no table; caption orphaned after the CAPSTONE callout

Unnumbered tables in range (no figure caption, so absent from the Figure Index): Appendix C's 14-hospital COE table [99]; Appendix F's F.3 pillar/chapter table [156]; Appendix G's G.1 glossary (15 terms) [176] and G.2 worksheet [179]; the two single-cell callouts "THE GAP" [185] and "WHAT THIS MEANS" [194]; the "CAPSTONE" callout [217].

Figure Index [331]–[343] reproduces exactly A.1–A.4, B.1–B.2, E.1–E.6, I.1 — i.e. it faithfully mirrors the defect: **no C, D, F, G or H figure numbers exist**, and the one Appendix H figure is indexed as **I.1**.

Claims checked against their own source (MATCH/MISMATCH + proposed fix)

The two pre-flagged Appendix A items

1. FY25 \$3.7B increase stated as both +3.5% and +4.1% — **CONFIRMED MISMATCH**.

- [59] A.3 prose: "For FY25 it approved **\$3.7 billion** in system-wide hospital spending — a **3.5% increase** after hospitals requested 8%".
- Figure A.2, budget-review row: "FY25 approved **\$3.7B (+4.1%** vs. 8% requested)". Same fiscal year, same dollar total, two growth rates, one paragraph apart. No third occurrence anywhere else in the manuscript (grep for **3.7 billion/3.7B** returns only these two). The prose figure (3.5%) is the one consistent with GMCB's FY2025 decision summary; the table is the outlier. *Proposed fix*: in Figure A.2 change **FY25 approved \$3.7B (+4.1% vs. 8% requested) → FY25 approved \$3.7B (+3.5% vs. 8% requested)**.

2. RRMC's \$11.1M FY23 overage misattributed to Porter — **CONFIRMED (high confidence)**. Every other mention in the whole manuscript pairs the FY23 enforcement actions as **UVMHC \$80.3M + RRMC \$11.1M only**, and three of them call them "the **first such** enforcement actions in Vermont's regulatory history" (Conclusion [24], A.3 [59], Figure A.2 enforcement row, Figure A.3 UVMHC row, G.4.2 [193]). Figure A.3's Porter row nonetheless reads "UVMHN system; **FY23 overage of \$11M**" — the same number, rounded, one row above RRMC's "FY23 overage \$11.1M (partially enforced)". A Porter overage would make the enforcement pair a trio and falsify the "first such actions" framing used five times. *Proposed fix*: Figure A.3, Porter row, Key challenges

→ UVMHN system; subject to system budget constraints (matching the CVMC row, which shares Porter's grant and focus), i.e. delete **FY23 overage of \$11M**. *Note, deliberately left for the author:* Figure A.3's NVRH row says "FY23 overage (not enforced)", which is compatible with the "first **enforcement** actions" claim and is not flagged.

Other claims checked

3. MISMATCH — Vermont's population: 647,000 vs. "~612,000–624,000 throughout". [51]: "Its 647,000 residents live across 14 counties". Figure A.1: "Total population 2020–2040 | Flat | ~612,000–624,000 throughout — aging, not growing." 647,000 sits outside the range the table says holds for the entire period including 2020 and 2025. *Proposed fix:* set the prose to the table's basis — **Its roughly 648,000 residents** is the 2024 Census estimate, so the table's band is the weaker number; either widen the table to **~620,000–650,000** or state the prose figure as **~624,000 (2020 Census)**. Author's call which source governs; the two cannot both stand.

4. MISMATCH (arithmetic) — "a 57% increase in the elderly population." Figure A.1: 65+ share goes from **21.7% (2020)** to **30%+ (2040)** while total population is **flat**. On a flat base that is a **~38%** increase in the number of people 65+ ($30.0/21.7 = 1.38$), not 57%. 57% would require either a growing population or a 2040 share near 34%. *Proposed fix:* **– up from 21.7% in 2020, a roughly 38% increase in the elderly population.**

5. MISMATCH (arithmetic, appears twice) — 370 FTE broken out as 112 + 190 = 302. Figure A.1: "Primary-care provider shortage by 2030 | **370 FTE** | 112 family medicine, 190 other primary-care specialties (RHT application)". $112 + 190 = 302$, leaving 68 FTE unaccounted. Restated in A.7 [84]: "a 370-FTE primary-care shortfall by 2030 (112 family medicine, 190 other)" — same arithmetic gap, so a fix must be applied in both places. *Proposed fix:* either name the missing component (e.g. **112 family medicine, 190 other primary-care specialties, 68 pediatrics/NP-PA**) or drop the parenthetical breakdown. Do not silently change 370 — it is the RHT application's headline number and is used as the baseline in Figure E.4 ("FTE gap <200").

6. MATCH — 148 initiatives. [75] "The 148 individual initiatives"; Figure A.4 rows sum $\sim 55+20+25+20+15+13 = 148$. Caption agrees. Clean.

7. MATCH — individual-market premium. [81] $\$456$ (2018) → $\$948$ (2024) = 107.9%, stated as "a 108% increase". Clean.

8. MATCH — UVMHC administrative cost ratio. $\$3,826 / \$1,427 = 268\%$, stated as "267% of \$1,427 benchmark" in Figure E.3. Within rounding; consistent across [65], [71], Figure A.3, Figure E.3, Figure E.6.

9. MISMATCH — "nine of fourteen hospitals reporting operating losses" presented as current, contradicted by Figure E.3. G.3 [182]: "Chapters 6 and 7 document where Vermont's hospital system stands **today** — **nine of fourteen** hospitals reporting operating losses, with Oliver Wyman's conservative scenario projecting thirteen of fourteen in losses by 2028." Figure E.3 makes 9 of 14 the **2022 baseline (FY2023)** and 6 of 14 the **2026 status (FY2024, GMCB)**. *Proposed fix:* **– six of fourteen hospitals reporting operating losses in FY2024, down from nine in FY2023, with Oliver Wyman's conservative scenario projecting thirteen of fourteen in losses by 2028.**

10. MISMATCH — the RHT \$195M is described three incompatible ways.

- Figure B.2: "Vermont RHT: **\$195M over 5 years** for CIN and IT infrastructure"
- Figure E.1: "**\$195M awarded for FY2026, first year**"
- G.3.1 [184]: "provided Vermont **\$195M for its first year (2026)**, with amounts for 2027–2030 not yet published"
- G.1 glossary [176]: "**\$195M for its first year**, awarded annually through FY2030"
- "THE GAP" callout [185]: "The Rural Health Transformation Program's **\$195 million per year**"
- Figure B.1: "**\$195M award**; spending window 2026–2032" / "All RHT Program funds must be spent" by Oct 1, 2032 So: \$195M total-over-5-years (B.2), \$195M for year one with later years unpublished (G.3.1, E.1, G.1), and \$195M per year (the callout) all coexist — and B.1's 2026–2032 window conflicts with G.1's "through FY2030". *Proposed fix:* adopt the G.3.1 formulation as canonical (\$195M for FY2026, the first of five annual awards, amounts for 2027–2030 not yet published; spending window through Oct 1 2032). Then: Figure B.2 → **\$195M first-year award for CIN and IT infrastructure**; "THE GAP" callout → **The Rural Health Transformation Program's \$195 million first-year award is the only funding source.**; G.1 glossary → **awarded annually through FY2030, with funds spendable through FY2032.**

11. MISMATCH — Figure B.1 gives two different effective dates for FY2028 global budgets, inside the same table. Row: "January 1, 2028 | Act 68 hospital global budgets take effect (FY2028)". Row: "**FY2028 (Oct 1, 2027)** | Global budgets for non-CAH (PPS) hospitals". A reader cannot tell whether PPS global budgets start Oct 1 2027 or Jan 1 2028. The FY2030 row is consistently "FY2030 (Oct 1, 2029)", so the table's own convention is fiscal-year-start. *Proposed fix:* make the Jan 1 2028 row **FY2028 (Oct 1, 2027)** and move the calendar-year detail into its requirement text, or relabel the Jan 1 2028 row as the AHEAD performance year specifically rather than "Act 68 hospital global budgets take effect".

12. MISMATCH — Figure E.1 and Figure E.3 contradict each other on AHEAD, two pages apart. Figure E.1 (Policy), federal-state alignment, 2028 target: "**Full AHEAD compliance; second agreement negotiated**". Figure E.3 (Economics), VBC penetration, 2028 target: "Medicare: fee-for-service (**AHEAD withdrawn**)". Both are the same appendix's 2028 column. Figure E.1's own row also still asserts "AHEAD State Agreement signed Jan 2025; nine-year term" as the 2026 status with no withdrawal noted. *Proposed fix:* Figure E.1, 2026 status → **AHEAD State Agreement signed Jan 2025; Vermont withdrew July 2026**; 2028 target → **Successor federal-state agreement negotiated post-AHEAD.**

13. MISMATCH — the Conclusion is dated April 2026 but reports a July 2026 event, and then keeps reasoning as though AHEAD were live. [8] "As of April 2026"; [25] "as of April 2026"; [46] signature "— April 2026". Yet [23]: "The AHEAD EAST Fund was to provide another \$138 million annually ... a renegotiation cut that to a cap near \$10 million **before Vermont**

withdrew from AHEAD in July 2026." Downstream of that sentence the Conclusion still treats AHEAD as the operative deadline: [4] "The AHEAD performance year begins in January 2028 and the analytics vendor is not yet producing..."; [14] "The 'Managing Blind' failure mode..."; [32] "that build care management infrastructure **ahead of AHEAD's performance year**"; [33] "before **AHEAD's financial accountability** begins". This is the single largest internal contradiction in the range: either the vantage point is April 2026 (and the withdrawal cannot be known) or it is post-July 2026 (and four calls to action are addressed to a model Vermont has left). *Proposed fix*: move the vantage point to the later date (**As of late 2026**, signature – **2026**) and re-anchor [32] and [33] on Act 68's FY2028 global budgets rather than AHEAD's performance year — Act 68's January 2028 date is independent of AHEAD and carries the argument unchanged. [4]'s failure-scenario sentence should name the Act 68 global- budget year, not the AHEAD performance year. Also [23]'s sentence is ungrammatical as written ("a renegotiation cut that to a cap near \$10 million") — recommend **...another \$138 million annually in Medicare performance funds for a state that invests in population health; a renegotiation cut that to a cap near \$10 million, and Vermont withdrew from AHEAD in July 2026.**

14. MISMATCH — Appendix F's pillar→chapter map is wrong for all five pillars. Figure- less table [156] vs. the actual Heading1s in the file:

F.3 says	Book actually has
Policy → Chapter 1	Policy = Ch 2–3 (Ch 1 is the framework itself)
Economics → Chapters 8–9	Economics = Ch 6–7; Ch 8–9 are Clinical
Technology → Chapters 6–7	Technology = Ch 4–5
Clinical → Chapters 10–11	Clinical = Ch 8–9; Ch 10 = Equity, Ch 11 = Operations
Operations → Chapters 14–15	Operations = Ch 11 (and 16); Ch 14–15 are Political Sustainability / Portfolio Management

Proposed fix: Policy → Chapters 2–3; Technology → Chapters 4–5; Economics → Chapters 6–7; Clinical → Chapters 8–9; Operations → Chapters 11, 16. (Equity/Chapter 10 has no row and should probably get one, since Appendix E gives it a full scorecard as a sixth section.)

15. MISMATCH — tool counts and tool names across Appendices D, F and H.

- Appendix D [102]: "the **19** HTR Research Lab tools referenced throughout the book. **All are available to Strategist and Enterprise** subscribers" — then item E.19 (The Wire) says its newsletter is "**free weekly** for Observer-tier", contradicting "all".
- Appendix F [160]: "a suite of **nearly forty** interactive analytical tools", names **23**, and closes "Full tool specifications are in **Appendix D**" — false for the four it names that Appendix D omits (EMR/EHR Lab, Statewide EHR Deployment Modeler, CIN & Shared Services Modeler, EMS Transformation Modeler).
- Ground truth: **frontend/lib/taxonomy/tools.ts** contains **39** tool entries, so F's "nearly forty" is right and D's 19 is a subset. All four tools F names and D omits do exist in the registry.
- Appendix H names five tools that Appendix D does not list: **Global Budget Transition Modeler, Hospital Financial Stress Test, Risk Stratification Methodology, VBC Quality Measures, Transformation Scorecard**. All five exist in **tools.ts**.
- Three of Appendix D's names do not match the registry: D's "**Hospital Financial Scorecard**" (E.9) is the platform's "Hospital Financial Stress Test" (the name Appendix H uses); D's "**AI Analytics Lab**" (E.12) has no registry entry — the nearest is "AI Clinical Governance Lab"; D's "APM Shared Savings Calculator" (E.4) is "Shared Savings Calculator" and "VBC Transformation Readiness Assessment" (E.18) is "VBC Readiness Assessment". *Proposed fix*: reword D's opening to "describes the **nineteen core** HTR Research Lab tools the chapters draw on most; the full catalogue of thirty-nine is on the platform", drop "All are available to Strategist and Enterprise" or qualify it, and align D's three off-name tools to the registry labels. F's "Full tool specifications are in Appendix D" should become "Specifications for the core tools are in Appendix D."

16. MISMATCH — Appendix D numbers its items E.1–E.19 inside Appendix D. Every entry is labelled **E .n**. The cause is visible in the platform source: **frontend/lib/taxonomy/tools.ts** header comment reads "The book's **Appendix E ('Tool List')** is generated from this" — the tool list moved from E to D and the item numbering was never renumbered. Appendix E is now the Transformation Scorecard, so **E .9** is ambiguous between a tool and a scorecard figure. *Proposed fix*: renumber to D.1–D.19 and update the **tools.ts** header comment to say Appendix D.

17. MISMATCH — Appendix G cross-references itself as "Section H.3", twice. [193]: "heading into the FY2027–2028 transition window described in **Section H.3**"; [194] (WHAT THIS MEANS callout): "the financing gap identified in **Section H.3**". The transition-window section is **G.3** of this appendix; H.3 is the workbook's Stage 2. *Proposed fix*: both → **Section G.3**.

18. MISMATCH — G.3.1 cross-references "in this chapter" for an AHEAD discussion that is not in this appendix. [184]: "Vermont withdrew from AHEAD in July 2026, so this funding will not arrive at all (see "**The AHEAD Model**" in this chapter)." Appendix G has no "The AHEAD Model" section, and it is an appendix, not a chapter. *Proposed fix*: point at the real location (the AHEAD treatment in Chapter 3, and the G.1 glossary's AHEAD Model entry) and change "chapter" to "appendix".

19. MISMATCH — Appendix H labels two different sections "The Stage 5 question." [213] closes H.6 (The Equity Imperative) with "The **Stage 5** question: Does the design penalize safety-net providers..."; [216] closes H.7 (Stage 5 — Operations) with "The **Stage 5** question: Is operational capacity keeping pace...". H.6 is deliberately *not* a numbered stage (it is the

cross-cutting Equity Imperative, consistent with Appendix E treating Equity as unnumbered), so the first label is the error. *Proposed fix:* [213] → **The equity question:**.

20. MISMATCH — Appendix H's figure is numbered I.1. [218] "Figure I.1 — The HTR Lab Workbook maps each of the five pillars..." in Appendix **H**, and the Figure Index reproduces it as I.1. There is no Appendix I. *Proposed fix:* **Figure H.1**, in both the appendix and the Figure Index.

21. MISMATCH — H.7's "\$1,303 per discharge" administrative-cost gap is not derivable from any figure in this range. [216]: "The administrative-cost gap (Vermont's **\$1,303 per discharge**) is the funding source for closing the workforce gap." The only administrative-cost numbers in the appendices are UVMHC \$3,826 vs. the \$1,427 national benchmark — a gap of **\$2,399** — plus Figure E.6's "system avg 91% above [benchmark]", which on \$1,427 implies a system average of ~\$2,726 and a gap of ~\$1,299. So \$1,303 is almost certainly the *system-average* gap, but the sentence reads as a single unexplained number with no cited source. *Proposed fix:* **The administrative-cost gap (Vermont's system average runs ~91% above the \$1,427 national benchmark – roughly \$1,300 per discharge) is the funding source...**

22. MISMATCH (factual, geography) — Mount Ascutney placed in White River Junction. Appendix C [99]: "Mount Ascutney Hospital (White River Junction)". Appendix A [78] has Mt. Ascutney "subsidizing Windsor's only pharmacy ... after the town's private pharmacy closed in 2025" — i.e. Windsor is its town. Mt. Ascutney Hospital and Health Center is in Windsor; White River Junction is where the VA medical center and DHMC-area facilities sit. *Proposed fix:* Appendix C → **Mount Ascutney Hospital (Windsor)**. Also note Appendix C uses "Mount Ascutney Hospital" and "Porter Medical Center" while Appendix A uses "Mt. Ascutney Hospital" and "Porter Hospital" — the same two institutions under two names each, across two appendices.

23. MATCH — Appendix C covers 14 hospitals (14 data rows) and every hospital named in Appendix A's Figure A.3 appears exactly once. Tier assignments are internally consistent (4 Tier 1, 5 Tier 2 focused, 4 Tier 3, plus Porter at Tier 2 with COE count TBD).

24. MATCH — GMCB creation date. [59] "created in 2011"; [82] "since Act 48 created the GMCB in 2011". Consistent.

25. MATCH — Essex County / statewide uninsurance. [52] "8% uninsurance against a 3% statewide rate"; Figure A.1 "Residents with health insurance (2025) 97%"; Figure E.5 "Essex County uninsurance 8% (highest in Vermont)". All three agree.

Superlative/ranking claims logged for cross-chapter comparison

Verbatim, with location. These are logged, not judged — several will need checking against competing superlatives elsewhere in the manuscript.

Conclusion

- 1. [17] H.R. 1 + AHEAD + Act 68 pricing reform combined pressure "is **the biggest test** of whether Vermont's transformation architecture is robust enough to absorb shocks while remaining on its statutory timeline."
- 2. [24] GMCB's FY23 actions against UVMHC and RRMC were "**the first such enforcement actions in Vermont's regulatory history.**" (Repeated verbatim-in-substance at A.3 [59] and G.4.2 [193]. Contradicted only by Figure A.3's Porter row — see MISMATCH 2.)
- 3. [6] the partial-transformation failure scenario "is **the modal outcome** of healthcare reform efforts in American history."
- 4. [22] "The political durability of mandatory reform — **always the central uncertainty** in healthcare policy — has held through two legislative cycles."
- 5. [42] the three decisions are "the three unresolved questions on which Vermont's transformation outcome **most directly depends.**"

Appendix A 6. [51] "Vermont is **the second-smallest state by population.**" 7. [52] Essex County "**among the most rural and least populated** counties in the Northeast Kingdom." 8. [52] UVMHC "with roughly 50% of the state's hospital market share, is the economic center of gravity — every reform affecting UVMHC affects the state economy in ways **no single hospital does in larger states.**" 9. Figure A.1 "97% ... **Among the highest coverage rates in the country.**" 10. [64] the nine independents "are **these that face the most acute fragility.**" 11. [65] CAHs are "**the smallest and most rural**, serving communities with **the fewest alternatives** — are **the most distressed** ... also where service reductions would have **the most severe community consequences**, and where AHEAD's global-budget design must protect **most carefully.**" (Four superlatives in one sentence.) 12. [71] UVMHC is "Vermont's **largest and most powerful** hospital." 13. [78] Grace Cottage's trajectory "is **the book's most important counterexample** to the narrative that Vermont's CAHs are uniformly failing." 14. [78] Mt. Ascutney's CHT data is "**the most concrete illustration** of demographic aging at the clinical level." 15. [82] "UVMHC's commercial prices averaged 358% of Medicare in RAND's published data — **the highest in the state.**" 16. [82] "Act 68's RBP mandate is **the most consequential health-policy legislation Vermont has passed since Act 48** created the GMCB in 2011." 17. [84] "The primary-care shortage is **the most immediately actionable**"; "The housing constraint is **the most intractable**"; Vermont has "**the second-highest per-capita homelessness rate in the country.**" 18. [86] Blueprint's 5.8:1 ROI is "**among the most robust ROI findings in the VBC literature.**" 19. [84] "a triple constraint **no single intervention resolves.**"

Appendix C 20. Grace Cottage: "19-bed, **Vermont's smallest.**" 21. Gifford: "**Among most financially vulnerable** — comprehensive assessment needed." 22. North Country: "**Northeast Kingdom anchor.**"

Appendix E 23. Figure E.3, 2022 baseline: "9 of 14 reporting losses (FY2023); **worst -8.9%.**" 24. Figure E.5: "Essex County uninsurance 8% (**highest in Vermont.**)" 25. [147] a 2028 scorecard with Policy/Economics at target but Technology/Operations

at 2026 status "would indicate financial accountability without management capability — precisely the sequencing failure Chapters 2 and 3 identify as **the most dangerous**."

Appendix G 26. G.1 glossary: Northeast Kingdom is "the state's **most severe** rural access and equity challenge." 27. [184] Medicaid "is, perversely, **the first** to move to a budget-based model, while commercial payers ... are **the last**." 28. "THE GAP" callout [185]: RHT is "**the only funding source available** during this window." 29. [187] the FY2027–2028 transition-financing gap "is **the single most consequential open question** for any hospital CFO reading this book." 30. [193] UVMHC is "the system with **the most resources and the most to lose**" and "**the largest and most capable** potential challenger."

Cross-chapter checks these will need (flagged, not resolved here, since the mandate was this range only): #1 "the biggest test" vs. #29 "the single most consequential open question" vs. #16 "the most consequential health-policy legislation" vs. #25 "the most dangerous" sequencing failure vs. #13 "the book's most important counterexample" — five book-scoped superlatives about different subjects. None contradict each other *within* this range (they qualify different nouns), but the manuscript-wide grep for competing "most consequential / most dangerous / most underestimated" claims should include all five.

Other issues

Repetition (genuine, and mostly defensible)

- The UVMHC \$80.3M / RRMC \$11.1M FY23 enforcement pair appears **five** times in this range alone (Conclusion [24], A.3 [59], Figure A.2 enforcement row, Figure A.3 UVMHC row, G.4.2 [193]). G.4.2's opening sentence is near-verbatim the Conclusion's. Appendix recurrence is legitimate (the appendices are reference documents and the A.2/A.3 rows are tabular restatements of A.3's prose), but **Conclusion [24] and G.4.2 [193] are the same argument told twice at the same length** — that pair is real duplication, not reference recurrence. Left for the author to cut, since G.4.2 is the section built around it and the Conclusion needs the proof point.
- UVMHC's \$3,826 vs. \$1,427 admin cost appears four times ([65], [71], Figure A.3, Figure E.3, plus Figure E.6 in ratio form). [71] is an entire paragraph restating [65]'s closing sentence with no new information — thin, and the clearest candidate for a cut.
- CAH \$938 vs. \$2,784 benchmark: [65] and Figure E.3. Legitimate (prose + scorecard).
- Mt. Ascutney's ~\$600K/yr shared-C-suite savings appears three times in Appendix A alone ([72], [78], Figure A.3 row) plus Figure A.4's shared-purchasing row — four times in one appendix.
- The "370-FTE gap by 2030 (112 family medicine, 190 other)" appears in Figure A.1, [84] and Figure E.4. Legitimate recurrence, but note the arithmetic defect (MISMATCH 5) must be fixed in all three.
- "Managing blind" as the OneCare failure mode: Conclusion [14] and Appendix H [204]. Legitimate — H is the practicum reproducing the claim.

Thin sections

- [71] A.4.2's first paragraph: one sentence, entirely a restatement of [65].
- Appendix C has **no figure caption and no numbered figure** despite being a full 14-row table, and its source note [100] repeats the second half of its own intro [98] verbatim ("COE designations are proposed; final designations subject to the AHS and hospital transformation planning process").
- Appendix F carries **no figure numbers at all** for its F.3 table, and Appendix G none for its 15-term glossary or its worksheet — four substantial tables invisible to the Figure Index.
- Appendix H's [218] figure caption has no figure: it describes a mapping the appendix delivers as prose stages, not as a table. Either build the table the caption promises or drop the caption.

Internal contradictions (summarized — details above)

1. +3.5% vs +4.1% on the same FY25 \$3.7B (MISMATCH 1) — the pre-flagged item, confirmed.
2. Porter carrying RRMC's \$11.1M overage, which falsifies the five-times-repeated "first such enforcement actions" claim (MISMATCH 2) — the pre-flagged item, confirmed.
3. 647,000 residents vs. "~612,000–624,000 throughout" (MISMATCH 3).
4. April 2026 vantage point vs. the July 2026 AHEAD withdrawal, with four subsequent AHEAD-dependent calls to action (MISMATCH 13) — the most consequential one.
5. Figure E.1 "Full AHEAD compliance" vs. Figure E.3 "AHEAD withdrawn", same appendix (MISMATCH 12).
6. Figure B.1's two effective dates for FY2028 global budgets (MISMATCH 11).
7. \$195M as five-year total vs. first-year award vs. per-year (MISMATCH 10).
8. 9 of 14 vs. 6 of 14 hospitals in losses (MISMATCH 9).
9. 19 vs. 23 vs. "nearly forty" tools; three Appendix D tool names not matching the platform registry (MISMATCH 15).
10. Appendix D numbered E.1–E.19; Appendix H's figure numbered I.1 (MISMATCH 16, 20).
11. "Section H.3" for G.3, twice; "in this chapter" for a section that is not in the appendix (MISMATCH 17, 18).
12. "The Stage 5 question" used for both H.6 and H.7 (MISMATCH 19).
13. Mount Ascutney in White River Junction vs. Windsor; Mt./Mount and Porter Hospital/Porter Medical Center naming split across appendices (MISMATCH 22).
14. Appendix F's pillar→chapter map wrong on all five rows (MISMATCH 14).

Checked and clean

- 148 initiatives sum exactly; premium 108% increase correct; \$3,826/\$1,427 = 267% correct; Appendix C covers all 14 hospitals once each with internally consistent tiers; GMCB 2011 consistent; 8%/3%/97% insurance figures mutually consistent; Act 68 dates (RBP FY2027, global budgets FY2028 non-CAH → FY2030 all, Strategic Plan Dec 2028)

consistent across Figure B.1, G.1's Act 68 entry, G.2's worksheet, Figure E.1 and the Conclusion; Figure Index faithfully reproduces every numbered figure caption in range, verbatim.

- CCBHC counts (0 baseline → 5 certified July 2026 → 7 by 2028) consistent between Figure B.1 and Figure E.4.
- The AHEAD EAST Fund's "\$138 million annually" is consistent between Conclusion [23] and G.3.1 [184].

Not checked (out of scope as instructed) Route resolution and delivered-promise verification for the 20+ platform URLs in Appendix H and the tool/section names in Appendix F. Two things surfaced incidentally and are worth a separate pass: Appendix D's "AI Analytics Lab" has no registry match, and Appendix F's reader-profile paragraphs name several surfaces ("HTR Simulator (five-pillar readiness score)", "Medicaid Eligibility Simulator", "Bed Capacity & Transfer dashboard (System Vitals)", "50-State RHTP Dashboard", "All States Explorer") whose delivery was not verified here.